

CITATION: K.M. v. Marson, 2018 ONSC 3493
COURT FILE NO.: CV-08-0283-00
DATE: 2018 June 29

ONTARIO
SUPERIOR COURT OF JUSTICE

BETWEEN:

K.M.

Plaintiff

– and –

ROBERT TERRANCE MARSON and THE
HASTINGS AND PRINCE EDWARD
DISTRICT SCHOOL BOARD

Defendants

)
)
)
) *Robert J. Reynolds*, for the Plaintiff
)
)
)

) *S. Wayne Morris and Teri Liu*, for the
) Defendant, The Hastings and Prince Edward
) District School Board
)
)

) *Michael J. Pretsell*, for the Defendant,
) Robert Terrance Marson (not appearing)
)
)
)

) **HEARD at BELLEVILLE:** March 20, 21,
) 22, 23, 24, 27, 28, 29, 31, 2017; May 1,
) 2017; written submissions received by July
) 31, 2017; oral submissions September 5, 6,
) 7, 2017

Publication Ban:

By Court Orders dated February 10, 2017 and March 20, 2017, under the authority granted in sections 135 and 137 of the *Courts of Justice Act*, R.S.O. 1990, c. C.43, there is a publication ban in effect in this case as to the identity of the plaintiff, his parents, siblings or child, and any complainant identified in the criminal proceedings as set out in Section (E) of Exhibit #2, or any information that has the effect or could have the effect of identifying the plaintiff, his parents, siblings or child, and any complainant identified in the criminal proceedings as set out in Section (E) of Exhibit #2 filed in this proceeding. Initials only will be used to identify these individuals and other persons as is necessary in my Reasons for Judgment.

REASONS FOR JUDGMENT

MACLEOD-BELIVEAU J.

Index

	<u>Paragraph</u>
1. <u>Introduction</u>	[1]
2. <u>Issue</u>	[7]
3. <u>Result</u>	[9]
4. <u>Litigation History</u>	[10]
5. <u>Admission of Liability</u>	[18]
6. <u>Position of the Parties</u>	
a. <u>K.M.'s Position</u>	[22]
b. <u>The School Board's Position</u>	[24]
7. <u>Relief Sought</u>	
a. <u>K.M.</u>	[27]
b. <u>The School Board</u>	[29]
8. <u>The Evidence – Overview and Analysis</u>	[30]
9. <u>The Sexual Abuse by Marson, 1978 to 1980</u>	[32]
10. <u>K.M.'s Life before the Sexual Abuse by Marson, 1965 to 1978</u>	
a. <u>K.M.'s Family and Friends</u>	[51]
b. <u>K.M.'s Education and School History</u>	[67]
c. <u>K.M.'s Problems with Learning</u>	[84]
d. <u>K.M.'s Drug Use</u>	[97]
e. <u>K.M.'s Psychological and Mental State</u>	[103]
f. <u>Summary</u>	[108]
11. <u>K.M.'s Life after the Sexual Abuse by Marson, 1978 to 2017</u>	
a. <u>K.M.'s Family and Friends</u>	[113]
b. <u>K.M.'s Education and School History</u>	[128]
c. <u>K.M.'s Problems with Learning</u>	[142]
d. <u>K.M.'s Relationships</u>	[143]

e. <u>K.M.'s Employment, Work Experience, and Taxable Income Sources</u>	[161]
i. <u>K.M.'s Ontario Disability Support Plan Application (O.D.S.P.)</u>	[184]
ii. <u>K.M.'s Canada Pension Plan Disability Benefit Application (C.P.P.)</u> ...	[197]
f. <u>K.M.'s Drug Use</u>	[199]
g. <u>K.M.'s Psychological and Mental State</u>	
i. <u>Overview</u>	[214]
ii. <u>Dr. C. C. Brant – 1989 to 1994</u>	[222]
iii. <u>Dr. E. B. Conn – 1994 to 1996</u>	[246]
iv. <u>Dr. M. Schwarz – 1996 to 1997</u>	[248]
v. <u>Dr. E. B. Conn – 1997 to 2006</u>	[251]
vi. <u>June 2006 to July 2007</u>	[280]
vii. <u>Dr. A. Singh – July 2007 to June 2012</u>	[285]
viii. <u>June 2012 to present</u>	[322]
h. <u>K.M.'s Current Psychological and Mental State</u>	[328]
i. <u>Summary</u>	[337]
12. <u>Credibility of Lay Witnesses</u>	[362]
13. <u>Expert Psychiatric Medical Evidence</u>	[373]
a. <u>Dr. A. Singh</u>	[374]
b. <u>Dr. D. Silver</u>	[460]
c. <u>Summary</u>	[510]
14. <u>Causation</u>	[519]
15. <u>General Non-Pecuniary and Aggravated Damages</u>	[584]
16. <u>Damages for Past and Future Income Loss</u>	[611]
17. <u>Damages for Future Loss of Interdependent Relationship</u>	[677]
18. <u>O.H.I.P. Past Loss Subrogated Claim</u>	[720]
19. <u>Management Fee</u>	[733]
20. <u>Conclusion – Judgment to Issue</u>	[736]
21. <u>Pre-judgment Interest</u>	[739]
22. <u>Costs</u>	[740]

23. <u>Cross-claim</u>	[741]
24. <u>Further Calculations, Errors, Omissions, and Ancillary Matters</u>	[743]
25. <u>Schedule ‘A’ – K.M.’s Earnings and Taxable Income Sources, 1987-2014</u>	[752]
26. <u>Schedule ‘B’ – O.H.I.P. Past Loss Subrogated Claim Amounts</u>	[753]

1. Introduction:

[1] This is an action in negligence by an adult plaintiff against his teacher and his school board for childhood sexual abuse committed by his teacher at his elementary school.

[2] The plaintiff's Statement of Claim seeks damages for assault and/or battery and breach of fiduciary duty as against his teacher, and for vicarious liability and alternatively negligence as against his school board. Liability for the plaintiff's damages in negligence was ultimately admitted by the school board and the matter proceeded to trial as a contested assessment of damages. The teacher did not actively participate in the trial, although he remains a party defendant.

[3] The plaintiff K.M. (hereinafter referred to as "K.M.") was sexually assaulted from 1978 to 1980 by his science teacher, the defendant Robert Terrance Marson (hereinafter referred to as "Marson"), when he was a student at Harry J. Clarke School in Belleville, operated under the *Education Act*, R.S.O. 1990, c. E.2, by the defendant, The Hastings and Prince Edward District School Board (hereinafter referred to as the "School Board"). K.M., now 51 years old, was 13 and 14 years old and in grades 7 and 8 at the time of the sexual abuse. Current age references in these Reasons for Judgment are as at the time of trial.

[4] To sexually abuse K.M., Marson used a small remedial reading lab room in the school, a petting or mini-zoo that he created in a room across from his own classroom at the school, and a storage room at the back of his science classroom. Marson used the reading lab to groom and sexually assault K.M. over his clothing multiple times. He used the mini-zoo as a means of luring K.M., who loved animals, for the purpose of sexually abusing him. Marson performed oral sex on K.M. in the mini-zoo and in the storage room at the back of his classroom on a total of five occasions.

[5] K.M. has had learning difficulties since early childhood. Because of his learning difficulties, K.M. struggled to learn throughout his schooling, from Kindergarten and grade 1 onward, and in particular with his math and reading. He had poor grades. He learned differently than others. He is more effective at processing information that he hears, rather than information that he sees or confronts more directly. K.M. is considered to be a "verbal learner" according to Dr. A. Wolfgarth, a clinical psychologist who assessed K.M. on December 19, 2002 when he was applying for income support from the Ontario Disability Support Program (hereinafter "O.D.S.P."). K.M.'s learning disability is associated with "dyslexic tendencies". He has difficulty with spatial perception, in that he does not see what is actually there. He sees things differently. He has difficulty with numbers, road maps, cell phones and gadgets. He has difficulty, for example, backing up a vehicle with a mirror. Importantly, Dr. Wolfgarth assessed K.M. as falling into the average range of intellectual ability. To be clear, the evidence in this case, including the evidence of Dr. D. Silver for the School Board, is that K.M. is an intelligent man.

[6] K.M. suffers from serious mental health issues and injuries as an adult. It is his position that his mental health injuries are the result of the sexual abuse perpetrated on him by Marson. K.M. suffers from major depression with dysthymia, post-traumatic stress disorder, substance

abuse, and personality disorder of the antisocial and borderline type. K.M. is unable to sustain an interdependent personal relationship, and he has difficulty in keeping employment. He was assessed as being a person with a substantial disability and has received O.D.S.P. benefit payments since April 2003. K.M. was later approved for a Canada Pension Plan (hereinafter “C.P.P.”) disability pension in October 2012, with payments retroactive to September 2009, the maximum time allowed.

2. Issue:

[7] The central issue between K.M. and the School Board is one of causation: whether or not Marson’s sexual abuse of K.M. is “a” cause of some or all of the injuries K.M. claims. It is not necessary that the sexual abuse by Marson be the sole cause of K.M.’s injuries. The issue is whether K.M. has proven, on a balance of probabilities, that his injuries would not have occurred “but for” the sexual abuse by Marson. If it is determined that Marson’s abuse is “a” cause of K.M.’s injuries, then a further determination is needed of what damages, if any, are appropriate. The assessment of damages begins by considering what the original position of K.M. would have been had the sexual abuse not occurred. Damages are then awarded to place K.M. in the position he would have been in if Marson’s sexual abuse had not occurred.

[8] The parties disagree on the law of causation, its application to the facts of this case and how it affects the damages assessed. The parties differ on the application of the “crumbling skull” principle, which, if applied, would reduce any damages assessed for K.M. The School Board appeared to advance a crumbling skull qualification argument as applying to this case, although it did not explicitly acknowledge it. As a result, the School Board did not deal with whether the qualification applied at all, or whether or not it had established the requisites for the application of the qualification.

3. Result:

[9] The action is allowed. The School Board is 100% liable for K.M.’s damages for the sexual abuse by Marson. The impact of the sexual abuse on K.M. was severe. I assess K.M.’s total damages at \$250,000.00 for general non-pecuniary and aggravated damages; \$1,192,550.00 for past loss of income and \$762,450.00 for future loss of income, for a total of \$1,955,000.00 in total loss of income; \$135,587.00 for future loss of interdependent relationship; \$27,953.11 for the past loss subrogated claim of the Ontario Health Insurance Plan (hereinafter “O.H.I.P.”); and \$44,902.00 for a management fee, for a total damage award of \$2,413,442.11 plus pre-judgment interest. The issues of the quantum of pre-judgment interest and costs are reserved for further submissions or agreement between the parties. The assessments of damages for K.M.’s total income loss claim, loss of future interdependent relationship and management fee are reserved and subject to the terms as set out in Item #24 of this judgment (“Further Calculations, Errors, Omissions, and Ancillary Matters”), only as to further calculations as specified.

4. Litigation History:

[10] K.M. commenced his civil action by way of a Statement of Claim issued September 23, 2008 and claimed damages in the amount of \$3,500,000 and punitive damages in the amount of

\$500,000, plus interest and costs. The parties agree that there are no limitation period issues under the *Limitations Act, 2002*, R.S.O. 2002, c. 24, Sched. B, due to the sexual nature of the claim. The parties agree that the cause of action arose on October 1, 2006, approximately when the claim became discoverable. Counsel have agreed that the time period from October 1, 2006 to the commencement date of this trial, March 20, 2017, is 10.5 years.

[11] An Amended Statement of Claim was issued May 26, 2015. The trial was scheduled to commence May 9, 2016 but could not proceed due to a lack of judicial resources in the East Region beyond the two-week assigned trial time and defence witness unavailability during the two-week trial time assigned. The trial was re-scheduled and started March 20, 2017. Submissions were originally scheduled to begin April 3, 2017. Counsel requested more time to prepare their submissions, which was granted. Submissions were re-scheduled to start on May 8, 2017.

[12] On May 1, 2017, a motion was brought by plaintiff's counsel concerning the use of audio recordings in making his final submissions in the trial, in order to make direct references to the evidence as required by me. As a result of my motion decision released on May 15, 2017, found at 2017 ONSC 2972, reported as *M. (K.) v. Marson*, 138 O.R. (3d) 709, additional time was required for transcripts of all of the evidence to be completed and to be provided to counsel for use in their submissions, and to be provided to the court. Costs of the motion were reserved to be included in the cost submissions of the trial.

[13] The evidence in this case was complex. Complete transcripts of each witness's evidence were received and made available to counsel and to the court. Written submissions in the trial were filed by July 31, 2017. Oral submissions were heard from September 5 to 7, 2017, which were transcribed for the benefit of the court. The decision was reserved. On February 27, 2018 and May 1, 2018, extensions were granted for delivery of these Reasons for Judgment to June 30, 2018 pursuant to section 123 (5) of the *Courts of Justice Act*, R.S.O. 1990, c. C.43. Counsel were so advised.

[14] In addition to K.M.'s own evidence, the plaintiff's case consisted of evidence from five other lay witnesses—his mother A.F.; his younger sister S.G.; his maternal aunt M.I.C.; his former common-law spouse Y.G.; his current neighbour and friend T.A.—and evidence from two experts—Dr. A. Singh, a psychiatrist; and Professor J. Carr, an economist. The defendant's case consisted of evidence from two experts—Dr. D. Silver, a psychiatrist; and Professor M. Charette, an economist. All four expert witnesses signed an acknowledgement of expert's duty form as required pursuant to Rule 53.03 of the *Rules of Civil Procedure*, R.R.O. 1990, Reg. 194.

[15] Other initialled individuals referred to in these Reasons for Judgment are: J.C., K.M.'s older sister; D.M., K.M.'s father; M.M., K.M.'s son; K.W., a former girlfriend of K.M.; C.P., a former girlfriend of K.M. and the mother of M.M. (the son of K.M. and C.P.); and B.H., a high school friend of both K.M. and Y.G.

[16] The trial proceeded before me as a contested assessment of damages. Exhibits #1, #2 and #3 are joint briefs containing 682 pages of relevant documents of medical notes, school reports, employment records, criminal proceedings and related materials, O.D.S.P. and C.P.P.

disability pensions materials, the subrogated claim amounts of O.H.I.P., and K.M.'s income tax returns from 1987 to 2014.

[17] The contents of Exhibits #1 to #14 were agreed by both counsel to be admissible on consent as evidence of the truth of their contents. The weight and effect of these documents is a matter for determination by the court. The Experts' Reports Brief, Exhibit A, was filed for the assistance of the court and the expert witnesses during their testimony.

5. Admission of Liability:

[18] Just prior to the original trial date of May 9, 2016, by faxed letter to K.M.'s counsel on May 4, 2016 (Exhibit #4), the School Board admitted its liability in negligence as alternatively pleaded in paragraphs 14 (2) and 14 (3) of K.M.'s Amended Statement of Claim. Specifically, the School Board admitted it was negligent in that it failed to establish a system of reasonable checks and precautions so as to minimize the risk of such behaviour on the part of Marson and failed to take reasonable steps to monitor the activities of Marson so as to ensure that such activity would be discovered and halted.

[19] The School Board admitted that it failed to take steps to deal with Marson in and around September 1976 and thereafter, so as to prevent his abuse, assault and battery against the plaintiff K.M. Specifically, the School Board admitted that the school's principal was alerted to a parent's suspicion about Marson in or around September 1976, and failed to take sufficient steps over the ensuing years to supervise Marson's interaction with the students.

[20] The School Board advised, as confirmed by plaintiff counsel's letter dated May 16, 2016 (Exhibit #5), that it would neither admit nor deny the alleged assaults, and would be calling no evidence to rebut K.M.'s evidence of those assaults. In particular, the School Board advised that it would not be calling Marson as a witness. It was agreed that records, of which notice had been given under section 35 of the *Evidence Act*, R.S.O. 1990, c. E.23, had been created in compliance with the requirements of section 35 of the *Act*.

[21] Marson and his counsel did not appear at this trial, but knew the trial was proceeding. Marson does not contest his liability to K.M. and made a \$50,000.00 advance payment to K.M., depleting the equity in his home, before the commencement of this trial. The date this payment was made was not provided to the court.

6. Position of the Parties:

a. K.M.'s Position

[22] K.M.'s position is that the childhood sexual abuse perpetrated on him by Marson is a major cause of his mental health injuries in adulthood. K.M. has been diagnosed as suffering from major depression with dysthymia, post-traumatic stress disorder, substance abuse, and a personality disorder of the antisocial and borderline type. K.M. has been diagnosed as being unable to sustain an interdependent spousal relationship, and he has difficulty in keeping employment, such as a regular job. K.M.'s position is that the School Board is liable for K.M.'s

entire injuries if the sexual abuse is proven to be at least “a” cause but not the only cause of K.M.’s injuries. K.M.’s position is that even if other factors may have been contributing causes to K.M.’s disorders, it does not reduce the School Board’s 100% liability for K.M.’s damages. K.M.’s position is that his claim has been amply proven on a balance of probabilities and that he is entitled to an award of damages in the amounts as claimed by him.

[23] K.M.’s position is that prior to the sexual abuse by Marson, he was a happy, energetic child who functioned well in his family and in the classroom despite his learning difficulties that made learning a struggle for him since he began school. His position is that after the sexual abuse, he became a disruptive influence in the classroom, a runner from school and a heavy user of drugs. Within a few years K.M. was suffering from severe psychological problems, including chronic major depression, anxiety attacks, and diagnosed personality disorders. K.M. has been unable to hold a job despite being a hard and effective worker when he could work. K.M.’s psychological and mental health problems will prevent him from sustaining an interdependent spousal relationship.

b. The School Board’s Position

[24] The School Board’s position is that K.M.’s ongoing mental disorders have been caused not only by the sexual abuse but by K.M.’s genetic makeup, his life experiences, and conditions in K.M. that existed prior to the abuse by Marson.

[25] The School Board submits that it need not put K.M. in a better position than his “original position” and it should not have to compensate K.M. for any damages that he would have suffered anyway. K.M.’s “original position” is the position he would have been in absent the School Board’s negligence. It is therefore necessary not only to determine K.M.’s position after the tort, but also to assess what his “original position” would have been. It is the difference between his “original position” and his “injured position” that is the measure of K.M.’s loss. The School Board’s position is that K.M. has suffered injuries from traumas other than the sexual abuse by Marson and that the court must therefore untangle the multiple interlocking factors and confine any damages award to only those arising from the sexual abuse committed by Marson. The School Board’s position is that K.M. suffered from trauma and mental health injuries prior to the childhood sexual abuse by Marson and that there is a “measurable risk” that it would have led to the development of at least part of K.M.’s mental disorders, even if the sexual abuse had not occurred. The School Board submits that as a result, K.M.’s damages should be reduced by 50%.

[26] The School Board’s position is that prior to the sexual abuse by Marson, K.M. had a history of behavioural issues, drug use, and poor grades. His prior family life was dysfunctional. K.M. had a genetically determined characterological depression, which has caused or contributed to his disorders, and all these factors had a negative impact on K.M.’s psychological development. The School Board’s position is that K.M.’s multiple disorders, including his depression and personality disorder, are genetically and environmentally determined, quite apart from any of the abuse committed by Marson. K.M.’s grades were uniformly poor, both before and after any interaction with Marson, because of K.M.’s learning disability. He failed grade 6 and had to repeat it. There was never any expectation that K.M. would finish high school. K.M.

is not motivated to work and works intermittently. He has some success with relationships although he is not in a spousal relationship currently. He maintains a positive relationship with his son, age 18 at the time of trial. K.M. maintains himself on disability on the basis of his depression, dyslexia and learning disability. He has not sought meaningful employment since 2003. The School Board does not dispute that K.M. is now unable to form an interdependent relationship, based on the evidence of its own psychiatrist, Dr. Silver, but the School Board disputes the quantum of the claim.

7. Relief Sought:

a. K.M.

[27] K.M. submits that \$2,867,625.80 in damages for sexual abuse, assault, and battery—plus pre-judgment interest of \$639,176 for a total of \$3,506,801.80—is a reasonable assessment of his damages. His claims are for general damages, damages for loss of past and future income, damages for impairment of his ability to sustain an interdependent relationship (hereinafter referred to as “loss of interdependent relationship”), an O.H.I.P. subrogated claim, a management fee covering the cost of obtaining advice as to how to invest any award obtained, pre-judgment interest, and costs. K.M. makes no claim for damages for the effect that his learning disability has had on his life. K.M. did not pursue his claim for punitive damages.

[28] The damages claimed by K.M. are as follows:

a. General Damages	\$ 250,000.00	
b. Damages for loss of past and future income (Past loss \$1,425,630; future loss \$918,538)	2,344,168.00	
c. Damages for loss of interdependent relationship	178,366.00	
d. O.H.I.P. subrogated claim	40,246.80	
e. Management fee	<u>54,845.00</u>	
TOTAL DAMAGES		\$2,867,625.80
f. Pre-judgment interest on general damages (5% for 10.5 years, 52.5%)	131,250.00	
g. Pre-judgment interest on the past income loss (\$1,425,630) and O.H.I.P. past loss (3.3% for 10.5 years, 34.65%)	<u>507,926.00</u>	
TOTAL PRE-JUDGMENT INTEREST		<u>\$639,176.00</u>
K.M.’S TOTAL DAMAGES:		<u>\$3,506,801.80</u>

b. The School Board

[29] The School Board submits that a reasonable assessment of K.M.’s damages is in the range of \$896,187.50 to \$934,312.50, broken down as follows:

a. General Damages	\$ 100,000.00	
	to 125,000.00	
b. Damages for loss of past income (\$375,000) and loss of future income, future loss of earning capacity (\$200,000)	575,000.00	
c. Damages for loss of interdependent relationship	10,000.00	
d. O.H.I.P. subrogated claim	0.00	
e. Management fee	<u>28,750.00</u>	
TOTAL DAMAGES		\$713,750.00
		to \$738,750.00
f. Pre-judgment interest on general damages (5% for 10.5 years, 52.5%)	52,500.00	
	to 65,625.00	
g. Pre-judgment interest on the past income loss (3.3% for 10.5 years, 34.65%)	<u>129,937.50</u>	
TOTAL PRE-JUDGMENT INTEREST		<u>\$182,437.50</u>
		to \$195,562.50
K.M.'S TOTAL DAMAGES:		<u>\$896,187.50</u>
		<u>to \$934,312.50</u>

8. The Evidence – Overview and Analysis:

[30] I have separated the evidence and the exhibits in this case into various sections for review. Within the various headings are a number of sub-headings that may necessarily overlap as the lifetime of K.M. is reviewed together with the financial evidence and legal principles in this case. The headings and sub-headings are not mutually exclusive, and the judgment is to be read as a whole.

[31] The evidentiary review in these Reasons for Judgment about K.M. is very comprehensive in order to obtain an accurate picture and understanding of K.M.'s life, both before and after the sexual abuse by Marson, and the timing of important intervening events in K.M.'s life. The approach taken has been generally chronological. Exhibit numbers and page locations in the exhibits, where appropriate, have been added for ease of reference and review. I have used the terms sexual abuse, sexual assault, assault and battery to refer to the sexual acts that K.M. suffered by the actions of Marson. These terms reflect how the actions of Marson are referred to depending on the nature of the proceedings and the circumstances, and by the evidence and by the witnesses in their evidence.

9. The Sexual Abuse by Marson, 1978 to 1980:

[32] The background facts as to the nature of the sexual abuse by Marson on K.M. are not in dispute, but are important in placing the events occurring between the parties in context.

[33] Marson was a grade 7 and 8 elementary school teacher at Harry J. Clarke School from September 1, 1972 to June 30, 1985. He began teaching for the School Board on September 1, 1962. He taught at other schools before and after Harry J. Clarke School. Marson retired from the School Board on June 30, 1997. He continued to volunteer after his retirement at a school within the School Board until the fall of 2006, when the police contacted school officials about the investigation into Marson's criminal conduct and he was removed as a volunteer. Marson was in a position of trust and authority as a teacher in relation to K.M. as a student. Marson, born May 9, 1940, was 38 years old in 1978 when he began to sexually abuse K.M. who was 13 years old.

[34] It is not disputed that Marson established a petting or mini-zoo while at Harry J. Clarke School. Marson kept animals—including chinchillas, rabbits, mice and rats—in stacked metal cages in a room across the hall from his classroom. Marson then selected students within the school to “volunteer” to help him care for the animals, usually grade 8 boys. This extra work involved working with Marson during recess and after school and allowed Marson to develop a relationship with his victims.

[35] K.M. needed extra help with his reading and was kept in at recess and other times to do extra work to improve his reading abilities. This work was done in a small remedial reading lab, which had a number of carrels or small cubicles with privacy screens and a projector flashing words across a screen in a darkened room. K.M. would often be left there alone and unsupervised during his sessions, which were set up by the remedial teacher responsible for the lab.

[36] K.M. testified that he was 13 years old and in grade 7 (1978 to 1979) when Marson first began to sexually assault him by coming up behind K.M.'s chair in the reading lab. The reading lab was some distance away from Marson's classroom and had no windows. The door would sometimes be left open and sometimes closed. In K.M.'s words, Marson would “rub my privates, through my pants, squeeze my neck”. K.M. said “it was brief and fast, thankfully”. K.M. testified that it made him “super nauseous. Not the normal nauseous that I get at times – like extreme nausea”. Marson would squeeze K.M.'s neck to hold him in place. Then Marson would leave. K.M. said nothing to anybody. It happened “multiple, multiple times”. To this day, when K.M. watches television at home, sometimes the closed-captioning on the T.V. gives him “freak-outs ... [e]ven after all these years”. He tried to deal with it by getting sent to the office or any type of task. He felt ashamed. He felt terrible and continues to today, even all these years later.

[37] Marson gave K.M. Polaroid photographs of some young kid and cut-outs of “porno pictures”. The porno pictures were from magazines, some depicting oral sex from a gay magazine. Sexually explicit details of the Polaroid photographs of a boy with “a penis in his mouth”, and of a boy with a penis “½ way ...in his bum”, are referred to in K.M.'s statement to police (Exhibit #2, Tab #E2, pp. 322-26). In his statement, K.M. stated that he was sickened by those photographs and always will be for the rest of his life. K.M. stated he still feels sorry for that child in those pictures and that they truly affected his life, even before he became a parent.

[38] The sexual abuse escalated beyond rubbing K.M.'s genitals over his clothing. In grade 7 and into grade 8 (1979 to 1980) when K.M. was 14 years old, Marson “chose” K.M. to clean the

cages in his mini-zoo. When K.M. was getting wood shavings or bales of pine bedding for the cages, in K.M.'s words, Marson "come up behind me, grabbed my neck, rubbed my arse a little bit, then reached around, rubbed my front, moved the shavings, got in there, dropped to his knees, and performed oral sex on me". Nothing was said. K.M. would leave and go home. It made K.M. feel terrible, and he had extreme nausea. The work in the mini-zoo took place at recess and after school. Often, K.M. was the only student there with Marson. K.M. recalled three such incidents of oral sex in the mini-zoo.

[39] There were two more similar incidents of oral sex in the supply storage room at the back of Marson's science classroom. The supply room was behind the desk in the classroom and was used for storing "science stuff". Marson would put K.M. in there, pinching his shoulder. One or two of the five total incidents of oral sex, K.M. recalled, took place in grade 8.

[40] K.M. testified that he never fully took his pants off or removed his clothing. Marson never removed his clothing and there was no nudity. There was no touching of Marson by K.M. K.M. had no interaction with Marson outside of the school. The abuse then stopped. When asked "why", K.M. testified that he thought it was because he was starting to mature, and maybe someone else came along, and he was getting bigger. K.M. dealt with these events by trying to "forget all that part" of his life. He got a paper route, and did yard work—anything to get home and anything not to be at school. K.M. took odd jobs after school to get away from Marson. In his statement to police, K.M. wrote that he did not wash his "privates" or genitalia, trying to discourage Marson from sexually assaulting him. K.M. described that he became jaded and angry, and noticed differences in his mood. He was angry. After grade 8, K.M. did not see Marson again as K.M. attended a different school.

[41] The police began their investigation of Marson on August 8, 2006 as a result of an incident report that they had received from another victim. On October 13, 2006, Marson was arrested for sex-related offences on another victim. Marson's arrest was reported in *The Intelligencer*, the local Belleville newspaper, on October 25, 2006 (Exhibit #2, Tab #E8), advising of who the investigating officer was and asking for anyone who may have additional information about this case to contact the police. K.M.'s mother was reading the newspaper article and told K.M. that Marson had been "picked up for pawing at some kids". That's when K.M. told his mother, "He was pawing at me, too".

[42] K.M. came forward and first spoke to police on October 30, 2006 (Exhibit #2, Tab #E2, p. 296). Over the next several days, K.M. prepared a detailed, 51-page hand-written/printed statement (Exhibit #2, Tab #E2) outlining for the police his recollection of the sexual abuse by Marson on him and the impact it had on his life. He began writing the statement on October 30, 2006. He wrote again on November 1 and 2, 2006. He went to the Belleville General Hospital (November 7, 2006) to get medication for his anxiety. He tried to start writing again on November 12, 2006 and could not as he was having such a hard time dealing with it. Portions of his statement were then re-written on November 13 at 2:30 pm when he was in a "better mood". He wrote on November 13, 2006 for the last time. I note pages 10 and 36 of the hand-written statement, which would have been pages 305 and 330 of Exhibit #2, have been lost since 2006 when K.M. had the statement photocopied, although part of page 10 appears to be reproduced at page 295 of Exhibit #2.

[43] In his statement, K.M. referred to his learning disability, which he was led to believe had something to do with being “born blue”. He wrote that it’s believed that it had “a life-long affect [*sic*]” on him and his ability to learn and on some of his mental health issues to this day, as he was lacking oxygen at birth (p. 297). He wrote that at Harry J. Clarke School, the class he was in was referred to as the “ACCO squad”—ACCO being “dumb”. He wrote that he was struggling at school while his sisters were doing well. He was put in “special needs” classes because of his problems with learning. Marson had the trick of always knowing where he was (p. 301). K.M. felt that Marson’s private rubs went on far too long, writing “As I got my first hard ons, I had to wonder maybe this is O.K, maybe I was special??” (p. 301). His statement detailed how K.M. felt he was disclosing secrets to a complete stranger and that he never personally believed he would get to tell how the “actions of one man” could have affected his life and his ability to show his feelings in what may be considered “normal”. One of the reasons he never told anyone was because of the experience of his younger sister, who did tell about events with a male neighbour and nothing came of it, even after detectives came to their house, and how much it hurt his mother. K.M. thought no-one would believe him.

[44] His statement reflected that K.M. felt guilty, dirty, ashamed, and depressed and that he had his childhood stolen from him. He just felt like it was his fault, that he must have done something. K.M. expressed his anger at the school for allowing the animal cages that provided cover for Marson to prey on K.M. and other kids (p. 318). K.M expressed his anger that he “had enough problems trying to learn let alone having to worry about when that sick ‘f...r’ would come around and grab you or rub against you – mini-zoo, my ‘f...en’ ass, that was his personal play land” (p. 318).

[45] In his statement, K.M. states that Marson had touched him, “well over 100 times easy, rubbed against me probably more” (pp. 331-32). K.M. stated that he did not wash his “privates” during the years of the sexual abuse as he thought that would keep Marson away from him. It did not work. K.M. wrote that he compulsively cleans there now, 2-3 showers or baths a day, as he thinks he’s dirty there (pp. 332-33). K.M. wrote, “By no uncertain terms should my first boyhood erections be at the hands of a man” (p. 300). He wrote that it affected his relationship with his father and his ability to be at ease or trust him. He stated that it was his feelings for his Mom that have kept him alive for all these years. He hoped that it would not affect his relationship with his girlfriend, Y.G., who is “very, very special”. He wrote that he has not tried to kill himself since his son was an infant. He expressed at the end of his statement that he wished he had never come forward, but he would do what is asked of him in hopes that this “petifile” [*sic*] will never touch any more boys. He then stated, “God help anyone who ever touches my child” (pp. 343-44).

[46] On January 23, 2008, Marson was convicted by Justice S. Hunter in the Ontario Court of Justice of four counts of indecent assault and one count of common assault on a total of five victims (one count per victim), including K.M. Marson was sentenced to 2 years in penitentiary followed by 3 years of probation on terms. Marson pled not guilty to the charge involving K.M. (and one other victim), as Marson had medical conditions that had a substantial impact on his memory and his ability to recall past events. Marson did not deny the facts as alleged by K.M. in the criminal trial and offered no evidence to contradict K.M.’s allegations of the sexual abuse. Counsel agreed that a statement of evidence would be read in and be accepted as admissible evidence in the criminal case. Marson was found guilty of indecently assaulting K.M. contrary to

section 156 of the *Criminal Code*. Marson did not appeal his criminal conviction or his sentence (Exhibit #2, Tabs #E4-7).

[47] I find as a fact in this civil trial that the evidence supports the finding that K.M. suffered sexual abuse, assault and battery by Marson as alleged in his Amended Statement of Claim and as testified to by K.M. The assaults included rubbing K.M.'s genital area over his clothing well over 100 times, and five acts of forced oral sex. The abuse lasted one to two years and did, I find, involve use of physical force by Marson in the grabbing and pinching of K.M.'s neck and/or shoulder area to maintain control of him during the abuse.

[48] The disclosure of these most personal events, then and now, was clearly difficult for K.M. I find delayed disclosure is not unusual in victims of sexual abuse, and I draw no negative inferences from the delay, which was triggered by the external event of Marson being arrested in relation to another victim. I find K.M. was more vulnerable because of his learning disability, which isolated him in a remedial reading lab to help him learn and made him an easier, more accessible target for a predator like Marson. It provided Marson the opportunity to sexually abuse K.M.

[49] While there was physical force used, there was no violence, sexual penetration, or nudity by Marson. I find that it did not lessen the effects of the sexual abuse on K.M. The sexual abuse violated K.M.'s personal integrity. The repeated fondling and the forced oral sex by his teacher while physically controlling him to keep him in position had devastating mental health consequences for K.M. and changed the course of his life forever.

[50] I find the impact of the sexual abuse by Marson on K.M.'s life was severe.

10. K.M.'s Life before the Sexual Abuse by Marson, 1965 to 1978:

a. K.M.'s Family and Friends

[51] K.M. was born August 10, 1965 in Belleville, Ontario. He was part of a family of five consisting of his parents and two siblings: an older sister J.C., born in 1962; and a younger sister S.G., born in 1969. His parents married in 1959. His father D.M. had a university degree and worked as a warehouse manager. His mother A.F., now aged 77, completed grade 11 in high school and worked primarily in the banking field when not rearing the children, retiring from the bank around the year 2000. His mother continues to work today as a drug store clerk and demonstrates a strong work ethic.

[52] K.M. in his evidence described his early years in positive terms. K.M. testified that he and his sisters were close when they were younger. His sisters learned differently than he did. They got to play outside while he had to stay in a lot to do flash cards with his mother to improve his learning. Even now, K.M. struggles to learn new things.

[53] K.M.'s mother A.F. testified that prior to Marson, K.M. had trouble in school compared to his sisters, but out of school, K.M. was "just a normal, happy boy", "always a happy boy". He had lots of friends, and loved animals. She never observed any signs of any mental problems or

depression or anything like that in K.M. She was unaware of any drug use by K.M. before Marson.

[54] A.F. was cross-examined about K.M.'s childhood being a happy one. Dr. Singh's comments on K.M.'s past history in his June 15, 2009 expert report—about K.M.'s childhood being unhappy—were put to her. Specifically, Dr. Singh had reported that K.M.'s childhood was unhappy, that his mother would tie him up to a tree so he could not take off, that his father was aloof, and that to K.M., his mother had a tendency to be overprotective and overbearing. A.F. testified that she had no idea why K.M. would have told that to Dr. Singh. She did not agree that K.M. was unhappy. She testified that maybe because he was not doing well in school like the girls (his sisters), it bothered him. She agreed she may be overprotective, but she denied being overbearing, even now.

[55] His mother testified that as a child, K.M. was very helpful, always doing painting and snow shovelling. She could see him as an adult doing things like painting or cement work—something in a trade. She thought K.M. would grow up and have a normal family. He loved kids and animals.

[56] K.M.'s younger sister S.G. testified that she also attended Bayview Public School and Harry J. Clarke School. She achieved grade 12 in high school with average grades. She attended Algonquin College but did not complete her course. She began instead training and then working as a phlebotomist at the local Belleville General Hospital. She married and moved to the Kitchener area. She has worked at various jobs: as a phlebotomist; as a pharmacy assistant; entering computer data; and as an embroiderer. She divorced and re-married and had two children. She worked part-time until her children were older.

[57] S.G. testified that she was hired by GE in 2008. She has some welding, braising, and soldering credentials and she put rotors together in a factory. She has her fork-lift, crane and jib-boist tickets. When laid off, she works at her own embroidery business, which she started in 2010. She separated in 2011 and is now in a new relationship. During her last layoff, she cared for her ailing father, D.M.

[58] S.G. testified that her older sister J.C. completed high school like she did and did not do any formal post-secondary schooling. J.C. began instead working at a bank, which she did for many years. J.C. now works for social services as a welfare fraud investigator. She has been married four times, with one of her husbands passing away. She has been married to her present husband for almost eight years. S.G. testified that she and her sister have not been on their own very often, as they have been in relationships.

[59] S.G. testified that to her knowledge, neither she, her sister, nor her mother have ever been treated for depression. She testified that her father D.M. had a nervous breakdown from his job and he could not remember things. Her father once forgot her at church. She testified that her father's problem was with his memory. He has been now diagnosed as having *myasthenia gravis*, which affects his memory and his mobility and causes difficulties in his eating and swallowing. She testified that D.M. does not have depression or a personality disorder. In cross-examination, she testified that she was unaware that her father was being treated by Dr. Conn, a psychiatrist.

Dr. Conn's letter of January 11, 2005 (Exhibit #1, Tab #A58, p. 138) was put to her, where Dr. Conn stated that K.M.'s home environment had its problems and that it seriously affected K.M.'s development emotionally. If there were such problems, S.G. testified, she was unaware of them.

[60] S.G. testified that her home environment when she was a little kid was "normal". It was not dysfunctional. She does not recall seeing K.M. tied up to a tree, which came up in other evidence, but agreed that these sorts of harnesses with kids were commonplace at that time.

[61] S.G. described her relationship with K.M. before Marson as "awesome". She always was his little sister and got to tag along. He was happy, and they were a happy family. K.M. being in a remedial class did not seem to bother him. There were no signs of mental health injuries, disorders, depression, or personality problems. K.M. was very laid back, friendly and out-going. He had lots of friends. Animals would flock to him. He played hockey. Their parents went to his games. She described him as being her protector, and he always made her feel welcome with his friends. He had a great sense of humour, and he liked to joke. His learning disability at the time was never a big deal and there was no drama about it. He flipped his letters and had trouble with numbers.

[62] It was news to S.G. that K.M. had stated in his application for O.D.S.P. (Exhibit #1, Tab #F1, p. 430) that he had been "mentally ill [his] entire life" and that he believed it was "passed to [him] genetically" from his father, and that K.M. had told Dr. Singh that his childhood was not a pleasant one. She agreed that from her perspective K.M.'s childhood was happy, but she could allow for the possibility that from K.M.'s point of view, it was not.

[63] Looking back before the sexual abuse, she testified that as a little kid, she never saw anything in K.M. that was a sign or indication of depression or some other personality disorder. In cross-examination she was asked to look at K.M.'s referral form from the mental health clinic when he was age 11 for his inability to learn (Exhibit #1, Tab #A5, p. 63). S.G. would have been age 7 at the time. She knew that K.M. was in the remedial class run by Mrs. Whiting at school. She was asked if she had ever heard any discussion within the family or was aware that K.M. had been diagnosed by a medical professional as having a "childhood behaviour disorder", and she answered "no". She was further cross-examined about the reference (Exhibit #1, Tab #A35, p. 110) that K.M. said "he has been depressed since age 10". She did not see anything to indicate that.

[64] K.M.'s sister S.G. testified that if K.M. had not been assaulted, she thought "[K.M.] would have been married, with a whole bunch of kids, and lots of animals around". She said K.M. was always a "tinker" when he was little and would help out. He always loved animals and kids.

[65] K.M.'s maternal aunt M.I.C., now aged 75, testified. She is the younger sister of K.M.'s mother and has a grade 12 education. She has lived in Stouffville, Ontario most of her life. She owned and operated a sanitation supply distribution business until she sold it in 2007, with a transition period ending her involvement in the business completely in 2011. She was married and had two sons. Her husband passed away in 2002. She would see her nephew K.M. on family occasions in Belleville about four times a year prior to her husband passing away.

[66] His aunt M.I.C. described K.M. as a little boy as being “funny, laughing, loved animals, loved kids. Just a nice little boy”. He was a “happy little boy”. In the early years, she did not see any indications or signs of depression or behavioural problems. She was aware K.M. had some difficulties and did not do particularly well in school. She would not have said he was a “star pupil”. Her first awareness that there were some sort of problems arising from K.M.’s behaviour was before he was sent to Saint John’s School when K.M. was in his teens, but she cannot say how much before. The evidence is that K.M. started at Saint John’s School in the summer of 1980 and enrolled in grade 8 in September 1980 when he was 15 years old.

b. K.M.’s Education and School History

[67] K.M.’s school records are before the court (Exhibit #2, Tabs #B1-24). Due to their age, some of school records have been photocopied from microfiche and are difficult to read, and some are incomplete. I have referred to the school records from Saint John’s School of Ontario as such, but they are also referred to as “St. John’s School”. They are one and the same school.

[68] Prior to K.M. starting school, the family moved from Belleville to Thornhill for his father’s work. From the school records, K.M. attended grades 1 to 3 at Baythorn Public School within the jurisdiction of the York County Board of Education. There was reference to Kindergarten by both K.M. and his mother A.F. in their evidence, but there were no school records about Kindergarten. K.M.’s medical records do indicate that he had a school medical on June 8, 1970, at 4 years of age, which may have been for Kindergarten, as he did not start grade 1 until he was 6 years of age.

[69] K.M. started grade school in September 1971 at 6 years of age. In grade 1 (1971-1972), his teacher described K.M.’s efforts at school as trying very hard, having a good attitude, being co-operative, helpful, considerate, getting along very well with his peers, being enthusiastic about each new situation, and eagerly participating. His teacher recorded that in reading, K.M. relied totally on memory and that he was steadily improving in math. The signs of his difficulty in learning at school were present at an early age. A.F. wrote a parent note on K.M.’s grade 1 report card that said, “We are disappointed in [K.M.’s] progress and upset that it was not brought to our attention sooner.”

[70] In grade 2 (1972-1973), his teacher described K.M.’s efforts at school as being usually a hard worker, usually putting a good effort into his work, listening carefully and quickly grasping meaning from stories, being an eager student, and willing to help. His teacher recorded that K.M. was still having difficulty with math. His reading was improving. His grades overall were C’s (average) with a D (fair) in math.

[71] In grade 3 (1973-1974 – first term), K.M.’s oral reading was improving and he still needed extra help with math. His first term grades overall were C’s (average) with a B (good) in oral reading and a D (fair) in math and social studies.

[72] After the first term of grade 3, K.M.’s family moved back to Belleville for his father’s work. K.M. then attended Bayview Public School in Belleville, within the jurisdiction of the defendant School Board, for the second term of grade 3 and for grades 4, 5 and 6.

[73] The grading system at Bayview Public School was different than at Baythorn Public School. Students in grade 3 at Bayview Public School were evaluated in reading, phonics and number work as either “introduced”, “understands” or “not mastered”. From grades 4 to 6 at Bayview, students were evaluated differently again, based firstly on “academic progress in relationship to their peers” (as either “above average”, “average”, or “below average”), and secondly on “effort” (as either “satisfactory” or “needs improvement”).

[74] In June 1974, K.M.’s grade 3 (1973-1974 – second term) teacher recorded that K.M. had made many friends since coming to Bayview. His assessment revealed that K.M.’s level of understanding of concepts was good, but that he had not mastered reading or math to his grade level. The teacher found that K.M. was going to have to put forth a more sincere effort and improve his work habits to be successful in grade 4.

[75] In grade 4 (1974-1975), his teacher indicated that she was pleased with the effort K.M. had put into his work. He understood what he had read and added a great deal to group discussions. K.M. had improved in math, but still had some difficulty understanding new concepts. She recorded that his reading was below average. She recorded that K.M. was “a cheerful, happy boy. He has a great sense of humor. He is a bit talkative at times however, which hampers the completion of his work on time. The neatness of his work has improved a great deal”. He had put “a great deal of effort into his work this year with very good results”. His overall academic grade was “average” or “below average” and his effort grade was “satisfactory” while being still weak in certain areas of reading and math.

[76] K.M.’s grade 5 (1975-1976) and grade 6 (1976-1977) reports from Bayview Public School appear to have been either intermingled in the photocopying process (Exhibit #2, Tabs #B6-7), or the grade 5 report is simply not available. There is very little material definitively from grade 5. Page 234 at Exhibit #2, Tab #B6 appears to be K.M.’s reading and math evaluations for grade 5 (1975-1976) as it refers to K.M. being in the “Low 5 Reading Program”. His teacher described K.M. as “working hard” and that he was “concerned about his progress”. The rest of Tabs #B6 and 7 appears to be related to grade 6 at Bayview. K.M. was in Mrs. Turrall’s remedial math class and scored “weak” in division and fractions. K.M. was reading below his grade level but scored “strong” in his comprehension level and “weak” in spelling.

[77] In his first attempt at grade 6 at Bayview (1976-1977), K.M. was 11 years old. K.M.’s teacher noted in March 1977 that K.M.’s “devil-may-care” attitude hampered his reading work. In June, she noted that he seemed to have the ability but not the maturity to use his ability consistently. K.M. scored 1/41 in the year end math class, having done one multiplication correctly. His teacher noted that K.M. was hampered by his reading level and did not seek out books suitable to his level. He had worked hard on his speech and improved his research skills. Overall, he was considered to have slightly improved in the second June term. His participation, working independently, completing assignments on time, relationship with his peers and completing his work neatly were all recorded as satisfactory or needs improvement. His teacher noted that K.M. needed to “strengthen the basics to ensure a strong foundation for future endeavours”.

[78] K.M.'s teacher reported that he should have worked much harder, that his efforts were inconsistent, and that he was indifferent. His teacher considered that K.M. may need to spend another year in grade 6 if only to "shock him out of his indifference and to ensure future success". A.F. agreed in cross-examination that K.M. was not only not doing well at school academically, but that he had a poor attitude at school and was "indifferent". This was clarified in re-examination to mean indifferent to the "work" at school but not to the kids and sports. His overall academic grade was "below average" and his effort grade was "needs improvement". He was failing in math and reading. K.M.'s placement for September 1977 was to repeat grade 6, which he did but at a different school.

[79] K.M. described his experience at Bayview Public School as "normal", but he acknowledged that he struggled with his learning. He played any sport he could. He was good at sports. He was a class clown and recalls getting sent to the principal's office for that, but little or no mention is made of this in his school records. He was not truant and did not try to run away from school. In cross-examination, K.M. agreed that he had to go to the principal's office regularly, meaning once a week, and he had to "write lines". He said there were lots of kids who had to go to the principal's office. He agreed that he went to the principal's office just as often before and after encountering Marson.

[80] During this time, K.M. had lots of good friends. He played street hockey, pond hockey, and baseball, skateboarded, and enjoyed fishing. He testified that his home life was fine. He described himself as a "regular" kid with lots of friends coming around to his house all the time.

[81] K.M.'s parents decided to have K.M. change schools to repeat grade 6. Initially, A.F. thought that it was grade 4 that K.M. repeated, but when the school records were reviewed with her in cross-examination, she agreed that it was grade 6 that K.M. repeated. K.M. started at Harry J. Clarke School in September 1977. Marson was a teacher at this school, but K.M. had no contact with Marson while in his repeat year of grade 6.

[82] The grading system at Harry J. Clarke School was different again than at Bayview. Students were evaluated first by "academic achievement" related to other students at the same grade level by way of letter codes A (80-100) to E (below 50), and second by "social development and work habits", or generally, their effort at school, by V (very good), S (satisfactory) and N (needs improvement).

[83] In K.M.'s repeated year of grade 6 (1977-1978), he was 12 years old. K.M. continued to be very active in sports. He played floor hockey and soccer, and went out for all the teams that were playing when he started at Harry J. Clarke School. K.M.'s teacher recorded that his effort had been good and his improvement was apparent in areas of oral expression and comprehension. His assignments were "satisfactory" and on time. In math, his teacher recorded that he had a good knowledge of basic concepts but became discouraged by his careless mistakes. His effort was consistently described as "satisfactory" and his responding well to suggestions and/or criticisms as "very good". K.M.'s overall grade was C (60-69) with a B (70-79) in spelling. He was advanced to grade 7, where he came into contact with Marson as his science teacher. Whether he "achieved" grade 6 on his second attempt or was "pushed through"

or “advanced” to grade 7 is unclear from the records. K.M. testified in cross-examination that he failed grade 6 twice.

c. K.M.’s Problems with Learning

[84] K.M. struggled to learn since the beginning of school. It was noted as early as grade 1 that K.M. had particular difficulties in reading and math. He was in remedial learning classes at school from at least grade 5, including in his repeat year of grade 6. At Harry J. Clarke School he was in “special ed”. K.M. testified that he was “a slow learner” and admitted that he had “struggled through school with math and reading” since “Kindergarten”.

[85] K.M. did not learn like his sisters. K.M.’s mother A.F. would do a lot of flash cards and review fractions with him. Dr. Manderville, his family doctor, saw K.M., aged 11, on June 24, 1977 and made a note that K.M. had “learning difficulties – failing grade 6”. He made a referral to the “mental health clinic” (Exhibit #1, Tab #A1, p. 2). K.M. had just failed grade 6 at Bayview Public School. Before any encounter with Marson, K.M. was referred by Dr. Manderville to the Edith Cavell Clinic, which is a mental health clinic in Belleville.

[86] On July 13, 1977, a consultation note was sent back to Dr. Manderville from a clinic (Exhibit #1, Tab #A5, p. 63). In the diagnosis section of the referral form, the diagnosis was written as “childhood behaviour disorder” and the treatment program was written as “psychological testing”. His condition on termination of the assessment was listed as “improved”.

[87] Of note, it has been specifically agreed and stipulated by both counsel that neither Dr. Manderville nor anyone in his office authored or created this referral form. This form came back to Dr. Manderville, after he made the referral, from some clinic as part of their intake process. Simply put, we do not know who authored the information on the form. I note it is a form used by the Ministry of Health.

[88] K.M. recalls that at the Edith Cavell Clinic, they were trying to make programs to help him learn. He had problems with repetition, number sequences, and fractions, and his letters would turn on him. He would use repetition and practice techniques to the point of frustration, but eventually he would grasp things. He recalls doing some puzzles. K.M. does not know if he had any psychological testing. K.M. was cross-examined in detail about the reference on the form to “childhood behaviour disorder”. He testified that no-one discussed with him that he had a “childhood behaviour disorder”. He had no knowledge of it at 11 years old. K.M. testified that no-one, from age 11 until now, has raised with him that he had some form of behavioural disorder.

[89] In cross-examination, K.M. disagreed with the suggestion put to him that his learning difficulties caused him problems in getting along with other kids, being teased, or being given a hard time at school. K.M. testified that he was a “good, strapping sized kid”. He was not picked on or called “dummy”. He just had a problem learning. K.M. testified that he “kept up” with the other kids at school in grades 1, 2, 3 and 4, and that he was not a complete “retard”. He appeared to take offence to the suggestion that he was not a good student. He had to repeat grades, but he

testified he kept up with the other kids in the class. He testified that his learning disability was not a problem for him within the family.

[90] K.M. agreed in cross-examination that his learning disability had been called a disability in relation to spatial perception. K.M. described it in relation to backing up trucks. What he saw in the mirror was not what was actually there. He does not do well with road maps and gadgets. K.M. does not have a cell phone as to him, they are complicated. It's worse if he's under stress and trying to learn. No-one ever offered him any treatments or therapies to help him with his disability. At Moira Secondary School, he was in the occupational class in grade 9, which was designed as a two-year course for those students who would not finish high school.

[91] K.M. agreed in cross-examination that he never thought he would finish high school and that his lot in life would be that of a labourer, which is what he has done for most of his working life when he could work.

[92] A.F. testified that as parents, they were aware of K.M.'s spatial perception issues, which surfaced particularly when K.M. was playing hockey. She took K.M., age 11, to the Edith Cavell Clinic to help him with his reading as recommended by Dr. Manderville. No-one ever told her a diagnosis for K.M.'s learning problem. K.M. did not do well at school. He spelled things differently. He saw things differently and sometimes they were backwards. She testified that "dyslexia" is what they call it now.

[93] A.F. did not agree in cross-examination that K.M. had a general "inability to learn", but that he had "school problems". He could learn other things—he could skate, and ride a bike. A.F. agreed that inside the classroom, K.M. had an inability to learn. A.F. testified that she did not recall any behavioural issues at school with K.M. before Marson other than when K.M. was sent home once for throwing a snowball. She did not recall phone calls from the principal's office. She did not agree in cross-examination with the referral form diagnosis in Dr. Manderville's records of K.M. having a "childhood behaviour disorder".

[94] She testified that K.M. never had a behavioural problem, except that he had difficulty learning at school. A.F. testified she recalled K.M. being tested by Mrs. Whiting, who ran the remedial class at Bayview Public School. She did not recall anyone doing any psychological testing on K.M. at that time. No-one told A.F. that K.M. had a "childhood behaviour disorder".

[95] A.F. was cross-examined about K.M.'s attitude towards school. She did not agree that K.M. had a poor attitude towards school, but agreed he had found school very difficult because he was having trouble learning. She testified, "He loved school, he loved the kids, he loved all of that. He just had trouble learning". A.F. testified that she never thought K.M. would finish high school because of all the troubles he was having.

[96] K.M.'s aunt M.I.C. testified that she was aware that K.M. had difficulties in school and did not do particularly well, and that K.M. was not a "star pupil". She testified in cross-examination that she was unaware that K.M. had to repeat grade 6.

d. K.M.'s Drug Use

[97] K.M. testified that prior to the sexual abuse by Marson, he used drugs to some extent. K.M. testified in chief that his drug use was “just a little bit of messing around, but nothing...nothing serious”. After the sexual abuse, he regularly used “marijuana, LSD, uppers”. In cross-examination, K.M. testified that before any encounter with Marson, he used “marijuana, ‘bennies’ we used to call them – caffeine pills – amphetamines”. The drugs were around. K.M. did not supply marijuana to others. Counsel put to K.M. that this started in grade 6. K.M. answered, “yeah” but that his “marijuana use, and amphetamines, and that kind of stuff, was just very gradual – the last part of Harry J., and then on into Moira, and Saint John’s”. That’s when, K.M. testified, he “started to develop dependency issues”. K.M. testified in cross-examination that before any sexual abuse was committed by Marson, “I experimented with marijuana and caffeine pills”. The caffeine pills, or “trucker pills”, could be bought at the drug store.

[98] K.M. agreed with counsel in cross-examination that his drug use started in his second year of grade 6, when he was 13 years old, if “that’s how the math goes”. K.M. agreed that he was 13 years old in grade 6. The school records, however, indicate that K.M. began his second year in grade 6 in September 1977 when he was actually 12 years old and he would have been 12 for the entire school year. I take this evidence as an error as to K.M.’s age but that the pre-Marson drug use was in fact, as K.M. testified, in his second year of grade 6 at Harry J. Clarke School. This does not appear to be in dispute. K.M. was 12, not 13, years old in his second year of grade 6. The abuse by Marson started in grade 7 when K.M. was 13 years old. The evidence demonstrates that K.M. is not a particularly good historian as to his exact age at various points of time in the past.

[99] The reason given by K.M. for taking the drugs pre-Marson was “curiosity”. The extreme caffeine gave him a “[j]ust a mood lift”. He testified that he was just experimenting with what was accessible.

[100] K.M.’s mother A.F. testified in cross-examination that before Marson, when K.M. was in his repeat year of grade 6 at Harry J. Clarke School, she was not aware of any drug use by K.M. She was not aware that he was using marijuana and “bennies”, and that came as a complete shock to her. She testified, “I was only his mother”.

[101] K.M.’s sister S.G. testified that before the Marson period, she did not see any indication that K.M. was using drugs. In cross-examination, S.G. testified that she was unaware that before Marson, K.M. was taking street drugs occasionally, such as marijuana and “bennies”. It did not surprise her, but her clear understanding was that K.M.’s drug use started after Marson.

[102] K.M.’s aunt M.I.C. testified in cross-examination that she was not aware that K.M. was taking street drugs, such as marijuana and “bennies”, when he was in grade 6.

e. K.M.’s Psychological and Mental State

[103] K.M. self-described his mental state as a little kid before Marson as “regular”, “normal”. He said he did not want to get grounded, but he wanted to have fun. He testified, “I was just a normal child. I had a good neighbourhood. We had a beautiful home, with beautiful

things. I wasn't deprived, that way, in any way. I had a normal childhood". K.M. testified that before Marson, he did not really recall anything bad happening to him, just that he did not learn like his sisters. His sisters were having fun, and he was doing flash cards to help him with his learning.

[104] K.M. was cross-examined about Dr. Singh's reference to his mother tying him to a tree when he was a young child. K.M. testified that his mother was looking after three kids and that he liked to "take off". Their house backed onto a pond. His mother would put him in a lederhosen-type harness while she did the laundry, and his little sister (S.G.) would be in the playpen. The harness was leather with felt inside and had hooks on the back. In re-examination K.M. testified that this type of harness was used back in the 60's and he wasn't left in it for long. He was never hooked up and just left there. K.M. didn't think he was being abused—it was just part of how parenting was done then.

[105] He described his personality as a "kid" as "he's going to be a handful, but he's okay". He was happy. He testified that he was never diagnosed or treated for any kind of mental health issue when he was a "kid" before his encounter with Marson, other than going to the Edith Cavell Clinic for his learning disability. He first realized he had some "mental health issues" going on when he was at Saint John's School for boys in grade 8.

[106] K.M.'s pre-Marson childhood medical records, from January 13, 1966 to 1978 (Exhibit #1, Tab #A1, pp. 1-2), contain very few entries and are unremarkable. There are no references to or any history whatsoever of behavioural difficulties or mental health injuries other than the one note of Dr. Manderville on June 24, 1977, referenced earlier, about K.M. having "learning difficulties" and "failing grade 6" and being referred to the "mental health clinic". The childhood medical records of K.M. contain notations of having a school medical on June 8, 1970 at age 4 and otherwise, the limited entries detail K.M.'s symptoms of common childhood ailments such as tonsillitis, skin rash, and ear infections.

[107] In cross-examination, it was put to K.M. that before his encounter with Marson: as a young child, his mother tied him to a tree; his father had anxiety and depression; and he had learning disabilities and did not learn as much as his sisters, to which K.M. answered, "I had problems before I encountered Mr. Marson".

f. Summary

[108] I find that the preponderance of evidence is that prior to the sexual abuse by Marson, K.M. was a happy, energetic child who functioned well in his family and in the classroom despite his problems with learning. The evidence of K.M. and his family which I accept as credible is that K.M. was a normal, happy boy in spite of his learning difficulties, which limited K.M.'s academic success at school. He loved all the other activities at school—the other kids, the activities, and especially the sports. He was close to his sisters and had lots of friends who came over to his house all the time. K.M.'s childhood memories I find were generally positive. I do not draw a negative inference from A.F. using a harness attached to a tree to make sure K.M. was safe. It was a different era. I find that K.M.'s family life was not dysfunctional, and that he did not have a history of behavioural disorders.

[109] I find that K.M.'s problems before Marson were related to his difficulties with learning. In his early years at elementary school, I find that K.M. functioned in a positive way, as to his efforts made at school. He tried very hard, but he could not obtain good academic achievement results, particularly in math and reading. As he grew older, his learning difficulties caused him more frustration and problems with his learning at school. He was assessed as being indifferent and as having a poor attitude at school, and placed in special educational classes for slow learners. I find on the evidence that K.M. is not intellectually slow at all, as later confirmed by Dr. Wolfgarth and Dr. Silver. I find that K.M.'s poor performance at school does not mean that he suffered from intellectual "slowness", which later was relied upon by Professor Charette, the School Board's economist, as a reason for reducing K.M.'s income loss claim. His poor school performance, and his so-called indifference and poor attitude, were, I find, as a result of his misunderstood and undiagnosed learning disabilities. I find that K.M. is an intelligent man. Academically, I find that because of his learning difficulties, he achieved at least grade 5. Beyond that and based on the school records produced, K.M.'s "achieved grade" level as opposed to his "passed through" grade level at school is unclear. K.M.'s grades before Marson were poor and he was a failing student. He was made to repeat grade 6 and his parents sent him to a different school, one where Marson would later become his teacher.

[110] K.M. and his mother A.F.'s evidence, which I accept, is that K.M. would not have graduated from high school, even if the sexual abuse had not occurred. I accept the School Board's position that K.M. would not have graduated from high school, even if the abuse had not occurred, as being supported by the evidence which I find to be credible.

[111] There is no cogent evidence, I find, of any traumas suffered by K.M. prior to the sexual abuse by Marson. While no family's history is perfect, these kinds of broad assertions must be supported by cogent, credible evidence in order to be relied upon, which I find is lacking in this case.

[112] I accept K.M.'s position that prior to the sexual abuse by Marson, K.M. was a happy, energetic child who functioned well in his family and in the classroom despite his learning difficulties that made academics a struggle for him since he began school. This is supported by the evidence, which I accept as being credible.

11. K.M.'s Life after the Sexual Abuse by Marson, 1978 to 2017:

a. K.M.'s Family and Friends

[113] K.M.'s mother A.F. testified that there is "no comparison" between K.M. as a little boy before Marson and now. Before Marson, K.M. was always "loving, happy" and "had lots of friends". Now, K.M. is very different. He has good friends, but he does not have a lot of friends. K.M. is not the happy person that she thinks he should be.

[114] K.M.'s parents were divorced in 1989. Both A.F. and K.M. described that there was no fighting or name-calling in the home. A.F. testified there was no big reason for the divorce; they had "nothing in common anymore". D.M. had developed some form of Alzheimer's or other amnesic memory disorder a few years before and could not work, and the family home was sold.

A.F. testified in cross-examination that D.M. was diagnosed with Alzheimer's in 1988, that he left work because of stress, and that otherwise he did not have mental health problems. A.F. was not aware of any family history of mental illness. Her former husband was adopted and she did not know his family history. A.F. described the family as being a "normal" family. She testified that the family was not abnormal or dysfunctional.

[115] In cross-examination, A.F. agreed that the divorce was very hard on the children. The divorce was after the children had left home, so she did not think the divorce made K.M. depressed. She saw as much of K.M. before as after the divorce. Dr. Manderville's 1989 note (Exhibit #1, Tab #A1, p. 7) of K.M. being depressed and the referral to Dr. Brant, a psychiatrist, were put to her. A.F. agreed that she did not make any observations that would confirm K.M. was depressed because of the divorce. In re-examination, she testified that K.M. never has confided in her as to what was wrong or bothering him.

[116] A.F. testified that while K.M. was at Moira Secondary School in grade 9, she became aware for the first time that K.M. had some sort of mental or behavioural issue when the police came and told her and her husband that K.M. had not gone to school and was using marijuana. It was the first time she had heard about the drugs. They instituted "tough love" strategies and rules within the household. K.M. moved out.

[117] A.F. first became aware of an actual, diagnosed, psychiatric issue with K.M. when he was admitted to the Belleville General Hospital and she visited him. This was sometime after her divorce in 1989 and when K.M. was seeing Dr. Brant. The hospital records note that K.M. was admitted to the Belleville General Hospital from December 27, 1989 until January 19, 1990. She was not told the nature of K.M.'s psychiatric diagnosis.

[118] A.F. now lives in the same building as K.M., and they eat together nearly every night. A.F. described K.M.'s moods as "funks". If K.M. is in a "funk", he does not do anything and will not come down to her apartment. These "funks" happen a lot, weekly, and she has gone days without seeing him. A.F. testified that K.M. does not have a wide circle of friends. He is good with the people he knows. A.F. confirmed that K.M.'s employment has been sporadic. K.M. had problems with employers as he would go into a "funk" and he could not be counted on to show up for work. She confirmed that K.M. does odd jobs for people, like painting and cement work. They understand and wait if he is in a "funk".

[119] K.M.'s sister S.G. testified that in the time period after Marson, K.M.'s personality and behaviour had changed. She was no longer welcome. She could not tag along. She could not go and see him. It was like she was not his sister anymore. K.M. did not talk. He would sleep for long periods of time. He did not interact with the family anymore. S.G. testified that K.M. was a "totally different person". He was not funny anymore. He was sort of withdrawn. He did not have many relationships with other people. They did not have a family relationship anymore.

[120] S.G. has had very little contact with K.M. since their school years. She once found K.M. when he had tried to overdose at his kitchen table. Later, she would see K.M. at their mother's place. K.M. would never stay; he would say "hi" and disappear and not engage in family functions. She would see him maybe three times a year. She was aware of his problem with

depression. She testified that he does not get involved and that “[i]t’s like a detachment ... He doesn’t want to be with any of us”. She added: “He’s not the same happy person that I knew as a kid”.

[121] S.G. testified that she tries not to see her brother because she is afraid it will be the last time she will ever see him as he has issues with trying to commit suicide. It’s very hard on her to see him. She testified she does not know if K.M. will get any better. She found out from their mother about the sexual abuse by Marson when her mother asked her if she knew anything. She said “no”. K.M. never told her about the sexual abuse.

[122] S.G. was asked in chief about the difference in K.M. as a little boy before the assaults and today. She testified, “[K.M.] isn’t the happy-go-lucky kid he used to be. He’s turned into a loner. He doesn’t have the humour he used to have... It feels like he does not want to be part of our family”.

[123] K.M.’s aunt M.I.C. testified that since she resumed contact with K.M. in 2002, she observed that it was difficult for K.M. to deal with people. She thought he “feels, somewhat inferior” and that he had not done things in his life that “maybe his cousins have”. There were a number of times that he was unhappy and did not seem able to cope with “just life”. He would be working and he would just have to go home and leave. She was aware of his diagnosis of depression and K.M. has talked with her about it.

[124] His aunt M.I.C. sees K.M. “as being damaged” in how he feels and interprets things. He can take offence easily. She testified that neither she nor her sister A.F., nor any of her other siblings, had had any mental health issues such as depression.

[125] K.M.’s neighbour and good friend T.A., aged 58, testified. She has known K.M. since 2009 when she moved into the same apartment building in Belleville. She works as a waitress. Previously, she ran the café at the Belleville General Hospital for ten years. She visits and has coffee with K.M. a couple of times a week and they talk. K.M. never told her in detail about the sexual abuse and what happened to him. He told her about the abuse, in just a round-about way, a couple of years after she met him.

[126] T.A. described K.M.’s mood and behaviour as, “when he’s happy, he’s very happy. I can tell when he’s not feeling the best”. She can tell by his face. K.M. does not speak, or his speech is “very clipped”, and he does not want to talk. This could happen a couple of times a week, lasting anywhere from that day to a couple of days. She may not see him for a week. He will not answer the phone. What sets K.M. off is not being comfortable talking about anything. He is “a lot withdrawn”. He’ll talk about things after.

[127] She described K.M. as being friendly with everybody. He’s always joking. He spends more time by himself than around other people. He is not good at dealing with an interpersonal issue between himself and another individual. He will isolate himself. He does not put himself into too many situations where he does not know anybody. T.A. testified that she does not think K.M. has a good opinion about himself. She testified that as long as she has known him, K.M. has not had any kind of relationship with anyone.

b. K.M.'s Education and School History

[128] K.M. began grade 7 (1978-1979) at Harry J. Clarke School and encountered Marson in September 1978, at 13 years old. The first page of K.M.'s school records for the end of the third term of grade 7 recorded 15 out of 16 achievement entries for every term as "E" (for a mark below 50) and one entry as "D" (for a mark of 50-59). K.M.'s effort entries were recorded as 15 entries of "3" (meaning "unsatisfactory"), 6 entries of "2-3" (meaning somewhere between satisfactory and unsatisfactory), and 4 entries of "2" (meaning satisfactory)—one in music and the remaining three in physical education. He was reported to have "apparent lack of sincere, consistent effort" (comment 31), and more self-discipline was desired (comment 34).

[129] K.M. was a failing student before and after being abused by Marson. K.M. agreed in cross-examination that his school marks were about the same after encountering Marson as they were before. I find there is an indication of some decline in K.M.'s grade 7 school records, however, as to his "efforts at school". K.M. was nonetheless a failing student and failed grade 7. K.M. was "passed through" to grade 8 without "achieving" grade 7.

[130] This fact is confirmed in a letter of June 28, 1979 (Exhibit #2, Tab #B11) from the principal of Harry J. Clarke School to K.M.'s parents, stating that while K.M. was transferred to grade 8, his academic performance for grade 7 had not been satisfactory and "promotion to Grade 8 [had] not been achieved". The school stated that advancing K.M. to grade 8 was still preferable and that if K.M.'s achievement continued to be unsatisfactory he would be accepted only for the two-year occupational program at the secondary school level.

[131] In grade 8 (1979-1980) at Harry J. Clarke, K.M. was 14 years old. For the end of the third term, the school recorded 15 achievement entries of "E" (for a mark below 50), 9 entries of "D" (for a mark of 50-59), 6 entries of "C" (for a mark of 60-69), 1 "B" (for a mark of 70-79) in language arts, and 1 "A" (for a mark of 80-89) in technological and vocational studies, described as safe operation of power and hand tools by producing projects in sheet metal, wood, and plastic. K.M.'s efforts entries were recorded as 15 entries of "3" (for unsatisfactory), 9 entries of "2-3" (somewhere between satisfactory and unsatisfactory), 14 entries of "2" (for satisfactory), 1 entry of "1-2" (somewhere between exceptional and satisfactory), and 1 entry of "1" (for exceptional) in technological and vocational studies, as previously described. It was reported that K.M.'s assignments/projects were not complete (comment 23), he was "inattentive to instruction" (comment 26), he was a "disruptive influence in class" (comment 27), he had an "apparent lack of sincere, consistent effort" (comment 33) and "more self-discipline desired" (comment 36). K.M.'s efforts at school were poor in grade 8, and he remained academically a failing student as he had in the past.

[132] K.M. failed grade 8. K.M. was transferred to grade 9, which was "considered to be in the student's best interest, even though overall achievement does not qualify for promotion". K.M.'s achievement and effort were better in grade 8 than in grade 7, but not sufficient to "achieve" grade 8. His math and language skills still remained far below his grade level.

[133] To help him get a high school education, K.M.'s parents made the decision to send him to Saint John's School of Ontario—a private, bare-bones, residential boys boarding school in

Claremont, Ontario, where corporal punishment was regularly and freely used. The school had an excellent outdoor program, which appealed to K.M. Saint John's School registered K.M. in the summer of 1980. This school was not the reform school with the same name. K.M. started school in the summertime with the usual two week school canoe trip, not long after several students and staff had drowned on a similar school canoe trip in the past.

[134] In September 1980, Saint John's School placed K.M., age 15, in grade 8 (1980-1981)—K.M.'s second attempt at grade 8. His science teacher recorded that his mark was disappointing (35 out of 75) and that K.M. seemed unable to retain the material and put it to use. His other grades were poor, but he was noted to have a fine class attitude. By Easter, his average had improved to 52%. He was described as a cheerful, helpful boy with a very good heart. He worked well on the maintenance crew and it was recorded that he had a "successful year". He did very poorly in math and literature. I am unable to ascertain from the records produced whether or not he "achieved" grade 8. I infer from his overall grades that while he completed grade 8, he did not in fact "achieve" grade 8 academically while at Saint John's School.

[135] K.M. was cross-examined in detail about his treatment when at the school. He testified that you would be hit with a wooden "beat" stick for behavioural reasons. He was never hit because he was "stupid". He was hit because he ran away, he smoked, he didn't listen, he'd leaned on his chair, bit on his pen, or didn't say "sir" or "ma'am". His discovery transcript was put to him where he said that their learning method was "repetition, repetition, repetition" and that if you didn't get it, they'd go back to the stick to get your attention. K.M. said it was taken out of context. He testified that if you were not paying attention and not trying as hard as you could, there was no doubt that you'd get a couple of touches of the stick. He was never beaten because he did not learn like other kids. K.M. agreed that disciplinary use of the stick was a regular event for him, even for trivial offences. K.M. agreed it would constitute abuse today. In re-examination, K.M. testified that the wooden stick did not really bother him. Some kids were terrified, but K.M. said it did not hurt that much.

[136] K.M. went back for a second year at Saint John's in the fall of 1981 but ran away from the school and went home to Belleville. He described his experience at Saint John's as being terrible and overall horrible and that he was homesick like crazy. He was by then 16 years old and old enough to decide if he wanted to be in school or not. He told his parents not to pay the school, as he was not going back. K.M. never did go back to Saint John's School.

[137] K.M. testified that it was at Saint John's that he realized something was wrong. He felt "crushed". He testified that he started to "know what depression is – and I didn't think it would be my friend for all these years, but ... that's when it started, and it's stuck on, ever since". He was 15 years old when he started at Saint John's School. K.M. has suffered with depression for the past 36 years.

[138] A.F. testified that while K.M. was a student at Saint John's School, K.M. hitchhiked home three times. After the last time, K.M. moved back home to Belleville in the fall of 1981, and enrolled in grade 9 at Moira Secondary School in the two-year occupational class, designed for those students that would not complete a high school education. A.F. testified that K.M. was skipping school while at Moira.

[139] K.M. did not stay at his parents' home very long and he quit school. His parents kicked him out of the house; if he was not in school, he could not live with them. K.M. testified that his parents got involved with "tough love" and that they had "rule-itis" and that he could not stay with them. K.M. never attended secondary school again and moved into a friend's (B.H.) basement.

[140] It is difficult, from looking at the old school records produced, to identify K.M.'s actual educational grade level "achieved" as opposed to the grade level to which K.M. was "passed though". The evidence is inconsistent. Anecdotal, Dr. J. McMillan, a doctor working with Dr. Singh, recorded in his notes made July 30, 2007 that K.M. had a grade 6 education (Exhibit #1, Tab #A71, p. 164). Other references in the evidence refer to it being grade 8 or even grade 9, but are not substantiated by the school records. K.M.'s C.P.P. application stated that he had "completed" grade 8. Dr. Singh reported on June 15, 2009 that K.M. achieved between a "grade six or eight education".

[141] K.M. testified in cross-examination that he agreed that he had failed grade 6 twice, failed grade 8 twice, and never finished grade 9. The school records indicate that K.M. definitively failed grade 6 at Bayview Public School, grade 7 and grade 8 at Harry J. Clarke, and grade 8 at Saint John's school, and that he never completed grade 9 at Moira. Whether K.M. failed his second attempt at grade 6 at Harry J. Clarke or was "passed through" to grade 7 is difficult to ascertain from the school records. K.M. did slightly better in his second attempt at grade 6 at Harry J. Clarke, but overall he was a very poor student and his grades very low. He was below the level of his peers in math and reading. K.M. was not a particularly good historian on this issue either. K.M.'s evidence is that he failed grade 6 and 8 twice, but he did not mention that he also failed grade 7. Based on K.M.'s evidence, K.M.'s educational grade level "achieved" would be grade 5.

c. K.M.'s Problems with Learning

[142] K.M.'s problems with learning continued and have not significantly changed. The problems remain the same both before and after his encounter with Marson. After leaving Moira, K.M. did achieve some qualifications in the workplace, which I will review in K.M.'s employment history in these Reasons for Judgment. While K.M. may have grown out of some of his issues with learning disabilities in adulthood, Dr. Wolfgarth noted his learning difficulties were still present and reflective in his testing of K.M. in 2002.

d. K.M.'s Relationships

[143] K.M. testified that when he quit school in 1981, he moved into his friend B.H.'s basement. Shortly after that, he moved in with Y.G., a grade 10 female high school student he met at a dance at Moira Secondary School. K.M. and Y.G., both 16 years of age, rented and lived in a room at the Quinte Hotel in Belleville for a number of months. Y.G. continued to go to school. K.M. worked at an auction house and paid for their room. After Y.G. finished grade 12, she got a full time job at TAS, a telephone answering service.

[144] K.M. and Y.G. were together for about 8 years, from 1981 until the end of 1989. Shortly after referring K.M. to Dr. Brant, Dr. Manderville made a note in 1989 (Exhibit #1, Tab #A1, p. 7) that K.M. was separating from his common-law wife (Y.G.) and that he loves her but they cannot get along. K.M. attributed their breakup to his ongoing struggles and addiction issues. K.M. testified that Y.G. did not want to be a part of that and he could not blame her. His struggles were his mental health concerns and his depression. It was hard to be involved in a relationship with him as he did not care about himself. Dr. Brant reported to Dr. Manderville on February 20, 1990 that Y.G. had left because she was fed up with K.M. moping around and his lack of interest in sex, and she took up with someone else.

[145] K.M. then had a relationship with a new girlfriend, K.W., beginning around March 1990. K.M. lived with her and her two-year-old son. K.M. had symptoms of mood swings and reduced sex drive and he was home by himself, moping around the house. This relationship was over by the end of July 1993.

[146] His next relationship was with C.P., which began in 1997 and lasted about 2 ½ years. They had a son, M.M., born in July 1998. They did not stay together very long because of K.M.'s mental health issues, and they separated December 1, 1999. K.M. testified that he did not really love her and they had not planned on being parents. On December 22, 1999, K.M. ended up at the Crisis Intervention Centre at the Belleville General Hospital. K.M. attempted suicide on January 28, 2000.

[147] Their son lived with his mother C.P. as a child. K.M. exercised access, usually every other weekend, which was made difficult at times by C.P. He paid child support as required. There were some weekends when K.M. had to cancel access with his son as he was unwell, meaning, in K.M.'s words, that he was overwhelmed, depressed and anxious. M.M. is now 18 years old and lives away from Belleville, attends a Community College and hopes to become a police officer. He has a part-time job. K.M. gets along well with M.M.'s girlfriend. K.M. taught his son how to ride a bike and how to skate. His son is very important to him.

[148] K.M. next got back together briefly with Y.G. after the suicide of their friend B.H., who hanged himself. They met at his funeral in 2004 or 2005. K.M. and Y.G. did not live together this time, but saw one another and dated for about two or three years. In the intervening years between 1989 and 2004 or 2005, K.M. had not seen Y.G. at all. Y.G. had been in another relationship and had a daughter, about three years older than K.M.'s son M.M.

[149] Y.G. testified that she is now 51 years of age and she cleans houses for a living. She had an 8 to 9 year relationship with K.M. starting in late 1981 when they were both 16 years of age. They lived together for most of that time period. She had a second dating relationship with K.M. beginning in 2004 or 2005 when she reconnected with K.M. after meeting him at their friend B.H.'s funeral. This second relationship, she testified, lasted for two years. From the medical records during this time period—reviewed below in these Reasons for Judgment—this second relationship most likely began in 2005 and ended in 2007. Dr. Singh's medical note of October 2, 2007 stated that K.M.'s girlfriend (Y.G.) of 9 years had left him. Dr. Singh failed to mention that it was for the second time.

[150] Y.G. testified that in their longer relationship in the 1980's, K.M. used "pot" daily, three to five times a day. As to K.M.'s mood and personality, Y.G. testified that K.M. never talked about how he felt and that he kept his feelings and emotions to himself. Y.G. noticed problems with K.M. He slept quite frequently, which she found odd as they were teenagers or in their early twenties. K.M. did not talk about his emotions, which she also found odd. His moods would change within the day from being happy in the morning to being in a bad mood, down and depressed after lunch. He was up and down. He was more withdrawn. She testified that "he seemed down a lot. He didn't seem a very happy person." She was not able to figure out what triggered a particular mood change.

[151] In cross-examination, Y.G. testified that after living at the Quinte Hotel, she moved around a lot. She lived with her brother for a while, and then she and K.M. lived in apartments on Charles Street, Foster Avenue and Station Street in Belleville. K.M. would just sit in their apartment, not working or looking for a job. In cross-examination, Y.G. testified that the positive or good she saw in K.M. was that he was a nice guy, he was kind, and she knew he loved her.

[152] When K.M. was "down", he was not very sociable. If he did go to a family event, he would not interact. He does not do well interacting with other people in public. With a friend one-on-one, K.M. was fine. Y.G. testified that K.M. tried to make the relationship work in the 1980's but he reverted back to his old behaviour near the end.

[153] K.M. expressed thoughts of suicide to Y.G. at the end of both relationships. After the first and longer relationship ended, K.M. was in hospital because of depression and suicidal thoughts. He was pushing her away so she walked away. After the second relationship ended, K.M. mentioned suicide to Y.G. again. She told him "not to do that to his son" and she walked out. In cross-examination, Y.G. testified that K.M. was declining mentally before their first relationship ended. He was more withdrawn and was seeking help and that's why K.M. went to the hospital on his own accord.

[154] Y.G. testified in cross-examination that when she resumed her relationship with K.M., she thought he was more verbal and could talk more about his feelings, but it turned out that K.M. did not talk about his feelings. They saw one another about three times a week and would stay over at each other's places. They would do day trips together, go to a cottage, and do trail walking. K.M. mostly interacted well with her daughter.

[155] Y.G. testified that the factors that doomed the relationship with K.M. in the end were the "emotional ups and downs". It was a difficult future with someone who does not work and uses pot daily. In the second relationship, she did not want her daughter in that situation. She testified that it was "difficult to keep a relationship going with him, and very emotionally strenuous". She added: "It was just too strenuous on me. I just could not do it anymore."

[156] The medical reports of Dr. Singh, when he was treating K.M. after 2007, make a reference on August 3, 2010 to K.M. having started to see a new partner, but by March 7, 2011 it was reported that K.M. had no romantic partner. No further references were made about any other relationships of K.M.

[157] K.M. testified that he does not expect to ever be in another relationship. He would not risk it as he knows how he reacts when it does not work out and he cannot put himself in that position again. When a relationship ends, K.M. feels a complete loss, like “why push ahead, longing, that things aren’t ... that you’re never going to be fulfilled.” K.M. is not actively looking for a partner to “suffer in my misery”. He stated: “It’s extremely hard on people”.

[158] K.M.’s sister S.G. described K.M. as going into a deep depression and not talking to anybody when a relationship would break up. She said it was like he was not worth fighting for or worth having the love of another person, and he would not go back into another relationship. It was like he had come to the conclusion that he’s not worth being happy or having a partner. She testified that she thought K.M. would be alone for the rest of his life. She does not think that K.M. feels he deserves happiness, like everybody else.

[159] K.M.’s neighbour T.A. testified that over the eight years that she has known him since 2009, K.M. had not been in a relationship. She described one time when she and K.M. were at Tim Hortons and he saw a lady he likes who serves him coffee all the time. T.A. suggested to K.M. that he ask her out. K.M. said, “I can’t do that”. He just could not get up the nerve up to ask her. He told T.A., “What have I got to offer?”

[160] K.M. lives alone. He has not had a live-in, spousal-type relationship since he separated from C.P. on December 1, 1999.

e. K.M.’s Employment, Work Experience, and Taxable Income Sources

[161] At 13 years old and in grade 7, K.M. took a paper route and did yard work and odd jobs to get away from Marson. There is no evidence that K.M. had worked before that age.

[162] When K.M. first started working to try and support himself, he was 16 years old and living at the Quinte Hotel with Y.G. He worked various types of jobs over the years. He worked at Bunnett’s Auction for about 2-3 years, picking items up, getting things ready for sale, and helping at the auctions. This job was followed by a smaller job at C & B Woodworking, refinishing furniture. In the early 1980’s he supported himself more from social assistance than he did from employment income.

[163] In the later 1980’s he worked at Pre-Con (St. Mary’s Cement) for about three years doing concrete work, cleaning forms, pulling wire, helping in making the pour, and doing some cement finishing. The finishing involved putting a smooth or “broom finish” surface on concrete segments. He left that job as he had a mental breakdown and was on disability benefits for a year. He started to see a psychiatrist, Dr. C. Brant, in November 1989.

[164] In the 1990’s, he had a number of different jobs, including as a mover for MacCosham Van Lines for about three years, for Proctor & Gamble putting coupons in diaper boxes, and making deliveries for Gilly’s Express, a small courier company in Belleville.

[165] In early 1995, K.M. moved to the Guelph, Kitchener-Waterloo area. K.M. stated that he needed to get out of Belleville. He needed a change. He lived with his younger sister S.G. in Kitchener. He initially worked in the fall of 1995 at Traces Screen Printing Ltd. putting silk

screening on jerseys. The president of Traces gave him a good reference letter, dated August 28, 1996 (Exhibit #2, Tab #C5), and said that K.M. was an excellent worker with impeccable work habits. He was attentive, orderly, and took instruction extremely well and was willing to learn. They ran out of work for him.

[166] In February 19, 1996, K.M. started working at Tunnel Systems Inc. as a labourer doing production line work including mould opening, hardware installation and concrete finishing. They made large precast concrete liners for the TTC Toronto subway expansion. K.M. was an active member of their Joint Health and Safety Committee. It was a fixed term contract for about one year to mid-April 1997, after which time everyone was let go as the project was over. K.M. excelled and really liked working there. Tunnel Systems Inc. provided a positive reference letter dated February 27, 1997 (Exhibit #2, Tab #C6), stating that K.M. was a dedicated and punctual hard worker and that his attendance record was excellent. The medical records report that K.M. was still having mental health problems while he was in Kitchener.

[167] Tunnel Systems in addition trained K.M. on the overhead cranes and fork-lifts. On March 22, 1996, K.M. received a Certificate of Achievement and Certificate from Fork Truck Training Limited (Exhibit #2, Tabs #C1-2) as an overhead crane/slinger operator, limited to operating such equipment only at Tunnel Systems Ltd. premises. The crane was used to lift the large cement segments out of the moulds, rotate them, and place them in sequence and in order to ship to Toronto. This certificate was one of the first diplomas of K.M.'s life.

[168] On May 13, 1996, K.M. received a Certificate of Achievement and Certificate from Fork Truck Training Limited (Exhibit #2, Tabs #C3-4) as a counter balance operator/fork truck operator, limited to operating such equipment only at Tunnel Systems Ltd. premises. This was a fork-lift used to move the moulds. K.M. worked hard for them. He testified he could have done better but he could not grasp the computers and was limited in his ability to read plans. At times, K.M. found the work extremely confusing, but with the help of his co-workers, he got through it.

[169] K.M. agreed in cross-examination that he never thought growing up that he would finish high school, and that he would work as a labourer and do physical work. He described his work at Tunnel Systems as a cement finisher as being a skilled labourer job and he gained the qualifications by experience. K.M. agreed that his crane and fork lift tickets obtained at Tunnel Systems were not apprentice-type programs. It took a day and a half of training and a couple of weeks of hands-on training to obtain both tickets.

[170] In 1997, K.M. moved back to Belleville and met C.P., the mother of his son. After that time, K.M. worked at a number of jobs. Manpower offered him retraining and he qualified as a long distance truck driver. K.M. completed a St. John Ambulance course on safety orientated first aid. He completed an on the job Air Brake Course and received a certificate on March 22, 1999 (Exhibit #2, Tab #C7). He successfully completed a six week, 240 hour professional transport driver training program and received his Diploma on April 16, 1999 (Exhibit #2, Tab #C8). He drove a truck with ITS for a very short period of time, about six months in 1999, and had difficulties with it. He worked at Concrete Plus Cement as a labourer and cement finisher. He worked at Quinte Castings as a machine operator, operating a small crane like he had been

trained on at Tunnel Systems. He worked at Sears Warehouse as a warehouse person in the fall of 1999.

[171] K.M.'s aunt M.I.C. testified that in the 1980's and 1990's she did not see much of K.M. until after her husband died in 2002. She was aware, through family, that K.M. was having psychiatric issues in those intervening years. She needed help at home with her yard work, painting and general cleanup, and her sister A.F. told her that K.M. was unemployed. She hired K.M. to do this kind of work for her. He would come for three to five days and then leave. She paid him \$10.00 an hour, which would not amount to much over a year.

[172] K.M. testified that he worked for his aunt, M.I.C., in Ajax in her sanitation supply distribution business in the GTA. He did pressure washing, deliveries, pick-ups, cleaning, truck loading, and painting. He helped her with yard work at her home. She was very generous and would pay him \$120.00 a month, whether he worked or not, to help him out.

[173] His aunt testified that she hired K.M. to work as well in her warehouse business with painting, clean-up and to "attempt to do some deliveries for us" until 2007, when the business was sold. In cross-examination, she explained that her business had a lot of work downtown but they could not send K.M. to do those deliveries. She testified that K.M. was nervous about dealing with the city. She was aware that K.M. had difficulty reading maps.

[174] M.I.C. testified that K.M. would not have been able to function as a full-time employee in her business. She could not count on him five days a week. He would appear to be unable to handle something and he would have to leave. He had trouble reading invoices and was nervous about driving, and she could not send him to downtown Toronto. He would get upset with the traffic, the buildings, and stopping in the middle of the street. She could send him to the suburbs, which she did sporadically. There were jobs in the business that she gave him, which he could do and which were not affected by his learning problems.

[175] K.M. worked at other times for brief periods at Ken Lynn Wood as an unskilled machine operator, at Precision Pad Printing putting logos on golf balls, and at Plane Carpentry as a cement finisher, but no job lasted very long. He was generally an unreliable employee and inconsistent in his attendance.

[176] K.M. agreed in cross-examination that many of his jobs over the years had ended for reasons unrelated to his mental health, but simply because the job came to an end. This was true for his work at Concrete Plus and Plane Carpentry. His job at Quinte Castings ended when a number of workers were fired over a union dispute. He left his truck driving job because of his difficulties with spatial recognition and his problems reading maps. He agreed that after his son M.M. was born in 1998, he limited his job search to the Belleville area as he did not want to be too far away from his son.

[177] K.M. picks up small jobs on the side when he can, but he is struggling and cannot make commitments to people. He testified that people for whom he does odd jobs—like masonry repairs, brick repairs, pouring concrete sidewalks, paint work, and yard work—know that he may not be able to attend, not "because it's sunny, because I'm just unable". K.M. testified that he

limits what he does in his daily life, in many forms. He testified that he stays home when he becomes overwhelmed and anxious. K.M. testified that if he had not been assaulted, he could have operated heavy specialized equipment. He could have run a bull dozer at a gravel pit or been a crane operator.

[178] K.M. agreed in cross-examination that after he received his O.D.S.P. pension, he never made an application for a job. The evidence is that he started receiving O.D.S.P. income support payments on April 1, 2003. K.M. is not actively job searching, but he has worked when it comes available to him. If he's able to handle it, he will take the work.

[179] K.M. testified that after the abuse, he had big blocks of time when he worked and big blocks of time when he did not work. When he did not work it was because he was unwell or he was working for cash—odd jobbing. He was often glad to get out of a job and just lacked motivation. Even if he needed money, he could not push himself. He had depression for years and years. He could fit in pretty well in the workplace, could chat people up and could hide that he had a problem. If he grew to hate the job, he would be laid off or be fired. K.M. testified that the depression affected his motivation and his mood.

[180] The way he felt affected everything. He stated, “Whether I’m working, driving, around people, the ‘cloud’ hangs on me, and it affects everything”. K.M. has not done anything except casual odd jobs since 2002, because it is all he is able to do. K.M. stated that his anxiety and depression make it hard for him to get up and get pushing to do things when he is not feeling well. When he felt unwell, he would not get so far ahead in a job that it could not be left for three to five days, and hopefully he’d be back. These odd jobs would provide him with a couple of hundred dollars a month, which he would report as income to the O.D.S.P. personnel.

[181] K.M.’s sister S.G. described K.M.’s work history as having the odd job here and there. His neighbour and friend T.A. testified that K.M. does odd jobs around the apartment building. He’ll come in and fix something for you, and he’s always shovelling, but not for money. He is not the superintendent of the building, but she testified that K.M. “paves the way for the super”.

[182] His former girlfriend Y.G.’s testimony about K.M.’s work history is consistent with the other evidence. She testified K.M. did not really have jobs. He worked first, she recalled, at an auction house. He got a job with St. Mary’s Cement but it did not last very long. He cannot get along with people, socially. He did not tend to keep jobs and if he did not have a job, he would not look for one. She recalled that he collected welfare.

[183] K.M.’s income tax returns from 1987 to 2014 are before the court (Exhibit #3, Tabs #I1-36). A summary of his earnings and taxable income sources for those years is attached as Schedule ‘A’. The summary confirms the other evidence that K.M. has had an inconsistent work history and that his highest income earning year was in 1996 when he worked for Tunnel Systems Inc. K.M. has had no employment income since 2003, when he earned \$1,800. Prior to 2003, his previous employment income was \$4,427 in 2000.

(i) K.M.'s Ontario Disability Support Plan Application (O.D.S.P.)

[184] K.M. has supported himself with some amount of public assistance almost continuously since 1992. The materials related to O.D.S.P. are before the court (Exhibit #2, Tabs #F1-11). K.M. applied for O.D.S.P. benefits on August 24, 2001 at age 36. In his application (Exhibit #2, Tab #F1, p. 430), K.M. wrote, "I strongly believe that I've been mentally ill all my life which I believe has been passed to me genetically. As my father has mental issues of his own that Dr. Conn is aware as he is both our doctor. From my years in grade school struggling through I now know that I had my problems from my dyslexia, A.D.T., 'ect' [sic]. All these problems I've had since birth."

[185] K.M. testified in cross-examination about this report to O.D.S.P. and how he felt he had been mentally ill his entire life and that it was perhaps connected to his father's own mental illness. K.M. testified, "That's what I thought then, yes". K.M. said that at the time of the application in 2001, he did not know to what he was referring, whatever his father's illness was. He was "grasping at straws why I was sick". When pressed, K.M. said that his father had "[a]nxiety, depression". In re-examination, K.M. testified that the reference to genetics was because his father was unwell. He did not know any more than that.

[186] K.M.'s mother A.F. was cross-examined on this issue as well. She testified that the mental illness K.M. was referring to was D.M.'s diagnosis of Alzheimer's. She agreed that Dr. Conn treated D.M. in the year before she left him. A.F. testified that her marriage broke up in 1989, so this would place the time of D.M.'s Alzheimer's diagnosis around 1988.

[187] On August 24, 2001, K.M.'s mother A.F. added a written note in support of K.M.'s O.D.S.P. application about K.M. being what she called a "blue baby". A.F. was cross-examined in detail about what she meant when she wrote, "When K.M. was born the cord was wrapped around his neck & he was very blue in colour. I often wondered if this had any impact on his life's problems". She testified that she was Rh negative (her blood type) when she had K.M., and that often a second child could be a "blue baby". She testified that they took blood out of K.M.'s heel to make sure he was not a blue baby, as those babies had to have their blood transfused. There was nothing in her evidence that any such blood transfusion ever occurred. When asked about the "cord" reference, she said it was a quick delivery.

[188] She was asked in cross-examination: was K.M. "blue due to lack of oxygen"? She said "No". She said the doctor told her that "maybe he was a blue baby". She was asked why it was relevant to his O.D.S.P. application. She said she did not know why she wrote that. She testified that K.M. did not have the normal colouring of his older sister when she was born. Ultimately she agreed that she was suggesting in the application that at birth, K.M. was blue and had the cord wrapped around his neck, and that it might be part of K.M.'s problems.

[189] In K.M.'s cross-examination, it was suggested to K.M. that he was a "blue baby" because there may have been a lack of oxygen at birth, and K.M. agreed. There is no medical evidence, however, about him being a "blue baby" and no medical records of a lack of oxygen at birth. There are no entries or notations in the family doctor medical records about K.M.'s birth, nor about any problems that he may have had at birth, nor about any problems immediately

following his birth. There are no other medical records produced from anywhere else indicating that there were any problems with K.M.'s birth or its aftermath.

[190] K.M.'s application for O.D.S.P. was initially denied on November 8, 2001, as stated in his Initial Application Adjudication Summary. He was found not to have a substantial physical or mental impairment that was continuous or recurrent. Further, the direct and cumulative effect of his impairment on his ability to attend to his personal care, function in the community, and function in the workplace was stated to not result in a substantial restriction in one or more of these activities of daily living. This decision was confirmed by a letter dated December 5, 2001. K.M. later obtained the assistance of Ms. Stapely from the office of Hastings County Ontario Works.

[191] K.M. was then tested and assessed on December 19, 2002 by Dr. Wolfgarth, a clinical psychologist, who filed his assessment report (Exhibit #1, Tab #A54) with a Health Status Report to O.D.S.P. on December 20, 2002 (Exhibit #2, Tab #F5). K.M.'s scores were considered reliable and valid measures of his demonstrated skills. It was noted that K.M. had a history of inability to hold continuing employment due to psychiatric problems. His problems involving reading deficiency had impaired his ready assimilation of instruction. Other symptoms were noted by the Dr. Wolfgarth in a checklist at the back of the Health Status Report form, without any knowledge that K.M. had been sexually abused.

[192] In the assessment report, K.M. was assessed by Dr. Wolfgarth on "Intellectual Measures" as falling into the average range of intellectual ability at the twenty-seventh percentile rank. There was an eighteen-point difference between his verbal I.Q. score and his performance I.Q. score, indicating that K.M. was considered a verbal learner. On tests involving visual and spatial perceptual abilities, K.M. scored in the low-average range. He had a lack of appreciation for linear and spatial relationships.

[193] On "Personality Characteristics", Dr. Wolfgarth determined K.M. to have experienced a chronic condition of clinical depression with hospitalizations. His learning difficulties were still apparent and reflected in his performance in the testing done, and were associated with dyslexic tendencies. He was assessed as having a history of inability to hold continuing employment due to interference in work steadiness caused by his psychiatric problems. He had not demonstrated a significant degree of progress over the years and his prognosis was guarded for the future.

[194] On April 16, 2003, K.M., then 37, was found by O.D.S.P. to be a person with a disability due to his condition of clinical depression, dyslexia and his learning disability, with a two-year review recommended. This was confirmed by letter from Paula Lam on April 16, 2003, with a review scheduled for April 16, 2005. By letter dated May 26, 2003, K.M.'s benefits were confirmed to begin on April 1, 2003. Subsequent reviews have maintained K.M.'s disability finding.

[195] As a result of this litigation, by formal request of the Ministry of the Attorney General dated June 15, 2011, K.M. was required to sign an agreement to reimburse O.D.S.P. for funds received as a condition of eligibility for continued income support. By letter dated February 29, 2016, it was confirmed that as of February 16, 2016, K.M. has received \$105,645.29 in O.D.S.P.

income support payments from April 2003 to February 2016, plus an overpayment of \$21,325.54.

[196] The O.D.S.P. notes made on September 13, 2010 (Exhibit #2, Tab #F11, p. 487) record that on June 24, 2010, K.M. received an award of \$12,500.00 from “Compensation for Victims of Crime”. This is exempt as income for O.D.S.P. purposes and is not deducted from K.M.’s income support payments. O.D.S.P. records confirm that K.M. reported his casual employment earnings to O.D.S.P. and that his income support payments were adjusted accordingly. K.M.’s prescription medications are paid for by his drug card from O.D.S.P.

(ii) K.M.’s Canada Pension Plan Disability Benefit Application (C.P.P.)

[197] On June 22, 2010, K.M. completed a questionnaire for disability benefits from the C.P.P. The materials related to his C.P.P. disability application are before the court (Exhibit #3, Tabs #G1-8). K.M.’s relevant medical history was described by Dr. Singh, in his medical report to C.P.P. of June 29, 2010, as “patient has suffered from childhood sexual abuse with post-traumatic stress disorder which turned in chronic major depression with dysthymia in 1987.” He also noted K.M.’s substance abuse. On September 23, 2011, K.M. was denied a C.P.P. disability pension. He was considered to be a “late applicant” and had to meet stricter criteria, in that his disability had to be severe and prolonged, and had to be continuous when making contributions to qualify. K.M. appealed the decision with the assistance of counsel. On October 26, 2012, following a review of the file, C.P.P. personnel agreed that K.M. had met the requirements for a disability pension and he was eligible as of May 2009. Legislatively, the earliest he could receive the pension was September 2009.

[198] On November 8, 2012, K.M. received notice of a credit for retroactive payments totalling \$22,805.44. His total taxable monthly pension amount was \$617.00. K.M. made arrangements with his bank to pay him \$1,000 per month from this lump sum until the funds were exhausted, as he was no longer eligible for O.D.S.P. income support payments while these lump sum funds from C.P.P. were available to him.

f. K.M.’s Drug Use

[199] I have gathered references to K.M.’s drug use after his encounter with Marson from the evidence, including the medical reports in the exhibits. A more complete review of K.M.’s overall medical status is found below in these Reasons for Judgment.

[200] After Marson, K.M. was using marijuana daily. Y.G. testified that when she first got together with K.M. in the fall of 1981, K.M. used “pot ... it could have been three to five times a day”. She had no knowledge of K.M.’s drug use before she met him. Y.G. testified in cross-examination that K.M.’s drug use was increasing over the years of their first relationship. In the second relationship, K.M.’s drug use was about the same as it was when they broke up before. K.M. paid for his drugs with welfare monies and with money from Y.G.’s wages.

[201] K.M. testified that he was developing a serious drug habit when he came back from Saint John’s to help him “feel something”, to alleviate his mood and to “feel normal, feel like everybody else”, but he did more drugs than everybody else. On July 14, 1982, when K.M. was

age 16, Dr. Manderville, his family doctor, made a note (Exhibit #1, Tab #A1, p. 5) that he had discussed K.M.'s drug use with him. Dr. Manderville noted, "discussion re use of illicit drugs – marihuana – uses it almost daily. Does not use other drugs or alcohol. Thinks it's alright because he likes it. Refuses to see a 'shrink'. Says he'll try to stop. Wants to get a job. Will not go back to school. Parents threatening to throw him out."

[202] On October 21, 1982, K.M., now age 17, was noted by Dr. Manderville (Exhibit #1, Tab #A1, p. 6) as, "Trying to stay off marijuana – very tense". He was prescribed 20 tablets of Ativan. On October 25, 1982, K.M. was admitted into a residential drug treatment program at Serenity House of Quinte in Belleville, with the knowledge of Dr. Manderville, to try and get him off drugs. K.M. left the facility without staff approval on November 1, 1982, telling the staff that he needed to "get high".

[203] K.M.'s mother A.F. testified that the first time she had heard about K.M. using drugs was when he was at Moira Secondary School, when the police told her K.M. was using marijuana.

[204] K.M.'s sister S.G. testified about their father D.M. being upset with K.M. when he was in grade 7 or 8, but she did not know why. When K.M. went to Moira, he stopped going to school. She was not aware of any problem K.M. had with drugs until her parents started "tough love" when K.M. was acting up at Moira. Their father had kicked K.M. out of the house. S.G. knew he was doing drugs then as she could "smell it".

[205] Dr. Manderville next noted K.M., age 24, using marijuana in 1989, around the time of his parents' divorce and when he was referred to Dr. Brant, a local psychiatrist. Dr. Brant on November 24, 1989 (Exhibit #1, Tab #A9) described K.M.'s depression as being compounded by his parents' divorce after 27 years of marriage, his breakup with Y.G., and his heavy marijuana use. He reported that K.M. "started smoking up when he was about thirteen years old and has done so on a daily basis since then ... up to four large joints of strong marijuana" per day.

[206] K.M. was admitted to the Belleville General Hospital on December 27, 1989 (not 1990 as recorded in some forms, discussed below). In the Admission History and Physical (Exhibit #1, Tab #A10), dictated December 28, 1989 and transcribed January 1, 1990, Dr. Manderville reported that K.M. had a major addiction to marijuana. He stated K.M. "has used this drug since about grade 7 and at that time his mother found out, after he had been using it for a couple of years, and went to various social agencies in an attempt to try and get him some help but he states that he has taken it nearly every day." In addition he noted that K.M. had used cocaine. The only evidence of any social agency being involved in helping with K.M.'s drug use was Serenity House in 1982, when K.M. was age 17.

[207] On December 27, 1989 (Exhibit #1, Tab #A11), dictated December 27, 1989 and transcribed January 1, 1990 (not 1989, as indicated on the form), Dr. Conn reported that K.M. was "utilizing hashish and marijuana since age 13 years. On a daily basis and to a considerable amount", which K.M. was trying to reduce. Dr. Conn could not have seen Dr. Manderville's report from K.M.'s hospital admission, as it had not yet been transcribed.

[208] In February 1990 (Exhibit #1, Tab #A15), Dr. Brant reported that K.M. had “given up marijuana to which he had been habituated for about eleven years”. This reference would place K.M.’s starting point of marijuana use “about” February 1979 when K.M. was age 13 and in grade 7.

[209] K.M. was admitted to the Kingston Psychiatric Hospital from July 19, 1990 until August 15, 1990. In Dr. H. Pytlak’s Discharge Summary dictated August 17, 1990 (Exhibit #1, Tab #A24), he reported that on August 14, 1990, K.M. had told Dr. G. Linn, a third year psychiatric resident, that “since he was 12 years old he began to use cannabis (marijuana, hashish), acid (form of ‘speed’), various ‘uppers’ and ‘downers’ and ‘everything I could get my hands on’”. K.M. reported that over the last year, he had become involved with cocaine when he provided his apartment as a safe house, and was given 5 gm per week as payment. Others involved with the cocaine were arrested and the supply dried up. It was reported that this is what had precipitated K.M.’s admission to the Belleville General Hospital.

[210] When K.M. was in the Kitchener-Waterloo area, Dr. Schwarz reported on February 27, 1997 that K.M. was using hash once a week to improve his mood.

[211] After the sexual abuse by Marson, K.M. became a regular street drug user. The reason given by K.M. for using drugs was that he just wanted to feel something, to make himself feel better, and it helped. There is no evidence of any criminal convictions of K.M. Over the years, K.M. reduced his dependency on drugs except for marijuana, which he uses today and for which he has a prescription. K.M. testified that Dr. A. Matveyev suggested it and Dr. R. Holmes sent him to Medi-Green in Smiths Falls. His current marijuana use, as medically prescribed, is legal and permitted by license from Health Canada.

[212] In cross-examination, K.M. testified that after Marson, as he got older, he started using different sorts of drugs and using more of them. K.M.’s use of illegal or other street drugs stopped when his son M.M. was born. The evidence is that K.M.’s son was born in July 1998, or 18 years ago, when K.M. was 32 years old. K.M. testified that he stopped with the drugs and with alcohol, but continued to use marijuana. He’s been “clean” for eighteen years, subject to his use of medical marijuana. He does not drink alcohol, except for a couple of beers he had on his 50th birthday. He does not do hard drugs.

[213] Dr. McMillan, dictating for and in consultation with Dr. Singh, noted in his July 30, 2007 initial assessment of K.M. that K.M. smoked two marijuana joints per day and that he was trying to cut down. It was noted that K.M. had “used cocaine in the remote past for approximately one year” and that he had “tried LSD in the 80s” (Exhibit #1, Tab #A71).

g. K.M.’s Psychological and Mental State

(i) Overview

[214] K.M. testified that after Marson, “everything’s a mess for me.”

[215] K.M. has seen different doctors since 1978 as they came and went from being working physicians to retiring or passing away over all these many years. Records from his childhood

family doctors, initially Dr. E. A. Manderville and Dr. A. Goddard-Hill in Belleville, begin in 1966. From July 26, 1996 to 1997, his family doctor was Dr. D. Choong when K.M. was in Guelph. After returning to Belleville in 1997, K.M.'s family doctor was again Dr. Goddard-Hill. Since September 8, 2015, his family doctor has been Dr. R. Holmes in Belleville.

[216] The medical records of these family doctors and of Dr. Conn, a psychiatrist, in relation to K.M.'s office visits are before the court at Exhibit #1, Tabs #A1-4. The records from the offices of Drs. Manderville and Goddard-Hill at Tab #A1 span from January 13, 1966 to the latest date of January 2, 2015. Dr. Choong's office records are mislabelled in the Joint Document Brief Index and are in fact at Exhibit #1, Tab #A2, pages 46-49, from July 26, 1996 to the latest date of May 20, 1997 (not 1970 to 1997 as indicated in the Index). I note that Tab #A2, page 49 indicates an appointment with Dr. Choong on March 10, 1995, however I find that this would have been on March 10, 1997 because it was a follow-up of K.M.'s meeting with Dr. Schwarz, whom K.M. met on February 25, 1997 (discussed later in these Reasons for Judgment), and because the following entry in Dr. Choong's records is May 20, 1997. Tab #A2, pages 38-43 appear to be mistakenly and partially duplicated from medical records in Tab #A1, while Tab #A2, pages 44-45 contain records from 1995 that did not appear in Tab #A1. Dr. Conn's records are at Tab #A3 from August 18, 2003 to May 17, 2006. Dr. Holmes' records are at Tab #A4 from September 8, 2015 to January 20, 2016.

[217] I have considered the family doctors' records and integrated relevant references in them, chronologically, with the records and reports of K.M.'s treating psychiatrists since he began psychiatric treatment in 1989, to reflect a more complete picture of K.M.'s overall medical history. Counsel as well have highlighted important notes and comments in the family doctors' records in the evidence.

[218] From 1978 to 1989, after his encounter with Marson, K.M.'s medical records from Dr. Manderville are relatively unremarkable except for the two important entries made on July 14, 1982 and October 21, 1982, which I have referred to earlier in relation to K.M.'s serious use of drugs. A second school medical was done on June 22, 1981. The other recorded entries contain references to medical issues such as a leg injury from football, an ankle sprain, acne, dermatitis, a cough and similar symptoms reflecting common physical ailments as K.M. grew up from a teenager into an adult.

[219] In 1989, K.M. began being treated by a psychiatrist. In November 1989, Dr. Manderville referred K.M. to Dr. Clare Brant, an Assistant Professor of Psychiatry, University of Western Ontario, who was then practicing on the Tyendinaga Mohawk Territory near Belleville. Dr. Brant died of cancer in 1994. After 1994, K.M. was treated by Dr. E.B. Conn in Belleville until 1996. There were consultation and assessment requests made between Drs. Manderville, Brant and Conn from 1989 to 1996. K.M. briefly saw Dr. M. Schwarz in Guelph from in 1996-1997. After returning to Belleville, K.M. was again treated by Dr. Conn from 1997 until he retired in 2006. K.M. was treated and assessed by other psychiatrists and psychiatric residents during his two hospital admissions to the Belleville General Hospital and his one admission to the Kingston Psychiatric Hospital.

[220] K.M. then started to see Dr. A. Singh in Belleville in 2007 until Dr. Singh semi-retired in 2012. K.M. has seen Dr. Singh a few times in Kingston since then for the purpose of preparing expert reports for this case. Since Dr. Singh semi-retired in 2012, K.M. has not had a regular, treating psychiatrist.

[221] After 2012, K.M. continued to struggle with his mental health. He was referred to Dr. A. Matveyev, a psychiatrist working at the Belleville General Hospital, who saw him on June 14, 2013 (not June 12, 2013, as incorrectly stated in the O.H.I.P. summary) and October 8, 2015. This was K.M.'s last appointment with a psychiatrist. K.M.'s medical care thereafter has been managed by his family doctor. K.M. testified that between October 8, 2015 and the civil trial date in March 2017, he needed a prescription for Ativan in May 2016. He used two tablets and still has the rest. He continues to use marijuana, which is now medically prescribed.

(ii) Dr. C. C. Brant – 1989 to 1994

[222] In 1989, Dr. Manderville referred K.M. and made a clinical note that K.M. was “depressed over parents’ divorce – may need something – uses marijuana – refer to Dr. Brant” (Exhibit #1, Tab #A1, p. 7). K.M. saw Dr. Brant as his treating psychiatrist beginning November 23, 1989. K.M. was feeling suicidal. His relationship with Y.G. had broken up and his parents were getting a divorce. A follow-up letter to Dr. Manderville was sent by Dr. Brant on November 24, 1989 (Exhibit #1, Tab #A9) describing K.M.'s depression as being compounded by his parents’ divorce after 27 years of marriage, his breakup with Y.G., and his heavy marijuana use. He reported that K.M. “started smoking up when he was about thirteen years old and has done so on a daily basis since then”. K.M. was described as suffering from a major depression compounded by the family breakups. He was prescribed Prozac.

[223] K.M. testified that 1989 is the first time that he can recall suicidal thoughts or that he had an “out”. It’s when he first considered suicide, and he still does. He never told Dr. Brant about the sexual abuse by Marson.

[224] With the assistance of Dr. Brant and Dr. Manderville and under the care of Dr. Conn, K.M., at 24 years old, was admitted to Belleville General Hospital on the psychiatric ward for his depression from December 27, 1989 until January 19, 1990. Dr. Brant had wanted K.M. to be admitted on December 22, 1989 but K.M. refused. Dr. Manderville, in his Admission History and Physical dictated December 28, 1989 (Exhibit #1, Tab #A10), reported that K.M. had seen him in his office approximately three weeks before and was very depressed, with suicidal ruminations. Dr. Manderville’s notes of this office visit are at the bottom of his records at Exhibit #1, Tab #A1, at page 7, but they are barely readable. It appears to say, “Depressed. On Ludiomil, Xanax, and marijuana. Seeing Dr. Brant - not happy with him. Suicidal ruminations. Refuses to come into hospital”. In the Admission History and Physical, Dr. Manderville reported that K.M. had a major addiction to marijuana and had used cocaine.

[225] Dr. Conn dictated his Belleville Hospital Consultation Report on December 27, 1989 (Exhibit #1, Tab #A11). K.M. had group therapy. Various medications were tried, including Prozac, Ludiomil, Xanax, and Restoril, with no improvement in his symptoms. K.M. had fatigue, loss of interest, some shortening of concentration span, decreased appetite, and disturbed sleep

with fitful insomnia. His symptoms had been present since October 1989. It was noted that K.M. had no criminal history or temper control problems. Dr. Conn reported that K.M.'s home environment was described as "average" and that K.M. had "obtained grade 9 in his school". Dr. Conn reported that K.M. was utilizing hashish and marijuana since age 13 on a daily basis and in a considerable amount, which K.M. was trying to reduce. K.M. testified that there was never any benefit from his hospital admissions on his mental health.

[226] On January 19, 1990 (Exhibit #1, Tab #A13), Dr. Conn dictated his Discharge Summary. I note the date of admission of "27-12-90" on the form is clearly a typographical error and should be "27-12-89". Unfortunately, this error is continued forward in some of the later medical records. K.M. was discharged on Sinequan, Chloral Hydrate and Xanax. He was stated to be improved and was warned about future use of street drugs.

[227] In February 1990 (Exhibit #1, Tab #A15), Dr. Brant reported in a progress note that he had seen K.M. on eight occasions in February. K.M. was constantly depressed and feeling hopeless, exacerbated by the fact that Y.G. had ended their relationship. K.M. had discontinued his medications. He had stopped using marijuana. He had frequent impulses to hurt himself but came to realize that he was going to recover. He denied any suicidal plan. Dr. Brant prescribed Parnate.

[228] On February 20, 1990 (Exhibit #1, Tab #A16), Dr. Brant reported that K.M.'s girlfriend (Y.G.) had left him because she was fed up with his moping around and his infrequent sexual advances, and she took up with someone else. K.M. was severely depressed and Dr. Brant arranged daily contact with him. K.M. was living alone and spent most of his time alone, which he preferred. He was receiving disability benefits from Proctor and Gamble, where he was working at the time. K.M. was seen at the emergency department on February 23, 1990 at the crisis centre, with follow-up by Dr. Brant.

[229] In March 1990 (Exhibit #1, Tab #A18), Dr. Brant continued to see K.M. for psychotherapy and medication monitoring. K.M. had an exacerbation of his depressive symptoms. Re-hospitalization was offered and declined by K.M. Dr. Brant reported in his progress note that K.M. advised that "he is getting so fed up with the way that he has felt for so many years that he intends to commit suicide unless relief is given to him by the middle of April 1990". K.M. testified that he still feels this way. Dr. Brant consulted with Dr. N. Delva from the Kingston Psychiatric Hospital. Dr. Delva recommended adding Lithium Carbonate to K.M.'s medications together with Parnate, which was then prescribed. Dr. Brant wanted K.M. to be hospitalized for electroconvulsive therapy. These medications only worked for a while.

[230] In April 1990 (Exhibit #1, Tab #A19), Dr. Brant reported in his progress note that K.M. had made a substantial improvement on his medications although he suffered stomach upset and diarrhea. His mood levelled out, but by the end of April, K.M. was emotionally upset, with crying and restlessness. His new girlfriend (K.W.) was distressed at K.M. being upset and did not know how to comfort him. Dr. Brant felt that K.M.'s depression was starting to resolve, with the exception of some restlessness and agitation in the evening. K.M. was optimistic about his future.

[231] In May 1990 (Exhibit #1, Tab #A20), Dr. Brant reported in his progress note that K.M. was doing well on his medications but that he had stopped taking Parnate as he did not have the money to pay for it. K.M. had not received disability payments since December 1989 and he felt embarrassed about borrowing more money from his parents. K.M.'s depression had regressed because of his inability to afford the medication. Dr. Brant provided him samples of Parnate and he re-started the medication along with Lithium Carbonate that K.M. still had.

[232] In June 1990 (Exhibit #1, Tabs #A21-22), Dr. Brant reported that K.M. had been assessed by Dr. Delva and arrangements had been made for K.M. to be admitted to Dr. Waldron's clinic at the Kingston Psychiatric Hospital for "definitive treatment of his long standing and chemically resistant depression". Dr. Delva asked Dr. Waldron to evaluate K.M. Dr. Waldron reported that K.M. was "an extremely disturbed young man who though giving a life history of dysphoria was never the less reasonably well adjusted, working and living in an eight year stable relationship with a common-law wife until the end of 1989. At that time, things seem to have become quite unstuck and he has become increasingly depressed. Trials of several tricyclics and Lithium have not helped". He recommended inpatient evaluation and treatment of K.M.'s depression.

[233] K.M., nearly age 25, was admitted by Dr. Waldron to the Kingston Psychiatric Hospital on July 19, 1990 and left August 15, 1990. Dr. Waldron's resident, Dr. H. Pytlak, reported in a Discharge Summary dictated August 17, 1990 (Exhibit #1, Tab #A24) that K.M. on admission had complaints of "sadness, loneliness, hopelessness, excessive sleep and average appetite". He had "occasional severe headaches, lack of energy, lack of motivation and some mild suicidal ideation with no plan". He continued to have poor sex drive. There was no evidence of a thought disorder. He was assessed as having mild to moderately depressed mood with appropriate affect. He had been living with his new girlfriend (K.W.) and her two-year-old son for the past four months and the relationship was so far very good.

[234] Dr. Pytlak's diagnosis was made in accordance with the *Diagnostic and Statistical Manual of Mental Disorders* ("DSM"). There is a five axis system of diagnosis that is used. For contextual purposes I note these are: Axis I (Clinical Disorders, other conditions that may be a focus of clinical attention); Axis II (Personality Disorders, Mental Retardation); Axis III (General Medical Conditions); Axis IV (Psychosocial and Environmental Problems); and Axis V (Global Assessment of Functioning (GAF) scale: with 1-10 as severe symptoms, 51-60 as moderate symptoms, and 91-100 as no symptoms). It is generally accepted that the GAF scale is used to rate how serious a mental illness may be, or how much a person's symptoms affect their everyday life—including their psychological, social and occupational functioning—on a continuum of mental illness.

[235] Dr. Pytlak's diagnosis of K.M. was: Axis I – psychoactive substance dependence and dysthymia associated with a bath; Axis II – specific learning disorder – unspecified, and mixed personality disorder – anti-social, dependent traits; Axis III – chronic cough – smoker two packages per day, hashish; Axis IV – mild; and Axis V – GAF in past year, 75. During K.M.'s course at the hospital, his girlfriend at the time (K.W.) visited him. He was tapered off his medications as K.M. felt that they did not make any difference and were giving him stomach problems. K.M. took a leave of absence from August 1 to 9, 1990 to spend some time with his

girlfriend (K.W.) and her son and then to attend his grandmother's funeral. It was noted that K.M. had problems at work when he transferred from a concrete finisher to a carpenter, "as he had many problems with measuring as he did not know numbers due to his specific learning disorder that was recognized early in his school years." K.M. told Dr. Linn, a third year psychiatric resident, on August 14, 1990 that he had used drugs since he was 12, as earlier discussed. While a patient at the hospital, K.M. was smoking hashish and snorting cocaine to make himself feel better. When Dr. Pytlak confronted him on August 15, 1990 about his use of drugs at the hospital, K.M. became very angry, rude and hostile.

[236] That very same day, August 15, 1990, K.M.—likely under the influence of drugs—discharged himself from the hospital against medical advice, despite being asked to stay. K.M. testified that the hospital was starting to suggest to him that he have electro-shock therapy, to try and "charge him out of his depression". K.M. just wanted out of there and felt his treatment was not helping him. He had phoned his mother and asked her for more money, which she suspected was to buy some cocaine.

[237] Dr. Brant reported to Dr. Manderville on September 7, 1990—not, as indicated in the report, 1989 (Exhibit #1, Tab #A8). I note that when the contents of the report are read, the date this report was written would have to be September 7, 1990, not 1989. Dr. Brant noted that K.M. had not been completely honest with him about his drug use and was snorting large amounts of cocaine. Dr. Brant stated he believed K.M. had discontinued street drugs in December 1989 (which I find is confirmation of the date error, as the report is dated September 7, 1989). Dr. Brant reported that K.M. was looking for an antidepressant drug that would allow him to abuse street drugs and make him feel well. That drug did not exist. K.M. made oblique referrals to suicidal ideation but not enough to force his hospitalization. Dr. Brant recommended residential treatment at the Donwood Institute, but it never came to be.

[238] Dr. Manderville (Exhibit #1, Tab #A1, p. 8) referred K.M. to Dr. Conn for a psychiatric consultation, which was done on February 7, 1992. K.M. was working at Gilly's Express and had been living with his girlfriend (K.W.) and her son for the past two years.

[239] Dr. Conn's report dated February 7, 1992 (Exhibit #1, Tab #A25) refers to K.M. being a patient of his on "Quinte 3 from December 27, 1990 to January 19, 1991". This is the same error in the Belleville Hospital records referred to earlier regarding the admission date. I am satisfied that Dr. Conn was referring to K.M.'s hospitalization from December 27, 1989 to January 19, 1990 at the Belleville General Hospital. Dr. Conn also refers to K.M. being at the Kingston Psychiatric Hospital "this past summer", which would be summer of 1991, whereas the Kingston Psychiatric Hospital records indicate it was the summer of 1990.

[240] Dr. Conn reported that K.M. continued to have problems with chaotic mood swings that varied from day to day. He had trouble with onset sleeping, varying levels of energy and interest, and varying levels of concentration ability. He did not experience a wealth of ambition. He was using marijuana in a very limited fashion and he maintained a close relationship with his mother. Dr. Conn stated that K.M. is a "personality problem" and had a number of "asocial personality features and certainly may well fit into the category of borderline personality". K.M. was developing anger towards his girlfriend (K.W.) and her child. Dr. Conn prescribed Neuleptil. Dr.

Conn disagreed with “Clare” (Dr. Brant) that K.M. had a bipolar mood disorder. Dr. Conn did agree that K.M. “does display some of the chaotic moods of the borderline personality”.

[241] At the request of Dr. Manderville, K.M. was re-assessed by Dr. Brant on December 23, 1992 (Exhibit #1, Tab #A26), over two years after he had last seen him. K.M. testified that Dr. Brant’s assessment was accurate of his situation at the time. In his report to Dr. Manderville, Dr. Brant stated that K.M. had reported signs and symptoms of major depressive episode. K.M. was feeling almost continuously dysphoric, with crying spells, severe middle insomnia, absence of sex-drive, and a poor appetite. K.M. had stopped using marijuana 60 days prior and now felt worse. K.M. had been only intermittently employed over the last three years.

[242] Dr. Brant described K.M. as feeling super depressed, staying at home by himself, watching television all day and moping around the house. His relationship with K.W. was seriously deteriorated and they had not been intimate since August due to his reduced sex drive and K.W.’s rejection of K.M. He looked, sounded and acted depressed. His thinking was normal in content and process. K.M. denied any suicidal ideation or plan. Dr. Brant reported that K.M.’s “recent and remote memory was intact. There was no sign of intoxication or brain damage”. His diagnosis was major depressive episode. He noted that numerous attempts at pharmacotherapy in the past had failed. He thought one of the newer antidepressants might be helpful and he prescribed Zoloft together with Buspar.

[243] Dr. Brant reported to Dr. Manderville on July 28, 1993 (Exhibit #1, Tab #A27) that he had seen K.M. in January, February and March of 1993. K.M.’s relationship with K.W. had ended and she had taken up and moved in with another man. K.M. was in a subclinical rage about the way he had been treated by the women in his life. K.M. had stopped his most recently prescribed drug, Luvox, which was restarted. After the breakup, K.M. was more depressed.

[244] On September 23, 1993 (Exhibit #1, Tab #A28), Dr. Brant reported that during his assessment on September 1, K.M. was feeling well. He had stopped taking Luvox and had cut off all contact with K.W. and her son. No plans were made to see Dr. Brant again. K.M. testified that he would sometimes feel optimistic, positive and somewhat normal, but it would only last about a month.

[245] On May 4, 1994 (Exhibit #1, Tab #A29), Dr. Brant reported that he had started K.M. on Prozac in February 1994 and that K.M. had made a remarkable improvement. He recorded that K.M.’s vegetative signs of depression were in remission and that his sex-drive had returned. Dr. Brant returned K.M. to Dr. Manderville’s care and recommended he remain on Prozac for another eight months. Shortly thereafter, Dr. Brant passed away.

(iii) Dr. E. B. Conn – 1994 to 1996

[246] Thereafter, K.M. was in the care of Dr. Goddard-Hill as his family doctor and Dr. Conn as his treating psychiatrist, except for 1996 to 1997 when K.M. was briefly in the Guelph, Kitchener-Waterloo area. Dr. Conn reported on March 23, 1995 (Exhibit #1, Tab #A30) that K.M., then age 29, was experiencing problems with depression and that Prozac appeared to have lost its effect. Amitriptyline was not helping. He had broken off with his girlfriend (K.W.) and

his other friends. He was utilizing alcohol and marijuana. Dr. Conn did not think any medication would completely address K.M.'s problems and recommended he stop using both alcohol and marijuana. Dr. Conn found that K.M. had a decided personality disorder and difficulty with interpersonal relationships over his lifetime. He was tried on Paxil. Dr. Conn's diagnosis of K.M. was (1) personality disorder with dysthymia and (2) substance abuse with its associated sequelae.

[247] Dr. Conn, in this same report, reported that K.M. came from a "rather dysfunctional family", noting: "Parents separated some years ago and father lives on his own on a disability pension on the basis of having marked memory problems". Dr. Conn had "never been completely convinced that this is an organic problem".

(iv) Dr. M. Schwarz – 1996 to 1997

[248] K.M. was now living in the Guelph, Kitchener-Waterloo area and still having mental health problems. He testified that he was struggling when he got there and needed to try and find some help. In 1996 and 1997, Dr. Choong (Exhibit #1, Tab #A31) had prescribed Effexor, Paxil, and Prozac for K.M.'s chronic depression. On July 26, 1996, he referred K.M. for a psychiatric assessment, which was conducted by Dr. M. Schwarz.

[249] On February 25, 1997, Dr. Schwarz, a psychiatrist at the Guelph Community Mental Health Clinic, assessed K.M. Dr. Schwarz did not have access to any of K.M.'s old psychiatric records. K.M. was not in any intimate relationship. According to the assessment report (Exhibit #1, Tab #A33), K.M. had excessive anxiety and worry that he found difficult to control, and he was scared for no reason. He had difficulty concentrating and felt overwhelmed. He had lost 30 lbs in the last 4-5 months. He had panic attacks. He was described as "inappropriately familiar". He was "chaotic and easily distracted" and it was "not easy to get a good history from him". His thought content displayed low self-esteem, some hopelessness and worthlessness. He denied suicidal ideations. He denied use of alcohol, but utilized hash once a week to improve his mood. K.M. had "good childhood memories". He denied a childhood history of any kind of abuse. His academic performance was poor and he was labelled as a "slow learner". He had to repeat many grades. He completed grade 8. There were no behavioural problems. He left home at 16 and started working at the same time. He held numerous small jobs. He had relationship problems with his supervisors and colleagues. He started dating at 16 and had had three serious relationships. He had a pattern of becoming jealous and possessive very quickly towards the person he dates. He had a past history of being physically abusive towards his girlfriends. Because of his temper, he had decided to remain single for the rest of his life.

[250] Mentally, K.M.'s capacity to focus his attention was impaired by his anxiety. His judgment was fair. He had some insight into his mental illness and limited functioning, but his motivation for treatment seemed to be compromised. K.M.'s diagnosis was similar to assessments in the past: Axis 1 – generalized anxiety disorder, panic disorder, dysthymia, and cannabis abuse; Axis IV (mild) – loneliness, upcoming job loss, chronic mental illness; Axis V (GAF) – 70. He was put on Paxil but Dr. Schwarz recommended that Lithium, Benzodiazepines, or Buspar be considered in the future. It was recommended K.M. stop using cannabis.

(v) Dr. E. B. Conn – 1997 to 2006

[251] K.M. was back in Belleville in 1997, having left the Kitchener-Waterloo area. He was 32 years of age and once again saw Dr. Conn as his treating psychiatrist.

[252] Dr. Conn reported on March 2, 1998 (Exhibit #1, Tab #A34) to Dr. Goddard-Hill, K.M.'s family doctor, that K.M. was chronically depressed, feeling sad, despairing and hopeless. He had a girlfriend of one year (C.P.) but his moods created some problem in the relationship. He lacked energy, had a decrease of interests, a marked decrease of capacity to enjoy life, felt anxious, had hypersomnia, had limited concentration span, had tendency for social withdrawal, and had increased irritability. Dr. Conn reported that K.M. had obtained grade 8 and because of his learning disabilities was in "special ed". He was out of work, which consisted mainly of concrete work. A family history of mental illness, apart from the father, was denied. Dr. Conn had previously questioned the father's amnesic problem. Dr. Conn reported that K.M.'s father D.M. suggested to K.M. that he be on a disability pension. K.M. maintained contact with both parents. Dr. Conn reported that K.M. had been off alcohol and street drugs for a period of time, but had in the past used them heavily for at least five years. He had had panic attacks.

[253] Dr. Conn reported that K.M. had no criminal history. He reported that K.M. had no close friends and has "never had the skill to make close friends". There was no evidence of significant cognitive impairment. K.M.'s depression was compatible with dysthymia, which is why Dr. Conn thought K.M. had not been responsive to antidepressant medications in the past. He noted that K.M. did seem to benefit from Xanax. He prescribed Trazadone and recommended hospital day care but K.M. refused, stating there was no benefit from his previous hospital involvement on Quinte III (Belleville General Hospital) or KPH (Kingston Psychiatric Hospital). K.M. had had personality problems in the past. Dr. Conn's diagnosis that day was: Axis I – chronic dysthymia, and Axis II – personality disorder, with a guarded prognosis.

[254] Dr. Conn stated in this same report (Exhibit #1, Tab #A34, p. 109) that, "I could not on the basis of his history to date indicate that he has a disability of such type that he could not be employable. I fear that he will suffer if he does not proceed and successfully obtains a disability pension since I feel this would commit him to a life of inactivity and lack of purpose".

[255] K.M. was cross-examined about Dr. Conn's statement. K.M. testified that Dr. Conn never told him that it was a bad idea or it would not be advisable for him to try and get a disability pension, nor did his family doctor, Dr. Goddard-Hill. Dr. Conn did not say to him that his life would be ruined if he got a pension.

[256] K.M. testified about his mental health issues, over these years and in the 1990's, as getting worse and knowing that he was not going to get any better.

[257] K.M. met his girlfriend C.P. in 1997 and their son M.M. was born in July 1998. K.M. testified that it was a very stressful situation for him. They were separated by December 1999. On December 20, 1999, K.M.'s family doctor saw K.M. and reported in an almost illegible note that he was having frequent attacks of crying and that his girlfriend (C.P.) had left him as of December 1, 1999 (Exhibit #1, Tab #A1, p. 29, typed note at p. 33). The abbreviated typed note

said “reactive depression. separation, son with wife. 2 wks.” The handwritten doctor’s note goes on to say that she was pregnant and “gave birth in July. Sees her still friendly and amicable. She wanted to get married, he did not”. K.M. agreed that when he had relationship issues, he would tend towards more depression and more anxiety and that he would require medical attention. K.M. testified in cross-examination that breakup of relationships has been particularly traumatic for him. K.M. agreed that when his parents divorced it caused a certain amount of anxiety and depression, which required some treatment for him.

[258] On December 22, 1999, K.M. ended up at the Belleville General Hospital emergency department and was seen at the Crisis Intervention Centre. His relationship with C.P. had ended. He was sad about the separation but felt that his son M.M. filled the void. The social worker P. Schroth recorded (Exhibit #1, Tab #A35, p. 110) K.M. as stating he had “been depressed since age 10”. He was having suicidal ideations, but denied any plans or current intentions. The social worker recorded that K.M. stated that “he may kill himself in the fall of 2000 because he likes fall”. The Emergency Room doctor recorded (Exhibit #1, Tab #A36) that K.M. had suicidal thoughts for a long time but would not act on it. He recorded that K.M. was adamant not to be admitted as he wanted to go to work. K.M. was working at Sears Warehouse at the time. He testified that he had tried to overdose, maybe on Parnate.

[259] K.M. was cross-examined in detail about his reference to being “depressed since age 10”. K.M. did not agree that this was the case. He agreed that the statement was probably made when he was “in a hell of a mess, and could probably not even think straight”. K.M. testified that, “I didn’t have any real depression problems when I was 10 years old”. He said that he did not check his journal before he went to the crisis centre, and testified, “I was a couple of years off”.

[260] On January 28, 2000, K.M., age 34, attempted suicide and had depressive symptoms with on-going suicidal threat. He was admitted to the Belleville General Hospital (Exhibit #1, Tabs #A37-40) on a Form 1 (Involuntary Admission) and discharged on February 14, 2000. The Initial Assessment and Emergency Room records state that K.M. overdosed. The medical records indicate he took 10 to 11 Oxazepam pills in a suicidal gesture and stated he felt hopeless and would attempt again. Dr. L. Ash recorded that K.M. had advised that he had been depressed and suicidal all his life and that he wanted to die by the fall of that year. Dr. Ash reported that K.M. tended “to be very negative about his life in general and does not seem to be able to make a commitment about anything”. He diagnosed him as having a long standing personality disorder and prescribed Paxil. He described K.M. as seeming to be “unable to make a commitment to anything which requires long-term effort on his part”, and that this was a problem, “not only in terms of his personal relationship but also in terms of his vocational career as well”. K.M. had lost his job at Sears Warehouse a few weeks before. K.M. had chronic suicidal ideation but denied any recent suicidal attempts. K.M. seemed quite willing to stay in hospital and did not have a thought disorder. On that basis, Dr. Ash cancelled his Form 1 on January 29, 2000.

[261] K.M. signed himself out of hospital and was discharged in the morning of February 3, 2000 (Exhibit #1, Tab #A41); he was considered as not being extremely stable. K.M. was quite disturbed that his common-law wife (C.P.) was attempting to get full custody of his son and he became agitated. He had second thoughts and decided to return. K.M. came back and was re-

admitted a few hours later the same day (Exhibit #1, Tabs #A42-43). His diagnosis was Axis I – acute chronic dysthymia; Axis II – personality disorder; and Axis IV – impaired psychosocial ability, with an Axis V GAF score of 65-70.

[262] While in hospital on Quinte III, K.M. was referred by Dr. Conn for MMPI-2 psychometric testing. He was tested on February 4, 2000. In the report dated February 7, 2000 (Exhibit #1, Tab #A44), the MMPI-2 yielded a valid protocol consistent with patients who are admitting personal and emotional problems and who are requesting help with these problems. His Axis I diagnosis was adjustment disorder, with secondary considerations of dysthymia, major depression, and psychoactive substance abuse. His Axis II diagnosis was antisocial personality disorder. It was noted that his prognosis was generally poor with respect to traditional interventions of psychotherapy.

[263] K.M. was discharged from hospital on February 14, 2000 (Exhibit #1, Tabs #A45-46) on Nozinan, Effexor, Epival, and Temazepam and with a diagnosis of acute depression and personality disorder. He was improved and had no evidence of suicidal ideation on discharge. According to K.M., his substance abuse had been brought under control. He was referred to the day hospital program and had follow-up appointments with Rick Bough, a counsellor at the hospital.

[264] On July 21, 2000 (Exhibit #1, Tab #A48), Dr. Conn saw K.M. for a follow-up appointment as he was experiencing progressive depression. K.M. was becoming more emotionally labile with unpredictable tearfulness. Dr. Conn decreased K.M.'s dose of Effexor, added Wellbutrin and maintained his Epival and Nozinan. Dr. Conn noted that “[h]opefully, this may break through this depression which is developing strains of being resistant depression”. K.M. denied alcohol and street drug abuse.

[265] On May 4, 2001 (Exhibit #1, Tabs #A49-50), K.M., age 35, went back to the crisis centre. He was diagnosed as having a personality disorder and a medication problem, as he had run out of Effexor. He was put on Ativan for 5 days until he could get in to see Dr. Conn. On December 20, 2001, K.M. was at the emergency room asking for Ativan, which he was prescribed.

[266] On January 7, 2002, K.M. attended at the Mental Health Clinic and was prescribed Effexor and Zyprexa. Dr. Conn reported on March 7, 2002 that K.M. in the last week or two had been feeling more depressed. Dr. Conn reported that it appeared to be “an ongoing situation with [K.M.] with some improvement followed by relapses”. His diagnosis was dysthymia, personality disorder and past substance abuse, with the latter under control. He reported that K.M.'s motivations for gainful employment were not particularly strong and that he was receiving welfare and hoped to be able to receive a disability pension. He was taken off Effexor and placed on Celexa and Risperdal. On July 25, 2002, Dr. Conn discontinued Celexa, as it was not working, and prescribed Remeron (Exhibit #1, Tab #A53).

[267] K.M. applied for financial assistance from O.D.S.P. and Dr. A. Wolfgarth, a clinical psychologist, did his assessment of K.M. on December 19, 2002 (Exhibit #1, Tab #A54).

[268] On March 10, 2003 (Exhibit #1, Tab #A55), Dr. Conn recorded that K.M. was experiencing considerable problem with memory and concentration span. K.M. also noted a decrease in his mood and an increase in his panic responses. He had been taken to task verbally by an employer in the Toronto area while doing deliveries. His depression was increasing. His prescription for Remeron was continued and Celexa was added.

[269] On May 7, 2003 (Exhibit #1, Tab #A56), Dr. Conn described K.M. as being particularly tense and anxious. His Celexa was discontinued, his Remeron was continued and additional Risperdal was prescribed. K.M. had just been approved for his O.D.S.P. benefits, which Dr. Conn felt would “allocate him to the ongoing disabled group”. In an addendum, Dr. Conn reported that K.M. was “displaying a progressive propensity to hypochondriasis as part of a somatoform disorder”. He felt K.M.’s depression was being contained by his antidepressant.

[270] Dr. Conn further stated in the addendum that K.M. “is the product of a family in which his father has continued to follow the path of a disabled person and often I have questioned the validity of his claim for marked memory impairment”. It was suggested to K.M. in cross-examination that Dr. Conn thought that part of K.M.’s problem was that he was following his father’s path of a disabled person. K.M. testified that Dr. Conn never suggested to him that it was a bad idea for him to get caught up in whole idea of disability like his father had.

[271] On June 20, 2003 (Exhibit #1, Tab #A57), Dr. Conn saw K.M. for a follow-up appointment and noted that K.M. was doing better in the past while, which Dr. Conn attributed to K.M. receiving the “Ontario pension” that relieved many of his financial worries. He was doing well on Risperdal and Remeron. K.M.’s hypochondriasis had remitted. Apart from his father’s history of memory impairment, there was no family history of mental illness per se. He noted that K.M. had not been very successful in maintaining any long term employment.

[272] On January 11, 2005 (Exhibit #1, Tab #A58), Dr. Conn reported that he had been seeing K.M. once every two months and that K.M. was generally improved. K.M. was prescribed Amitriptyline for back pain and was continued on Risperdal, and Mogadon. Dr. Conn recorded that K.M. “certainly was reared in a home environment which had it’s [sic] problems and this, of course, has obviously affected his development emotionally.” K.M. was cross-examined about Dr. Conn’s notation. K.M. testified that he did not know if his father had mental health issues, but that his father has had his struggles and they had the same psychiatrist. K.M. would not agree that growing up, his father had anxiety issues that affected his relationship with him. K.M. said his father was “stand-offish” and that his mental health issues were after his parents got divorced. Dr. Conn never suggested to K.M. that part of his problem was the challenges that he had at home as a young child. K.M. testified, “There’s no genetic link ... if that’s what you’re [School Board’s counsel] trying to imply”.

[273] In re-examination, K.M. testified that the first indication he had that his father might have some sort of mental or psychological issue was at the time his parents were separating and divorcing, which he guessed was in the “early 80’s, the 82, 83 block”. His mother A.F.’s evidence and K.M.’s doctor’s records indicate that the separation was actually in 1989. K.M. testified that his father’s issues were with his memory, more than anxiety.

[274] As to the “problems” in the home, K.M. testified in cross-examination that if there were problems, he did not know what they were. He would not speculate or guess.

[275] K.M.’s mother A.F. was cross-examined about Dr. Conn’s note regarding K.M.’s home environment. She testified she had no idea about the problems to which Dr. Conn was referring.

[276] In a follow-up appointment with Dr. Conn on May 17, 2005, K.M. was still doing well and his medications were maintained (Exhibit #1, Tab #A59).

[277] It is around this time in 2005 that K.M. began his second dating type relationship with Y.G., which, according to the definitive evidence of Y.G., lasted two years. I find their second relationship began in 2005. In order to ascertain the commencement of their relationship, I refer to medical records from 2007. An August 21, 2007 addendum by Dr. J. McMillan states that K.M.’s relationship with Y.G. was generally going well (Exhibit #1, Tab #A67), and an addendum by Dr. Singh from October 15, 2007 states that Y.G. had left K.M. (Exhibit #1, Tab #A72).

[278] On January 19, 2006 (Exhibit #1, Tab #A60), Dr. Conn reported that he was now seeing K.M. every three months and that he was doing quite well. He was continued on Amitriptyline and Risperdal. His dysthymia and personality problems generally appeared to have markedly improved. K.M.’s problem with substance abuse appeared to have been resolved. Dr. Conn, who was nearing retirement, determined that K.M. did not need to be followed by a psychiatrist and that Dr. Goddard-Hill could follow up with K.M.’s medication.

[279] On May 17, 2006 (Exhibit #1, Tab #A61), Dr. Conn advised K.M. that he was retiring at the end of June 2006. K.M. was described as being most anxious to see another psychiatrist. His dose of Amitriptyline was increased to help him cope with his chronic depression and he was continued on Risperdal. K.M. never told Dr. Conn anything about having been sexually abused by Marson.

(vi) June 2006 to July 2007

[280] After Dr. Conn retired, K.M. was temporarily without a treating psychiatrist. Marson was arrested on October 13, 2006. The *Belleville Intelligencer*’s newspaper article about Marson’s arrest was published on October 25, 2006.

[281] K.M. first spoke to the police on October 30, 2006. He provided a lengthy handwritten/printed statement to them that took until November 13, 2006 for him to complete, disclosing the details of the sexual abuse by Marson.

[282] On November 7, 2006 (Exhibit #1, Tab #A62), K.M. went to the emergency department at the Belleville General Hospital complaining of mental health issues. He denied suicidal thoughts and did not want to speak to the Crisis Intervention Centre. He was not taking any medications. Exhibit #1, Tab #A62 seems to be the intake form only and does not appear to have been completed as to his treatment or medications. This was during the time K.M. was writing his statement to the police.

[283] On November 24, 2006 (Exhibit #1, Tab #A63), Dr. Goddard-Hill referred K.M. to Dr. Singh at the Quinte Health Care Mental Health Centre with issues of mood disorders, including acute episodes of major depression, bi-polar disorder or dysthymia, and general psychiatric disorders. Dr. Goddard-Hill determined that K.M. needed ongoing follow-up and review. He was being prescribed Nozinan at the time. K.M. continued under Dr. Goddard-Hill's care for his mental health issues until he was first able to see Dr. Singh, which was scheduled for August 2007.

[284] On July 13, 2007 (Exhibit #1, Tab #A64), K.M., now age 41, went to the emergency department at the hospital. He was seen by the Crisis Intervention Centre, complaining of increased symptoms of depression and anxiety. Dr. Goddard-Hill had prescribed Ativan and Citalopram. K.M. had discontinued taking the Citalopram. K.M. advised that he was using "a lot" of THC but denied any other drug or alcohol use. He smoked a carton of cigarettes a week. K.M. advised that his sleep and eating patterns were poor and that since last October he had lost 45 pounds. It was recorded that K.M. was feeling "overwhelmed with increased emotionality" and that his current stressors involved a pending trial where K.M. may be required to testify. K.M. had constant suicidal ideation but denied any plans, noting that his 9-year-old son was his reason for living. K.M. was assessed as having increased depression and anxiety "pending sexual abuse (victim)". It was noted that Dr. Goddard-Hill had already referred K.M. to see Dr. Singh. K.M.'s appointment with Dr. Singh was August 9, 2007 but it appears it was moved up to July 30, 2007.

(vii) Dr. A. Singh – July 2007 to June 2012

[285] Dr. Singh was K.M.'s treating psychiatrist from July 30, 2007 until June 25, 2012 when he semi-retired, and he testified about his treatment of K.M. Dr. Singh was K.M.'s first treating psychiatrist with a complete history that included that K.M. was a victim of sexual abuse. Dr. Singh treated K.M. with cognitive psychotherapy supplemented by medications. As English is not Dr. Singh's first language, I have made some minor grammatical corrections in my summary of Dr. Singh's evidence.

[286] Dr. Singh and his residents prepared clinical consultation notes of K.M.'s care at his clinic in the Quinte Health Care Belleville General Hospital, and these notes are before the court (Exhibit #1, Tabs #A67, #A71-96). The notes begin July 30, 2007 and end September 6, 2011, recording 27 visits with K.M. O.H.I.P. has recorded four visits in 2011 and 2012 for which we have no consultation note. Dr. Singh's last O.H.I.P.-recorded visit with K.M. as a treating psychiatrist was on June 25, 2012, just before Dr. Singh left to work overseas.

[287] On referral from Dr. Goddard-Hill, Dr. Singh first saw K.M. on July 30, 2007 at the Quinte Health Care Belleville General Hospital in conjunction with Dr. McMillan, a psychiatric resident working with Dr. Singh. Together, they did a complete assessment of K.M. Dr. Singh testified that as with any new patient, his first meeting with K.M. was about 45 minutes to 1 hour. Dr. Singh would review presenting symptoms first, then a full history starting from childhood, educational life, family life, work and relationship or "marriage history", past illnesses, and past treatments. Then he would do a personality evaluation, a mood evaluation and present mental condition. Lastly, he would do investigation, tests and treatment with

recommendations made, usually for a combined regime of pharmacotherapy and psychotherapy. Supportive psychotherapy of managing the side effects of medications was important to ensure compliance. Follow-up meetings were about 30 minutes long and would begin with K.M.'s present mental condition, whether he had improved or deteriorated, and then an evaluation to determine if the treatment plan had to be changed. Every visit evaluated K.M.'s present mental condition and suicidal ideations.

[288] Dr. McMillan wrote the report in consultation with Dr. Singh on July 30, 2007 (Exhibit #1, Tab #A67, p. 154). K.M. had been seen due to his worsening and deteriorating depression since October 2006. The criminal proceedings against Marson were beginning to take a toll on K.M. The proceedings had stalled due to Marson's health concerns. K.M. had lost 35 pounds since October. His anxiety level was increasing. K.M. wanted to get better so he could take better care of his son. It was recorded that he found that his mother had a tendency to be overprotective and overbearing. K.M. was using two marijuana joints a day and taking Ativan as prescribed. He had discontinued Celexa due to gastrointestinal symptoms. He was advised to reduce his use of marijuana. He was prescribed Olanzapine rather than Ativan, which Dr. Singh testified was a benzodiazepine and an addictive drug. Blood work was ordered. It was noted that K.M. had used cocaine in the remote past for approximately one year and that he had tried LSD in the 1980s. His relationship with his girlfriend (Y.G.) was noted as generally going well.

[289] Dr. Singh testified about the addendum (written August 21, 2007) to the July 30, 2007 report, where it was noted that K.M. "[r]equired resuscitation" and had a grade 6 education. In testimony, Dr. Singh mentioned that he took K.M.'s history which included, I infer, K.M. telling him that he had been born with a cord around his neck and that resuscitation was needed.

[290] July 30, 2007 was the first time K.M. had told a psychiatrist about the sexual abuse. Dr. Singh testified that his impression was that K.M. was a chronically depressed person: "My impression was that we were dealing with a chronic case, which has more than one psychopathology". They were trying to first find the depression, which was the predominant concern because they did not want him to become suicidal.

[291] K.M. continued to attend as an out-patient at the hospital in Belleville, which is where Dr. Singh and his psychiatric residents worked. K.M. became a regular patient of Dr. Singh. K.M. had someone working with him from the Sexual Assault Centre. Dr. Singh's role was in treating K.M.'s depression.

[292] On October 2, 2007, Dr. Singh reported that K.M. was taking Olanzapine and was still using marijuana, which Dr. Singh wanted him to reduce. An addendum on October 15, 2007 noted that K.M.'s girlfriend of 9 years, whom I find was Y.G., had left him. As I have found earlier, Dr. Singh failed to note that it was for the second time.

[293] On November 20, 2007, Dr. Singh reported that the marijuana was becoming less effective. K.M.'s Olanzapine was increased due to his anxiety, some of which was related to the criminal court case involving Marson. K.M. did not want to take the anti-depressants because of the side effects. Dr. Singh wanted him to stop using marijuana. There was another reference to his girlfriend (Y.G.) having left him. Dr. Singh testified that K.M. was deteriorating.

[294] Dr. Singh testified that on February 25, 2008, K.M.'s mood was slightly better but he wanted to be prescribed Ativan, which Dr. Singh refused to prescribe. Dr. Singh wrote that he did not give any drug in large amounts "for which there are problem of addiction and also dependence" (Exhibit #2, Tab #A74). K.M. told him he wanted a different psychiatrist and that his sexual assault worker from the sexual assault clinic was going to find him another psychiatrist. It was an amicable discussion. K.M. was in a happy state of mind, particularly as Marson had been just recently convicted of the sexual offences (on January 23, 2008). K.M. was not concerned that his girlfriend relationship (with Y.G.) had broken down, and he wanted a new fresh start with time and with a slow pace. He was continued on Olanzapine. Dr. Singh noted that K.M.'s problem of substance abuse remained, but he did not focus on it as K.M. was going to see another psychiatrist.

[295] K.M. ultimately continued in the care of Dr. Singh at the clinic in the Belleville Hospital. On July 11, 2008, K.M. saw Dr. Singh's resident, Dr. N. Charriere, in consultation with Dr. Singh. K.M.'s mood was good. K.M. advised that things had been going fairly well for him. His visits with his son had been going very well and he reported that his son brought him great joy. He stated that he lived in fear of having another significant depression, he was not optimistic about his future, and he lived day to day. He did not involve himself in things that might upset him. He was continued on Olanzapine.

[296] On August 26, 2008, Dr. Singh noted that K.M. was maintaining his mood improvement and being compliant with medication. Dr. Singh was treating him as well with cognitive psychotherapy directed at bringing K.M. more self-confidence. There was no change to his medication. Dr. Singh testified that K.M.'s worry was a "little less" because the teacher (Marson) had "accepted the fault or guilt" in the criminal proceedings. Dr. Singh testified that clinically, K.M. was slightly better. The depression was still present and was diagnosed as dysthymia.

[297] Dr. Singh testified that dysthymia "is a kind of depression which doesn't cure itself, from treatment, completely. It can improve, but it will be fluctuating improvement. And stress will bring it down. And dysthymia happens in a long term depressive person, and he [K.M.] was one of them".

[298] On October 21, 2008, K.M. continued to show improvement and was enjoying visits with his son. K.M. still lacked confidence to build any kind of companionship or social activities.

[299] On December 22, 2008, K.M.'s mood was much better, although Christmas was not a good time for him. Cognitive psychotherapy was directed at improving his social life, as many times he felt lonely. His medication was not changed.

[300] On February 10, 2009, Dr. Singh reported that K.M.'s overall mood remained better though it fluctuated sometimes, but he bounced back to a normal mood. K.M. was building a relationship with his son, which was going very well. He had also started socializing slowly and gradually. This civil court case was a problem that was bothering K.M. His compliance with his medication was excellent and his mood was much better, and there were no side effects of the medication.

[301] On May 12, 2009, Dr. Singh reported that K.M. continued to show improvement and that his social life was improving. This court case was still a problem. His relationship with his son was progressing and he was getting along better with C.P., the mother of his son, as to access. Cognitive psychotherapy was directed at improving K.M.'s social life and him socializing more. Dr. Singh noted that K.M. was "not keen on keeping companionship with ladies" because of his previous traumatic breakups. His medication remained Olanzapine Zydis.

[302] On August 27, 2009, Dr. Singh noted that recently K.M.'s mood had been slightly down and that his fast thinking was bothering K.M. His problem had started with the sudden death of a very close friend, and his son was moving to Bancroft. His depression was not significantly increased but his fast thinking and worries had increased. His medication of Zyprexa Zydis or Olanzapine was prescribed at an increased dosage.

[303] On October 15, 2009, Dr. Singh reported that K.M. was doing very, very well. He was enjoying his time with his son and at the moment everything in life was satisfactory. His medication remained the same. On December 17, 2009, Dr. Singh reported that K.M. was doing very well except there was stress from his civil case and he was apprehensive about the questions being asked "of various private nature and of the past". Cognitive psychotherapy was directed at bringing confidence to him. His relationship with his young son was blooming. However, on the socialization side, K.M. was "not very much social". He was doing well on his current medication.

[304] On January 25, 2010, Dr. Singh reported that K.M. was getting "flooded with stressful event of life". His mother was being bullied by her landlord and K.M. was asked to go into rehab, which related to his past sexual abuse. He wanted more time with his 11-year-old son, but it was refused by the child's mother. Cognitive psychotherapy was directed at stress resolution. No change was made to his medication.

[305] On February 11, 2010, Dr. Singh reported that K.M. was a bit more stressed. There were problems with K.M.'s access to his son and he was going back to court. Cognitive psychotherapy was directed at resolving stress. His Olanzapine was continued, plus Nozinan for sleep. On March 5, 2010, Dr. Singh reported that K.M.'s mood was fluctuating, mostly because of this civil court case. It brought him anxiety and worry. His relationship with his son was improving. Cognitive psychotherapy was directed towards understanding his own past trauma and ways to deal with it. No change was made to his medication.

[306] On May 3, 2010, Dr. Singh reported that K.M. overall continued to improve. Cognitive psychotherapy was directed towards making K.M. confident in going forward and learning from the past but never going back to the past, and encouraging K.M. to make a plan for his life.

[307] On August 3, 2010, K.M. was seen by a resident of Dr. Singh, Dr. Y. Chuhan, in consultation with Dr. Singh. They described K.M. as continuing to improve. K.M.'s access issue to his son was resolved. K.M. had started to see a new partner. He was taking Warfarin for a previous DVT. He had had some financial relief (the evidence is that on June 24, 2010, K.M. received \$12,500.00 from the Criminal Injuries Compensation Board). K.M. was advised to

spend more time taking care of himself, physically and mentally. He had a marked improvement in the quality of his life.

[308] Dr. Singh testified that these improvements on August 3, 2010 do not mean that everything with K.M. was fine. If K.M.'s situation in life was better because his stressors had gone, his spirit would be boosted, but that does not mean that the stressors could not come back. Dr. Singh explained: "The phasic response is, 'The stress is gone, and I'm better'".

[309] On September 16, 2010, Dr. Singh reported that K.M. was going through a difficult and stressful path. He had to see Dr. D. Silver to be examined for the School Board in this civil case. K.M. could not visit his mother due to her landlord's restrictions. These stressful things had made his mood fluctuate, but he was not depressed. Cognitive psychotherapy was directed at bringing his self-confidence and self-image up. This depression was classified as reactive and his medication regime was not changed.

[310] On September 20, 2010, K.M. was seen by Dr. D. Silver, a psychiatrist, for a defence medical psychiatric assessment.

[311] On November 9, 2010, Dr. Singh reported that K.M.'s mood had been down in recent weeks. There were problems about child support, his mother's landlord and the prolongation of this legal case. Because K.M.'s mood was down, he was prescribed Remeron in addition to his regular medication. Cognitive psychotherapy was directed at ways and means to resolve the stresses of his life and create a happy outlet. K.M. was advised to improve compliance with his medications.

[312] On December 17, 2010, Dr. Singh reported that K.M. was doing very well and maintained improvement. He was seeing his son for Christmas. Cognitive psychotherapy was directed towards K.M. making an effort at socialization and improving the relationship "with the girlfriend but should not think of relationship as yet for long-term basis before knowing her". He was maintained on Olanzapine and Remeron.

[313] On January 21, 2011, Dr. Singh reported that K.M. was trying to cope with all the adversities and was doing better in his mental condition. His relationship with his mother had improved. K.M.'s relationship with his son was improving. Stress from the family side was now stabilizing. Cognitive therapy was directed towards developing patience and planning a goal for the future. His medications were continued.

[314] On March 7, 2011, K.M. was seen by Dr. Singh and a resident of Dr. Singh, Dr. G. Skelton. Dr. Skelton reported that K.M. was doing quite well currently and that he did not have any concerns at the time in terms of his mood. K.M.'s family relationships had significantly improved, including with his mother, and he was going to take her out for her birthday. He was happy with his relationship with his son, whom he had visited that past weekend. His mood was good and he was enjoying his life right now. He was beginning to socialize and seek more companionship amongst friends, which was going fairly well. K.M. reported that he currently did not have a romantic relationship but felt that this would come with time. Cognitive

psychotherapy was directed at reinforcing the positive changes that K.M. had made. His medications of Olanzapine Zydys and Remeron were continued.

[315] On May 12, 2011, Dr. Singh reported that K.M. continued to do better and better. His mood was good. His life was becoming better and better. His issue with his mother's landlord was resolved. His relationship with his son was excellent. His socialization was also increasing. Cognitive therapy was directed at patience for the legal system. His medications were continued.

[316] On July 4, 2011, Dr. Singh reported that overall K.M. was doing much better, but recently his legal stress (from this case) had caused mood fluctuation but that he still was taking it bravely and was not depressed. He was having excellent fun with his only son who had "school free time with him" and he was enjoying his life with him. Cognitive psychotherapy was directed towards letting his lawyer decide the legal problems in this civil case and to take time off from the stress. His medications of Olanzapine and Remeron were maintained.

[317] Dr. Singh testified that K.M.'s stress had come back and he was going down. He was not as good as before. The other aspects of K.M.'s disorders, the personality disorder and the post-traumatic stress disorder, did not improve very much. The Remeron with Olanzapine were helping him to manage his depression.

[318] On September 6, 2011, Dr. Singh reported that K.M.'s mood remained satisfactory, though financial and legal matters were still bothering him too much. The only bright spot was that his relationship with his son was improving by leaps and bounds. Cognitive psychotherapy was directed towards bringing socialization "because his loneliness because of financial lack" had also become a factor of mood fluctuation. He was continued on Olanzapine and Remeron with good compliance.

[319] K.M. was cross-examined about his knowledge of any treatment programs that may have been recommended by Dr. Singh on August 14, 2009, and in particular the Chrysalis program under O.H.I.P. for treatment of personality disorder. K.M. testified that it was recommended to him but he did not know any of the details and he could not give a reason for not taking the program. K.M. testified that doctors had also recommended medications that did not work either, implying that the counselling would be similarly unhelpful. In re-examination, K.M. testified that if Dr. Singh suggested he do something, he never declined to do it. K.M. testified that Dr. Singh was an excellent, excellent doctor and that he felt lucky to have him. In subsequent meetings with Dr. Singh and Dr. Matveyev, they have not suggested any treatment that K.M. has declined to take.

[320] K.M. was asked about Dr. Singh's recommendation of private cognitive therapy counselling by a psychologist. K.M. does not remember anything about this recommendation and never considered private therapy because of lack of funds. K.M. was cross-examined about this lack of funds to pay for private counselling with a psychologist. K.M. agreed that he received \$12,500.00 in June 2010 from the Criminal Injuries Compensation Board, which he did not use for counselling and which was not deducted from his O.D.S.P. pension. K.M. received a lump sum C.P.P. retroactive payment of \$22,805.44 in November 2012 but explained that he used that

to live on for twenty months, in lieu of O.D.S.P. payments, until it ran out. He received a lump sum from Marson of \$50,000.00, which K.M. agreed he did not use for counselling.

[321] Dr. Singh last saw K.M. as his treating psychiatrist on June 25, 2012. Dr. Singh then semi-retired and departed from Belleville and Kingston at the end of June 2012. K.M. testified that when Dr. Singh retired, it made his situation worse.

(viii) June 2012 to present

[322] After Dr. Singh retired, K.M. no longer had the benefit of a treating psychiatrist and was cared for by his family doctor. K.M. continued to struggle, in particular with his depressive symptoms. Dr. Goddard-Hill, his family doctor, referred K.M. to Dr. Matveyev, a psychiatrist at the Belleville Hospital clinic. Dr. Matveyev saw K.M. on June 14, 2013 (Exhibit #1, Tab #A69, pp. 157-159) and reported that K.M. was doing quite well managing his symptoms of depression and anxiety. He was using marijuana daily to level his mood. Dr. Matveyev noted that K.M. self-reported that during the time frame of the abuse, K.M. was “painfully ashamed, terrified, and fearful every time he would go to school”. K.M.’s diagnosis was consistent with complex post-traumatic stress disorder (“PTSD”), described as “repetitive subjection of a person to abuse in the situation where the person has no control over what is being done to him and feeling terrified and fearful”. His depressive episode was more consistent with dysthymia. There was insufficient evidence to support a diagnosis of borderline personality disorder. No psychiatric medications were recommended.

[323] In his testimony, Dr. Singh disagreed with Dr. Matveyev’s statement on June 14, 2013 that there was a lack of evidence for borderline personality disorder in K.M. Dr. Singh had seen K.M. on August 4, 2015 for the purposes of preparing an expert report (discussed below in these Reasons for Judgment), and testified that Dr. Matveyev must have seen K.M. in a charming stage or phase of this disorder. Dr. Singh testified that the Maudsley Personality Inventory—the “gold” standard test for personality disorder, which he administered in 2015—confirms K.M.’s diagnosis of borderline personality disorder. Dr. Singh testified that K.M.’s diagnosis of borderline personality disorder has existed for 27 or 28 years and it is part of his current complex of disorders.

[324] K.M. saw Dr. Matveyev for a consultation on October 8, 2015 (Exhibit #1, Tab #A70) at the request of K.M.’s family doctor, Dr. Holmes. K.M. reported that he continued to feel depressed and that he isolated himself due to lack of trusting people. He spent most of his day by himself smoking marijuana. He lacked enthusiasm and motivation and had to force himself to do activities. His memory and concentration had been deteriorating. Dr. Matveyev found that K.M. had symptoms of chronic disturbance in his mood, lack of trust in people, poor self-esteem, lack of self-confidence, and anxiety giving him bouts of diarrhea.

[325] Dr. Matveyev found K.M. to have a “chronic disturbance mood that should be seen as dysthymic disorder in the context of his complex post-traumatic stress disorder which is the result of sexual abuse that he was subjected to through his school”. In particular, he found that K.M. “was diagnosed with learning disabilities” that Dr. Matveyev believed made K.M. “particularly vulnerable to this abuse”. The memory of the abuse still haunted him and

contributed to K.M.'s daily depression. K.M.'s diagnosis of dysthymia was noted to be treatment resistant when it comes to medication. There was no evidence found of borderline personality disorder. Dr. Matveyev recommended stimulation with Ritalin or Concerta for K.M.'s cognitive impairment, which was in the form of difficulty sustaining his focus and concentration, as well as the anti-depressant Celexa. Alternatively, he recommended Effexor with Mirtazapine.

[326] Dr. Singh testified that cognitive impairment causes memory impairment, not memory deterioration. Dr. Singh, on August 4, 2015, and Dr. Matveyev, on October 8, 2015, both saw cognitive impairment in K.M. This is a reactive condition to a cluster of disorders such as dysthymia and post-traumatic stress disorder, which is not the same thing as memory deterioration such as is seen in dementia or pre-senile dementia. K.M.'s cognitive impairment, Dr. Singh testified, was connected particularly with his dysthymia and post-traumatic stress disorder. Dr. Singh testified that K.M.'s father's diagnosis of *myasthenia gravis* is not part of cognitive impairment, but of memory deterioration genetically—a different cluster of disorders.

[327] K.M. has been prescribed numerous drugs for his depression, anxiety and mental condition over a number of years but nothing has worked long term as the effect of the drug would wear off, aside from Xanax. He also suffered from many adverse reactions and side effects of the drugs prescribed, including headaches, bowel problems and insomnia. At present, his prescription drugs are for Coumadin for blood clots in his legs and for medical marijuana, prescribed on referral from K.M.'s family doctor, Dr. Holmes.

h. K.M.'s Current Psychological and Mental State

[328] K.M. testified that presently, his PTSD controls every day. K.M. becomes overwhelmed with outside stimulation and he just wants to be left alone. He is depressed. He does not make promises that he cannot keep as he may not feel well enough to carry through with any plans made, whether they be personal, social or work-related.

[329] K.M. testified that when he is feeling depressed, he feels hopelessness and loss. It's lonely, it's empty. It's a void. He does not feel so well about himself. He tries not to be around other people, as they care about him and know he is sick and it affects them too. He tries to avoid them.

[330] When he feels the depression coming on, he just stays home. He goes for long walks and tries to stay motivated, positive. His drug use is ongoing in his use of marijuana. Asked why he uses it, he testified that he uses marijuana because he just wants to feel "normal". It makes him level out and feel somewhat normal. He does not drink alcohol since his son was born in 1998, as he knows how it affects him—it's not for him.

[331] During the past years, his depression fluctuates, but generally it's always with him. He has to be aware of what he is doing, and who is going to be there. It was difficult before and it is even more difficult now. His depression steadily gets worse. K.M. testified that his symptoms have a pattern of getting better, getting worse, and getting better. This pattern happens when K.M. puts himself in a stressful situation and he gets pushed too far.

[332] K.M. testified that he tries to manage his life and avoid other people's problems to prevent becoming overwhelmed. When his son was younger, he found the access pick-ups and drop-offs and the Family Responsibility Office issues overwhelming. He knew he had to be careful in that type of situation. His son is an adult now so it's better.

[333] K.M.'s son M.M. still continues to fill the "void" in K.M.'s life. K.M. testified that if he did not have his son, he probably would not be still here as he is not getting any better. His son keeps K.M. "focused" and keeps him "pushing".

[334] K.M. is unable to make a commitment to work or to an interpersonal relationship. K.M. was asked about his current mental state. K.M. testified that he realizes he has problems, especially with PTSD, which he described as his "newest demon". His PTSD, he testified, is robbing him of his memory and his focus, and ultimately, probably, his ability to be alone. K.M. testified that his depression was not as bad but he was full of anxiety from the process of this trial. He has low self-esteem but knows there are people who care about him, and he cares about people. The anxiety he feels is a feeling of being overwhelmed, like something bad is going to happen. It can be all-consuming. This feeling of anxiety affects his sleep, and how he interacts with other people. K.M. testified that he just does not want to be around people. They may say something that he takes the wrong way and he does not want to snap at people. He just leaves or avoids that type of situation. The anxiety affects his bowels—it gives him the "runs" and makes him feel nauseous.

[335] When K.M. wakes up in the morning, he takes it slow. He does not think it will be a wonderful day, as that would be false hope. That does not happen. He just pushes through the day as it is. He has some good days—things he can look forward to. He looks forward to the summer—canoeing, fishing, and getting out of his apartment. The worst thing about the way he feels is "[c]onstant, crushing, lonely ...trapped" in his "own misery". When he gets feelings of being depressed and overwhelmed, K.M. can withdraw in his apartment for as long as a week, avoiding people. People that care about him know to leave him alone. When he feels like that, he smokes marijuana as it helps. K.M. uses medical marijuana to help him from being overwhelmed. When he is feeling really bad, even marijuana does not help, so he does not smoke it then. He does not drink alcohol.

[336] K.M. was asked by his counsel to think back to what Marson did and the way things are today, and how he felt about it all. K.M. testified in response:

It's ruined my life. It's ruined the way I can deal with people. It affects how I relate to people. I can't trust people. I'd like to have someone in my life, but I know they're going to be hurt, and I'm going to be hurt—but I'll be hurt much worse, when it "goes south". And I don't have that many more chances to ... to "walk the tightrope" anymore. I'm getting too old for that. So, I'm alone.

i. Summary

[337] After the abuse by Marson, I find that K.M. is, as testified to by his sister S.G., a "totally different person". He does not have a lot of friends and he is not the happy person he

once was. K.M.'s personality and behaviour have changed. It was like S.G. was not his sister anymore. He would sleep a lot and not interact with family. He would not talk. He was withdrawn and depressed. He withdrew from relationships, including his family. He's now a loner. S.G. has had little contact with K.M. since their school years as she is still afraid he will commit suicide, as he has tried to do since Marson, and she can't handle that. She does not think K.M. will get any better.

[338] His mother A.F. said K.M.'s mood swings and his "funks" last for days at a time, on a weekly basis, and he will not have contact with anybody. A.F.'s evidence is consistent with that of T.A., a neighbour in K.M.'s apartment building. K.M.'s work history is sporadic and he could not be relied upon to show up for work. This is consistent with the evidence of his aunt, M.I.C. He becomes angry with employers and with others easily. M.I.C. sees K.M. as being damaged and feeling inferior, and taking offence easily. This is consistent with K.M.'s reaction at trial when he felt his intellect was being challenged. He took great offence. I find the evidence of K.M.'s family—which I accept—of a very negative personality and behavioural change in K.M. after Marson to be overwhelming and credible.

[339] K.M.'s problems with his learning and school continued after Marson. After K.M. was cross-examined about the details of Marson's sexual abuse on him—which was difficult for K.M.—he was asked and he agreed that after the fifth episode (of forced oral sex), he had the same sort of behavioural problems as before, the same low marks as before, and got into the same amount of trouble with authority with the school as before. The inference I draw from K.M.'s answers, particularly since his answers were given right after being cross-examined about the sexual abuse by Marson, is that these problems were related to his difficulties with learning and its consequences for K.M. at school, which existed both before and after the sexual abuse. I do find, however, an indication of decline in K.M.'s efforts at school after the sexual abuse by Marson. Except for technological and vocational studies involving power and hand tools, K.M. stopped paying attention at school and became a disruptive influence in the classroom at Harry J. Clarke School. He was pushed through his grades at school without achieving his grade level. His parents, I find, really wanted K.M. to get a high school education. They paid for and sent K.M. to a private school in Claremont, Saint John's School. K.M. was placed again in grade 8 at 15 years old—his first time living away from home. He was very homesick and started to run away from school, and hitchhiked back home three times. K.M. did poorly academically there as well.

[340] The nature of K.M.'s learning difficulties were still unrecognized. Saint John's School used corporal punishment, which K.M. agreed would now be considered abusive. Again, it was a different era. K.M. testified that he received disciplinary touches of the wooden stick on a regular basis, but that it did not bother him or hurt that much. Dr. Matveyev reported in his June 14, 2013 report (Exhibit #1, Tab #A69) that K.M. reported pretty rough treatment at the school, doing their dishes or selling honey door-to-door to support the school financially, and that they were punished physically. He concluded that in such a way, this treatment contributed to K.M.'s previous history of abuse. No further mention of this issue is made in the evidence.

[341] Ultimately, K.M. came back to Belleville and briefly attended Moira Secondary School. He quit school at 16, was booted out of his parents' house as a result, and moved in with his

girlfriend Y.G. His formal schooling days were over. K.M.'s problems with learning have not significantly changed and were noted as still being present when he was assessed by Dr. Wolfgarth in 2002.

[342] K.M. has tried to have interdependent relationships in the past, but they have all failed. His true love, Y.G., gave up trying to live with K.M. after 8 years, from 1981 to 1989. She could not take his mood swings, his mental health issues, his depression and his drug addiction issues any longer. K.M. used "pot" three to five times a day, kept his feelings to himself, slept a lot and was withdrawn. K.M. would mope around their apartment, and showed no interest in sex. Y.G. took up with someone else. K.M. was hospitalized with suicidal thoughts.

[343] K.M.'s next three-year relationship, from 1990 to 1993 with K.W., did not fare much better. The same problems of K.M. moping around, his mood swings, and his reduced sex drive continued. His next relationship was C.P., from 1997 to 1999. They had a child together, M.M., born in July 1998. K.M.'s mental health issues continued. When that relationship ended, K.M. attempted suicide. Being involved in M.M.'s life was at times difficult for K.M. due to K.M.'s depression and anxiety. Ultimately, K.M.'s son gave him a reason to live, which he otherwise may not have felt he had. M.M. is now an adult and lives in another city. Their relationship from K.M.'s point of view is good.

[344] K.M. and Y.G. briefly got back together in a dating type relationship for 2 years from 2005 to 2007. Again, she could not make it work, and left K.M. for good. The emotional ups and downs were too much for her. It was just too strenuous and she could not do it anymore.

[345] I find that K.M. has not had a live-in interdependent relationship since 1999. He has had three domestic relationships lasting from 2 to 8 years, which have all failed. Y.G. was his longest relationship. She just could not take K.M.'s mood swings, depression, and poor interaction with others. The evidence of K.M., his family and his friend T.A., which I accept, is that he has lived alone for years. A partner would not be able to take his mental health issues for long, and the break-ups push K.M. into an even deeper depression with feelings of loss and suicidal ideations.

[346] K.M., I find, has had a sporadic, cyclical work history mainly doing construction type work when he could work. His best year working, in his damaged state, was when he was at Tunnel Systems when he worked as a labourer in the cement field. When he could work, I find that K.M., while unskilled, was dedicated, attentive, very hardworking and willing to learn. Reference letters from previous employers from the 1995-1997 time period confirm this. I find his mother and two sisters have a strong work ethic. His father also worked and provided for his family until he became unwell and had to leave work. K.M. had some limited training in crane operation and fork lift operation while at Tunnel Systems, working in that area while under its supervision. I find K.M. does not have the educational requirements to become a skilled crane operator or fork lift operator. I find that K.M., as he testified, would not have finished high school and that he would have worked as a labourer and done physical work.

[347] Some of his jobs ended in the past for reasons unrelated to his mental health due to the cyclical nature of the construction labourer field, jobs ending, or strike issues. Other jobs he would get and then lose or quit, and he would not look for work because of his mood swings.

Due to his mental health injuries, which, I find, have continued to deteriorate over the years, K.M. is unable to commit to a regular type of job, be it in Belleville or anywhere else. He becomes depressed, overwhelmed and anxious and cannot interact with people, and he retreats to his apartment until he can cope again, described as a “funk” by his mother A.F. His depression affects his motivation and his mood and his ability to interact with others. He takes offence easily. He gets irritated, angered, frustrated and unable to cope. His depression “cloud” is always hanging over him and never leaves. His depression has made work difficult for him. He has not worked in a meaningful way since 2000, I find, because of his mental health injuries. K.M.’s mental disorders are phasic and he has good periods and bad periods. K.M. worked when he could. When he could work, he did well, but he could not make it last. I find his mental health has deteriorated over the years. One indication of that in the evidence is his declining GAF scores assessed by different doctors over many years. K.M.’s mental health continues to deteriorate. I reject the School Board’s position that K.M. is unmotivated to work and prefers to live on government support.

[348] Much has been made of K.M.’s application and receipt of O.D.S.P. and C.P.P. benefits and the statements made on those applications. On K.M.’s O.D.S.P. application made in 2001, I find that K.M. and his mother A.F. did overstate his situation, but said what, I find, they each believed to be true at that time. K.M. stated that he thought he had been mentally ill all his life, which was passed to him genetically, by his father. There is no medical evidence before me of K.M.’s father being mentally ill, other than speculation and K.M.’s belief. A.F. said the father’s mental illness that was referred to was D.M.’s Alzheimer’s. A.F. added the note about the “blue baby and umbilical cord issue”, and the statement that she always wondered if it had had any impact on K.M.’s life problems. I find K.M. was desperate for answers as to why he was the way he was, and so was his mother. K.M. was then 36 years old and had suffered from depression since he was 15. K.M. later testified that he was grasping at straws as to why he was sick. I find that this indicates that K.M. did not himself, at that time, have the realization of the nature his mental health issues. He had suffered years with his depression by then, in what I find was a tortured existence. At this point in time, K.M. had not disclosed the sexual abuse to anyone.

[349] I find that these comments were what K.M. and A.F. actually genuinely believed at the time, but without any medical basis. I find they were not deliberate lies, but rather, acts of desperation by an adult survivor of childhood sexual abuse and his very concerned and caring mother. By the time of K.M.’s C.P.P. application in 2010, K.M. had counsel and was being treated by Dr. Singh, who fully supported K.M.’s application for C.P.P. disability benefits. I find that both O.D.S.P. and C.P.P., based on the documents filed in this trial, did thorough investigations and rigorous independent assessments of K.M.’s applications before awarding him any benefits of any kind. K.M. was ultimately found on April 16, 2003 to be entitled to receipt of O.D.S.P. benefits as fully disabled. K.M. was later found to be suffering from a prolonged and severe disability and incapable of any reasonably gainful employment by C.P.P. on October 26, 2012, with benefits retroactive to September 2009.

[350] K.M., I find, suffered from substance abuse after the sexual abuse by Marson. While there is some discrepancy about K.M.’s age when his heavy use of drugs began, I find that the preponderance of evidence is that it started when K.M. was 13 years old and in grade 7, after the sexual abuse by Marson. K.M., as I have found earlier, is not a good historian as to exact dates

and ages. By age 16, K.M. had a serious drug habit, in his words, to alleviate his mood and to feel normal like everybody else. He was using marijuana daily and, according to Y.G., from three to five times a day. By 1982, at age 17, he had been referred by Dr. Manderville to an in-treatment drug program. He left to go and get high. He was still using marijuana at age 24 in 1989 around the time of his parents' divorce. His relationship with Y.G. was ending. These events, I find, became too much for K.M. to handle combined with K.M. dealing with his sexual abuse. He was referred to Dr. Brant, his first psychiatrist. K.M. over the years has used marijuana, cocaine, hashish, acid, speed, and everything he could get his hands on to feel normal. It didn't work. He stopped using street and hard drugs when his son was born in 1998 when K.M. was 32 years old. His drug dependency today is limited to his use of marijuana. Dr. Singh encouraged K.M., as did others, to stop using marijuana, but nothing else gave K.M. any relief. His marijuana is now medically prescribed as recommended by his family doctor. When K.M. has a particularly bad day, even the marijuana does not help, and he isolates himself from others until he can cope again. He does not abuse alcohol.

[351] By age 24, K.M. was suffering from severe psychological problems: chronic major depression, substance abuse, anxiety attacks, and personality disorders. The evidence in this case, which I accept as being credible from K.M., is that K.M.'s depression began when he was 15 years old while at Saint John's School, and it has never left him. While at Saint John's, K.M. testified he realized that there was something wrong with him—he felt “crushed”. Depression had set in, and his depression has since never left. K.M.'s depression was later formally diagnosed as dysthymia. In the years that followed, there would be times of occasional improvement, but K.M.'s symptoms of depression, dysfunction and substance abuse would consistently return.

[352] K.M.'s first psychiatric treatment began in 1989 at age 24 with Dr. Brant. K.M.'s parents were going through a divorce, which was hard on him. K.M. became even more depressed than usual and was referred to Dr. Brant, who took K.M.'s condition seriously. By then, I find, K.M. had been suffering with depression for nine years and was unable to handle added combined stressors—such as his parents' divorce, relationship breakups, and job losses—which caused him additional reactive depression. Dr. Brant described K.M. as being desperately unhappy. In 1990, Dr. Brant found that K.M. had a longstanding and chemically resistant depression. K.M. had a lot of respect for Dr. Brant and the care that he received from him until he passed away in 1994.

[353] Dr. Conn cared for K.M. from 1994 to 1996, and from 1997 to 2006 when he retired. Dr. Conn, I find, appeared to have a particular aversion to persons on disability pensions that started with K.M.'s father, D.M., who Dr. Conn treated for a period of time and about which there is little evidence. In May 2003, Dr. Conn thought K.M. had a somatoform disorder, but by the next month, in June 2003, he found K.M.'s hypochondriasis, or his somatoform disorder, had remitted. In fairness to Dr. Conn, he knew nothing about the sexual abuse of K.M. and, therefore, he had a materially incomplete medical and psychiatric history of K.M. I find Dr. Conn's treatment of K.M. did not appear to be a positive therapeutic relationship from K.M.'s point of view, soured from the beginning, likely by Dr. Conn's views about K.M.'s father. I accept K.M.'s evidence that in 2005, Dr. Conn never suggested to K.M. that part of his problem was the challenges that he had at home as a young child, or that there was any genetic link to the

problems of his father. I accept and find credible the evidence of K.M. and his mother A.F. that if there were problems in the home as referenced by Dr. Conn, they did not know what they were. In 1998, Dr. Conn did describe K.M. as being chronically depressed since he was a teenager, and diagnosed him as having chronic dysthymia, and a personality disorder. By 2000, K.M. was experiencing progressive depression, and by 2006, Dr. Conn referred to K.M. as having chronic depression. In spite of Dr. Conn's scepticism, his diagnoses of K.M. were ultimately essentially the same as those of Dr. Singh and Dr. Silver, as I discuss below.

[354] Dr. Schwarz did a one-time assessment of K.M. without access to K.M.'s previous records, when K.M. was in the Guelph area from 1996 to 1997. Dr. Schwarz found K.M. to have excessive anxiety and worry, that he was chaotic and easily distracted, and that K.M. was not easy to get a good history from. K.M. had good childhood memories. He had problems with employers, colleagues, and girlfriends, and he had a temper, which he had trouble controlling. K.M.'s diagnoses were consistent with those made in the past. K.M. denied a childhood history of any kind of abuse.

[355] After Dr. Conn retired at the end of June 2006, K.M. returned to the care of his family doctor, Dr. Goddard-Hill. Marson was arrested and news about his arrest was published on October 25, 2006. K.M. was referred to Dr. Singh for much needed follow-up psychiatric care. Combined with his existing psychiatric diagnoses from the sexual abuse, K.M. was feeling overwhelmed, with increased emotionality and constant suicidal ideations with this added stress related to Marson's upcoming criminal proceedings. Dr. Singh was K.M.'s first treating psychiatrist who was aware that K.M. had been sexually abused.

[356] Dr. Singh treated K.M. from 2007 until the end of June 2012. K.M., I find, was at times a difficult patient and directly challenged Dr. Singh when he refused to prescribe certain drugs that K.M. wanted. It is clear, however, that they were able to move past those issues, and K.M. grew to respect Dr. Singh's psychiatric care. Dr. Singh treated K.M. at 27 appointments, in good times and in bad—which was helpful in treating K.M., whose condition, Dr. Singh testified, was phasic. Dr. Singh used a combination approach to the treatment of K.M. Dr. Singh treated K.M. pharmacologically as others had done in the past, with the same lack of success. Dr. Singh was not in favour of K.M.'s use of marijuana. He treated K.M. with cognitive psychotherapy to manage the side effects of the drugs being prescribed, and to deal with K.M.'s life stressors. The notes of those appointments demonstrate the ups and downs of K.M.'s moods and his dysthymia. Other than his substance abuse, K.M.'s disorders did not improve, and have deteriorated over the years. I find that K.M. has lived, and continues to live, in this up and down mode. When he's feeling up, he can go out of his apartment and do things. When the downturn comes, as it always does, he retreats to his apartment, sometimes for days at a time, until he feels he can handle interaction with others.

[357] Dr. Matveyev provided follow-up care to K.M. after Dr. Singh, from 2013 until he last saw him on October 8, 2015—K.M.'s last treatment with a psychiatrist. K.M. is now managed by his family doctor. It was Dr. Matveyev who recommended to K.M.'s family doctor, Dr. Holmes, that K.M. use medically prescribed marijuana to help alleviate his symptoms, which he continues to use under supervision as required.

[358] K.M. was asked about follow-up treatment for his anti-social personality disorder through the Chrysalis program and private counselling for his depression. I accept and find credible K.M.'s evidence that if Dr. Singh recommended something, he would have done it. In subsequent meetings with Dr. Singh and Dr. Matveyev, they did not recommend any treatment that K.M. declined to take. I draw no negative inference in K.M. being of the view that his mental health diagnoses and injuries are at this point treatment resistant. I find that this is consistent with the evidence in this case, particularly the evidence of Dr. Singh. I find this is not a case of K.M. failing to mitigate. I find that from his first psychiatric intervention in 1989 to his last on October 8, 2015, and continuing to the present, K.M. has done all that was reasonably asked of him as he struggled to get through his life in his damaged state, day to day, week to week, to deal with the abuse by Marson and his mental health injuries that followed.

[359] Due to the nature of his mental health injuries, I find K.M. will continue to struggle and that his current method of coping is appropriate. There is no evidence that I accept as being credible that K.M. will truly benefit from further counselling or therapy.

[360] As previously outlined, K.M. has tried a plethora of psychiatric drugs prescribed by his treating psychiatrists and his family doctors to help alleviate his conditions. While some worked for a while, they all ultimately failed. Even marijuana currently does not always help. In addition to K.M.'s intolerable side effects from some of these drugs, the treating psychiatrists did not think that medication would completely address K.M.'s problems. The inference I draw from the evidence is that K.M.'s mental health diagnoses and injuries—his dysthymia, his post-traumatic stress disorder and his anti-social personality disorder—are at this point treatment resistant and have been for some time.

[361] K.M. has had to deal with many tumultuous events in his life after Marson such as his parents' divorce, relationship breakups, job losses, hospital admissions, suicidal ideations and suicide attempts, as earlier reviewed, in addition to his mental disorders from which he suffers. He first considered suicide in 1989 as an "out" for him and he still does. He is fed up with the way he has felt for so many years. He feels hopeless, lonely, and empty. His depression continues to fluctuate, making it impossible for K.M. to make promises to do anything, be it in his personal life or otherwise. He never knows how he's going to feel. He stays home a lot and avoids other people's problems. K.M., I find, will continue to live in his phasic, mentally ill state for the rest of his life, and cope as best he can. The fact that he has a son keeps K.M. focussed. I find it gives K.M. a reason to continue to try and get through the day to day. K.M. is alone and, I find, living a most difficult life since the sexual abuse by Marson.

12. Credibility of Lay Witnesses:

[362] I found K.M. and the other lay witnesses A.F., S.G., M.I.C., Y.G. and T.A. to be credible. They testified in a fair, straightforward and impartial manner as to their best recollections about the relevant historical and current facts in this matter. There were some discrepancies and inconsistencies in their respective recollections as to ages, dates, grades and times and other such details, which I find not to be unusual in the normal course of trying to remember when things occurred over so many years in the past.

[363] I found K.M. to be a credible witness. He was generally a reluctant witness and found testifying in court about these most personal matters very difficult. He was nervous and anxious. He was overly agreeable at times, even on some uncontentious issues, with defence counsel in his thorough cross-examination. K.M. testified in his damaged state as an adult survivor of childhood sexual abuse. I find K.M. continues to be affected by the sexual abuse by Marson and that this factor has to be taken into account in assessing his evidence. As testified to by his aunt M.I.C., K.M. did take offence easily while testifying. K.M. took particular offence when he felt his intellect was being challenged, and retorted to defence counsel that “I’m not a retard, I can speak English, I can learn”. I found K.M. to be honest, forthright and straightforward in his evidence. I accept his evidence and found his evidence to be credible.

[364] K.M. did, as I have found, overstate his own situation in his application to O.D.S.P. and to other health care providers as to his medical past about the blue baby issue and the cord around his neck based on what he believed to be true, as I have discussed. I do not find that this factor affects his overall credibility. K.M. acknowledged this frailty in his evidence.

[365] There were other discrepancies in K.M.’s evidence as to his age and the dates of certain events, either made by him or by those recording them in his various medically related interviews. I have considered all of these inconsistencies and find that they were minor inconsistencies and not deliberate material inconsistencies, and do not affect K.M.’s credibility as a whole. I find K.M. was a credible witness. I have no reason to reject his evidence, which I accept as being credible.

[366] I found K.M.’s mother A.F. to be a credible witness. She was a hard-working, loving, caring, and somewhat overly protective mother who wanted only the best for her son. She testified in a straightforward and candid manner. The fact that she thought K.M. failed grade 4, and not grade 6, is not a reason, I find, to disbelieve the entirety of her other evidence or to find that she was a poor observer of overall events.

[367] A.F. did, I find, overstate her son K.M.’s situation in her handwritten comment on his O.D.S.P. application about K.M. being a “blue baby” and this causing his problems, in an effort to help K.M. obtain financial assistance. I find that her comments were those of a concerned protective mother doing what she thought was right to help her son based on what she believed. A.F. was unaware at the time that K.M. had been sexually abused. Her comments were overstatements designed to add support to K.M.’s application. She acknowledged that frailty in cross-examination. The comment, I find, does not have the effect of making her other evidence non-credible. I find A.F. was a credible witness. I have no reason to reject her evidence, which I find to be credible.

[368] I found S.G. to be a credible witness. Her evidence of her close relationship with K.M. as his younger sister and protector before the abuse, and of K.M.’s rejection of her after the abuse, is tragic. Before Marson, K.M. was her best friend and her hero. She has lost her family as she sees it because of the changes in K.M. She has not been close to K.M. for some years as it is too hard on her. She’s afraid K.M. will commit suicide. I have no reason to reject her evidence and I find her evidence to be credible.

[369] I found M.I.C. to be a credible witness. Her observations of K.M. as an aunt from more of a distance outside of the immediate family were helpful and filled in some of the salient factual background facts. I have no reason to reject her evidence and I find her evidence to be credible.

[370] I found Y.G. to be a credible witness. She has not been in a relationship with K.M. since 2007, 10 years ago. She was forthright about her relationships with K.M. and why they ended. The detailed and personal nature of her evidence was given in the presence of K.M. in the courtroom, which added to its difficulty. I have no reason to reject her evidence and I find her evidence to be credible.

[371] I found T.A. to be a credible witness. As a neighbour and friend of K.M. since 2009, she was forthright about the things in K.M.'s life that she had observed and talked with him about. She was straightforward and candid in her evidence. I have no reason to reject her evidence and I find her evidence to be credible.

[372] I note my findings on the credibility of the four expert witnesses are made within the relevant sections of their evidence.

13. Expert Psychiatric Medical Evidence:

[373] The expert reports of psychiatrists Dr. Singh and Dr. Silver were filed as part of the Experts' Report Brief (Exhibit A, Tabs #1-2) for the assistance of the court and for the purposes of cross-examination. The expert evidence reviewed and considered is from their testimony given at trial.

a. Dr. A. Singh

[374] Dr. A. Singh gave expert psychiatric medical opinion evidence on behalf of K.M. as to the effects of childhood sexual abuse and in particular as to the effects of childhood sexual abuse on K.M. Dr. Singh is a qualified psychiatrist and has significant expertise on the effects of childhood sexual abuse. His extensive Curriculum Vitae as at 2012 (Exhibit #6) and his updated Curriculum Vitae (Exhibit #7) are before the court.

[375] Dr. Singh has testified in court on about 10 prior occasions. In this case, Dr. Singh testified as both a participant expert and a litigation expert, to which there was no objection taken. Dr. Singh may testify as both a treating physician (participant expert) and a Rule 53.03 expert witness (litigation expert) if he has complied with Rule 53.03 to the extent that he offers opinion evidence beyond the scope of his participation in the events: *Westerhof v. Gee Estate*, 2015 ONCA 206, at paras. 60-63. Dr. Singh complied with the requirements of Rule 53.03 and signed an acknowledgement of expert's duty, Form 53, on April 26, 2016.

[376] Dr. Singh became qualified as a psychiatrist in 1971. Dr. Singh trained in England and is a Fellow of the Royal College of Psychiatry, London, England and a Fellow of the Royal College of Physicians and Surgeons, Canada. He has a Diploma in Psychological Medicine. He was the former Program Director of the Hamilton Psychiatric Hospital at McMaster University for over 24 years. He came to Queen's University in 1999. He was Chief of Psychiatry and

Director of the Academic Psychiatric Unit at the Belleville General Hospital (now known as the Quinte Health Care Belleville General Hospital) from 2006 to 2012, which was a joint project with Queen's University. In addition, he was a Professor of Psychiatry and Pharmacology, Toxicology and Neurosciences, Faculty of Medicine, at Queen's University in Kingston.

[377] In July 2012, Dr. Singh left Canada to become an ACPM World Health Organization (W.H.O.) international Professor of Psychosomatic Medicine and Psychopharmacology, and a lecturer, researcher, and research advisor. He was invited to set up a medical school in Mongolia and China and has worked in many other countries. He has held various research and teaching positions, has published articles in international journals, and has been a professor in medical schools around the world. He was a Clinical Professor of Psychiatry and Psychopharmacology at the Institute of Psychiatry, Postgraduate Medical Education and Research, UNDCP Nodal Centre for Drug Abuse Prevention. During this time, Dr. Singh remained a Professor of Psychiatry and Pharmacology, Toxicology and Neurosciences at Queen's University, Faculty of Medicine, and continued to do occasional work for Queen's University.

[378] Dr. Singh has years of experience in clinical psychiatry, teaching and research. He has encountered over 100 childhood sexual abuse cases in his career. He has published in the areas of presentation and management of childhood sexual abuse and depression and has written other articles involving adult victims of childhood sexual abuse. Dr. Singh did research in 1990 in the United Kingdom in the area of diagnosis and treatment of individuals who have suffered sexual abuse in their childhood. His work has been referred to over the years up to two thousand times, as recently as 2012. His most recent presentation was in 2016. At the end of 2017, after a distinguished full time career, Dr. Singh—born in 1935 and aged 81—planned to reduce his workload and return to Canada to do research work at Queen's University part-time.

[379] Dr. Singh was qualified as an expert to give opinion evidence to the court in the field of psychiatry including, but not necessarily limited to, the diagnosis, treatment, prognosis and impact of psychiatric disorders arising from childhood sexual abuse.

[380] Dr. Singh testified about epigenesis. He received a significant research grant ten years ago to do research into the genesis of mental illness. "Epigenesis", in its simplest terms, is the genetic change taking place after birth caused by external factors, such as post-traumatic stress disorder, sexual disorder in children or any severe trauma. The impact of this research for depression, dysthymia, and post-traumatic stress disorder arising from childhood sexual abuse is in its treatment in the future with gene therapy, which is on the horizon but in the early stages of development. Dr. Singh testified that based upon his research and the research of others in this field, trauma such as childhood sexual abuse causes actual genetic changes, which may explain why some treatment responses are irreversible and others are manageable.

[381] While providing hope of gene therapy treatment in the future for those afflicted with psychiatric disorders—such as what we presently see in the treatment of cancer, as Dr. Singh testified—I find it is not necessary for me to rely upon the opinion evidence of Dr. Singh about epigenesis, agreed by him to be a "novel scientific theory", in coming to my factual findings, determinations or assessments in this case. I accept that epigenesis is in the early stages of informing an understanding of mental illness, is in rapid development, and is accepted in most of

the medical community. Dr. Silver did not challenge Dr. Singh's expertise in this field. In the end result, after much testimony, I find that epigenetic evidence is not central to Dr. Singh's assessment, opinion and diagnosis of K.M., as accepted by Dr. Silver. I find that epigenesis was first reported on by Dr. Singh in his March 2011 report in response to Dr. Silver's 2010 report on which he was asked to comment, and that it was offered more by way of explanation as to why K.M.'s disorders had become so resistant to treatment. The evidence of epigenesis does not assist me in coming to my findings in this case.

[382] I do accept and rely upon the expert opinion evidence of Dr. Singh as a highly experienced clinical psychiatrist as to the diagnosis, treatment, prognosis and impact of psychiatric disorders arising from childhood sexual abuse. This evidence by its very nature includes considerations of environmental, physical, emotional, social and genetic factors affecting human life.

[383] On May 11, 2009, plaintiff's counsel wrote Dr. Singh asking for Dr. Singh's expert opinion as to the causal connection, if any, between K.M.'s current situation and the childhood sexual abuse. Dr. Singh testified about his opinions given in his June 15, 2009 expert report (Exhibit A, Tab #1A). Dr. Singh reviewed K.M.'s history of past illness and his medical records back to 1987 from the Kingston Psychiatric Hospital (now Providence Care) and the Belleville General Hospital, and his interactions with K.M. at their appointments. It was Dr. Singh's opinion that K.M.'s multiple disorders started due to childhood sexual abuse. K.M.'s previous treating psychiatrists were not told of the sexual abuse and they diagnosed chronic depression, personality disorder of the anti-social type. They did not have the full psychopathy.

2009 Diagnoses:

[384] Dr. Singh was asked: what is wrong with K.M., and what is K.M.'s current complex of disorders? Dr. Singh testified that there were five diagnostic components to his answer:

[385] 1. Major depression with dysthymia: This diagnosis involves K.M.'s major depression, chronic depression, called dysthymia, which is not responding to treatment. The diagnosis was based on K.M.'s mood evaluation, its fluctuating nature, his not responding to treatment, and his not being cured of his depression. The depression developed in K.M.'s teenage period. He was seen by Dr. Brant, Dr. Conn, Dr. Ash and others, but his original depression was all the time present in him.

[386] Dr. Singh testified that the diagnosis was made with the knowledge that the depression started after the sexual abuse, as did K.M.'s substance abuse. K.M.'s post-traumatic stress disorder was developing. K.M. had residual depression that needed to be resolved. Personality deficit is the real damage seen in the patient who suffers from childhood sexual abuse, and they are very difficult to treat and responses to treatment are unsatisfactory. Dr. Singh agreed that the chronic nature of the condition is influenced by the fact that it has been caused by childhood sexual abuse.

[387] 2. Substance abuse: Substance abuse, which becomes a vehicle in the patient to get short-term pleasure even at the cost of long-term pain, was seen in K.M. Dr. Singh testified that

most sexual abuse patients have been found to get involved in substance abuse. Childhood sexual abuse brings lack of faith in relationships and creates lack of trust in relationships, and its victims have difficulty in building relationships in the future. Substance abuse exists in 80% of the people who are sexually abused. Dr. Singh agreed that sexual abuse is major cause of pathology or psychopathy in K.M.

[388] Dr. Singh agreed that K.M.'s abuse of drugs is a disorder, rather than just a choice K.M. made. Because he was going through depression and trauma, he could not cope. To get pleasure, he started taking them, then it becomes a habit, and it becomes a disorder. Trying drugs is not a disorder. Taking drugs regularly, becoming dependent or addicted, becomes a disorder. Sexual abuse is an important cause—not the only cause, but an important cause—of substance abuse.

[389] 3. Personality disorder of anti-social and borderline type: This diagnosis means that the personality of a person like K.M. is not fully developed. It is fixated in a place where the deficit personality comes out in many different behaviour disorders—substance abuse, depression. All those things are part of personality disorder. Dr. Singh testified that in the anti-social or borderline type of personality disorder, the patient's behaviour is above normal people's behaviour. He becomes more of a problem in acting out. He takes drugs knowing it is wrong, to keep himself "up". He does not know if what he is doing is right or wrong, he does not learn from his past and he does things without knowing the consequence—these are the three characteristics of personality deficit. For example, K.M. takes drugs knowing that is a problem, but he still takes the drugs because he enjoys them. K.M. does not care for anybody's opinion. He does not learn from other people's advice. That is why the treatment of personality disorder becomes very difficult. If there is improvement, it is only temporary. The disorder becomes chronic because he does not learn and it is hard to treat, and it has some genetic element; even if improvement happens, it is a temporary improvement, or transitory improvement. One time he can be better, but the next time you see him he can be in full blaze. Sometimes they become a charmer and sometimes they are enemies of everybody. The diagnosis is very easy to make when the patient is showing bizarre or wrong behaviour. But when he's a charmer, many psychiatrists miss that fact unless they continue to see him, long term.

[390] 4. Difficulty in building interpersonal relationships due to past sexual abuse: This diagnosis means that when sexual abuse takes place, a person like K.M. feels guilty and blames the other person, but that also takes away the elements of his trust in other people. He feels that other people will take advantage of him too. His interpersonal relationships after sexual abuse are so deteriorated that he starts to become asocial. He was not aware of what he should have done to stop the sexual abuse, and he is full of guilt. Due to the past sexual abuse, K.M.'s interpersonal relationships are lost. K.M. will have difficulty in keeping relationships of any type and difficulty in keeping a job. He loses his self-confidence and has a poor self-image. He considers himself an inferior person, and not able to do anything right.

[391] When a relationship is lost or when somebody leaves him, the person becomes angry. He avoids future relationships and becomes lonely and depressed. He is not capable of keeping a job. He really is a very unhappy person and depression is the most presenting symptom.

[392] 5. Difficulty in keeping employment: Dr. Singh testified that all five components of the diagnosis are interconnected. Depression, substance abuse, loneliness, and poor social relationships become a vicious circle and are inter-connected. As to maintaining employment, afflicted persons see themselves as inferior to others. Their self-image is poor and their confidence is lost. They have difficulty in keeping a long term situation going because of their impulsive nature, which is created by their frustration that comes easily to them. They consequently always have financial problems because they cannot keep a job.

[393] Pharmacological treatment: Dr. Singh testified that some improvement in K.M.'s mood had been achieved by drug therapy. To keep the mood, the drug will have no effect, particularly if you are suffering from various psychopathies. Drugs are not the only answer because there is reversible change and there is irreversible change. Some part of depression and substance abuse is reversible, but some is ingrained and irreversible due to genetic change.

[394] Causation: Dr. Singh was asked his opinion as to whether or not there is a causal connection between the childhood sexual abuse and K.M.'s current disorders. He answered that it is a major cause, but that it is not the sole cause. It is multi-factorial in that no one can say that mental illness is due to one cause, but there is a hierarchical order of importance of symptoms. The symptom of sexual abuse is a major factor. It is not the only factor. What leads Dr. Singh to the conclusion that the childhood abuse is a major cause of K.M.'s current disorders is K.M.'s symptoms, history, psychopathy, and response to treatment. In K.M.'s case, the response to treatment has not been very satisfactory. Drugs have "no unlimited" therapeutic value. Other causes are his inability to keep a job, his financial stress, his impulsive nature, his mistrust, his lack of relationships, his loneliness. When K.M. has difficulty, he gets depressed more, he gets flashbacks of what happened to him more, and he has to recall the events again for court. Normal people do not get sick like this from life stressors, such as losing a job or a relationship break-up.

[395] Dr. Singh further noted that it is important that K.M.'s history of sexual abuse happened at a very tender age where an individual has all the constructive elements of his/her personality growing. Where the personality growing element is lost because of the trauma, you get personality disorder. You feel inferior, angry, and frustrated, causing depression. This causes substance abuse later on. So the psychopathy goes up and certain psychopathy can be treated. Dr. Singh testified mood can be treated to stabilize, and substance abuse can be treated, but it is phasic or "time-being" success. It is improvement, but it cannot be cured.

[396] Dr. Singh testified that the commonly recognized consequences of sexual abuse are post-traumatic stress disorder, depression, substance abuse, and personality deficit, and there can be borderline or anti-social personality. The person's personality development is completely changed and not growing normally.

[397] Prognosis: Dr. Singh testified that in 2009 his prognosis for K.M. was guarded. While K.M. had shown improvement in mood and depression, it would come back through stress or other factors, and a long-term follow-up plan and drug treatment were required.

[398] Dr. Singh testified about the importance of the treating psychiatrist knowing that K.M. had been sexually assaulted in childhood. Sexual abuse is a major factor in life, of trauma. A

treating psychiatrist not knowing this would think that K.M. was not telling the truth or was a hypochondriac, such as was reported by Dr. Conn on May 7, 2003 (Exhibit #1, Tab #A56). Dr. Conn was sceptical of K.M. and diagnosed him with a somatoform disorder, which is part of a psychosomatic disorder now. Dr. Conn was not aware of K.M.'s true history.

[399] 2010 additional diagnosis of severe post-traumatic stress disorder: Dr. Singh prepared a further expert report dated January 27, 2010. This was in response to a request from K.M.'s counsel for his opinion, for the purposes of applying for C.P.P. disability payments, of whether K.M.'s condition was severe or prolonged. Dr. Singh testified that he made the additional diagnosis at that point in time that K.M. was suffering from post-traumatic stress disorder. Prior to this point in time, Dr. Singh had observed minor symptoms of this disorder, but not sufficient to form a formal diagnosis due to lack of the cardinal symptom: flashbacks. By this point in time, K.M. was having flashbacks and the nature of his illness became more severe.

[400] Dr. Singh testified that post-traumatic stress disorder means severe trauma which happens of any nature, whether it is sexual abuse, war, experiences of any difficult nature, or a car accident if it is very violent or severe. Stress becomes ingrained as first, a symptom, and then it does not respond as quickly. His experience is that it responds 80% after long-term treatment. The addition of this diagnosis to K.M.'s five other diagnostic components means that his condition is becoming irreversible and that it is not going to be completely resolved, or even stabilized, by treatment, including drug treatment.

[401] As to its severity in K.M., Dr. Singh testified that the suffering from flashbacks is severe. It will all of a sudden wake you up if you are sleeping. It can make you agitated or uncomfortable and it is difficult to dissolve. His prognosis was very guarded.

[402] 2012 Report: Dr. Singh left the Belleville and Kingston area and semi-retired in 2012 and worked overseas. Prior to his departure and on the same day that he last treated K.M., Dr. Singh prepared a further expert report dated June 25, 2012 in response to a request from K.M.'s counsel for his opinion and a status report of K.M. at that time—was K.M. improving, the same or getting worse, and what was his prognosis? Dr. Singh testified that K.M. was deteriorating and was not as good as before. His reactive depression could either respond or deteriorate. Treatment had reached its limitation. There was no hope of benefit from drug treatment on the irreversible part of his depression.

[403] Dr. Singh testified that as to chronicity, K.M. had suffered 23 years of depression (as of 2012). While his reactive stress could get better, long-term depression was not going to respond completely to drug treatment and K.M. had become dysthymic. By this stage, K.M. had become compliant with drug treatment and his co-operation with that treatment was much better than a few years ago. His treatment response, however, had not been much better.

2015 Diagnoses:

[404] Dr. Singh returned to Canada for the summer of 2015 and prepared a further expert report dated August 14, 2015 in response to K.M.'s counsel's request for his opinion and a status

report of K.M. Dr. Singh saw K.M. on August 4, 2015 for the purposes of this report, which covered the period since June 25, 2012 when Dr. Singh had last seen K.M.

[405] Dr. Singh testified that he took a full history again from K.M. Prior to any testing, Dr. Singh found that K.M.'s reactive depression was chronic and fluctuating in the mild to moderate range. He had dysthymia or chronic depression, which was unresponsive to treatment. He had anxiety and panic attacks in the moderate range. There was no suicidal ideation. His social interaction was poor and he was doing better in 2012 when Dr. Singh had last seen him. Apart from his mother, K.M. had no relationships. He remained single and alone. He had poor response to treatment. K.M. described mild symptoms of confusion and impaired memory, which was not present before and reduced his work capacity. He had financial stress, mood fluctuations, anxiety occurrences, and personality deficits of introversion and loneliness. He had more flashbacks of the sexual abuse because of the legal situation with this lawsuit, which affected his mood, anxiety and sleep. His sleep pattern was erratic and occasionally restless. He felt anger and inadequacy in himself because of the sexual abuse in his teenage years. The length of the civil legal proceedings had made him frustrated, confused, angry, depressed and anxious, and this reactive factor was prolonging his misery.

[406] As he had not kept in touch with K.M. since 2012, Dr. Singh did an extensive evaluation of K.M. using seven rating scales:

[407] 1. Hamilton Depression Rating Scale: This is the most famous rating scale for major depression, by Professor Hamilton in England. K.M.'s depression was moderate, fluctuating to mild, with erratic sleep. Anxiety was mild. K.M. was suspicious and paranoid, which was a new symptom that Dr. Singh attributed to the chronicity of the legal proceedings.

[408] 2. Montgomery-Asberg Depression Rating Scale: This is another famous scale for measuring clinical depression. K.M.'s depression was confirmed. Dr. Singh found gloominess of moderate and fluctuating type depending on external circumstances. K.M. had difficulty concentrating and sustaining thoughts, with reduced ability to read. Difficulty in starting activities was of the moderate to severe type. No suicidal ideation. Depression was of moderate type and not responding to treatment.

[409] 3. Mood Disorder Questionnaire: This scale measures mood disorders. K.M. was not bipolar. Depression was moderate, of the uni-polar type.

[410] 4. Hamilton Anxiety Rating Scale: This scale measures anxiety. K.M. had fluctuating mild anxiety.

[411] 5. Maudsley Personality Inventory: This is a world-wide famous scale to measure personality. The scale confirmed the presence of borderline personality disorder in K.M. with introversion.

[412] 6. Self-Analysis Form: This measures how K.M. sees himself in his illness. Dr. Singh found the following: presence of inadequacy, frustration, mild suspiciousness and paranoia. K.M. had moderate difficulty in coping with challenging situations. He was easily upset, and mild anger came quickly particularly if anybody was critical of him. Frequent occurrence of

restlessness and feeling people were not interested in him. Annoyance level and tension development were increased. He became absent-minded and forgot details of anything he was doing. He was rattled and over-excited in upsetting situations. Vivid dreams, due to flashbacks, had increased. Lack of energy and stamina when confronted with difficulties. He felt most people were mentally odd. He often felt loneliness and worthlessness. Flashbacks made him wake in the night, and he worried and had difficulty in sleeping again. His spirits were low. He had feelings of guilt and remorse over small matters. His nerves were on edge and any kind of sounds made him frequently nervous. He trembled and perspired, a sign of flashbacks, when he experienced tension or turmoil due to recent concerns and stresses.

[413] 7. Cognitive Abilities Form: This test measures memory and other cognitive functions. This test confirmed mild impairment of memory, affecting remembering of information and application to tasks. He had impaired computing abilities, difficulty tolerating stimuli, and reduced abilities tolerating emotional situations. Confusion caused stress and difficulties in performance. He had inability to tolerate confrontational situations in which assistance may or may not be available. His memory was impaired and he had difficulty remembering things from before 2012. He had poor ability to remember information and apply it to work tasks, and this had been reduced. Confusion in him caused increased stress and difficulties in performance.

[414] Dr. Singh testified that as of 2015, K.M.'s complex of disorders was as follows:

[415] Axis I – Major depression with dysthymia and post-traumatic stress disorder.

[416] Axis II – Borderline personality disorder with introversion and unspecified learning disorders, with interpersonal failure for keeping relationships and employment. This was due to K.M.'s memory lapse, not his old learning disorder, and was affected by post-traumatic stress disorder, which is why it is called "unspecified". It is the product of cognitive abilities lost.

[417] Axis III – Leg clot. This was improving with drug treatment.

[418] Axis IV - Financial stress, social stress, and stress due to failure in establishing relationships, thereby causing his loneliness, and the impact of his teenage sexual abuse.

[419] Axis V – GAF 30. This is a measure of his working capacity as a human being. A normal working range is about 45 or 50 to 90. Good working people are up to 90. Below 30, people are not able to work.

[420] Dr. Singh testified that compared to his previous diagnosis of K.M.'s complex of disorders, K.M. was deteriorating. Dr. Singh was really concerned with K.M.'s memory problem. In particular, Dr. Singh was concerned about K.M.'s cognitive abilities decreasing in the areas of learning anything new or keeping a job. K.M.'s previous complex of disorders of depression, personality disorder, interpersonal relationships, and difficulty in employment was a little bit worse.

[421] 2015 Updated Prognosis: Dr. Singh testified that his 2015 prognosis for K.M. was guarded and poor, which he testified was worse than before. "Poor" was added because of K.M.'s cognitive abilities deterioration. Dr. Singh confirmed in his evidence that the complex or

disorders displayed in K.M. are very similar to those commonly seen in childhood sexual abuse cases.

Other Evidence of Dr. Singh

[422] Dr. Singh was asked to comment in his testimony about Dr. Silver's report of December 8, 2010. Dr. Silver had agreed with Dr. Singh's diagnosis of major depression with dysthymia, personality disorder of anti-social and borderline type, difficulty in building interpersonal relationships due to post-traumatic experiences, and difficulty in being employed. Dr. Singh testified that Dr. Silver attempted to minimize the diagnosis of dysthymia in his report and, according to Dr. Singh, dysthymia is a chronic condition that does not improve with treatment.

[423] Dr. Singh testified about Dr. Silver's opinion that K.M.'s multiple diagnoses over the years were genetically and environmentally determined, quite apart from the alleged abuse by Marson. Dr. Singh testified that Dr. Silver cannot have it both ways. Dr. Silver failed to consider the genetic input of the symptoms and the disease and the genetic effect the abuse would have, besides the environmental, physical and emotional impact.

[424] Dr. Singh testified that the importance of symptoms is hierarchical as to what is causing more damage to the person or is the most dominant in his illness. From sexual abuse, there is post-traumatic stress and dysthymia. K.M. has had dysthymia for years and nobody has treated and cured him.

[425] Dr. Singh testified that the childhood sexual abuse did not have any impact on K.M.'s existing childhood learning problem. Many childhood learning disorders resolve in adulthood. Depression is common with a history of a learning disorder. As to the "blue baby" issue, 80% of blue babies have a short term learning disability. If K.M. was a "blue baby", he would have lacked oxygen but as the person grows up, the effect of that is gone in the brain and the illness (disability) goes away. Dr. Singh did not find any evidence of an adulthood learning disorder in K.M.; rather, his problems with numbers and the like revealed themselves on the cognitive abilities test that Dr. Singh administered, which showed that K.M. had computing difficulties. Dr. Singh testified that any depression from a learning disorder would play a very minor part. Dr. Singh testified that K.M. has a depression of over twenty years, and it has become dysthymia; nobody gets dysthymia from a learning disorder.

[426] Dr. Singh testified about Professor Charette, the School Board's economic expert. Dr. Singh disagreed with Professor Charette's opinion in three major aspects. First, Dr. Singh disagreed that the working capacity of a person in a damaged psychological condition can be used as a valid predictor of what his capacity would be undamaged. Sexual abuse has a major impact on the performance of education and employment. Dr. Singh's opinion was that K.M. would have achieved a much better education and employment if he had not been sexually abused.

[427] Second, Dr. Singh disagreed with Professor Charette's opinion that the educational and employment performance of K.M.'s parents and siblings were of no use in predicting what K.M. would have done if he had not been assaulted.

[428] Third, Dr. Singh disagreed with Professor Charette's opinion that individuals actually attain higher levels of education and employment after being sexually assaulted. The reference Professor Charette relied upon was from two psychologists whose study was done only on women. Dr. Singh testified that men deteriorate more or do worse in education than women after abuse. Women internalize behaviour after abuse, while men externalize with more behavioural problems. There is gender differentiation, between the profile of men and women who have been abused, in education and in performance in life.

[429] Dr. Singh was specifically asked about K.M.'s drug use in the year before he was assaulted by Marson and K.M.'s experimentation or messing with marijuana and "bennies" (benzadryl pills) around age 13. This information about K.M.'s drug use did not change Dr. Singh's opinion about K.M.'s condition. Dr. Singh testified that many people have recovered from experimentation. Experimentation has no impact on anything.

[430] Dr. Singh confirmed in his evidence that he never discussed private treatments with K.M. and that in his opinion these extra-cost treatments would be of no benefit to K.M. in the long term, as K.M. had dysthymia and epigenetic difficulties. K.M. will either have to wait for gene therapy or some new drug of the future. Right now, we have no pharmacotherapy, no psychotherapy, to change those illnesses—dysthymia, post-traumatic stress disorder, and the impact of sexual abuse.

[431] Dr. Singh was asked his opinion about the July 13, 1977 consultation note that was sent back to Dr. Manderville from a clinic (Exhibit #1, Tab #A5), where "childhood behaviour disorder" is written in the diagnosis section of the referral form. Dr. Singh testified that is an intake form written by non-physician people. It is a referral for psychological tests, so whatever is written here is not a diagnosis.

[432] Dr. Singh was vigorously cross-examined in all aspects of his evidence. I find that in cross-examination, Dr. Singh did not derogate from his substantive expert opinion on causation in this case: the sexual abuse by Marson was a major cause of K.M.'s mental health injuries, but not the only cause.

[433] Dr. Singh was cross-examined in detail about the July 13, 1977 consultation note mentioning a "childhood behaviour disorder". He testified that in his opinion, this form was not written by a doctor, as no doctor would write psychological testing as a treatment. The form was meant to go to a psychologist. Dr. Singh did not refer to it in his reports. He testified that as Chief of Staff, he changed the forms and processes at the Belleville General Hospital when he started there, to make them more accurate. He testified that he did not make inquiries as to what psychological testing was done of K.M. at age 11 because at that time the hospital did not have psychologists—Dr. Singh's clinic hired them when the clinic came to Belleville.

[434] He testified that he was not aware of any abnormal behaviour that K.M. had been demonstrating at age 11. It was referred to as a symptom in documentation that Dr. Singh reviewed but nobody described it except Dr. Silver in his written report.

[435] Dr. Singh was not aware of the exact details of the sexual abuse by Marson on K.M. and testified that as K.M.'s treating psychiatrist—and as K.M. was getting sexual abuse treatment elsewhere (at the Sexual Assault Crisis Centre)—it would be harmful to K.M. for Dr. Singh to inquire as to the details of the abuse, as it was painful for K.M. to describe in full detail. Dr. Singh confirmed in cross-examination that he had some feedback from the sexual assault centre that was treating K.M. for the sexual abuse. Dr. Singh confirmed in re-examination that he understood that K.M., in 2008, had embarked in a law suit against the teacher he alleged had sexually assaulted him while he was in grades six, seven and eight, from 1976 onwards. It is clear that Dr. Singh had also reviewed K.M.'s past history and treatment contained in K.M.'s clinical notes and records. I cannot fault Dr. Singh for taking this approach as K.M.'s treating psychiatrist.

[436] In Dr. Singh's opinion, sexual abuse is sexual abuse. On a hypothetical basis, Dr. Singh was asked if there was a difference between the extremes of fondling over clothing versus violent, forced anal rape. Dr. Singh testified that sexual abuse is a wholesome process; whether "small wholesome" or "big wholesome", the impact is roughly the same. In psychiatric terms, there is no "extreme" in sexual abuse. Sexual abuse is sexual abuse. A horse is a horse. There is no extreme horse. It is not described by quantity. The treatment is the same. In psychiatric terms, sexual abuse is sexual abuse. The diagnosis of sexual abuse remains the same whether Dr. Silver had more details of the abuse than Dr. Singh; it was sexual abuse.

[437] Dr. Singh testified about his June 15, 2009 report where he wrote that K.M. had the cord wrapped around his neck at birth. The School Board's counsel put to him the possibility of some sort of oxygen deprivation at birth (to which Dr. Singh said, "Yes - blue baby") and that oxygen deprivation to the brain at birth might account for some aspect of K.M.'s mental illness, to which Dr. Singh replied, "Fair". Again, there is no actual reliable medical evidence of K.M. ever being a blue baby.

[438] Dr. Singh testified about his note in the June 15, 2009 report that K.M.'s childhood was not a pleasant one. This was his determination after hearing that K.M. was tied to a tree so he would not take off, that his mother was over-bearing and over-protective, that his father was aloof, that K.M. had a learning disorder, and that academically he was a poor student, achieving between grade 6 or 8. Dr. Singh testified that this was not a diagnosis; many people have a happy or unhappy childhood, but still they have no mental illness.

[439] Dr. Singh agreed that K.M.'s history of the cord around his neck, his unpleasant childhood, his learning disorder, and his being academically poor at school are relevant to an assessment of K.M.'s mental health. Dr. Singh testified that one third of people who have a cord around their neck have a learning disorder of a temporary nature. When Dr. Singh diagnosed K.M., they did not find any evidence of a learning disorder in him, which had been present in childhood, and therefore they did not test him for that as it would have been inappropriate. Dr. Singh was aware that K.M. had spatial recognition difficulties. This symptom is present in many disorders, including personality deficit disorder (which is not the same thing as borderline personality disorder), and is not static. Dr. Singh thought it was part of K.M.'s personality deficit disorder but not part of a learning disorder, as he did not find any symptom of a learning disorder

in K.M. when he saw him. Again, there is no actual reliable medical evidence of K.M. ever having any birth trauma from having his umbilical cord around his neck, if in fact it was.

[440] Dr. Singh confirmed that K.M. is very close to his son and has a deep commitment. His relationship with his son is strong and a positive aspect for K.M.

[441] Dr. Singh confirmed that he did not approve of K.M.'s use of marijuana, which he described as chronic, and that he wanted K.M. out of the drug scene, as use of marijuana long term can develop into a psychosis. K.M. had stopped and started, and stopped and started using marijuana. Dr. Singh had prescribed Olanzapine, which was an anti-psychotic drug, to help with his mood and depression, sleep, and drug abuse, since K.M. had refused to try any more anti-depressant medications as they had not helped him in the past.

[442] Dr. Singh was asked about K.M.'s drug use in grade 6, before Marson. Dr. Singh testified that he was not aware of K.M.'s drug history before his encounter with Marson, other than when asked by counsel in chief about what Dr. Singh described as K.M.'s experimentation with drugs pre-Marson. Dr. Singh agreed that if someone uses marijuana routinely from age 12 to age 51, it becomes a disorder, an illness. Dr. Singh testified that when drug use starts, as in any symptom, it progressively becomes a disorder. You start taking drugs, you become addicted and dependent, and then you have a disorder. The disorder is in the habitual taking of the drug and a sign of dependence to make a DSM-V diagnosis.

[443] Dr. Singh was asked if he was aware of a period when K.M. had stopped using marijuana as an adult. If K.M. were again using marijuana, Dr. Singh agreed that K.M. would have a substance disorder, and that someone with a substance disorder is likely to have difficulty in obtaining and maintaining employment. Dr. Singh had no direct information on K.M.'s use of "bennies" but had read about it in the report of the other psychiatrist.

[444] Dr. Singh agreed that he had not discussed the Chrysalis program in Kingston with K.M. The Chrysalis program, funded by O.H.I.P., was started by Dr. Singh and involved cognitive psychotherapy, which Dr. Singh was already doing with K.M. The program was not available in Belleville and there would be transportation costs to get to Kingston. The difference is that Dr. Singh was seeing K.M. every 4-6 weeks whereas patients are seen every week in the Chrysalis program. The program was limited to the treatment of personality disorders. Dr. Singh was treating K.M. for multiple disorders, with depression being his primary concern. The cost of the program was high and the program was terminated. The only place now with concentrated treatment available is Homewood, a private hospital.

[445] Dr. Singh was specifically questioned about K.M.'s hospital visit and suicide attempt on January 28, 2000. He agreed that it is one of the common responses to a break-up of a relationship and the reactive depression to a break-up, whether the patient has suffered from sexual abuse or not. Dr. Singh agreed that he could not say "yes" or "no" as to whether this hospital visit and medical treatment for K.M. was required because of the sexual abuse committed by Marson. Dr. Singh testified that for all the times K.M. presented at the emergency department after losing a job or a relationship, he cannot say whether it was the sexual abuse, a reactive depression, or some other circumstance or a combination of both that

caused K.M. to require treatment. As to depression, Dr. Singh again said that K.M.'s treatment may be related to a combination of both the sexual abuse and some other circumstance.

[446] Dr. Singh confirmed that his initial expert reports did not refer to epigenesis. In fairness, Dr. Singh does refer to the epigenetic process in his March 28, 2011 expert report, about which he was thoroughly cross-examined. This reference in the 2011 report had come about as a response to Dr. Silver's 2010 expert report stating that K.M.'s multiple diagnoses are genetically and environmentally determined. Dr. Singh testified in cross-examination that he first published on epigenesis in 2006 and that by 2010 more proof of the theory had been obtained. He agreed it was, as of now, a novel scientific theory.

[447] Dr. Singh confirmed his opinion that K.M.'s psychiatric issues are not situational but a product of genetic, environmental, and epigenetic process. Dr. Singh described the word "situational" as meaning environmental, related to an external factor. The genetic factor is what a person is born with (DNA), and many disorders like bi-polar disorder and schizophrenia have a DNA component. Depression is DNA orientated but it is recessive, not dominant, which means it can miss a generation. It is the RNA part that changes with any external factors, through epigenesis. Dr. Singh agreed that you can have depression that is genetically based, as well as epigenetically based.

[448] Dr. Singh was asked about Dr. Conn's diagnoses of depression and borderline personality disorder, which were similar to Dr. Singh's diagnoses except that Dr. Conn did not know that K.M. was sexually abused. Dr. Singh agreed that Dr. Conn's diagnoses would say that K.M.'s depression had a genetic and environmental basis. Dr. Singh testified that Dr. Conn had called it a somatoform disorder, which is now called a psychosomatic disorder. Dr. Conn saw depression and dysthymia. The depression is genetic, but the dysthymia or chronic depression is not. K.M. had a different kind of depression. Dysthymia means chronic depression, which is not good at responding to treatment.

[449] Dr. Singh was then asked to confirm that you do not need the additional factor of sexual abuse to come to the diagnoses of depression and borderline personality disorder. Dr. Singh testified that there are two kinds of depression: there is indigenous or genetic depression, and there is reactive depression, "due to the girlfriend problem". The reactive depression led to the genetic depression. Dr. Conn diagnosed the reactive depression, which does not require any history of a sexual disorder.

[450] Dr. Singh was cross-examined about his November 21, 2011 expert report, specifically in relation to his extract from Dr. Conn's consultation note of January 11, 2005 where Dr. Conn stated that K.M. "certainly was reared in a home environment which had it's [sic] problems and this, of course, has obviously affected his development emotionally". Dr. Singh agreed that what Dr. Conn was saying was that K.M.'s home life had a negative effect on his emotional development. Dr. Singh agreed that he had also said, in his own report, that K.M.'s personality has been negatively affected by his home environmental factors. Dr. Singh agreed that patients can have these personality disorders quite apart from suffering from any sexual abuse, which is what Dr. Silver has said. The difference is that Dr. Singh and Dr. Silver knew that K.M. had been sexually abused.

[451] Dr. Singh testified that he and Dr. Silver could both realize the severe impact of the sexual abuse, while Dr. Conn could not since he was unaware of the sexual abuse. Dr. Conn diagnosed K.M. with a psychosomatic disorder. Dr. Singh and Dr. Silver knew more of the history and could better assess the therapeutic process than Dr. Conn. Dr. Singh was questioned as to how it was that Dr. Conn and Dr. Silver came to the same diagnoses, quite apart from the evidence of sexual abuse. Dr. Singh testified that the diagnosis is right, but a diagnosis changes with the progression of the disease, both in severity and in developing further diagnosis—both of which happened. Dr. Silver and Dr. Singh both noticed post-traumatic stress disorder. They both noticed dysthymia. These diagnoses are a progression of the disease. If the disease is not cured, it does not remain static. The disease intensifies, and in K.M.'s case it produced more diagnoses, like dysthymia and post-traumatic stress disorder.

[452] Dr. Singh confirmed that he has never said that the sexual abuse is the sole cause of K.M.'s etiologies. Dr. Singh's opinion is that the sexual abuse is one of the most important causes of K.M.'s mental health problems. K.M. has gone through so much progression of other diseases that it cannot be the sole cause; there are multiple causes. It is fair to say that Dr. Singh's opinion is that there are multiple causes of K.M.'s different mental health injuries.

[453] Dr. Singh testified that most mental illnesses have multiple causes. Other than depression, which can be genetically determined, other causes of K.M.'s mental health injuries are as outlined in Dr. Singh's evidence in reference to his November 21, 2011 report. Dr. Singh testified that K.M.'s psychopathy and psychiatric disorders are:

1. Major depression with dysthymia.
2. Substance abuse.
3. Personality disorder of the antisocial and borderline type.
4. Difficulty in building interpersonal relationships due to his past sexual abuse. The patient does not trust enough to build a relationship with anybody, because of his past traumatic experience. The most important factor remains in interpersonal failure. In these kinds of patients, it is their mistrust which spoils relationships in their life.
5. Difficulty in keeping employment. Because of the multiple psychiatric problems described above, he cannot keep employment.

[454] Dr. Singh was asked whether some of these diagnoses are related to sexual abuse and some are not. Dr. Singh testified that when you have multiple psychopathy, everything is related to each other. You cannot differentiate.

[455] Dr. Singh agreed that in summary, it was his opinion that the sexual abuse experienced by K.M. has had an impact with respect to his various psychiatric disorders, but it is not the sole explanation for them. Dr. Singh added that it was his opinion that the sexual abuse had a major impact on K.M.'s psychiatric disorders.

[456] Dr. Singh testified that where he and Dr. Silver disagree is as to the intensity and the genetic and epigenetic change of the effect of the sexual abuse on K.M. Dr. Silver talks about genetic change but he does not explain the kind of genetic change. Dr. Singh explains genetic change, which is both the patient's genetic structure they are born with and epigenetic change caused by external factors, and the two are inter-linked. Not every traumatic event in a person's life is going to cause genetic change.

[457] Dr. Singh testified that there are two kinds of trauma, major trauma and minor trauma. Sexual abuse is major trauma, as is war trauma. Minor trauma could be someone getting slapped. In major trauma, the patient suffers more. Dr. Singh testified that the classification system has defined any level of sexual abuse as a major trauma. The details of the sexual abuse are not needed by the psychiatrist for treatment.

[458] Dr. Singh testified that epigenetic change is accepted as a scientific fact and it is accepted in the medical community. Millions are being spent on laboratories, including one at the University of Toronto. Newer cancer drugs have come through the epigenetic process. The current state of scientific knowledge on this issue is always progressing.

[459] In re-examination, Dr. Singh confirmed that his knowledge of the sexual assaults on K.M. was as stated on page 1 of his June 15, 2009 report, in that the sexual assaults began on K.M. in grades 6, 7 and 8 from 1976 onward.

b. Dr. D. Silver

[460] Dr. D. Silver gave expert psychiatric medical opinion evidence on behalf of the School Board. He conducted an independent medical examination of K.M. on September 20, 2010 for the purposes of assessing K.M.'s mental status as it relates to the issues in this case. Dr. Silver is a qualified psychiatrist with years of clinical, teaching, and research experience and has published numerous refereed and non-refereed papers and chapters of books, with significant interest and expertise in the area of borderline personality disorders. Dr. Silver's updated, extensive Curriculum Vitae as of September 2013 (Exhibit #14) is before the court. Dr. Silver complied with the requirements of Rule 53.03 and signed an acknowledgement of expert's duty, Form 53, on May 4, 2016.

[461] Dr. Silver became qualified as a psychiatrist in 1969. He trained in Montreal at McGill University and since 1970 has practiced as both as a psychiatrist and a psychoanalyst, doing long-term, intensive study and treatment of individual patients. Dr. Silver has been head of both inpatient and outpatient departments of psychiatry at hospitals affiliated with McGill University and the University of Toronto, responsible for the diagnosis, management, and treatment of a wide variety of psychopathologies. He has taught, supervised, and trained psychiatric residents, and his practice has always been hospital-based.

[462] From 1994 until the present, Dr. Silver has been a clinical consultant with the Department of Psychiatry at Mount Sinai Hospital. From 1976 to 1994, he was an associate professor at the University of Toronto in the field of psychiatry. From 1968 to 1973, he was a psychiatric consultant to Weredale House, which was a residential treatment centre for young

boys who had got themselves into trouble with the law. The boys would receive treatment from the nearby Montreal Children's Hospital adolescent unit. Dr. Silver would attend there one or two times a week with the aim of trying to get them back to their families.

[463] From 1968 to 1971, Dr. Silver was a consultant to the Teenage Clinic at the Montreal Children's Hospital. From 1970 to 1972, Dr. Silver was Head of Youth Emergency, Diagnostic and Management Clinic at the Jewish General Hospital in Montreal. Dr. Silver described this period as the height of the drug scene, with adolescents and mainly young people getting into drugs. They were using pot, LSD and amphetamines. He ran a "no hassle clinic" to provide this group with treatment.

[464] In 1992, Dr. Silver participated in the publishing of a book, *The Handbook of Borderline Disorders*, as part of a symposium of experts in the United States and Canada. He was the editor and wrote a chapter in the book. This was a major publication of some 700 pages looking at the broader issue of borderline personality disorders from many different medical aspects in an effort to help those affected. Currently, Dr. Silver is trying to get some of the same people together to provide an update to the book, as their understanding of borderline personality disorders has shifted in terms of treatment and these patients continue to be challenging, with modest success in treating them.

[465] From 1973 until the present, Dr. Silver has been the Supervisor for Individual Psychotherapy at Mount Sinai Hospital, Department of Psychiatry. This work is part of the psychiatric training provided to residents as to how to treat people psychologically, in addition to other training they receive in psychotherapeutics and psychopharmacology. At age 88, Dr. Silver still sees and follows approximately 24 patients in general psychiatry and psychoanalysis at Mount Sinai Hospital, and works about 30 hours per week.

[466] Dr. Silver was qualified as an expert to give opinion evidence to the court, as requested by defendant's counsel, in the field of psychiatry and psychoanalysis. In cross-examination, Dr. Silver confirmed that he has treated quite a number of cases of adult victims of childhood sexual abuse, but could not guess the actual number. He has given expert evidence in court for 25 to 30 years and has testified for both plaintiffs and defendants, but could not say in what percentages he had testified for which party.

[467] Dr. Silver testified about the opinions given in his December 8, 2010 expert report (Exhibit A, Tab #2A) and his one-page report comment dated May 24, 2011. Because of Dr. Silver's vision problems, counsel was allowed to read significant portions of Dr. Silver's written reports to him during his examination in chief and then to ask questions. Dr. Silver acknowledged that prior to seeing K.M., he had read six volumes of medical briefs, the transcript of the examination for discovery of K.M., a cost of care report, K.M.'s hand-written statement of the alleged abuses in the police file, and the material from the Criminal Injuries Compensation Board.

[468] Dr. Silver testified that he found K.M. to be a delightful person, very co-operative and forthcoming, open, frank and quite even-handed about many things. Dr. Silver testified that

K.M.'s learning difficulties started in primary school, but that he seemed to be a happy child and had some difficulty with the family that did not seem unusual.

[469] Early in their discussions, K.M. told Dr. Silver that he was a "blue baby". Dr. Silver testified that is a common term used where a baby is delivered with the umbilical cord around their neck, cutting off some of their oxygen supply, so they turn out as "blue". The most severe of these cases results in cerebral palsy. He testified that when a child of 6, 7 or 8 begins to have learning difficulties, behavioural difficulties and positional difficulties, the birth delivery is checked out. K.M. was seen as a "bad boy", instead of a child who is having difficulties, particularly learning difficulties. Dr. Silver testified that we're much better at understanding these things now, and we see these children as needing a particular kind of attention and a particular kind of learning help. These children have problems reading and writing, or doing homework. They can't sit for a long time and they are usually hyperactive, and there are a whole lot of problems. K.M. began to show many of these problems as a young child. Back then, they were disciplined as if they were bad; they weren't being good children.

[470] Dr. Silver testified that these same children may be extremely bright in other areas. K.M. was bright and had "street smarts". These children could not do their homework or they were truanting. They were seen as "bad" instead of "troubled". They can have problems with peers as they are often seen as "dummies", and were bullied or became bullies, or do things to show off to be part of the gang in adolescence, but they are never quite accepted as they would wish. They get into drugs, become mules, and have other difficulties like truanting, coming late, and not completing tasks, and the family comes down hard on them. K.M. was tied to a tree, an example to Dr. Silver of disciplinary issues. Dr. Silver did not dispute that K.M. had a positive relationship with his mother, his two sisters and his aunt.

[471] Dr. Silver reviewed K.M.'s education and looked at the records. He explained that the reports say that "K.M. wasn't trying hard enough", when they should have said, "He's troubled, and needs remedial work, remedial help". Another example in the school records was the remark that it may be necessary for K.M. to repeat grade 6, "to shock him out of his indifference". Dr. Silver testified that K.M. was in a smaller town, but back then, it was misunderstood. After K.M. was transferred to Harry J. Clarke School in grade 7, the teachers noted projects not done, homework often not completed, notebook unsatisfactory, inconsistent results, and that "self-discipline was required". Dr. Silver noted K.M. spent a year at Saint John's School in grade 8.

[472] Dr. Silver testified about K.M.'s adolescence and how K.M.'s adolescent years were very difficult for him and school was a challenge. K.M. told him that there were constant arguments with his parents with regards to his smoking cigarettes, and particular his excessive use of marijuana. They were upset that he was abusing tobacco and marijuana when he was only 12 or 13 years old. K.M.'s anti-social behaviour during adolescence was rampant, stealing bicycles just to get a rush and things from the mall, but he was rarely caught. Dr. Silver testified that as an early adult, K.M. abused cocaine for about two years and supported his habit by being a "mule", that is, bringing drugs in from Montreal. K.M. had street smarts.

[473] Dr. Silver testified that the traumatic and very erratic developmental story of K.M. is of a man with a distorted self-image who is seen by others in a distorted sort of way. He tries to find

acceptance where he can. He's vulnerable. He's sought out by people who act as if they are befriending him, but they are abusing him. He goes with whomever will accept him.

[474] Dr. Silver testified about K.M.'s employment. He testified that when K.M. needed money, he could always find something to do. One problem he had was with consistency; he'd start something, he'd like it, he'd be good at it, then he'd get bored or he'd get unhappy with the people he worked with, and he would go from job to job. Dr. Silver thought K.M. was gifted in many ways and a likeable kind of guy, and when he worked he would be quite dependable. As to K.M.'s inability to hold a particular job for any length of time, Dr. Silver testified it had two aspects: firstly, his attention span, and secondly, his basic personality structure, which was that of a borderline personality disorder. One of the hallmarks of people suffering from that syndrome is that they get bored easily, then they are unhappy and start to drop off. Their work and relationship records are erratic. Their anger "hangs" and then slowly settles down. Consistency is not one of their strengths.

[475] Dr. Silver testified about K.M.'s mental status during his examination. He found K.M.'s long hair to be a bit disheveled. He was pleasant, verbal, congenial, co-operative and forthcoming. He missed no opportunity to impress Dr. Silver with how smart he was. He was very likeable. He was very intelligent as to his street smarts. He did not trust people. Dr. Silver reviewed K.M.'s history of hospital admissions and suicide attempts and testified that K.M. felt he was "wired" wrong, but had refused electro-convulsive therapy ("ECT") as a treatment for his depression. Dr. Silver agreed he was not an expert in ECT treatment.

[476] Dr. Silver testified that K.M.'s positive relationship with his son is relevant because it means there are healthy parts in K.M. that make him able to function in a caring relationship with his son. It does not transfer into healthy relationships with the women in K.M.'s life, however.

[477] Dr. Silver testified about K.M. becoming uncomfortable and angry when he inquired about the "alleged assault by Mr. Marson". Dr. Silver testified that he read a great deal about the "alleged abuse", including K.M.'s statement to the police. Dr. Silver decided not to pursue the details with K.M. The details of the sexual abuse were important to Dr. Silver. He had many queries about the details, and how the "alleged behaviours were acted out". Dr. Silver knew his role was not that of the prosecuting attorney, but he put his feelings about what he thought about the "alleged abuses" in his report.

[478] Dr. Silver testified about his opinion of K.M.'s long list of diagnoses over the years by various doctors, beginning with Dr. Brant on November 20, 1989. Many of these diagnoses, according to Dr. Silver, could come under the broader heading of a borderline personality disorder. Dr. Silver had never seen as many different diagnoses as this young man has.

[479] Dr. Silver then referenced Dr. Singh's diagnosis and testified that, "Dr. Singh's diagnosis was similar, but condensed, to what other psychiatrists have noted over the years: Major Depression with dysthymia, Personality Disorder of anti-social and borderline type, difficulty in building interpersonal relationships due to post-traumatic experiences, and difficulty in being employed." Dr. Silver stated that he is essentially in agreement with Dr. Singh's

diagnoses and psychopathological descriptions, but that he disagrees with Dr. Singh's clinical and particularly his etiological conclusions. By etiological conclusions, Dr. Silver testified he meant that the etiology of the reason for one being the way he is now is because "this" and "this" had happened before. Etiology refers to the basic cause or reason for what you're examining at this moment in time.

[480] Dr. Silver went on to agree that psychiatry is currently moving more and more towards evidence-based knowledge and treatment. It is a totally over-simplified reductionism to suggest that there is any one major etiological cause of K.M.'s long, complicated and complex psychological profile. Evidence-based knowledge is looking for the hard evidence for your assumptions that "this" has been caused by "this". Clinical wisdom is still very highly respected, but medical training now is looking for hard evidence for things looked at clinically. There cannot be one cause of a psychiatric diagnosis. One reason is that psychiatric diagnoses are not exact to begin with. There are not any real hard markers to say, for example, you have depression because the blood results show "this", and a lot of our diagnosis is "soft", based on clinical impressions. Dr. Silver testified that psychiatric discipline has no hard evidence for any single diagnosis, and to think that there is one etiological factor responsible for that is rather a stretch, to say the least.

[481] Dr. Silver testified that he disagrees with Dr. Singh's conclusions that K.M. suffers from the impact of what happened to him from 1976 onwards in grades 6, 7, and 8 and that many problems present in him can be directly based on those traumatic years of suffering. Dr. Silver disagrees with Dr. Singh's statement that most of the sexually abused patients have been found to get involved with substance abuse. Dr. Silver disagrees with Dr. Singh's statement that the two main damages done to K.M. from the childhood sexual abuse are the changes in personality, mood and substance abuse.

[482] Dr. Silver testified that to say that sexual abuse and later difficulties are linked is one thing. But when talking about causality, one have to be very, very careful because all the information we have is beginning to change now. Most of our past and current information has come from retrospective studies; if someone tells you they've had sexual abuse, you make the assumption that the sexual abuse is what caused the problem. It's certainly an important linkage, but the reality is that to know whether one is the cause of the other, you need prospective studies. They are just beginning to get some and they could take 15 to 20 years of follow-up. There are a few prospective studies now on borderline personality. One has to be very, very careful, where a psychiatric diagnosis is not based on hard data or any hard markers, about even implying that a single event or occurrence can lead to a particular problem.

[483] Dr. Silver testified about K.M.'s life-long depressive illness being likely a kind of genetically-determined characterological depression. There are depressions and different causes for it. Reactive depressions are a great loss, like a death in the family; it's a reactive depression to an event. There are other kinds of depression that happen for one reason or another. There are depressions that seem to be inherent, as if this person has a diopetre for depression. They seem to be in a depressive state from the get-go, and those kinds of depressions do not respond very well to psychotherapeutic or psycho-pharmacological interventions. They become very resistant and very intractable depressions, as if they are "baked into" the character of the human being. Dr.

Silver defined “genetically-determined” as being part of a person’s basic personality structure, inherent characterological differences. Dr. Silver said that in a simple-minded way, it’s the “wiring” that goes in, the DNA, the genetics. When it comes to depression, there are some people that are depressive characters, with a cloud over their head all the time. Dr. Silver testified that he’s not sure if this applies to K.M., as he has not seen him enough times to distinguish his depressions. He did note, however, that K.M.’s depression does not seem to respond well to medication. To really know and understand, you have to see a patient many times over a long period of time, and not sporadically, to really get to know what makes them “tick”.

[484] Dr. Silver testified about his report summary that K.M. has been challenged all his life by his learning disabilities, and probably problems of an Attention Deficit Disorder. He has been compromised mainly by affective dysregulation reflecting his mood disorder and Borderline Personality Disorder. By “learning disabilities”, Dr. Silver was referring to K.M.’s description of not being able to do math. Certain things he can be good at and able to understand, comprehend and manage to deal with, but on basic things he gets all muddled, mixed up and unable to accomplish and perform. Dr. Silver described Attention Deficit Disorder as an off-shoot of hyperactivity—he can’t stick with one thing for any length of time, he can’t apply himself for any length of time without losing patience, wanting to get on and get it over with or get going. Dr. Silver confirmed that K.M. does have a Borderline Personality Disorder, among other problems.

[485] Dr. Silver testified that K.M. would be affected by having a Borderline Personality Disorder in his life, relationships and holding a job. It would have an affect dysregulation, that is, he has mood fluctuations. He can get in and out of good moods and bad moods unexpectedly and very quickly. He has problems with irritation, in that things irritate him. He has problems with interpersonal relationships. He has problems with basic trust. He has problems with being able to relate, for any length of time. He has problems with anger. He has problems with his own sense of self and his own continuity. He doesn’t feel the same all the time, and he is often puzzled why he feels so differently at different times. This is part and parcel of Borderline Personality Disorder. This dysregulation goes on in one’s sense of being, and it’s not a happy set of events.

[486] Dr. Silver states that he is unable to say with any certainty how much, if any, K.M.’s alleged sexual abuse by Marson may have contributed to any of his psychiatric diagnoses or how much it has compromised his earning capacity. He can say, however, that K.M.’s multiple diagnoses over the years of Attention Deficit Disorder, anti-social personality disorder, learning disabilities, Dysthymic and Para-Suicidal behaviour, dependent personality, hysterical conversion, somatization, and major depressive and reactive depressions are genetically and environmentally determined, quite apart from any of the alleged abuses by Marson.

[487] By way of explanation, Dr. Silver testified that he would see patient after patient with similar diagnoses who have no history of sexual abuse. He testified that sexual abuse certainly can be one of the causes of some of these diagnoses, but you can have any and all of these diagnoses without any sexual abuse whatsoever—that is, independent of the sexual abuse.

[488] Dr. Silver's report—where he stated that based on his own personal experience and particular interest in Borderline Personality Disorder, he did not believe that K.M.'s primary diagnosis of Borderline Personality Disorder was caused directly by the alleged sexual abuse—was put to him by the School Board's counsel, who then said to Dr. Silver, "Now, we know that he was sexually abused. How do you make that statement?" Dr. Silver answered by way of a question, "We know that he was sexually abused?" to which counsel answered, "Yeah". Dr. Silver answered with another question, "Yes...?". It appeared that this was news to Dr. Silver, as his references had always been to the "alleged" sexual abuse of K.M.

[489] Dr. Silver eventually answered that the diagnosis of Borderline Personality Disorder is so multi-determined and there are so many factors that it would be hard to say the degree to which, if any, and how much any sexual abuse has to do with the ultimate Borderline Personality Disorder. It was the first time that he did not say "alleged" sexual abuse. Dr. Silver re-iterated his opinion that no single event can account for anything so complex and so complicated as the kind and variety of human behaviours that we see in K.M.

[490] Dr. Silver was asked about Dr. Singh's comment that Dr. Silver had forgotten the evidence of research on the process of epigenesis, and that sexual abuse can lead to genetic changes. Dr. Silver made it clear that he was no expert, as he thought Dr. Singh was, on epigenetic issues. Dr. Silver acknowledged the importance of epigenetics and the possibilities that DNA can be altered or changed in certain ways. Dr. Silver testified that the testing done so far was animal-based and may not translate into human advantages. Dr. Silver stated that trauma cannot predict how any given individual person is going to come out of it and the effect it has on him. It's not only the event that's important, but the personality and character structure of the person it's being acted on.

[491] Dr. Silver was thoroughly cross-examined in all aspects of his evidence. I find that while Dr. Silver agreed with Dr. Singh's diagnoses, he did not deviate from his opinion on causation: that it cannot be said that any particular event caused K.M.'s mental illness.

[492] In cross-examination, Dr. Silver was asked about his May 24, 2011 report and his reference to an article about epigenesis dated May 2007. Dr. Silver acknowledged that he had no involvement in and had done no work in the field of epigenesis. Dr. Silver acknowledged that Dr. Singh had done significant work in this area and was well-placed to explain this field to the court, and that he would defer to Dr. Singh as to its relevance to this case.

[493] Dr. Silver acknowledged in cross-examination that he is highly respectful of Dr. Singh as an eminent psychiatrist and that Dr. Singh is a very skilled, interesting and interested psychiatrist in some of the very organic issues, certainly psychopharmacology. Dr. Silver testified that Dr. Singh is an expert in that field. Dr. Silver said there are differences in psychiatrists. They are trained differently and operate differently. The issue explored by K.M.'s counsel was the nature of the opportunity to observe the patient over time, as opposed to on one occasion. All other things being equal, in terms of the treating psychiatrist having the necessary skill and ability to treat a patient of this kind (an adult victim of childhood sexual abuse), Dr. Silver did eventually agree that more information is always helpful and that Dr. Singh, through his experience, is qualified to make that judgment.

[494] Dr. Silver was cross-examined in detail about his definitive negative statements as to K.M.'s credibility in his report of December 8, 2010 and his doubt that the sexual assaults on K.M. had even occurred (hence his reference throughout his report to the "alleged" sexual assaults). I note that the report was written when the School Board was denying liability, even after Marson had been convicted of the sexual offences on K.M. two years earlier, on January 23, 2008.

[495] K.M.'s counsel put to Dr. Silver his comments on pages 9-10 of his report, where he stated:

As psychiatrists, we generally believe what our patients tell us. The credibility of the patient is an essential pre-condition for arriving at a diagnosis and forming a treatment program. Thus, to make a definitive diagnosis in any given case the clinician must feel comfortable that the primary motive derived from the patient's narrative is not, for example, for the purpose of gaining official medical validation for the illness, in pursuit of financial benefits.

It is indeed an unexpected clinical problem when the clinician is confronted with some doubt in completely accepting the patient's narrative and the difficulty in having to challenge it.

In this case, the alleged sexual abuse occurred over a period of more than 2 years, repeated supposedly "100 times" mostly in 2 settings, and in a public building. Yet it seemed to escape the attention of any faculty members or students. Given the number of times the alleged abuse occurred over so long a period, it is surprising that there is no corroborative evidence available.

Another unusual aspect of the alleged abusive encounter in this case is that the relationship never went beyond the two school settings. ... However, I find it difficult to believe that, during such a long relationship, he [K.M.] could not have been invited and joined Mr. Marson for an ice-cream, a cup of coffee, or a juice.

[496] Dr. Silver denied knowing that the School Board was denying liability at that time he wrote his report. Dr. Silver agreed that he was doubtful as to K.M.'s credibility and that he was doubtful that K.M. was telling the truth about the assaults. The reason he was doubtful was that in all the cases he had seen there was never, in his recollection, a case where the abuse occurred where there aren't other things in the relationship that went on—like inviting him to stay over at night, and sleep in his bed, taking him out fishing, and hunting, so they had time together. There were other things going on. It was never as cut and dried, and sporadic, as that. That was the only reason that it raised that question in his mind. Dr. Silver testified that "it wasn't there for me to judge whether it occurred or not occurred. That wasn't what I had in mind at the time. I was just sharing an opinion of what was going through my mind, as I was listening to the story".

[497] The obvious issue was then explored by K.M.'s counsel: if the credibility of the patient is an essential pre-condition for arriving at a diagnosis and Dr. Silver doubted K.M.'s credibility, how then did Dr. Silver not lack an essential pre-condition to form a diagnosis? Dr. Silver

answered, “No”. He then said he did not *not* believe K.M. but he had doubts based on his personal experience in these kinds of cases. Dr. Silver agreed with K.M.’s counsel that he was saying that, notwithstanding his doubts about K.M.’s credibility, he was nevertheless in a position to formulate a diagnosis. Dr. Silver agreed that he had become aware “very recently” that the School Board no longer disputed that K.M. was assaulted as he described.

[498] Dr. Silver agreed that it was fair to say that at the time he formulated his opinion as to how important or unimportant the sexual assaults were as a cause of K.M.’s disorders, he had a doubt as to whether the sexual assaults had even occurred. Dr. Silver testified that he was there to assess K.M. in totality, whether he was a functioning adult, and his total developmental process, from the time he was born. Dr. Silver testified that whatever he thought about the assault and whether it occurred or not would not change his overall way of getting at the information he did and formalizing it into the report he gave. Dr. Silver agreed that he was not asked to do a new report based on the School Board admitting that the assaults had occurred.

[499] Dr. Silver agreed in cross-examination that in terms of trying to figure out what was wrong with K.M., it would be important to know that he had been sexually assaulted as a child. Dr. Silver would not agree that since Dr. Brant and Dr. Conn did not know of the assaults, this meant that their diagnoses should be taken “with a grain of salt”, but did agree that the knowledge of the assaults would have given them some cause to try and understand why they came to their diagnosis. He agreed that if they had known, it might have had an impact on their diagnoses of the disorders.

[500] Dr. Silver testified in cross-examination that he broadly agreed with Dr. Singh’s description and diagnosis of the overall constellation of K.M.’s disorders, and that K.M.’s prognosis is poor. Dr. Silver agreed that any future employment for K.M. would be sporadic. Dr. Silver agreed that K.M.’s chance of a stable, spousal relationship is small.

[501] Dr. Silver agreed that his opinion on causation was that it was difficult, perhaps impossible, to say with any degree of scientific precision or assurance that the childhood sexual abuse of a particular individual caused a particular disorder in adulthood. Dr. Silver agreed that at that point, all a psychiatrist can do is to take into account as much as he can from the available science, look at all of the available evidence about this individual, and come to his best clinical judgment, on all of that material, as to whether there is a causal link in this particular case.

[502] Dr. Silver would not agree that the only person who is in a position to form that clinical judgment is Dr. Singh, as K.M.’s lengthy treating physician. Dr. Silver would not compare one psychiatrist to another based on opportunity to see the patient, as their basic knowledge base and what they are interested in is very different from each other. Dr. Silver did testify that what Dr. Singh does as a psychiatrist, he does very, very well. Whether Dr. Singh does it better than someone else, or is as good as someone else, is not for Dr. Silver to decide.

[503] Dr. Silver confirmed that his position on causation was as stated in his report on page 13, put to him as follows:

I am unable to say with any certainty how much, if any, K.M.'s alleged sexual abuse by Mr. Marson may have contributed to any of his psychiatric diagnoses or how much it has compromised his earning capacity.

[504] Dr. Silver's opinion was that you cannot pull out and assign a percentage responsibility for other possible causes, and that it was a mixture of all causes.

[505] As to the issue of the cord around K.M.'s neck at birth, Dr. Silver confirmed in cross-examination that all we know is that K.M. said he had been told this and that his mother talked about it. There is no medical record of it that Dr. Silver knows of or has seen. Dr. Silver agreed that nowhere in the family doctor's records or the hospital records is there any reference to the birth or to damage at birth, or anything. Dr. Silver agreed that if there is such a cord, there may or may not be any damage done to the baby and there may or may not be any noticeable consequences that can be related back to that; even if a child had a cord (around its neck in childbirth) and later has a learning problem, it may be related but we do not know for sure. Dr. Silver agreed that based on what little we know, we would be speculating if we were to conclude whether K.M. was damaged in any way (at birth) or whether it had any effect.

[506] All we can say, Dr. Silver testified, is that K.M. had early learning difficulties. If we have a history of a blue baby, either anecdotal or actually written, we cannot say that is the reason. Dr. Silver agreed that given the limited evidence we have as to the blue baby issue—and the possibility that there was no damage or that if there was, it was not connected—in terms of the learning difficulties, it is a matter of speculation, not probability or possibility.

[507] Dr. Silver was asked about how K.M. was treated by his teachers as a child with a learning difficulty. He testified that we used to see these children as “bad” when they were inattentive or oppositional, or didn't do their homework or keep their desks neat, instead of “troubled”, either physiologically or psychologically. Dr. Silver was of the view that the teachers were misinterpreting K.M.'s learning troubles. The teachers' notes were relating their frustration and their lack of sensitivity and ability to see what was going on, rather than what was wrong with the boy, K.M.

[508] Dr. Silver agreed in cross-examination that notwithstanding the apparent learning problem, K.M. in many respects was a very intelligent individual and that he seemed to compensate very well for his learning disability, including his ability to be a very effective and successful employee. Dr. Silver testified that K.M.'s learning difficulties made him more vulnerable to the kind of sexual predator that Marson was.

[509] Dr. Silver would not agree that, if the evidence was that K.M. appeared to be normal, happy and cheerful, got along with other kids and did not appear to show any signs of any sort of depression for the first several years of his life after birth, it would be inconsistent with it being a characterological depression. Dr. Silver described characterological depressions as being “treatment resistant” or “intractable”, and this term is used primarily about adults who seem to be always in a low depressed mood and cannot seem to ever derive any real joy out of life. Importantly, Dr. Silver's earlier evidence was that he could not say that K.M. had a

characterological depression, as he would have had to have seen K.M. many more times to try to distinguish his depression.

c. Summary

[510] I accept and find credible Dr. Singh's evidence that the complex of disorders displayed in K.M. is very similar to those commonly seen in childhood sexual abuse cases. I accept and agree with Dr. Singh's evidence that K.M.'s previous treating psychiatrists were not told of the sexual abuse, and therefore did not have K.M.'s full psychopathy to consider in coming to their assessments and diagnoses of K.M.

[511] Dr. Silver agrees with Dr. Singh's diagnoses of the overall constellation of K.M.'s disorders, that K.M.'s prognosis is poor, and that K.M.'s chance of having a stable, spousal relationship is small. Dr. Silver's opinion of K.M.'s future employment is that it would be sporadic as it has been in the past, whereas Dr. Singh's opinion is that K.M. is unable to be employed.

[512] In 2009, Dr. Singh's diagnoses of K.M. were major depression with dysthymia, substance abuse, personality disorder of the anti-social and borderline type, difficulty in building interpersonal relationships due to past sexual abuse, and difficulty in keeping employment. In 2010, Dr. Singh made the additional diagnosis of post-traumatic stress disorder. In 2015, Dr. Singh's diagnoses of K.M. were major depression with dysthymia and post-traumatic stress disorder; borderline personality disorder with introversion and unspecified learning disorders (not his old learning disorder, but affected by post-traumatic stress disorder) with interpersonal failure of keeping relationships and employment; financial and social stress and stress due to failure to establish relationships causing loneliness, and the impact of his teenage sexual abuse; and a GAF score of 30. A normal GAF range for working people is 45-50 to 90. Below 30, people are not able to work. K.M.'s substance abuse had improved. Dr. Singh's evidence is that K.M.'s diagnoses were deteriorating. Dr. Singh's prognosis for K.M. was guarded and poor. "Poor" was added because Dr. Singh's testing showed that K.M.'s cognitive abilities were deteriorating.

[513] K.M.'s chronic condition of dysthymia does not improve with treatment and is not related to his earlier learning difficulties. I accept and find credible and reliable, Dr. Singh's assessment and diagnoses of K.M. Dysthymia, or chronic depression, Dr. Singh explained, is the kind of depression that does not cure itself from treatment completely. It can improve but it will be a fluctuating improvement, with stress bringing it down. Dysthymia is present in a long-time depressed person such as K.M. Dr. Singh diagnosed K.M. in addition with substance abuse, personality disorders of the anti-social and borderline type, difficulty in building interpersonal relationships, difficulty in keeping employment, and post-traumatic stress disorder. I find the most serious and longstanding diagnosis of K.M. is his major depression with dysthymia.

[514] I accept Dr. Singh's evidence that in his opinion, further counselling or other extra-cost treatments would be of no benefit to K.M. Dr. Singh's opinion is that there is no pharmacotherapy and no psychotherapy that would change K.M.'s dysthymia, his post-traumatic stress disorder, and the impact of the sexual abuse. I find that the evidence supports that further

programs or counselling will have little if any impact on his mental health injuries, which are of a permanent nature. It will not get him employment or an interdependent relationship. I note K.M. makes no claim for future care.

[515] I find that the essence of the disagreement between Dr. Singh and Dr. Silver's opinions is as to the causation of K.M.'s complex of disorders. Dr. Singh and Dr. Silver agree that childhood sexual abuse constitutes at least "a" cause of K.M.'s psychiatric disorders, and that the disorders are indivisible. They differ as to the degree of the effect of childhood sexual abuse in K.M. Dr. Singh testified that childhood sexual abuse is a major illness. It has a major impact on the development of human life and is recognized as such in psychiatry. He testified that K.M. has a cluster of five illnesses and we cannot separate them, and Dr. Silver agreed with that. Dr. Singh testified that he was taken aback that Dr. Silver, however, reported that childhood sexual abuse is not a major mental illness.

[516] Dr. Silver's opinion is that it is not possible to say that childhood sexual abuse is "a" cause of any one specific mental disorder in adulthood. I do not accept Dr. Silver's evidence and opinions in that regard, and prefer and accept the evidence and opinions of Dr. Singh.

[517] In Dr. Singh's opinion, which I accept, normal people do not get sick like this from life stressors, such as losing a job or a relationship break-up. Depression may be common with a history of a learning disorder, but nobody gets dysthymia from a learning disorder. Dr. Singh found that K.M.'s personality growing element was lost because of the trauma of the sexual abuse at a very tender age. This causes feelings of inferiority, anger and frustration, causing depression. K.M. suffers from flashbacks of the sexual abuse. I find that these symptoms noted by Dr. Singh are consistent with K.M.'s medical history and with the evidence of K.M., his family members, and the other lay witnesses. These feelings lead to substance abuse, which was present in K.M. for many years.

[518] My findings on the issues of causation, raised in the expert evidence of Dr. Singh and Dr. Silver, and on the credibility of their opinion evidence are expanded upon in the causation section following.

14. Causation:

[519] The parties disagree on the issue of causation. K.M. submits that the sexual abuse by Marson is a major cause of his mental health injuries as an adult. The School Board submits that K.M.'s injuries were caused not only by the sexual abuse, but by other factors in K.M.'s life, occurring prior to or since the sexual abuse. It is the School Board's position that K.M.'s symptoms of depression are likely a kind of genetically determined characterological depression, and that K.M.'s multiple diagnoses, including his depression, are genetically and environmentally determined, quite apart from the sexual abuse by Marson.

[520] The first question is: has K.M. established on a balance of probabilities that the childhood sexual abuse perpetrated by Marson on K.M. constitutes at least "a" cause of K.M.'s complex of mental disorders in adulthood, so as to render applicable the general principle in *Athey v. Leonati*, [1996] 3 S.C.R. 458, that the School Board is liable for 100% of K.M.'s

damages arising from his complex of mental disorders? I answer that question—yes. Moreover, I find that the childhood sexual abuse by Marson constitutes a major cause of the mental disorders affecting K.M. as an adult.

[521] If the answer to the first question is yes, the second question is: has the School Board established that K.M. suffered from a pre-existing condition prior to the childhood sexual abuse and that there is a “measurable risk” that that condition would have led to the development of at least part of the complex of K.M.’s mental disorders, even if the sexual abuse had not occurred, so as to make applicable the “crumbling skull” qualification or exception to the general principle in *Athey v. Leonati*? I answer that question—no.

[522] K.M.’s position is that in order to establish causation, he must show that on a balance of probabilities the defendant’s tort was a “but for” cause of his psychiatric injuries. K.M. submits that under *Athey v. Leonati*, so long as the defendant’s tort is “a” cause of the injury the defendant remains liable for the full extent of the damages. You do not try to carve it out and apportion it, even if the damages may have been contributed to by other factors like K.M.’s genetic predisposition or stressful events in his life before or after the abuse. K.M.’s counsel noted that liability would not extend to an unrelated event if the injury in question were divisible from that caused by the defendant’s negligence. K.M.’s counsel observed, however, that divisibility does not appear relevant in this case; no one has suggested that there is any way to divide up K.M.’s psychiatric disorders—they represent one complex of symptoms.

[523] K.M. submits that so long as the School Board is part of the cause of K.M.’s injury, it is liable, even though its act, alone, is not enough to create K.M.’s injury. It is K.M.’s position that the School Board’s argument—that his injuries may have been caused by or contributed to by other things in life, aside from the sexual assaults—does not mean that it is not responsible for 100% of K.M.’s injuries. It is not necessary or appropriate to tease out what part of the injuries was caused by the abuse as opposed to other causes.

[524] K.M. acknowledges the “crumbling skull” qualification to this general principle: if the plaintiff would have suffered certain symptoms absent the tort, the defendant is not liable for that portion of the damages. However, K.M. submits that it applies only if there is a “measurable risk” that a non-tort event or pre-existing condition “would have” caused an injury on its own or detrimentally affected the plaintiff in the future. It is not enough for the School Board to present evidence that other factors pre-dating the abuse might have contributed to K.M.’s overall condition; there has to be tangible, cogent evidence establishing a measurable risk that those factors would have—not might have—caused K.M.’s disorders, even without the sexual abuse. While K.M. did not disagree that his learning disability is a pre-existing condition, he submits that this does not trigger the crumbling skull because K.M. has not claimed damages arising from or related to the learning disability, as it was there before and after the assaults; it is a “red herring” with no relevance to the damages.

[525] K.M. submits that *Blackwater v. Plint*, 2005 SCC 58, is an example of an application of the crumbling skull exception. *Blackwater* does not, however, stand for the proposition that the court must engage in this damages allocation process in every multi-factorial case, whether or not the defendant has proven that which is necessary to trigger the crumbling skull qualification.

Rather, for the court to be persuaded that it must apportion responsibility for K.M.'s disorders between the sexual abuse and other contributing causes, the School Board must establish that the other cause is sufficient, on the evidence, to represent a measurable risk of causing K.M.'s disorders absent the abuse. If the School Board cannot show that, it cannot justify apportionment. K.M. argues that the evidence put forward by the School Board in trying to achieve that objective—such as K.M.'s genetic predisposition or other traumatic events in his life—is speculative and simply insufficient to overcome the other evidence showing that K.M. had no disorders before the assaults that would have caused him difficulties in later life. It is K.M.'s position that the School Board has fallen far short of that burden, with the result that the general principle applies: the School Board is liable for 100% of the damages.

[526] The School Board's general submission is that it is incumbent on the court to assess plaintiff's "original position", the language used by the Supreme Court, to consider the tort that was committed upon the plaintiff, in this case the instances of abuse, and determine how those events have detrimentally affected the plaintiff. The School Board's position is that the court's task is to disentangle the situation that K.M. is in and to endeavour to assess damages based on the specific tort in question. The School Board argues that *Athey* is clear: the general rule is that the plaintiff must be returned to the position he would have been in, with all of its attendant risks and shortcomings, and not in a better position. The court's assignment is to extricate from the evidence the damages associated with the sexual abuse committed by Marson against the backdrop of K.M.'s other circumstances, and to do its best to determine what damages flow directly from the sexual abuse versus what non-compensable outcomes flow from other circumstances in K.M.'s life.

[527] The School Board submits that this is exactly what the judge did in the trial decision in *Blackwater*, 2001 BCSC 997. At paras. 327-28 of *Blackwater*, C.J.B.C. Brenner outlined the general principle in *Athey* that the plaintiff must be placed in the position he or she would have been in absent the tort, and explained that this requires consideration of the specific injuries caused by the sexual assaults, together with a measure of the loss to each plaintiff caused by the injuries. The School Board submits that K.M.'s counsel is trying to make the same argument as the plaintiffs in *Blackwater*—that *Athey* results in determining damages based on the entirety of the plaintiffs' psychological outcomes, without the challenging, daunting effort of extricating therefrom the specific damages flowing from the specific tort. This reading was rejected by Brenner C.J.B.C. Further, the court must keep in mind the onus on the plaintiff, always and throughout, to prove on a balance of probabilities that the psychological injuries for which damages are sought are, in fact, caused by the admitted sexual abuse.

[528] The School Board submits that while K.M.'s counsel spent a great deal of time discussing the evidentiary burden relating to the crumbling skull, we don't need to go there; it is not a relevant approach or consideration for the court in this case or in any case involving psychological damages flowing from sexual abuse. The appropriate methodology from *Blackwater* is to determine the plaintiff's original position, consider his current position given the occurrence of the sexual abuse, and determine the damages that flow from that difference. It is the School Board's submission that the trial decision in *Blackwater* is a useful roadmap in this case, particularly given that Brenner C.J.B.C.'s approach was approved of by the Supreme Court in 2005.

[529] The School Board relies on McLachlin C.J.C.’s statement in *Blackwater* at the Supreme Court: “Untangling the different sources of damage and loss may be nigh impossible. Yet the law requires that it be done” (para. 74). Counsel submits that K.M.’s submissions on causation are basically to the opposite effect of this statement and fail to distinguish between causation and damages assessment. The School Board agrees that it is liable if the tort is “a” cause of K.M.’s loss, but the next step in the process is the damages assessment, which is what this trial is about. The court’s challenge is to assess fair damages based on the untangling of multiple interlocking factors and confining the award to damages arising only from the sexual abuse committed by Marson. *Blackwater*, applying *Athey*, is what applies to this case.

[530] The School Board submits that if there is evidence of crumbling skull—as they submit there is in this case—it is a question to be put to the trier of fact. The original position of K.M. must be assessed, including not just circumstances that were obvious or manifest prior to the sexual abuse but all of the evidence, and particularly the medical evidence of Dr. Silver and Dr. Singh.

[531] In reviewing this case law, I find that the leading test for causation is the “but for” test, which requires the trier of fact to find that but for the defendant’s tort or negligence, the plaintiff would not have suffered the injuries in question: *Athey*, at para. 14. “But for” remains the appropriate test for negligence actions and can be applied in multi-cause injuries: *Resurface Corp. v. Hanke*, 2007 SCC 7, at paras. 21-23. K.M. bears the onus of proving causation on a balance of probabilities: *Athey*, at para. 13.

[532] If a defendant is found to be “a” cause of the injury, the presence of other, non-tortious causes does not reduce the extent of the defendant’s liability. Pre-conditions are not a basis for reducing liability. K.M. need not prove that the School Board’s negligence was the sole cause of his injuries; so long as the School Board is part of the cause of K.M.’s injuries, it is liable, even if its negligence alone was not enough to create K.M.’s injuries: *Athey*, at paras. 12-17.

[533] The School Board need only pay for the full amount of K.M.’s damages if the injury to which it contributed is indivisible, like the injury in *Athey*. Where a plaintiff has suffered divisible injuries—such as, paradigmatically, an injury to the arm and a subsequent injury to the foot—each defendant is only liable for the injury resulting from his or her own wrongdoing: *Athey*, at para. 24; *B.(M.) v. 2014052 Ontario Ltd.*, 2012 ONCA 135, at para. 40, leave to appeal refused, [2012] S.C.C.A. No. 213.

[534] The *Blackwater* trial decision considers these principles in the context of psychological injuries. At paragraph 370, Brenner C.J.B.C. adapted the general causation principles in *Athey* to the context of psychological injuries and sexual assault—an approach not questioned by the Supreme Court on appeal:

- (1) If the psychological injury would have occurred at the same time, without the injuries sustained in the sexual assault, then causation is not proven;

(2) If it was necessary to have *both* the sexual assaults and the other life circumstances for the psychological injury to occur, then causation is proven since the psychological injury would not have occurred but for the sexual assaults;

(3) If the sexual assaults alone could have been a sufficient cause, and the other life circumstances alone could have been a sufficient cause, then it is unclear which was the cause in fact of the psychological injury. The trial judge must determine, on a balance of probabilities, whether the defendant's sexual assault(s) materially contributed to the psychological injury. [Emphasis in original.]

[535] Once causation and liability are established, a separate inquiry considers the extent of the harm caused by the tort. While causation is concerned with the cause of the plaintiff's injury, the rules of damages focus on the plaintiff's loss and consider the plaintiff's original position absent the tort. A finding of causation results in responsibility for the entire injury, but a defendant need not put the plaintiff in a better position than his or her original one: *Blackwater SCC*, at para. 78; *B.(M.)*, at paras. 31-32; *Athey*, at paras. 32, 35. The court must therefore consider K.M.'s original position to assess the role of the sexual abuse in the loss suffered by K.M.

[536] Two principles assist in assessing K.M.'s original position and his damages: the thin skull principle and the crumbling skull principle.

[537] The thin skull principle makes the defendant liable for the plaintiff's full injuries, even if they were more severe than expected due to the plaintiff's pre-existing conditions or vulnerabilities. The tortfeasor must take the victim as he finds him: *Athey*, at para. 34.

[538] The crumbling skull principle recognizes that a pre-existing condition may have been inherent in the plaintiff's original position; a defendant "need not compensate the plaintiff for any debilitating effects of the pre-existing condition which the plaintiff would have experienced anyway": *Athey*, at para. 35. Damages should be reduced where there is a "measurable risk that the pre-existing condition would have detrimentally affected the plaintiff in the future, regardless of the defendant's negligence": *Athey*, at para. 35.

[539] If the expert evidence in a case raises no more than a "vague possibility" that the plaintiff's injuries would have been suffered in any event, this is not reasonable evidence of a "material risk" sufficient to trigger the crumbling skull principle: *B.(M.)*, at para. 59. In *B.(M.)*, the plaintiff had depression and post-traumatic stress disorder following sexual assaults, but had also suffered a prior assault, an abusive marriage, a therapeutic abortion or miscarriage, and a car accident (para. 11). The expert witnesses could only say that these other incidents "may have" caused or contributed to her injuries, but did not offer anything tangible with respect to the risk that she would have suffered her psychological conditions in any event (para. 57). The Court of Appeal agreed that on that evidentiary record, there was no need to leave the crumbling skull question with the jury.

[540] Similarly, in *John Doe v. O'Dell* (2003), 230 D.L.R. (4th) 383 (Ont. S.C.), Swinton J. declined to apply the crumbling skull principle to reduce the plaintiff's damages for sexual

assault, holding that there was insufficient evidence that he would have suffered his injuries absent the assaults; at most, he was a thin-skulled plaintiff (para. 308). The Court found, at para. 306:

We are dealing here with a child who was vulnerable and lonely. He was passing at school, with some difficulty in math and perhaps a circumscribed learning disability, but he was not presenting with behavioural problems. He was part of a loving and supporting family. He may not have been popular, with many friends, nor an athlete, nor an extrovert, but there is no evidence of pre-existing problems which were likely to cause the serious emotional and psychological damage that JD has suffered.

[541] Based on the preponderance of credible evidence which I accept, I find K.M.’s position on causation to be correct on the law and consistent with the evidence in this case. I find the sexual abuse of K.M. by Marson is a major cause of K.M.’s psychiatric injuries and disorders and that but for the abuse by Marson, K.M. would not have suffered his mental health injuries. I find that K.M. had no pre-existing conditions other than a learning disability, which did not contribute to his disorders and for which he claims no damages, and it does not trigger the application of the crumbling skull principle. The School Board’s position suggests that the crumbling skull exercise is inherent in the causation analysis when the plaintiff’s “original position” is considered, and that the evidentiary burden associated with proving the crumbling skull is not a relevant approach or consideration. The School Board wants to rely on the crumbling skull principle without having to prove it. I reject this. The evidence relied upon by the School Board, I find, falls far short of establishing that there was a “measurable risk” that other factors would have caused K.M.’s disorders even if the sexual abuse had not occurred. I find there is no cogent, tangible or credible evidence to that effect on the facts in this case.

[542] Both Dr. Singh, K.M.’s treating psychiatrist, and Dr. Silver agree in their evidence that the childhood assaults constituted “a” cause of K.M.’s psychiatric disorders. They agree that K.M.’s psychiatric disorders are not divisible. I accept Dr. Singh’s evidence and opinion that in psychiatric terms, while one can never say that anything is the cause of a disorder, the childhood sexual assaults constituted a major cause of the complex of psychiatric disorders affecting K.M., while not being the only cause.

[543] Dr. Silver seemed to be of the view that Dr. Singh’s opinion was that the sexual abuse was the only cause of K.M.’s psychiatric disorders, but that in fact was not Dr. Singh’s evidence. In cross-examination, Dr. Silver was asked, “are you contesting that the childhood sexual assaults were at least a part of the cause of what happened?” and answered, “I would say that childhood sexual assaults would certainly be an important factor in later development problems – yes”. The difference between the expert psychiatrists is one of degree.

[544] I accept and prefer the evidence of Dr. Singh over the evidence of Dr. Silver. I find that Dr. Singh has significant experience in treating adult survivors of childhood sexual abuse and had more opportunity to observe and examine K.M. as his treating psychiatrist and to determine K.M.’s diagnoses. Dr. Silver’s evidence was that he could not say that K.M. had a characterological depression, as he would have had to have seen K.M. many more times to try to

distinguish his depression. Dr. Singh, importantly, had that opportunity to see K.M. many more times. In my view, Dr. Silver was significantly negatively influenced in his evidence and opinions by his belief that the sexual abuse did in fact not occur (hence his numerous references to the “alleged” sexual abuse) and that he did not find K.M. to be credible. At the time of Dr. Silver’s assessment of K.M. on September 20, 2010, Marson had been criminally convicted of the sexual offences on K.M. since January 23, 2008, but significantly, the School Board that had retained him was still denying liability at the time.

[545] At trial, Dr. Silver seemed genuinely surprised to hear from counsel for the School Board, in a preamble to a question, that we know the sexual abuse occurred. Dr. Silver testified that he found out that the School Board accepted liability and that the sexual abuse had in fact occurred “very recently”. K.M. was sexually abused by Marson and I found K.M. to be a credible witness about his sexual abuse by Marson. I find Dr. Silver’s evidence was not objective and demonstrated partisanship in favour of the defendant School Board. I give Dr. Silver’s evidence little weight on the central issue of causation for which he was primarily retained. Dr. Silver did not believe K.M. at the time of his assessment, which I find is a material negative factor affecting Dr. Silver’s credibility, particularly on the central issue of causation in this case.

[546] I find that K.M. was sexually abused at age 13 and 14 by Marson. The nature of the sexual abuse by Marson is not disputed. Soon thereafter, K.M.’s personality and behaviour negatively changed and, I find, his mental disorders began. His sister S.G., his mother A.F., and his aunt M.I.C. all testified as to the changes they saw in him, evidence I accept as being credible, and that he was a totally different person. K.M. did not interact with the family anymore. He became withdrawn, and a loner. He took offence and angered easily. His sense of humour was gone. K.M.’s own evidence was that he felt extreme nausea during and after the assaults. He became angry and noticed differences in his mood.

[547] I find that by the time he was 15 years old and at Saint John’s school, depression had set in for K.M. and has never left him over the last 36 years. It was later diagnosed as dysthymia. He became anti-social and distrustful of other people, and developed a borderline personality disorder. He became a heavy drug user for a number of years, which declined after his son was born. He could not keep a job. He could not last in an interdependent spousal-type relationship. He has suicidal ideations and has attempted suicide. Post-traumatic stress set in and his overall mental condition and his ability to function, while phasic, continued to deteriorate.

[548] Y.G. was K.M.’s first love and knew K.M. well in the early years following the sexual abuse by Marson. I accept her evidence that K.M., from the age of 16 when they started out together, slept a lot and had mood swings, depression, and suicidal thoughts. He could not keep a job. He became a regular drug user and was admitted to a drug rehabilitation program in the fall of 1982 at 17 years of age, and he promptly discharged himself. Y.G. could not take living with him and his mood swings in particular.

[549] I find the sexual abuse has permeated all aspects of K.M.’s life and that the impact on him has been severe. I find it has been established on the evidence in this case that K.M.’s complex of psychiatric disorders began to develop soon after the sexual abuse by Marson occurred. These disorders include his depression, his drug abuse, his anti-social personality, his

difficulty with his interpersonal relationships, and his difficulty with employment and in keeping a job. These disorders became major depression with dysthymia, substance abuse, personality disorder of the anti-social and borderline type, difficulty in building interpersonal relationships, difficulty in keeping employment, and severe post-traumatic stress disorder. I accept and find credible the evidence of Dr. Singh that the mental disorders seen in K.M. including his depression, his drug abuse, his personality deficits, and his post-traumatic stress disorder, are the types of disorders commonly or regularly seen in adult victims of childhood sexual abuse. Dr. Singh's evidence in that regard was not challenged. I further accept and find credible Dr. Singh's evidence that K.M.'s abuse of drugs is a disorder, rather than just a choice K.M. made, and helped him cope with the depression and trauma associated with the sexual abuse. I find that but for the sexual abuse, K.M. would not have developed his mental health injuries.

[550] K.M.'s DSM axis GAF score, as assessed by different assessors over a span of 25 years, declined from a GAF score of 75 by Dr. Pytlak on August 15, 1990 (when K.M. was in the Kingston Psychiatric Hospital); to a GAF score of 70 by Dr. Schwarz on February 25, 1997 (when K.M. was in the Guelph, Kitchener-Waterloo area); to a GAF score of 65-70 by Dr. Ash on January 28, 2000 (when K.M. was admitted to Belleville General Hospital); to a GAF score of 30 by Dr. Singh (when he assessed K.M. on August 4, 2015). The declining GAF score is consistent with the other evidence in this case, and the evidence of Dr. Singh, that K.M.'s mental health continues to deteriorate. K.M.'s prognosis is guarded and poor.

[551] I find that K.M. has lost the ability to form a spousal-type relationship in the future and that his inability to sustain an interdependent relationship is the result of the sexual abuse by Marson and the mental health injuries it caused. I find that K.M.'s mood swings, drug abuse, depression, suicidal thoughts, mistrust, inability to keep a job, and lack of interaction with others arose as a result of the sexual abuse and made K.M. unable to sustain an interdependent relationship. I further find that the sexual abuse by Marson made K.M. angry, which at times crossed over into his domestic relationships. I find that the evidence of K.M., K.M.'s family and friend T.A. which I accept as credible, together with the evidence of Dr. Singh and Dr. Silver, supports a finding that K.M. will not be able to form a future interdependent relationship, as a result of the sexual abuse by Marson. I accept and find that K.M. is genuinely condemned to live alone. The sexual abuse by Marson, and its resultant mental health injuries, I find, took away K.M.'s ability to form a future interdependent relationship.

[552] I find that K.M.'s inability to sustain work as an adult is as a result of the sexual abuse by Marson. Dr. Singh and Dr. Silver noted that due to his mental disorders, K.M. could not stay employed in spite of being good at his work and likeable, because he was unable to be consistent. The sexual abuse by Marson, and its resultant mental health injuries, I find, took away K.M.'s ability to function and succeed in the workplace.

[553] I accept and find credible Dr. Singh's opinion that the childhood sexual abuse suffered by K.M. was a major cause of K.M.'s mental health disorders as an adult, but not the only cause. Dr. Singh's opinion is based upon K.M.'s symptoms, history, psychopathy, and response to treatment. K.M.'s response to treatment has not been very satisfactory. Other causes of K.M.'s injuries are K.M.'s inability to keep a job, his financial stress, his impulsive nature, his mistrust, his lack of relationships and his loneliness. While I find that these other causes are currently

present in K.M. as an adult, I find that his mental health disorders began with his chronic depression with dysthymia; these other more minor causes and psychopathies arose as a result of and compounded after the sexual abuse, and made his mental health disorders more severe, leading to the severe mental injuries that K.M. now has as an adult. I find that the short-lived diagnosis made by Dr. Conn of a somatoform disorder or hypochondriasis in K.M., from May to June 2003, is not a relevant factor in this case; it was withdrawn after only one month and is not supported by the other evidence which I find credible and reliable. Further, Dr. Conn was not aware of K.M.'s history of sexual abuse.

[554] Having found that the childhood sexual assaults constituted a major cause of the complex of psychiatric disorders affecting K.M. and that K.M. has satisfied the but for test on a balance of probabilities, K.M. need not establish anything more to find the School Board 100% liable for his injuries, subject only to the possibility of the “crumbling skull” exception. I find the “crumbling skull” exception does not apply in this case. I reject the evidence that prior to the sexual abuse by Marson, K.M. suffered injuries from traumas other than the sexual abuse by Marson, which had a measurable risk of contributing to K.M.'s mental disorders as an adult, even if the sexual abuse by Marson had not occurred, as being unsupported by the evidence which I accept as being credible.

[555] I find that K.M. and his family members did not have a history of depression or borderline anti-social personality disorder. His father D.M. had *myasthenia gravis* which affected his memory and mobility, and he was being treated by Dr. Conn, a psychiatrist, who later treated K.M. Dr. Conn was not supportive of D.M. and his being on disability from his work. Dr. Conn seems to have transposed those opinions onto K.M. As Dr. Conn was not aware of the history of K.M.'s sexual abuse, I give his evidence little, if any, weight. There is very little other reliable evidence as to the history of D.M., other than that he had a breakdown and had to leave work. Dr. Singh's evidence that borderline personality disorder has some genetic element is not sufficient, I find, to prove that K.M. would be pre-determined to have such a disorder manifest absent the abuse. Dr. Singh testified that depression may have a genetic component, but was clear that dysthymia or chronic depression like K.M.'s is not genetic. Other than putting it forward, even Dr. Silver's evidence did not ever appear to suggest any genetic pre-disposition that would have caused K.M.'s disorders to arise in any event, even if the assaults had not occurred—as opposed to just putting him more at risk or making him more susceptible to such conditions. I am satisfied that in this case, any potential genetic pre-determination in K.M. does not rise to the level of a measurable risk of K.M. developing his disorders in any event. There is no credible evidence of any pre-existing condition in K.M. or his family, or of genetically pre-determined depression or borderline anti-social personality disorder in K.M., I find, that goes beyond mere suggestion, conjecture, or speculation.

[556] There is no reasonable basis to find that K.M., at the time the abuse began, suffered from any pre-existing mental disorders or behavioural issues which created a material risk of causing or developing into the serious emotional and psychological damage that K.M. has suffered. K.M.'s learning difficulties, which were present from an early age, are not disputed and K.M. claims no damages in relation to his learning disorder. Importantly, I accept Dr. Singh's evidence and find credible that a person does not get dysthymia from a learning disability.

[557] K.M., I find, is not a particularly good historian in his damaged state, not only as to dates and ages of various times and events but as to his recollections of childhood—which varied as his mental health deteriorated. He told Dr. Schwarz in 1997 that he had good childhood memories. Dr. Schwarz found K.M.’s capacity to focus was impaired by his anxiety. Dr. Singh reported in 2009 that K.M.’s childhood was unhappy. K.M.’s recollections of his childhood are somewhat inconsistent, and phasic like his mental illnesses. I accept the evidence of K.M.’s family members as credible that K.M. was a happy kid and showed no signs of any mental disorders prior to the abuse by Marson.

[558] The evidence that I accept and find credible is from K.M. and K.M.’s family—his mother A.F., his sister S.G. and his aunt M.I.C—that as a young boy, before Marson, K.M. was a happy, normal, regular kid who loved to play sports and loved animals, and he had lots of friends, many of whom came over to his house regularly. He functioned well in the family and in the classroom. He liked the activities at school and he liked going to school. I find that any “indifference” or “devil-may-care” attitude noted by his grade 6 teacher and in A.F.’s evidence about his poor attitude and indifference were related to his difficulties in his academic work at school. K.M. struggled with his learning as a result of his learning disability and did not resent his two sisters being better at school than he was. K.M.’s mother helped him with flash cards and he did the best he could. He struggled with math and reading in particular, which admittedly was frustrating for him. K.M.’s learning disability, and the requirement that he work alone in the remedial reading room at school, made him vulnerable to the sexual abuse by Marson in that it provided Marson an opportunity to abuse him; however, I reject that K.M.’s learning disability made him vulnerable to the effects of the sexual abuse or pre-disposed him to his mental health injuries as an adult. There was no evidence of K.M. being depressed or anyone seeing any signs of depression or of any of the other psychological problems he developed after Marson in interacting with others. There are no medical or school records making references to these kinds of issues. The school records from grade 1 to March 1978 (K.M.’s second time in grade 6) make reference to K.M.’s problems with learning, but also report that K.M. was helpful, engaged, eager, a cheerful, happy boy, had a good sense of humour, and was making a good effort at school.

[559] K.M. does recall going to the principal’s office for being the class clown, as were other kids, but if he did, the school did not keep a record of it and his recollection could be somewhat overstated. I accept his evidence that he was never told he had any sort of behavioural disorder as a child. He was a big, tall, kid who I find could hold his own in the school yard. He has no record of truancy. I find that K.M.’s sister S.G. and mother A.F.’s evidence is consistent with K.M.’s in that he was a happy kid, had lots of friends, loved animals, and was not a behavioural problem, and that there were no signs of any mental health issues in K.M. before Marson.

[560] The medical records of K.M.’s family doctors when he was young are unremarkable. In 1977, K.M. was referred to the Edith Cavell Clinic for an assessment of his learning issues as he had just failed grade 6 for the first time. I find that his grade 6 failure was upsetting, particularly to his mother, who set about to get him help. I find and accept K.M.’s and his mother A.F.’s evidence that the clinic tried to help him with his learning issues, and that K.M. did not have behavioural issues before Marson.

[561] The School Board's position, which I reject as not being supported by the evidence, is that K.M. was already affected by psychological, psychiatric or behavioural issues before he was sexually abused by Marson, which would have led to K.M.'s psychiatric disorders later in life. In short, the School Board's position is that K.M. was damaged before the abuse by Marson and that he, in his original position, had pre-existing problems sufficient to cause a measurable risk that would have led to his difficulties and mental health injuries as an adult, even if the sexual assaults had never occurred.

[562] In support of their position, the School Board relies on the pre-Marson evidence of (1) the 1977 referral form where the phrase "childhood behaviour disorder" is used in reference to K.M.; (2) the 1999 social worker intake form where it is recorded that K.M. "has been depressed since age 10"; (3) that K.M. was a "blue baby" and had his umbilical cord wrapped around his neck at birth, which contributed to his learning difficulties which contributed to his mental health injuries later in life; and (4) that K.M. was a drug user in his second year in grade 6 and "messed with" marijuana and "bennies" (caffeine pills) in the year before any sexual abuse occurred by Marson. I will review these in more detail.

(1) The 1977 Referral Form

[563] The medical records of K.M. contain a form, dated July 13, 1977 when K.M. was age 11, from a clinic sent to Dr. Manderville, his family doctor, that states K.M. had a diagnosis of "childhood behaviour disorder" (Exhibit #1, Tab #A5). The School Board relies on this document as evidence that K.M. had more than merely a learning disability before being abused by Marson. The parties agreed at trial that this form did not come from Dr. Manderville or his office but from "some clinic". There is no evidence as to who authored the contents of this Ministry of Health form. I accept Dr. Singh's evidence and find credible that this form was an intake form used at the time and would have been created by non-medical personnel, and that the so-called diagnosis of "childhood behaviour disorder" was not made by a qualified physician. The point of this referral, as confirmed by other evidence, was to assess and assist K.M. with his learning issues as he had just failed grade 6. As such, I give the evidence of this form and its contents no weight on the issue of K.M. having such a disorder before the abuse by Marson.

[564] If K.M. had such a diagnosis, I find it would have been reflected in his medical records and made known to his parents. No other records state that K.M. had a "childhood behaviour disorder" and the evidence from K.M. and the family is otherwise to the contrary. I find that K.M. did not have a medically diagnosed "childhood behaviour disorder". I accept K.M. and his mother A.F.'s evidence that the referral to the Edith Cavell Clinic was to assist K.M. with his learning difficulties as he had just failed grade 6 at age 11, and nothing more. There was no evidence of psychological testing of K.M. having been done. I find that K.M. did not have general behavioural problems as a child before Marson and I accept the evidence of K.M., A.F. and S.G. as being credible in that regard. K.M.'s learning difficulties have never been denied.

(2) The 1999 Social Worker Crisis Intervention Centre Intake Form

[565] The Crisis Intervention Centre initial assessment report, dated December 22, 1999, records that when K.M. went to the hospital at age 34, he told the social worker that he had been

“depressed since age 10” (Exhibit #1, Tab #A35). The School Board relies on this evidence as establishing that K.M. had depression before the abuse by Marson. K.M. testified that when he went to the hospital, he was in a “hell of a mess”, not thinking straight, and was more worried about getting help than checking his journal. This statement was made during a crisis intervention in his life when he was very emotional after the breakup with his girlfriend C.P., and I find was more likely a mistaken recollection on his part, given in his damaged state. He was chronically depressed and had suicidal ideations at the time. I accept K.M.’s evidence that he was not thinking straight and he was “a couple of years off”.

[566] I accept K.M.’s evidence as credible that he was never depressed as a young child before Marson and that this “age 10” was a simple error as to his recollection of his age at the time and in the circumstances. I find that there is no evidence that K.M. “has been depressed since aged 10”. This is consistent with his medical history and records, and the evidence of K.M. and his family, which I accept as credible. I give this evidence no weight on the issue of K.M. having been depressed before the abuse by Marson. The preponderance of evidence, which I accept as being credible, is that K.M. was not depressed as a child before the sexual abuse by Marson and that K.M.’s depression began after Marson, when he was 15 years old, as testified to by K.M.

(3) The 1965 birth of K.M. as a “Blue Baby”, with the “Cord around his Neck”

[567] The School Board relies on Dr. Silver’s expert evidence given at trial that K.M. was a “blue baby” at birth and might have suffered damage at birth leading to his learning difficulties and causing his problems later in adulthood, including some of K.M.’s symptoms and disorders of behavioural problems, poor self-image and drug abuse. The evidence of the “blue baby” and/or of K.M. being born with a cord around his neck begins with the evidence of A.F., K.M.’s mother, and it seems to have ballooned from there. The evidence from her is that the doctor told her that “maybe he was a blue baby” (because of A.F.’s Rh negative blood type) and had he had the cord around his neck. According to A.F., K.M. was tested at birth to make sure he was not a blue baby, as those babies require a blood transfusion. There is no evidence that K.M. was given a blood transfusion at birth or that anything came of this testing. She denied that K.M. was deprived of oxygen at birth. There is no indication in any of the medical records of any trauma, problems or concerns with K.M. at birth whatsoever.

[568] Dr. Silver’s evidence was, in fact, that all we can say is that K.M. had learning difficulties. Given the lack of medical evidence of any damage at birth, the lack of any diagnosis in childhood, and the possibility that there was no damage (or that if there was, it may not be connected to the learning difficulties), it is a matter of speculation, not probability or possibility, to suggest that these issues had any connection to K.M.’s learning difficulties or his adult disorders.

[569] This issue first came up in K.M.’s application for O.D.S.P. when A.F. added a note to his application in 2001 about her wondering if these two things, the blue baby and the cord issue, impacted on K.M.’s life’s problems. As I have discussed earlier, I find this to be A.F.’s attempt at assisting her son in obtaining a disability pension, based on what she believed at the time. I find A.F. struggled to make sense of K.M.’s learning difficulties, as did K.M., and was doing

what she thought best to help her son. This idea was started by K.M.'s mother A.F. as a result of a doctor saying that K.M. "maybe" was a blue baby. Based on his mother's reports, K.M. adopted and repeated this information to others as being factually based and it shows up in his adult psychiatric medical documentation. While it may give both A.F. and K.M. a potential reason in their own minds for K.M.'s learning difficulties, it is not based on any factual medical reality. There is no evidence that either of these two issues ever existed or can be related to any of K.M.'s learning difficulties or to any of K.M.'s adult psychiatric disorders that followed later in his life after the abuse by Marson. This evidence is purely speculative and completely unsafe to rely upon. There is nothing in either the medical or school records about these issues.

[570] That K.M. was a "blue baby" and the umbilical cord was wrapped around his neck at birth—contributing to his learning difficulties, which contributed to his mental health injuries later in life—is, I find, another unsupported suggestion and speculation by the School Board. There is no medical evidence of any trauma of any kind at K.M.'s birth or aftermath. There was nothing in A.F.'s evidence about K.M. requiring resuscitation at birth. In fact, her evidence is to the contrary. It appears to have been a way for A.F. to explain K.M.'s difficulties with learning to others, as he did not learn like his two sisters did. K.M. was obviously told this as well and then came to believe it, and repeated it to others involved in his care. The problem is there is no evidentiary basis that K.M. was ever a "blue baby" or that a cord around his neck caused him any difficulties whatsoever. I reject this evidence as being supportive of the School Board's position that K.M. suffered from learning difficulties that led to behavioural and mental health problems and poor self-image prior to being abused by Marson. The preponderance of evidence, which I accept as being credible, is to the contrary. I give this evidence no weight on the issue of K.M. having suffered any such trauma before the abuse by Marson.

(4) K.M. was a Drug User before the Sexual Abuse by Marson

[571] The School Board relies on K.M.'s evidence at trial in cross-examination—that prior to the sexual abuse by Marson he had experimented or "messed" with drugs, namely marijuana and "bennies" (caffeine pills), in his repeat year of grade 6 at Harry J. Clarke School when he was age 12—as evidence that K.M. was habituated to street drugs and had a mental disorder of substance abuse prior to the sexual abuse by Marson. I accept and find credible K.M.'s evidence that his drug use was that of experimentation at age 12, prior to Marson. S.G., K.M.'s sister, was very close to K.M. prior to the abuse, and her evidence was that she was not aware of any experimentation with drugs by K.M. She was, however, very aware of K.M.'s drug use after Marson, as she could smell it.

[572] I find and accept as credible that, as testified to by K.M., he did experiment with marijuana and caffeine pills or "bennies" available at the drug store when he was in his second year of grade 6 at Harry J. Clarke (which I found was when he was age 12, before Marson). As I have found, K.M. is not a good historian as to dates and his age at various times and events. I find, as testified to by K.M., his drug use was nothing more than experimentation, and that K.M. was not habituated to drugs before Marson. I accept and find credible Dr. Singh's evidence that many people experiment with drugs, but most do not develop a diagnosed substance abuse

problem. I find that it was only after the sexual abuse by Marson that he developed a serious drug dependency, became a heavy drug user and habituated to drugs for a number of years, and had a substance abuse disorder. The preponderance of evidence in this case is that K.M. became habituated to drugs from age 13 as a way of coping with the sexual abuse which started in 1978 and lasted for approximately two years.

[573] K.M.'s substance abuse, I find, arose after and as a result of the sexual abuse, and was a method used by K.M. to cope with the sexual abuse. His drug dependency issues began in the last part of Harry J. Clarke, at Saint John's School and onto Moira Secondary School. At age 17, K.M. was referred to a drug treatment program in 1982 with the assistance of his family doctor. He checked himself out without completing the program. K.M. kept silent about the abuse until he came forward to the police in 2006, some 28 years later when he was 41. I find K.M. never expected that he would ever talk about the abuse and was sorry at times that he came forward. The stress of talking about these events and the legal process involved, I find, were particularly difficult for K.M. I am not surprised that his recollection as to his exact age at the time of various events is not always accurate. In spite of various ages being referred to, the evidence which I accept is that K.M.'s habituated drug use began after his encounter with Marson.

[574] I accept and find credible K.M.'s evidence that his heavy drug use was later in his time at Harry J. Clarke, at Saint John's School and then Moira Secondary School, and ballooned from there. I accept and find credible Dr. Singh's evidence that the sexual abuse was a major cause of K.M.'s drug abuse. Dr. Singh testified that many people experiment with drugs in their youth, but go on to have no problem with drug abuse in later life.

[575] I therefore reject this evidence of K.M.'s pre-Marson drug use as being supportive of the School Board's position that K.M. suffered from mental health issues prior to being abused by Marson, and in particular, that he suffered from substance abuse prior to the abuse. The preponderance of evidence from K.M. and his family, which I accept as being credible, is to the contrary.

[576] I therefore reject the above four areas of pre-Marson evidence as being supportive of the School Board's position.

[577] I find that there is no merit in, and I reject, the School Board's position that prior to the sexual abuse by Marson, K.M. suffered from any genetically pre-determined characterological or environmental depression, personality disorders, behavioural disorders, poor self-image, psychiatric disorders, trauma at birth, substance abuse or any otherwise pre-existing condition that would have caused his serious mental health injuries as an adult, even if the abuse by Marson had not happened. I find that the evidence relied upon by the School Board does not support a finding of a measurable risk that K.M. would have developed his mental disorders even if the sexual abuse had never occurred.

[578] I find that prior to the sexual abuse by Marson, K.M. was a normal, happy, intelligent child with a learning disability. Soon after the abuse, by age 15, the symptoms of that abuse began to materialize and have now worsened to become, over the many intervening years, the complex of serious mental disorders that K.M. suffers from as an adult at age 51 and from which

I find he will continue to suffer into the future. I find the sexual abuse by Marson is a major cause of K.M.'s mental health disorders and that but for the sexual abuse, K.M. would not have suffered his mental health injuries. K.M.'s mental disorders are typical of those suffered by adult survivors of childhood sexual abuse. The preponderance of evidence is that there is nothing else that could have caused K.M.'s mental health injuries without the sexual abuse. There is no plausible evidentiary basis that there was something else in K.M.'s circumstances that could give rise to K.M.'s disorders without the sexual abuse. I find that the sexual assaults by Marson are at least "a" cause of K.M.'s disorders, and further find that the sexual assaults are a major cause of K.M.'s disorders.

[579] As in *John Doe v. O'Dell*, I find that this is not a "crumbling skull" case. The evidentiary basis for this exception to 100% liability has failed to be established by the School Board. The preponderance of evidence does not support that K.M. is a "crumbling skull" who would have suffered these mental disorders absent the abuse. There was insufficient evidence that he would have suffered his injuries absent the assaults. Any evidence as to other pre-existing causes of K.M.'s disorders are purely speculative. It is not enough for the School Board to suggest that other factors "might" have contributed to K.M.'s disorders. The School Board has failed to provide any cogent or tangible evidence to establish a "measurable risk" that K.M.'s mental disorders would have developed even if the assaults had not occurred.

[580] The School Board submits that K.M. had post-abuse issues that contributed to his current complex of mental injuries. I find the evidence of corporal punishment and the time K.M. spent at Saint John's School (post-abuse)—suggested by the School Board as having affected K.M.'s adult mental disorders—is speculative and has not been proven on the evidence to have contributed to K.M.'s mental disorders. The School Board raised this issue in the cross-examination of K.M., but otherwise led no further evidence relating it to K.M.'s adult mental disorders. While K.M. acknowledged in his evidence that he experienced corporal punishment and while K.M. told Dr. Matveyev that he was required to wash dishes and sell honey door-to-door to support the school financially, I find that K.M.'s corporal punishment and time at Saint John's School has failed to be established on the evidence as having affected or caused K.M.'s mental health injuries as an adult, other than it happened and forms part of his history.

[581] K.M. has had a difficult life since the abuse of Marson. In the fall of 1989, K.M. first talked to his family doctor, Dr. Manderville, about his depression. K.M. could not handle additional situational life stressors such as his parents' divorce or problems with any work colleagues or employers or the breakups with his girlfriends. I find that these types of life stressors, in 1989 and in the years following, caused additional reactive depression to K.M.'s existing depression, later diagnosed as dysthymia. This made life even more difficult for K.M., but I find they were not the basis of his mental health injuries.

[582] In summary, I find that K.M. has proven on a balance of probabilities that the sexual abuse by Marson on K.M., liability for which is admitted by the School Board, is a major cause of K.M.'s adult mental disorders. I find that but for the sexual abuse, K.M. would not have suffered his current complex of mental injuries. I further find that the School Board has failed to establish that K.M. suffered from any pre-existing condition prior to the sexual abuse by Marson which gave rise to a measurable risk that K.M. would have developed the mental disorders he

has in adulthood, even if the sexual assaults had not occurred. The “crumbling skull” exception therefore does not apply here. I further find that any suggestion of post-abuse issues having contributed to K.M.’s injuries has not been made out on the facts of this case.

[583] In the result, I find the School Board is liable for 100% of K.M.’s damages.

15. General Non-Pecuniary and Aggravated Damages:

[584] K.M. claims significant general damages for the consequences of his sexual abuse by Marson. The impact of sexual abuse can be devastating and severe. Its effects on a particular victim can vary significantly.

[585] K.M. claims \$250,000 in general non-pecuniary and aggravated damages. K.M. submits that the courts have determined a range, rising with inflation, of \$125,000 - \$250,000 in this sort of case and cites *G. (B.M.) v. Nova Scotia (Attorney General)*, 2007 NSCA 120, and *Evans v. Sproule*, [2008] O.J. No. 4518 (S.C.), as support for this range. He submits that where you are placed in that range depends on the evaluation of a number of factors in each case. Factors to be considered in the damage award are: the circumstances of the victim at the time, the nature of the assaults, the circumstances of the defendant, and the consequences for the victim. In K.M.’s submission, the most significant element is the consequences for the victim. K.M. argues that the simple reality is that a given assault or level of intrusiveness may cause one level of damage in one victim, and another level of damage in another. The courts’ objective in these cases is compensation. Once that objective is identified, the focus has to be on the consequences and asking: what did the sexual abuse do to this victim? K.M. submits that his damages should fall at the upper end of the range because the consequences for K.M. have been at the serious end compared to other cases.

[586] K.M.’s counsel reviewed K.M.’s circumstances that relate to the determination of his damages. First, as a child of 13 to 14 years old, K.M. was quintessentially vulnerable to a predator like Marson. Second, the nature of the assaults was, “on the scale of awfulness”, not as bad as some of the other cases disclosed. Third, Marson was in a position of trust and authority, and betrayed the very core of his obligations as a teacher to K.M. Fourth, the consequences for K.M.—the severity of symptoms, duration, and the extent to which they have impacted on his ability to function, earn a livelihood, and maintain domestic relationships—are at the higher end. It is K.M.’s position that the court’s appreciation of the terrible consequences of the abuse of children by those expected to protect them has been growing over the years, and with a realization that earlier damages may have failed to adequately reflect the magnitude of those consequences.

[587] The School Board’s position is that general damages for K.M. should fall in the \$100,000 - \$125,000 range. Counsel for the School Board submits that the four factors outlined by K.M.’s counsel have been approved as guidelines in the case law, but they are not all-inclusive; they are simply useful to consider. The School Board provided a sample of cases that it submitted were relevant to the damages assessment in this case. In *B. (A.) v. D. (C.)*, 2011 BCSC 775, a female grade 12 student was involved in seven incidents of sexual touching by her teacher, touching that was not particularly invasive and did not include force or threats of physical harm. General

damages were assessed at \$50,000. In *M. (D.F.) v. D. (J.)*, [2000] O.J. No. 559 (S.C.), a female victim was sexually abused by her uncle between ages 12 and 15 years old, involving fondling, digital penetration, masturbation and oral sex. The victim was, 25 years later, still fearful, trusted very few people, and had panic attacks and difficulty controlling her temper and anger. She was awarded \$75,000.

[588] The School Board relies on *H. (S.G.) v. Gorsline*, 2001 ABQB 163, where a female plaintiff was sexually abused by her physical education teacher for about a year, over the same time frame as K.M.—grades 7 and 8. The plaintiff received \$110,000 in general and aggravated damages. The School Board’s counsel submits that *Gorsline* sounds pretty close to K.M.’s case, though admittedly it took place in 2001 and the acts appear more invasive than K.M. had to endure. It is the School Board’s position that appropriate general damages for K.M. are in the \$100,000 - \$125,000 range.

[589] The evaluation of non-pecuniary damages is “a philosophical and policy exercise more than a legal or logical one”: *Andrews v. Grand & Toy Alberta Ltd.*, [1978] 2 S.C.R. 229, at p. 261. In sexual assault cases, a functional approach to non-pecuniary damages takes into account the award’s capacity to provide solace to the victim, to vindicate his or her physical autonomy and dignity, and, through aggravated damages, to take account of the humiliating and degrading nature of a defendant’s conduct: *G. (B.M.)*, at para. 120. An important function of non-pecuniary damages in this case is to demonstrate, to K.M. and to the broader community, the vindication of his fundamental and intangible rights that were violated by Marson: *G. (B.M.)*, at para. 130.

[590] General or non-pecuniary damages may be augmented in certain cases by aggravated damages, for example where the defendant’s conduct is especially high-handed, oppressive, humiliating, or undignified and thereby increases a plaintiff’s distress and injury. Aggravated damages are not awarded separately or in addition to general damages; rather, general damages are assessed taking into account any aggravating features of a case and to that extent increasing the amount awarded: *Norberg v. Wynrib*, [1992] 2 S.C.R. 226, at p. 263; *M.(D.) v. W.(W.)*, 2013 ONSC 4176, at paras. 122-23. While aggravated damages will frequently cover conduct which could also be the subject of punitive damages, they can be distinguished; punitive damages are designed to punish whereas aggravated damages are designed to compensate: *Norberg*, at p. 264. As I have said earlier, K.M. did not pursue his claim for punitive damages.

[591] In *Blackwater SCC*, at para. 89, the Supreme Court approved the factors relied on by Brenner C.J.B.C. when assessing non-pecuniary damages in that case (*Blackwater BCSC*, at para. 398 ff). Subsequent cases have confirmed that the *Blackwater* factors include (*Lucas v. Prince*, 2015 ONSC 7450, at para. 26; *M.(D.)*, at para. 128; *G. (B.M.)*, at para. 134):

1. The circumstances of the victim at the time of the events, including factors such as age and vulnerability;
2. The circumstances of the assaults, including their number, frequency and how violent, invasive and degrading they were;
3. The circumstances of the defendant, including age and whether he or she was in a position of trust; and

4. The consequences for the victim of the wrongful behaviour, including ongoing psychological injuries.

[592] K.M. and the School Board are agreed that these factors are to be considered.

[593] As explained in *M.(D.)*, at para. 129, “[n]umerous decisions have emphasized that the last identified factor, (gravity of damage caused to the particular victim), is by far the most important factor that the court must consider”: see e.g. *C.(J.C.) v. Keats* (1995), 133 Sask R. 65 (Q.B.), at para. 15. Other cases have similarly focused on the injuries suffered by the plaintiff: see e.g. *R. v. A.(W.)*, 2003 ABQB 50, at paras. 128, 132; *M.(K.M.) v. Roman Catholic Episcopal Corp. of the Diocese of London in Ontario*, 2011 ONSC 2143, at paras. 86-90. A focus on the consequences of the abuse reflects how general damages are an attempt to compensate for and substitute what has been lost by a particular victim. On the particular facts of this case, I agree that the consequence of the sexual abuse on K.M. is the most important factor.

[594] Case law helps to determine the range of general or aggravated damages that have been awarded in cases of sexual abuse with a similarly devastating impact on the plaintiff. As explained in *Andrews*, “[t]he award must be fair and reasonable, fairness being gauged by earlier decisions; but the award must also of necessity be arbitrary or conventional” (p. 261). The damages awarded in prior cases “must be viewed flexibly in future cases in recognition of the inevitable differences in injuries, the situation of the victim, and changing economic conditions” (p. 263). In *G. (B.M.)*, Cromwell J.A. (as he then was) explained that this assessment proceeds as a “‘horizontal comparison’, that is, by determining from the case law a range of acceptable awards and then placing the present case within that range” (para. 136). When assessing the appropriate range of damages, the court should identify the important characteristics of the case in order to define the types of cases to be considered in establishing the range. Then, the court should review the cases of this sort and determine the range of awards that have been made (paras. 138-40). Cromwell J.A. agreed with the trial judge that the range of damages for sexual abuse in that case was between \$125,000 and \$250,000—a range now over ten years old.

[595] Counsel have referred me to a series of cases to help establish the range in this case. I have focused on and considered those cases involving sexual abuse and a plaintiff who still suffers significantly from such abuse. The core characteristic in this case is the effect of the sexual abuse on K.M. Damage awards for childhood sexual abuse that has resulted in depression, post-traumatic stress disorder, or borderline personality disorder fall within the \$50,000 - \$285,000 range, subject to being adjusted upwards for inflation as discussed below. The following are awards made in those types of cases, unadjusted for inflation. See: *A.(D.A.) v. B.(D.K.)* (1995), 27 C.C.L.T. (2d) 256 (Ont. Gen. Div.) (\$100,000 in general and aggravated damages); *L.M.N. v. M.(M.J.)*, [1998] B.C.J. No. 2591 (S.C.) (\$175,000 in general damages); *H.(S.G.) v. Gorsline*, 2001 ABQB 163 (\$110,000 in general and aggravated damages); *R. v. A.(W.)*, 2003 ABQB 50 (\$285,000 in general damages); *H.(C.) v. British Columbia*, 2003 BCSC 1055 (\$135,000 in general and aggravated damages); *John Doe v. O’Dell* (2003), 230 D.L.R. (4th) 383 (Ont. S.C.) (\$175,000 in general and aggravated damages); *B.(L.) v. M.(W.)*, 2003 BCSC 261 (\$125,000 in non-pecuniary damages); *S.A.D. v. E.E.P. Estate*, 2003 BCSC 1535 (three plaintiffs awarded \$180,000, \$100,000 and \$120,000 in general and aggravated damages); *K.(K.) v. G.(K.W.)* (2006), 40 C.C.L.T. (3d) 139 (Ont. S.C.) (\$100,000 in general and aggravated

damages), varied on other grounds, 2008 ONCA 489; *G. (B.M.) v. Nova Scotia (Attorney General)*, 2007 NSCA 120 (\$125,000 in non-pecuniary and aggravated damages); *B.(A.) v. D.(C.)*, 2011 BCSC 775 (\$50,000 in non-pecuniary damages); *M.(K.M.) v. Roman Catholic Episcopal Corp. of the Diocese of London in Ontario*, 2011 ONSC 2143 (\$190,000 in general and aggravated damages); *Milne v. Betts*, 2012 ONSC 5565 (two plaintiffs awarded \$225,000 and \$250,000 in general and aggravated damages); *M.(D.) v. W.(W.)*, 2013 ONSC 4176 (\$95,000 in general and aggravated damages); *Lucas v. Prince*, 2015 ONSC 7450 (\$175,000 in non-pecuniary damages).

[596] K.M. in addition relies on *Langstaff v. Marson*, 2013 ONSC 4246, a case involving this same defendant where a civil jury awarded \$347,293 in general and aggravated damages. This damage award was the maximum amount of the \$100,000 *Andrews* cap adjusted for inflation to January 2013 when the case was heard. The Ontario Court of Appeal in *Langstaff v. Marson*, 2014 ONCA 510, set aside the entire jury verdict and the subsequent judgments by Tausendfreund J. and ordered a new trial. The matter then settled. I find it is not appropriate to rely on this jury verdict damage award for two reasons: firstly, the award has been completely set aside, and secondly, even if it was not set aside, the evidentiary and factual basis for the award is unknown as a jury is not required to give reasons. I have not considered the award of damages in the *Langstaff* case in my assessment of the range of damages for this type of case or in my analysis of what is the appropriate measure of damages for K.M.

[597] Since this case was argued in September 2017, two cases have been decided with claims similar to those K.M. makes as an adult victim of childhood sexual abuse. The first case is *John Doe (G.E.B. #25) v. The Roman Catholic Episcopal Corporation of St. John's*, 2018 NLSC 60, which was released on March 16, 2018 and involved four plaintiffs, identified by numbers, claiming damages for abuse during their childhood while living in an orphanage funded by the Catholic church. There were several other actions before the Court involving other men who were abused during the same period at the same orphanage. Those actions were held in abeyance pending the outcome of the *G.E.B.* proceeding. Despite not being formal representatives or a formal class action, these four plaintiffs' cases were put forth as being somewhat representative of the issues and damages that would arise in all of the claims and to provide a precedent for resolution of the other outstanding actions. *G.E.B. #25* suffered long-lasting effects of the sexual assaults into his adulthood, but was not impaired to the extent that he was unable to follow a long and relatively successful military career; he was awarded \$125,000 in general and aggravated damages. *G.E.B. #26* experienced many of the symptoms of P.T.S.D. and was awarded \$240,000 in general and aggravated damages. *G.E.B. #33* was variously diagnosed with alcoholism, depression, and P.T.S.D., and the Court found him to be "a profoundly unhappy man" whose sexual abuse had materially contributed to the problems in his life (para. 584); he was awarded \$325,000 in general and aggravated damages. *G.E.B. #50* suffered more minor symptoms, which were attributed more to physical abuse and other traumas than sexual abuse, and he was awarded \$60,000 in general damages. Based on the totality of the case law including *G.E.B.*, I therefore find that the range of non-pecuniary damages for cases similar to K.M. is \$50,000 - \$325,000, unadjusted for inflation.

[598] The second case, *MacLeod v. Marshall & the Basilian Fathers of Toronto*, Toronto, CV-13-481822 (Ont. S.C.), was decided by a civil jury on April 26, 2018. The jury awarded a total of

\$425,000 in general and aggravated damages, the current highest award for this type of claim. Again, as in *Langstaff*, the evidentiary and factual basis for the award is unknown as a jury is not required to give reasons. I have not considered the award of damages in the *MacLeod* case in my assessment of the range of damages for this type of case nor in my analysis of what is the appropriate measure of damages for K.M.

[599] *Andrews* established a general upper cap of \$100,000 for non-pecuniary damages, though acknowledged that “exceptional circumstances” could push such an award higher (p. 265). The Court of Appeal for Ontario has left it open as to whether the *Andrews* cap applies to sexual assault damages: see *B.(M.) v. 2014052 Ontario Ltd.*, at para. 77; *Padfield v. Martin* (2003), 64 O.R. (3d) 577 (C.A.), at para. 29. Regardless, the Supreme Court has confirmed in subsequent cases that this \$100,000 limit should be subject to increases based on the effect of inflation: *Lindal v. Lindal*, [1981] 2 S.C.R. 629, at p. 641; *ter Neuzen v. Korn*, [1995] 3 S.C.R. 674, at para. 104. As I have discussed, the range I determined above of \$50,000 - \$325,000 has not been adjusted for inflation.

[600] The approaches to calculations used by the courts to take inflation into account have varied. More recent cases have referred to the Bank of Canada’s Inflation calculator: see e.g. *Ellsworth v. Singer*, 2016 ONSC 4281, at para. 293; *Camilleri v. Brunet*, 2016 ONSC 7312, at para. 16. Older personal injury cases have assessed inflation by using the Consumer Price Index (CPI): see e.g. *Overholt v. Stratford (City)* (1996), 1 O.T.C. 161 (Gen. Div.); *Mozersky v. Cushman* (1997), 48 O.T.C. 161 (Gen. Div.). In reality, the two approaches are the same if the CPI calculations are done accurately.

[601] I use the *Andrews* case based on the month of June 2018 as an illustrative example. The first method, the Bank of Canada Inflation Calculator, uses the CPI to show changes in the cost of a fixed “basket” of goods. The Calculator uses the most recent month for which the CPI data is available (in June 2018, the most recent CPI data is April 2018) and then compares that same month in the historical year. According to the Inflation Calculator, the \$100,000 upper limit in *Andrews* (the \$100,000 “basket”) in April 1978 would cost \$372,346.37 in April 2018.

[602] The second method is to use the CPI tables directly. Each month, Statistics Canada calculates the purchasing power of a dollar against the current base year period (currently, 2002 = 100), and these can be compared against historical summary tables of the Canadian CPI that are published online. The purchasing power of a sum of money between any two years can be compared by dividing the CPI of the later year by the CPI of the earlier year, then multiplying this by the sum of money. For example: 133.3 (the CPI in April 2018) $\div$ 35.8 (the CPI in April 1978) = 3.72346369 $\times$ $\$100,000$ = $\$372,346.37$, the same sum as calculated under the Bank of Canada Inflation Calculator in the first method, above. Since the Bank of Canada Inflation Calculator uses monthly figures in its calculation and not yearly averages, the monthly CPI historical tables must be used to reach the same figure as the Bank of Canada’s inflation calculator for a given month.

[603] Applying inflationary increases to the range of cases provided above, the inflation-adjusted range of non-pecuniary damages for cases similar to K.M. is \$55,634.39 to \$371,000.98 in general and aggravated damages (based on the \$50,000 award in 2011 in *B.(A.) v. D.(C.)* and

the \$285,000 award in 2003 in *R. v. A.(W.)*). These figures were calculated using the Bank of Canada Inflation Calculator (which used the most recent available figures, from April).

[604] While comparison with other cases is helpful as a rough guide, it is damages for the impact on K.M. and the particular circumstances of his sexual abuse that must be assessed in this case. I have considered several factors in assessing K.M.'s non-pecuniary damages. I have considered that general damages in sexual assault cases may include an element of aggravated damages. K.M. was a vulnerable victim because of his learning disability and an easy target for a predator like Marson. He was required to be isolated within the school setting to work on his learning issues and he was left essentially unsupervised. K.M. was required to attend school or face punitive consequences. He was a young, sexually inexperienced boy of 13 and 14 years of age. Marson was much older and was K.M.'s science teacher and in a position of trust and authority over K.M. He used K.M. for his personal sexual gratification. K.M. felt no-one would believe him and he felt trapped. I find and accept K.M.'s evidence that he was painfully ashamed, terrified, and fearful every time he would go to school, and he had no control over what was happening to him. He endured living with the secret of the sexual abuse until external events brought them to light. The assaults happened well over 100 times over his clothing, and there were five acts of forced oral sex. Marson exercised physical control over K.M. by pinching his neck and shoulder to maintain control of him. The sexual abuse was invasive and degrading to K.M. and he did everything he could to stay away from Marson outside of the school setting, to protect himself as best he could.

[605] The effects of the sexual abuse on K.M. I find were severe and life-altering. The sexual abuse has drastically interfered with K.M.'s enjoyment of life, his ability to work and his ability to form and maintain satisfying, meaningful and stable relationships with his family, friends and domestic partners. K.M. has endured a severe degree of suffering in his adult life as a result of the abuse. His mental health prognosis is poor, and unlikely to improve in the future.

[606] Since the abuse, K.M. has endured a life—from being a teenager at 13 years of age into adulthood at 51 years of age, a period of 38 years—of shame, guilt and degradation. He felt the abuse must have been his fault. He feels guilty, dirty, ashamed and depressed. He feels he has had his childhood stolen from him. He wishes he had never come forward about the details of the abuse. He has suffered the symptoms of chronic depression, dysthymia, substance abuse, post-traumatic stress disorder, personality disorder of the borderline anti-social type, and the economic and social consequences of his inability to sustain employment or an interdependent relationship. He feels inadequate and inferior. He has had a life since the abuse of misery, emptiness, and loneliness. He has contemplated and attempted suicide as his “out”, to make his suffering go away. He lives under the constant “cloud” of depression that never leaves him. He lives with the ups and downs of his mental disorders. The downs can last for days, requiring him to isolate himself from others in order to cope. Medical marijuana offers him some relief, but when he is really bad, not even that helps.

[607] K.M. has lost his close family relationships, which are not the same as they were before Marson, particularly with his sister S.G. He is no longer her hero and contact between them is minimal compared to what it used to be. He cannot live with anyone else. No-one could take his moodiness, his inability to get along with others, his lack of interest in sex, and his lack of trust

in others for long. The consequences of relationship failures are life-threatening for K.M. He feels he does not deserve happiness like everyone else does.

[608] K.M. has endured years of mental health treatments, assessments and drug therapies, all of which have eventually failed. He has been hospitalized for investigations. His condition is treatment resistant. I cannot fault him at this point for not trying any more therapeutic drug therapies or counselling. His son M.M. and the love of his mother have kept him alive, but it is a life full of difficulties and challenges, lived day to day, with no expectations for tomorrow. He is genuinely trapped in his own misery.

[609] I have considered the four specific factors in *Blackwater v. Plint* and find as follows:

- (1) the circumstances of the victim at the time of the events, including factors such as age and vulnerability;

K.M. was a young teenage boy of 13 and 14 years of age when he was sexually abused, and K.M. was vulnerable to a sexual predator like Marson. His first sexual erections were with Marson.

- (2) the circumstances of the assaults, including their number, frequency and how violent, invasive and degrading they were;

The sexual assaults of rubbing K.M.'s privates over his clothing happened well over 100 times and there were five acts of forced oral sex where Marson pinched K.M.'s neck and shoulder to maintain physical control over K.M., all taking place over a protracted period of time of about 2 years, at his school.

- (3) the circumstances of the defendant, including age and whether he or she was in a position of trust;

Marson was K.M.'s teacher, a lot older than K.M., and in a position of trust and authority over K.M. Marson's sexual abuse for his personal sexual gratification betrayed the very core of his obligations and responsibilities as a teacher to K.M.

- (4) the consequences for the victim of the wrongful behaviour, including ongoing psychological injuries.

The consequences of the sexual abuse on K.M. were severe and life-altering. It has affected every aspect of his daily existence and his ability to function. His mental health disorders are serious and his prognosis is guarded and poor. The abuse has severely negatively impacted his ability to live a normal life and his interactions with others and his family. He lives a life full of depression, loss, emptiness, isolation and loneliness. He is unable to work as a labourer and cannot cope with a regular job where he has to be relied upon to show up for work. His mental health disorders have made him unable to live with anyone else and form and maintain a domestic relationship. He has been forced to live alone because of his mental disorders, and he is condemned to live alone in the future. The abuse has ruined his life and how he relates to other people.

[610] In light of these factors, I find that K.M.'s general and aggravated damages, placed on a horizontal comparison, are required to be at the higher end of the range. K.M. has established that he is entitled to a significant award of general and aggravated damages on a balance of probabilities for the abuse that he has suffered. While the evidence is that the sexual abuse itself was at the lower end of the intrusive scale, the acts of fondling and forced oral sex over a two-year period were humiliating and degrading, and I find the impact on K.M. was severe. I find that the damages in this case would not exceed the *Andrews* cap once adjusted for inflation. I therefore assess K.M.'s general and aggravated damages at \$250,000.00. Any lower award, I find, would ignore the severe impact of the sexual abuse on K.M. by Marson, K.M.'s grade 7 and 8 teacher, while K.M. was at school for Marson's sexual gratification.

16. Damages for Past and Future Income Loss:

[611] K.M. claims significant economic loss damages for his past and future income loss or, more accurately, loss of his income earning capacity from age 18 to 64. Income loss is a matter of assessment of damages, not a matter of strict mathematical calculation, and, by its very nature on the particular facts of this case, is based on hypothetical events. Damages for loss of income are compensation for the loss of the ability to earn income, and the damages should, so far as any amount of money can, restore K.M. to the position he would have been in but for the sexual abuse by Marson. As K.M. was very young when he was sexually abused from 1978 to 1980 and before he had a chance to enter the workforce, the court must make a finding of what his likely career track would have been, and what K.M. would have been able to earn, had he not been assaulted. All of K.M.'s limited earnings from age 18 to 51, when he could work, were earned when he was in his damaged state after the abuse.

[612] Assessing income loss is often an inherently speculative exercise, particularly where the plaintiff had not yet begun his or her career at the time of the tortious events. A pecuniary damages award should not require the School Board to pay for damages that K.M. would have suffered in any event or place him in a better position than he would have been. Rather, the amount for which the School Board is liable is dependent on K.M.'s likely path had he not been abused: *Lucas v. Prince*, at para. 43.

[613] Assessment of damages for loss of earning capacity involves estimation of the value of work that would have been performed (from which the value of the work the plaintiff may now perform is subtracted); determination of the period during which the loss will continue; a consideration of fringe benefits; determination of the appropriate discount rate and the effect of general productivity; adjustment for contingency; and consideration of any overlap in recovery or deduction of collateral benefits. There does not appear to be dispute that the claim for loss of income is valued as the difference between: (1) the income that the plaintiff probably would have earned had the injury not occurred (the "without incident" scenario referred to by the economists), and (2) the income that the plaintiff has earned, and probably will earn in the future, given the effects of the injury (the "with incident" scenario). Both expert economists in this case have factored in K.M.'s actual earnings in their calculations and have assumed zero income for K.M. from 2004 onwards.

[614] Past lost income, in this case, is from age 18 to K.M.'s age at the time of trial, 51. K.M.'s future loss of income, in this case, is his projected income from age 51 at the date of trial to age 64, his agreed-upon age of retirement. The plaintiff must prove past pecuniary loss on a balance of probabilities: *Basandra v. Sforza*, 2016 ONCA 251, at para. 24. However, hypothetical events (such as how the plaintiff's life would have proceeded without the tortious injury) and future events do not need to be proven on a balance of probabilities but rather are given weight according to their relative likelihood: *Athey*, at para. 27. Future income loss can be proven on the basis of a real and substantial possibility or risk, and compensation for future loss is "not an all-or-nothing proposition"; it depends in part on the degree of risk of loss that is established, as well as on the possibility, if any, that a plaintiff would have suffered some or all of those projected losses even if the wrong done to her or him had not occurred: *Graham v. Rourke* (1990), 75 O.R. (2d) 622 (C.A.), at p. 634; see also *Athey*, at para. 27.

[615] K.M.'s sexual abuse by Marson has prevented him from being able to obtain or maintain employment. His ability to work and have a career track, I find, was harmed by the sexual abuse. K.M.'s mental health injuries are of such a nature that he cannot be relied upon to be dependable in the workplace. His mood swings, irritability, anger, and difficulties interacting with other people would make him an undesirable employee in the workplace. I accept and find credible Dr. Singh's opinion that K.M. is not employable and is unable to work. This evidence is consistent with that of K.M., his mother A.F. and his aunt M.I.C. I find that he is unable to work or hold a regular job in the future, and that K.M. is unsuitable for any regular type of employment, even part-time. I find that K.M. is permanently unemployable and that, as his mental condition continues to deteriorate, I find his income will be zero going forward as a result of the sexual abuse by Marson. I find K.M. does not have the ability to work and hold down a regular job as a result of sexual abuse by Marson. His mental health issues and medical diagnoses, as outlined by Dr. Singh, which I accept, prevent him from working. As I have found earlier, the "crumbling skull" exception does not apply to the facts of this case and I reject the 50% reduction sought to K.M.'s total income loss claim by the School Board. I find that K.M. has proven on a balance of probabilities that his total loss of income, to be quantified, was caused by the sexual abuse by Marson, and I find that K.M. is therefore entitled to have his damages assessed for that loss.

[616] K.M.'s work history has been sporadic since he left school at age 16. His last earned income was in 2003. The preponderance of evidence is that, absent the abuse, K.M. would not have graduated from high school or achieved his high school equivalency because of his learning disabilities. I find, however, that K.M. is an intelligent man and but for the abuse, he would have worked as a general labourer for his entire working career in the construction field, and he would have had stable employment. Reference to the educational level of his siblings and his parents is of limited value due to K.M.'s learning disabilities, however I find that but for the sexual abuse, K.M. would have had a strong work ethic like his siblings J.C. and S.G., his mother A.F. and, for the most part, his father D.M. who was a good worker and provider until he became unwell. While some adult survivors of sexual abuse are able to carry on with some form of employment, this is not the case for K.M. The sexual abuse by Marson was too much for K.M. to bear and he has struggled since age 16 in all aspects of his life, and in particular with employment. This case, I find, is not one where K.M. is trying to avoid work. This is a case where K.M. is unable to work because of the sexual abuse by Marson, which in turn has caused K.M.'s substantial past

and future income losses. The court must now determine K.M.'s most probable without incident earnings from age 18 to 64.

[617] A series of expert reports of economists Professor Jack Carr and Professor Michael Charette were filed as part of the Experts' Report Brief (Exhibit A, Tabs #3-4) for the assistance of the court and for purposes of cross-examination. Professor Carr's most recent substantive report is dated March 15, 2016. Professor Charette's most recent substantive report is dated September 23, 2016. Both experts were qualified by the court to give expert opinion evidence in the field of economics, including the evaluation of the economic loss sustained by individuals as a result of the impairment of their capacity to earn income and their capacity to form and maintain an interdependent relationship (discussed below). Professor Charette was additionally qualified by the court, as requested by counsel, to give opinion evidence as a labour economist and econometrist. Professor Charette acknowledged that he had no training in doing psychological or learning disability assessments on individuals as a psycho-vocational assessor to assess the effect of a learning disability, and agreed that he had no expertise to assess the impact of a learning disability on K.M.'s intellectual capacity to work. As I discuss further below, I find that Professor Charette is not qualified to give the opinion he did as to whether or not K.M. would graduate from high school or to assess the nature of K.M.'s disabilities and their effect on K.M.'s pecuniary earnings. In contrast, Professor Carr's evidence was that no economist has the expertise to judge or assess the effect of learning disabilities, of whatever kind, and how those disabilities would affect pecuniary earnings or an individual's productivity or working potential. Therefore, he did not take into account any learning disability on the part of K.M. in his income loss analyses.

[618] Professor Carr's analysis and economic loss evidence is as at the original trial date of May 9, 2016. He testified that the total amounts would not be that much different as at March 20, 2017. Professor Charette's analysis and economic loss evidence is as at the start of this trial on March 20, 2017. Both Professor Carr and Professor Charette's analyses are from age 18 to the same retirement age of 64, as provided by Statistics Canada.

[619] K.M. claims \$2,344,168 in damages for his total income loss, and it is his position that this figure is valued as the difference between (1) the income K.M. probably would have earned had the injury not occurred (the "without incident" earnings) and (2) the income K.M. has earned and probably will earn in the future given the effects of the injury (the "with incident" earnings). He submits that expert witnesses are generally required to quantify the loss, but their opinions on matters that are common-place or on which the expert has no special education, training or experience will be inadmissible and, in any event, accorded no value.

[620] Here, K.M. submits that the "with incident" earnings can be calculated fairly easily based on his tax returns and assuming zero income moving forward. For the "without incident" earnings, K.M. states that it does not fall within the expertise of an economist witness to predict what career track K.M. would have been capable of; this is why he did not ask his expert witness, Professor Carr, to opine on K.M.'s likely career track. Professor Carr was asked to assume that K.M. would have graduated from high school, but Professor Carr testified that this was only for the purpose of deciding when K.M. would have commenced employment at age 18. By the end of the trial and in closing submissions, counsel for K.M. acknowledged—in light of

the evidence at trial, including the evidence of A.F. and K.M.—that K.M. likely would not, in fact, have graduated from high school absent the abuse because of his history of poor academic achievement. K.M. submitted, however, that he may have obtained his high school equivalency in the workforce.

[621] It is K.M.’s submission that certain pieces of information are relevant to the court’s task of developing the best prediction of his career track without the assaults:

1. K.M.’s actual employment, damaged, is a reasonable indicator of the lower bound of what he could have achieved undamaged;
2. K.M. likely would not have finished high school;
3. When he could work, K.M. demonstrated excellent working qualities;
4. K.M. learned to compensate for his learning problems, and he was familiar with the skilled trades and satisfied he could have undertaken such employment;
5. K.M.’s family shows commitment to being in the workforce;
6. K.M.’s 2003 psycho-vocational assessment for O.D.S.P. indicated that his intellectual abilities were no worse than average; and
7. Dr. Silver confirmed that K.M. is a very intelligent individual who coped very well with his learning disability and demonstrated ability to be a successful employee.

[622] I agree that the above information is consistent with the evidence in this case that I accept as being credible, and that it is reliable and necessary for the assessment of K.M.’s career track absent the abuse.

[623] For the “without incident” earnings, Professor Carr was asked to assume that K.M. would have graduated from high school in Grade 12 in June 1983 and begun work at age 18 as the commencement date of the analysis, until a retirement age of 64. He was asked to assume two occupational earning scenarios for K.M. in his uninjured state: Scenarios A and B.

[624] Scenario A is based on the occupational assumption that K.M. would work in the skilled construction trade as an overhead crane operator or a heavy machine operator, category H621 of Statistics Canada, for Ontario males who worked full-year, full-time from age 18 to 64. Scenario A is an upper bound scenario that takes into account an “upside” and the probability that K.M. would have done better in his career track had he not been abused by Marson. Scenario A, including loss of fringe benefits at 11.5%, results in a past income loss of \$1,583,080.00 and a future income loss, including an allowance for productivity on future loss from the valuation date, of \$1,029,386.00—for a total income loss of \$2,612,466.00.

[625] Scenario B is based on the occupational assumption that what K.M. earned in 1996 at Tunnel Systems (the only year of full-time work available to analyze for K.M.) would have been representative of his lifetime earnings, each and every year, adjusted only for inflation. In

Scenario B, K.M. would have worked as a labourer in the construction or industrial trade. This is a lower bound scenario that accepts that K.M.'s career performance at Tunnel Systems, while damaged, is representative of the market value of his work at the time and the lower end—if anything, an underestimate—of what K.M. could and would have earned undamaged. In that regard, Scenario B is a conservative lower bound scenario that takes into account the “downside” and the fact that K.M.'s earning potential would have been higher had he not been abused by Marson. Scenario B, including the loss of fringe benefits at 11.5%, results in a past income loss of \$1,268,179.00 and a future income loss, including an allowance for productivity on future loss from the valuation date, of \$807,689.00—for a total income loss of \$2,075,868.

[626] For past and future income loss, K.M. claims the midpoint of Scenarios A and B: \$2,344,168. The midpoint is actually \$2,344,167 (not 8) but I have done the analysis based on numbers as rounded to \$2,344,168. Broken down, the midpoint of past income loss is \$1,425,629.50 rounded to \$1,425,630.00; the midpoint of future income loss is \$918,537.50 rounded to \$918,538.00; for a total income loss of \$2,344,168.

[627] In calculating Scenarios A and B, Professor Carr relied on the mean income from the 2001 census and calculated the lifetime income that K.M. would have earned in each scenario to retirement age of 64. He included a 0.8% productivity allowance on future loss to reflect an overall increase in productivity in the economy and, based on a 1998 KPMG study, added 11.5% in fringe employee benefits that K.M. would have received. In Professor Carr's analysis, the term “fringe benefits” refers to employer's share of statutory benefits (C.P.P., E.I., and W.S.I.B.) plus pension plans, disability, health and dental plans, and insurance benefits that are paid by the employer.

[628] Professor Carr made no further adjustment for either positive or negative contingencies. Professor Carr made it clear in his evidence that that is not something an economist should do. Rather, it is a judgment the court has to make based on the evidence before it. It is also why Professor Carr begins his analysis from clearly established assumptions, which I find to be the correct approach. Professor Carr approached the analysis of K.M. as being within the “average”, as he was not given any indication of any reason or specific assumption to depart from the “average” in doing his analysis. Professor Carr left such any departure from the “average” analysis for the court to determine, which I find is the correct approach.

[629] It is the School Board's position that K.M.'s damages for his total income loss are \$575,000. The School Board submits that Professor Carr did as he was told in this case; he did not consider the plaintiff's history or review any transcripts, and he had no information on K.M.'s educational trajectory up to Grade 7. Regarding Scenario A, the School Board submits that to the extent that this scenario was informed by K.M. getting a “crane ticket” at Tunnel Systems, it is not correct; this ticket is not a skilled trade, and in all likelihood, a crane operator as envisaged in Scenario A requires a community college diploma and, at the very minimum, a high school diploma with work experience. If the court concludes that K.M. would not likely have completed community college or high school, or had spatial recognition difficulties that would have prevented him from working in Scenario A, it follows that Scenario A does not assist the court. Regarding Scenario B, the School Board submits that to “cherry-pick” a single year—not coincidentally, K.M.'s best year in terms of financial success—and say that it is indicative of

a forty-year work experience does not accord with logic. Further, it is not reasonable or logical to look at post-incident capacity to determine pre-incident capacity; one cannot inform the other, and an economist should take the average over all the ensuing years.

[630] The School Board submits that in comparison, its expert, Professor Charette, was provided with this case's underlying documentation and K.M.'s discovery transcript. Rather than accept any assumption from counsel, Professor Charette exercised his expertise as a labour-market economist. The School Board states that Professor Charette did not come to any opinion about K.M.'s learning disability; rather, Professor Charette looked at the educational records, saw the trajectory, and, within the exercise and scope of his expertise, determined that a reasonable assumption would be that K.M. would not graduate from high school, even without the sexual abuse. Professor Charette stated in his report of September 23, 2016 and in his evidence that, eight years after Marson's conviction, his instructions were based on the "alleged" sexual assault of K.M.

[631] Like Professor Carr, Professor Charette assumed retirement at age 64. However, Professor Charette's starting point was that K.M. would not have graduated high school. Professor Charette relied on the median, not mean, income value of 2006 census data for Ontario males without high school to produce a single figure rather than an upper and lower bound. The Ontario employment rates for males with less than high school education were then applied to produce what Professor Charette termed "contingency adjusted" median earnings. Professor Charette declined to apply the 0.8% productivity value applied by Professor Carr because he found K.M. to be part of a lower skilled cohort, and he applied 8% in non-wage benefits to reflect basic statutory benefits. After subtracting K.M.'s actual earnings, this produced a past loss earnings shortfall of \$689,002, plus non-wage benefits of \$55,120 (8%), for a total past income loss of \$744,122. The same approach was used to establish the future loss, using a present value multiplier, plus 8% non-wage benefits and assuming zero income from 2004 to age 64 (as did Professor Carr), for a total future loss claim of \$407,463. The total of past and future income loss calculated by Professor Charette, based on these contingency-adjusted earnings, was \$1,151,585. The School Board then reduced this figure by 50% based on causation and K.M.'s original position, rounding K.M.'s past income loss to \$375,000 and his future income loss to \$200,000, for a total income loss of \$575,000. As I have already rejected any such 50% reduction of damages based on causation as being inappropriate, the School Board's total income loss for K.M., after the many negative contingencies applied by Professor Charette, is \$1,151,585.

[632] Counsel for K.M. and the School Board each made submissions regarding the calculations made by the other party's expert.

[633] K.M.'s counsel submits that notwithstanding the conservativeness of Professor Charette's assumption for calculation purposes (that K.M. would not have graduated high school), Professor Charette's starting number, without negative contingencies applied, is near Professor Carr's midpoint number of \$2,344,168. It was Professor Charette's negative adjustments that had the effect of bringing K.M.'s claim down to the actual amount of \$1,151,585—the total of \$744,122 for past income loss, and \$407,463 for future income loss—which counsel for the School Board reduced by a further 50% to \$575,792.50 to account for K.M.'s pre-incident state, rounded to \$575,000.

[634] It is K.M.'s position that Professor Charette's broad approach was flawed in at least three respects. First, it was based on his own opinion, which he was not qualified to form, that K.M. would never have finished high school or gotten his high school equivalency given his problems in school and learning disability. Second, Professor Charette's approach ignores other evidence relevant to predicting K.M.'s "without incident" career path. Third, Professor Charette made no effort to allow for an "upside" or explain why there should be no positive contingencies for K.M., other than those implicitly included in the income ranges, to reflect that he may have been above average; he allows for only a "downside" for K.M.

[635] Regarding Professor Charette's use of median figures, K.M. submits that simple logic and common sense tell us that the average (mean) captures all of the information about the income of all members of the category. Professor Charette used the median based on his own judgment about K.M.'s employment capabilities, and his approach of using the median was previously found, in *Gordon v. Greig* (2007), 46 C.C.L.T. (3d) 212 (Ont. S.C.), to be an unjustified means of arriving at a lower figure. Professor Charette's other adjustments, such as excluding a productivity increase or adding only 8% in fringe or "non-wage" benefits, rather than the usual 11.5%, were based on Professor Charette's opinions and views of K.M.'s limitations in the workforce. His further reductions were based on employment rates that included voluntary absences from the work force and did not consider K.M.'s individual characteristics. It is K.M.'s position that Professor Charette's approach to negative contingencies is untenable on the evidence and contrary to the approach taken by the courts; the court does not automatically make a general contingency adjustment, and any general contingency that is made should be modest or small, in the range of 0-20%.

[636] The School Board's counsel submits that Professor Carr was unable to use median figures because he was working with an older census data set that only includes average figures. Professor Charette worked with the 2006 set that includes both sets of figures, and he relied on the median. The School Board submits that Professor Charette's use of the median is not question of judgment or opinion; statistically speaking, the median gives you the most probable outcome. The School Board submits that while older cases used the average (mean) figures, median figures are now available and are the appropriate approach.

[637] The School Board also submits that while Professor Carr assumed full-year, full-time employment in both Scenarios A and B, employment rates for Ontario males without high school from age 18 to 64 must be applied in order to achieve utility in a long-term calculation such as this. It is the School Board's position that these calculations by Professor Charette do not incorporate any individual consideration of K.M., other than the assumption that he would not have completed high school. While Professor Charette was cross-examined at length about his opinions as to K.M.'s actual abilities or potential, it is the School Board's submission that none of that factored into his actual approach. It is the School Board's submission that to the extent the employment rate is the application of a contingency, it is a general contingency because it is general to the entire population of Ontario males without high school education.

[638] Regarding Professor Charette 8% non-wage benefits, the School Board submits that Professor Charette observed that the cohort of non-high school Ontario males falls into a different sort of employment category and is not entitled to some of the enhanced non-wage

benefits, beyond statutory benefits, that are enjoyed by the skilled workforce. Regarding productivity, the School Board submits that K.M.'s cohort would generally not be participating in the technology changes that generate increased productivity in the economy.

[639] Professor Carr was asked by me at trial: if the court determines that K.M. would not have graduated high school, are there categories in the economic analysis for each grade level achieved? Professor Carr responded that the lowest or bottom category in the latest census is “no certificate or diploma”, which means that you have no high school. He agreed that it does not matter if you have grade 8, 10 or 11—you are still in the lowest category for analysis. Professor Charette agreed that there are no statistics available for each grade level; if the person lacks a high school diploma, that person is captured simply by the “no high school” category, with no differentiation between grade levels achieved.

[640] Professor Carr was asked by me at trial what kind of difference he would see in his total income loss figures, using the mean income analysis and the same assumptions he otherwise applied in Scenarios A and B, if he were told to make the assumption that K.M. would not graduate from high school. Professor Carr answered that in a scenario for full-year, full-time without graduating high school, the income loss figures would be close to Scenario B, and if anything Scenario B would be a conservative (low) estimate. This left a vacuum in the evidence as to what Professor Carr's numbers and calculations would be for full-time, full-year employment of Ontario males not graduating from high school, with the same fringe benefits and productivity being considered as in Scenarios A and B, and if the mean (average) income was used instead of the median as used by Professor Charette. Prior to the end of K.M.'s case, Professor Carr provided a signed schedule of calculations, filed on consent of counsel (Exhibit # 10), to answer that question—which I termed “Scenario C”—which avoided Professor Carr having to be recalled as a witness.

[641] In Scenario C, Professor Carr stated that the 2011 National Household Survey is the most recent available data from Statistics Canada on the income of Ontario males aged 45-54 who did not complete high school. This table refers to persons with no certificate, diploma or degree, and it is the National Household Survey's equivalent class to the class Professor Charette referred to in his calculations. The National Household Survey shows 2010 wages and salaries at \$47,493 (median) and \$52,085 average (mean). In 2016 dollars, wages and salaries are \$52,344 (median) and \$57,405 average (mean). This median figure is quite similar to Professor Charette's median and the difference is not really significant, with Professor Carr attributing it to the data being from a different data set. If you work out the numbers, the mean is 9.7% higher than the median. Counsel for the School Board had nothing to add to Professor Carr's evidence that the mean is 9.7% higher than the median.

[642] Factoring up Professor Charette's median by this 9.7% indicates that the mean income value for this population would be \$58,490. If one were to use the 2011 data set figure for the mean (i.e., that relied on by Professor Carr in Scenario C), it would be \$57,405. This indicates that a fair approximation of the mean income in 2016 dollars for the population being considered—here, Professor Charette's selection of the category of Ontario males, aged 45-54, who did not finish high school—is \$58,000. This \$58,000 can be compared to the upper and lower bounds of Professor Carr's Scenarios A and B approach (\$68,232 as the upper bound

Scenario A, and \$53,537 as the lower bound Scenario B); \$58,000 stands just below the middle of that range. In terms of the total loss of income claim, the midpoint of the Scenarios A and B total loss of income claims is \$2,344,168. At an assumed average (mean) income level of \$58,000—for Ontario males, aged 45-54, in the cohort of those who did not finish high school, with full-year, full-time work—the corresponding total loss of income claim for Scenario C, Professor Carr estimates, would be in the vicinity of \$2,300,000, just below the midpoint of Scenarios A and B.

[643] On the facts of this case, I find that K.M. would not have graduated high school, and likely would not have achieved his high school equivalency in the workforce, due to his learning disabilities. His income loss claim must flow from this core factual finding. Professor Carr was given the assumption that K.M. would have graduated from high school for analysis purposes in Scenarios A and B to provide an upper and lower bound, but only to establish the commencement date of the analysis at age 18. At trial, both K.M. and his mother A.F.’s evidence was that they never expected K.M. to graduate from high school and that his employment would be as an unskilled labourer. This is based, I find, on K.M.’s learning difficulties and not due to his intelligence. I therefore agree with the submissions of the School Board that K.M. would not have been able to complete the certification process necessary to be an overhead crane operator or heavy equipment operator because of his learning disabilities and his inability to achieve a high school diploma or high school equivalency. This skilled trade requires a high school diploma, together with other training, which I find K.M. could not achieve. Scenario A is therefore overly optimistic and is of limited assistance.

[644] I find, nonetheless, that K.M.’s intellectual abilities are no worse than average within the no high school cohort, even with his visual spatial perception and learning difficulties. This was confirmed in the psycho-vocational assessment of Dr. Wolgarth. I find that K.M. may have done optimistically better in the construction industry than a general labourer and may have in fact learned additional skills on the job. I find he would have had a strong work ethic. I find that K.M. would have had full-year, full-time work based on his strong work ethic and that of his family. K.M. had positive references letters from two previous employers. I find that K.M. did compensate for his learning difficulties very well and did work as cement finisher and operated heavy equipment. I find that within the cohort of Ontario males not graduating from high school, there is nothing that would take K.M. out of the “average” in this category as a general labourer. I reject that K.M. would have been below average in that cohort.

[645] With regards to Scenario B, I accept and find reasonable that K.M.’s actual income while at Tunnel Systems, despite being the best job he performed in his damaged state, is representative of the lower bound of what K.M. could have accomplished had he not been abused; in fact, I find that he may have done better. While it is preferable to have 3 to 5 years of without-incident earnings for purposes of analysis, that is not possible on the facts of this case, as K.M. was sexually abused from 1978 to 1980 before he could enter the workforce. Professor Charette did not fairly consider K.M.’s work at Tunnel Systems as being relevant to his analysis, whereas I found it to be of important evidentiary value. It was at least some evidence of what work K.M. could do in his damaged state. I accept Professor Carr’s opinion that the Tunnel Systems job was indicative of K.M.’s earning capacity as it was clear evidence of how the labour market valued K.M.’s skills at the time. That said, Professor Carr testified that using only actual

earnings data to estimate “without-incident” earnings is problematic, as the actual earnings data is tainted or affected by the incident and would be an underestimate of what K.M. would have earned. Further, the job at Tunnel Systems for 1996 included 7 weeks of unemployment prior to K.M. starting that job on February 19, 1996, as set out in their reference letter of February 27, 1997, and I find that the annual income from that job was therefore already reduced in Professor Carr’s Scenario B analysis. I therefore find Scenario B is an underestimate of K.M.’s income loss as he was working in his damaged state, which is below his potential if he were undamaged.

[646] I have considered Professor Charette’s calculations and Professor Carr’s Scenario C calculations from the perspective of the finding that I have made: K.M. would not have graduated high school or achieved his high school equivalency in the workforce. For reasons that follow, I accept and prefer the opinion and the approach to the income loss analysis taken by Professor Carr. I reject and disagree with Professor Charette’s approach and calculations. The only exception is the presumption he made—which he was not qualified to make, but which turned out to be my finding on the evidence at trial—that K.M. would not have graduated from high school. I find Professor Charette’s approach to be fundamentally flawed, but the finding that K.M. would not have graduated from high school does have a core effect on the income loss claim analysis, as it puts K.M. in a different and lower cohort than an overhead crane operator in the tables of Statistics Canada. In fact, it puts K.M. in the lowest cohort. I accept Professor Carr’s Scenario C total income loss calculation of \$2,300,000 as the basis for K.M.’s income loss claim because, as I discuss below, I accept Professor Carr’s approach to the use of the simple average (the mean figures), employment rates and other adjustments that he made, over the approach taken by Professor Charette. I accept and find the mean income of \$58,000 in 2016 dollars as being the appropriate income for analysis in Scenario C.

[647] At trial, the defence was critical of the manner in which Professor Carr accepted the assumptions provided by plaintiff’s counsel regarding K.M.’s likely path. However, I find this is the expert witness’s duty: to remain within the bounds of his or her expertise and work within the assumptions provided by the parties that are readily ascertainable. Both Professor Carr and Professor Charette—despite the evidence Professor Charette gave at trial—have previously followed this expert’s duty and worked within assumptions provided by counsel, assumptions that the courts have ultimately found to be unfounded or overly optimistic: see e.g. *Freeswick v. Forbes* (1996), 1 O.T.C. 23 (Gen. Div.), at paras. 26-27, 43, 48; *Kelly v. Perth (County)*, 2014 ONSC 4151, at paras. 184, 188. It is the court’s role, not the expert’s, to determine the reasonableness of the assumptions provided. The assumptions relied upon by economist experts, of necessity, must be clearly and plainly set out in their reports and in their evidence so that those assumptions can be assessed by the court as to their reasonableness in accordance with the other evidence in the case.

[648] I reject Professor Charette’s criticisms of Professor Carr’s use of assumptions rather than his own opinions, and I do not find Professor Charette’s general approach helpful in assessing K.M.’s loss of income. While the evidence and my finding at trial ultimately was that K.M. would not have graduated from high school or achieved equivalency, Professor Charette took it upon himself to make that decision and a number of other decisions about K.M. that I find were inappropriate and partisan in favour of the School Board. Professor Charette made limited allowance for an “upside” of K.M.’s earning potential in his undamaged state or for positive

labour market contingencies, yet made a significant allowance for the “downside” and applied large negative labour market contingencies. Professor Charette used the median, not the average or mean. Professor Charette then reduced K.M.’s non-wage benefit component to be rounded to 8%, as he decided K.M. would not have a job with anything but statutory benefits. He reduced K.M.’s assumed productivity increase altogether because of his belief that K.M. was in such a low category. He applied very large negative contingencies and no positive contingencies, failing to take into consideration that K.M. would have done better than he did in his damaged state.

[649] In Professor Charette’s report, it is not possible to calculate his gross beginning or starting number that gets him to his opinion of K.M.’s total loss of income of \$1,151,585. Based on Professor Carr’s analysis of Scenario C and its total income loss figure of \$2,300,000, which I accept as a reasonable starting number for K.M.’s total income loss claim, it became clear that Professor Charette—even working with median instead of mean figures, lower non-wage benefits and no productivity allowance—must have started from a gross income loss amount closer to \$2,300,000 before he applied his significant negative contingencies to drastically reduce K.M.’s total income loss claim to \$1,151,585. I agree with the submissions of counsel for K.M. I find that Professor Charette’s general approach to contingencies is untenable on the facts and the evidence, but also on the approach to contingencies developed by the courts for application to personal injury cases. In the end, as the analysis and calculations demonstrate, Professor Charette’s approach to loss of earnings using median incomes of Ontario males who do not complete high school—absent his negative contingencies—would be closer to \$2,300,000. This is just slightly below Professor Carr’s midpoint of Scenario A and B values, \$2,344,168, reached using average mean incomes and with Professor Carr leaving further contingencies to the court.

[650] I reject Professor Charette’s application of these large, unsupported, negative contingency adjustments to K.M.’s income loss claim. They are as a result of him relying on his own opinion about the impact of the learning disabilities on K.M., an opinion I find he is not qualified to make. Professor Charette formed an opinion and assigned K.M. a poor potential for employment based on his learning disability and his “slowness” from K.M.’s school records, and applied a negative, greater than average contingency rate. While Professor Charette testified that he had expertise in assessing the economic effects of learning disabilities on individuals (as it has an impact on their potential earnings), I find that he is not qualified to give the opinion he did as to whether or not K.M. would graduate from high school or to assess the nature of K.M.’s disabilities and their effect on K.M.’s pecuniary earnings. While he tried to call it an assumption when challenged in his evidence, I find it was his clear opinion and that it very negatively affected his analysis of K.M.’s probable without incident earnings. Professor Charette agreed in cross-examination that he had no expertise to assess the impact of K.M.’s learning disability on K.M.’s intellectual capacity to do work.

[651] I find on the evidence, when looked at as a whole beyond the school records, that K.M. is not mentally slow. Professor Charette presumed K.M. would not work full-time and full-year, would never have fringe benefits from his employment beyond statutory benefits, and would not take part in economy-wide productivity increases. He failed to account for K.M.’s positive work performance when he could work and his being no less than average vocational ability. These are judgment calls Professor Charette made that, I find, had the primary objective of reducing K.M.’s income loss claim for reasons not clearly stated. The reductions Professor Charette made

to the income loss claim, I find, are not objectively based; they are based on his own opinions and not supported by the evidence, which I accept as being credible.

[652] I further accept and find credible the evidence of Dr. Singh as to his criticisms of Professor Charette's opinions. Dr. Singh disagreed with Professor Charette's opinion that individuals attain higher education and employment after being sexually assaulted, which was based on a limited study done only on women. Dr. Singh's opinion was that Professor Charette was incorrect in his opinion that the working capacity of a person in a psychologically damaged state is a valid predictor of their undamaged working capacity. Dr. Singh's opinion was that K.M. would have achieved a much better education and employment if he had not been sexually abused, and that the educational and employment performance of K.M.'s parents and siblings were of use in predicting what K.M. would have done if he had not been assaulted. Similarly, Professor Carr's evidence was that work post-incident or in a damaged state is not a valid indicator of without incident working capacity; it would only be a valid indicator if there was no effect of the abuse altogether. Professor Carr's evidence is that economists are not qualified to evaluate that issue. I have found that while K.M. was limited in attaining academic success and would not have graduated from high school because of his learning difficulties, his work, while damaged, would have been on the low end of the spectrum of what K.M. could have achieved in his employment, if he had not been sexually abused.

[653] The negative adjustments made by Professor Charette, I find, are based on his own interpretation of the evidence, which, with respect, is the court's role. The result is that the figures produced have been adjusted inappropriately by him rather than the court. Since his gross starting number is unclear, his figures do not provide the necessary framework to allow the court to assess the reasonableness of the quantum of his deductions. The result is that I find Professor Charette's evidence to be of little assistance to me in assessing K.M.'s income loss claim.

[654] Professor Carr's use of mean figures and Professor Charette's use of median figures was another disputed area at trial. There is limited Ontario case law commenting on the propriety of relying on median or mean (average) values. While some older cases have held that medians help avoid the distortions that arise from averaging (see e.g. *Lyon v. Gill*, [1985] O.J. No. 626 (H.C.)), more recently in *Gordon v. Greig*, Professor Charette—again acting as a defence expert—used median figures when calculating income loss. Justice Glass found that the defence had “tagged” the plaintiff as low-skilled and that Professor Charette's use of median levels had the result of lowering the plaintiff's loss of income figure: “the way that Dr. Charette calculated the income is not acceptable because he used a method that simply cut down costs without justification” (para. 66). The Court found that using the average (mean), not median, incomes of persons in the fields of work as shown in government statistics would have been reasonable and would not have distorted or inflated the plaintiff's income (para. 66).

[655] I agree with the above analysis and find that use of the mean (average) incomes provides the most likely income that an individual would earn. I accept and rely on Professor Carr's opinion that he always uses the mean, as do most economists, whether he testifies for the plaintiff or the defence. I find that Professor Carr's approach to the analysis is appropriate on the facts of this case: there is no reason to depart from the simple average or mean figures in the analysis of K.M.'s income loss. The simple average or mean takes everybody's income, adds it

up, and divides by the number of people. This approach is used, according to Professor Carr, by most economists as it obtains a number that is the most likely number you would earn. Professor Carr's evidence is that once you decide what category K.M. is going to be in—a construction worker, or without high school—then within that category the appropriate summary measure is the mean, not the median. Professor Carr always uses the mean; it takes into account high-earning people and low-earning people. The median calculation does not work this way, and Professor Carr has not seen the median used. The median is almost always below the mean.

[656] I find Professor Charette made a judgment call to use the median. He made subjective decisions based on his own opinion that K.M. was “below average” based on his learning disabilities, his poor school performance and his “slowness”, which I find he was not qualified to make, and applied substantial negative adjustments as a result, contrary to the objective approach of Professor Carr. As I have discussed, on the facts of this case I find that in spite of K.M.'s learning disabilities, when K.M. could work he was motivated to work and had a strong commitment to the labour force, he had a positive work ethic as did his family, and he was intellectually average within the cohort of Ontario males not graduating from high school. I find that there is no reason to depart from the mean (average) in the income analysis of K.M. I reject Professor Charette's use of the median in his income analysis of K.M. and the significant negative adjustments to the income loss claim that he made.

[657] I now turn in greater detail to the adjustments and contingencies applied by each party's expert. Contingencies, both negative and positive, are the various factors affecting the degree of risk of future economic loss and the possibility that all or part of those losses may have occurred apart from the wrong which is the subject of the litigation: *Graham*, at p. 635. There are two types of contingencies: specific and general. Specific contingencies are peculiar to a particular plaintiff, such as a particularly marketable skill or consistently poor work record. They require a greater evidentiary basis capable of supporting the conclusion that the occurrence of the contingency is a realistic, not speculative, possibility: *Graham*, at p. 636.

[658] Generic examples of specific contingencies that Professor Carr gave in his evidence include, as a positive contingency, a long history of dedication to the labour market by an individual and a corresponding assumption that the retirement age for that individual would be above average and later than age 64. An example of a negative contingency would be an assumption that an individual intended to retire earlier than age 64. Another example of a negative contingency is that if there was medical evidence that an injured individual before an accident had sub-normal mortality rates and a higher chance of dying because of a genetic condition, then an economist would depart from the average mortality rates and use sub-standard mortality rates in their analysis. You need case specific evidence to make these positive or negative contingency adjustments, which comes from the assumptions given to the economist or the court's findings based on the evidence. In Professor Carr's words, “you need some sort of indication that this person is either above average or below average” to apply additional positive or negative contingencies. Having no specific evidence of this nature in K.M.'s case, Professor Carr applied neither a positive or negative specific contingency, and treated K.M. as being “average”. He left any other contingencies up to the court, which I find is appropriate.

[659] General contingencies are the other type of contingency, those which “as a matter of human experience are likely to be the common future of all of us”, such as promotions or illness: *Graham*, at p. 636. General contingencies operate to increase or decrease the predicted loss of earnings. General contingencies take into account the uncertainty of predicting the future and the chance that the assumptions on which the income loss has been assessed may turn out to be wrong. They must be based on the evidence in the case. As explained by the Supreme Court in *Andrews*, contingencies are often implicitly already contained in an assessment of the projected average earnings of the injured person, “for one must assume that this figure is a projection with respect to the real world of work, vicissitudes and all” (p. 253). Additionally, not all contingencies are adverse, and there are many public and private schemes that cushion an individual from possible negative contingencies (p. 253). The percentage deduction for general contingencies is generally “small” or “modest”: see *Graham*, at p. 636; *Andrews*, at p. 253; *Beldycki Estate v. Jaipargas*, 2012 ONCA 537, at para. 78.

[660] In this case, a large part of the discrepancy between K.M. and the School Board’s calculations can be explained by their experts’ differing approaches to K.M.’s risk of unemployment. Professor Carr did not adjust his average figures to reflect employment rates, and assumed full-year, full-time employment by K.M. from 18 to retirement at 64 for Scenarios A, B and C. Professor Charette applied employment rates for Ontario males who did not complete high school, which had the effect of significantly reducing K.M.’s projected income levels. The School Board submitted that to the extent the unemployment rate is the application of a contingency, it is a general contingency because it is general to the entire population of Ontario males without high school education. The unemployment rates used by Professor Charette were for Ontario males without high school and were listed between 22.1% and 45.1% (the reverse of the employment rates in his Table IV, by stated age ranges). I note, however, that Professor Charette used the age 25-34 employment rate for age 15-24 instead of the 15-24 rate, which in Professor Charette’s opinion is too low because it captures people who are leaving school and entering the labour market for the first time. His practice is to use the 25-34 employment rate for ages 15-24, which is higher (and therefore the unemployment rate, or contingency deduction, is less).

[661] There is no single way to take the risk of unemployment into account, though one method is to simply incorporate employment and participation rates into a single general contingency, along with sickness, early retirement, or career changes. In *Gray v. Macklin*, [2000] O.T.C. 866 (S.C.), for example, the defence expert economist incorporated a reduction in wage loss based on average labour participation rates across Canada. Justice Shaughnessy rejected this and found that “[t]he blind application of participation rates can lead to unfair results” by blending together various types of employees without sufficient regard to the particular plaintiff and by creating the potential for double counting if an additional general contingency is deducted (para. 199). The Court instead deducted a general 20% contingency to future income loss to cover “contingencies of life” common to all of us, including labour force interruptions, early retirement, sickness, accident, or unemployment (para. 203). Where sufficient data is available, the Court may make a deduction that corresponds to the employment rates of a particular industry: see e.g. *Rabideau v. Maddocks*, [1992] O.J. No. 2851 (Gen. Div.).

[662] Contingency deductions may not be necessary where the numbers provided by the experts incorporate an “upside” and “downside” in themselves. In *Butler (Litigation Guardian of) v. Royal Victoria Hospital*, 2017 ONSC 2792, at para. 211, McCarthy J. refused to reduce a future income loss claim by any negative contingencies, as general contingencies were already implicitly contained in the average earnings relied on by the expert witness in that case, who happened to be Professor Carr. This was upheld by the Court of Appeal, 2018 ONCA 409, released May 1, 2018 after the submissions in this case, where it agreed that this result was open to the trial judge; he was not obliged to make a further deduction for non-participation in the workforce (para. 12).

[663] As in *Butler*, I see no cogent evidence to warrant departing from the average data that was used by Professor Carr, that is the mean, and I would not apply the significant deductions based on employment rates made by Professor Charette. The employment rates that Professor Charette used included absences from the work force for any reason, including voluntary absences from the work force and taking time off work, which are not applicable here. Professor Charette acknowledged in cross-examination that voluntary absences from the workforce could be as much as half of the total absences. I have found that K.M. would be working to support his family if he had not been assaulted and he would not have been voluntarily absent from the workforce. The employment rate deductions by Professor Charette were, I find, another way of reducing K.M.’s income loss claim. In not allowing for any positive contingencies for K.M. and only negative contingencies, I find that Professor Charette has not taken a balanced, objective approach to the income loss assessment. As I have discussed, I accept and rely the approach used by Professor Carr—which considered both positive and negative contingencies, that K.M. could be above or below average, and that these contingencies in essence cancel one another out—as being the most reasonable, as I have already found that K.M. was average within his cohort. The income loss assessed by Professor Carr balances the “upside” and the “downside” of K.M.’s income loss claim.

[664] I accept and rely on Professor Carr’s other adjustments to K.M.’s income loss claim, namely his approach to fringe benefits and the productivity allowance. Professor Charette relied on and decided that K.M.’s below-average quality of employment would have attracted statutory benefits of 8%, whereas Professor Carr incorporated fringe benefits of 11.5%, relying on a 1998 study by KPMG that was recently accepted by this Court in *Butler* (ONSC), at para. 208. With regard to the productivity allowance, Professor Carr began with the discount rates mandated by Rule 53.09 for trials beginning in 2016 (as this case was slated to begin in May 2016): 0% for 15 years and 2.5% thereafter. He then adjusted them by the 0.8% productivity increase, reaching a -0.8% discount rate for 15 years and 1.7% thereafter. Professor Carr’s productivity increases are the average derived from the entire economy over a long period of time. There is no reliable data for productivity increases for a particular sector such as the construction field, so overall rates are used. Two recent Superior Court decisions have incorporated a 0.8% productivity increase: *Butler* (ONSC), at para. 210; *Pomeroy v. Misljencevic*, 2016 ONSC 5867, at paras. 77-80. I note that *Pomeroy* applied the 0.8% productivity increase with a relatively low-skilled plaintiff, who worked in retail and had achieved grade 11 in school (paras. 10-11). Professor Charette did not apply a 0.8% productivity increase but did apply the rates in Rule 53.09.

[665] I see no reason to depart from the economy-wide productivity increase added by Professor Carr, or to assume that K.M. would have received below-average fringe benefits. This is, again, another way that Professor Charette's adjustments reduced K.M.'s income loss claim. I do not agree that K.M. would not have taken part in the general economy-wide growth in productivity. For these reasons, I accept Professor Carr's adjustments for fringe benefits and productivity.

[666] I accept and prefer the evidence of Professor Carr to that of Professor Charette. I find that Professor Carr has significant experience in assessing claims of economic loss and that he approached his calculations of K.M.'s economic loss with appropriate objectivity. Professor Charette's evidence and opinion, I find, were significantly negatively influenced by his belief that K.M. was mentally slow because of his lack of academic achievement and that K.M. was below average within his cohort. I find that Professor Charette failed to consider any appropriate "upside" for K.M. and that he applied significant and untenable negative contingencies to reduce the quantum of K.M.'s economic loss claims. I find Professor Charette's evidence was not objective and demonstrated partisanship in favour of the School Board. As a result, I give Professor Charette's evidence very little weight as to K.M.'s economic loss.

[667] For the reasons given above, I find that K.M. has met the burdens of proof in relation to his total income loss. I find and assess K.M.'s damages for his total loss of income on the basis of K.M. being an average Ontario male not graduating from high school or obtaining high school equivalency. On the basis of Scenario C, I accept that \$2,300,000.00 is the appropriate beginning or starting number for K.M.'s total loss of income from age 18-64 based on the mean income which I have found to be \$58,000 in 2016 dollars.

[668] I next have to consider my findings, if any, as to the probabilities affecting K.M.'s without incident earnings and career track and decide whether, on the evidence as a whole, any adjustment, whether up or down, is warranted to those numbers as a general contingency.

[669] The trial date on the facts of this case is somewhat artificial, as what this case requires is an assessment of K.M.'s hypothetical earnings and career track in the past and the future, from age 18 until retirement at age 64. Counsel have approached this general contingency issue on the basis of the period from age 18 to retirement at age 64—K.M.'s without incident earning period. This calculation is based on hypothetical events of how K.M.'s life would have proceeded absent the sexual abuse from 1978 to 1980. Any factors affecting the risk of K.M.'s income loss claim must be given weight according to their relative likelihood and will be taken into consideration as long as it is a real and substantial possibility and not mere speculation: *Athey*, at para. 27. In assessing hypothetical events, I see no reason to distinguish between those that are applied before and those that are applied after trial. Because K.M. was abused before he could enter the workforce at a young age, I have to decide his likely career track for his entire without incident working period; the same hypothetical analysis on the facts of this case is therefore required for both past and future income loss. The question is whether or not there should be any positive or negative general contingency adjustment at all to K.M.'s total income loss.

[670] K.M.'s position is that we start with the beginning number (here, \$2,300,000.00 based a mean income of \$58,000 in 2016 dollars, given that I accept Scenario C) and apply the governing

principles in light of the evidence as a whole. K.M. submits that the percentage range for consideration for general contingencies in this case, if any, is 10% to 20%. The School Board's position is that K.M.'s contingency-adjusted damages, as already provided by Professor Charette, are \$1,151,585.00. While I accept the beginning or starting number in Scenario C of \$2,300,000.00 for the reasons I have indicated above, I find that it is necessary on the particular facts of this case to apply a modest negative contingency to K.M.'s beginning number for his total loss of income claim. This reflects negative labour market contingencies that I find would have affected K.M. throughout his entire working life without incident from age 18 to age 64 as a general labourer.

[671] Professor Carr confirmed in his testimony that his total figures, on which I rely, do not allow for general contingencies, positive or negative, of illness, injury, or economic downturn, or labour market contingencies, since it is the court's task to make that assessment on a case specific evidentiary basis. The "average" of Ontario males without high school applies to all occupations and is not, I find, particularly adapted to reflect the category of a general labourer in the construction field, as submitted by the School Board.

[672] *Andrews* was clear that the percentage deduction in any scenario will depend on the facts of the individual case, "particularly the nature of the plaintiff's occupation, but generally it will be small" (p. 253). If applied at all, as I have discussed, the plaintiff submits that a range of 10% to 20% is appropriate. I have found that K.M. would not have graduated high school or achieved his high school equivalency in the workforce, and would have been involved in physical work as a general labourer or in a construction type of job had he been able to sustain employment. K.M. did well in his technological and vocational studies courses at Harry J. Clarke School, which involved the use of power and hand tools. He could have worked with his hands provided it did not involve mathematical calculations, which ruled out work such as carpentry. K.M. was able to do cement finishing types of jobs when he could work. Professor Carr's Scenario C incorporates Ontario males who did not complete high school, but is not specific to the construction or labourer type of work.

[673] Taking into account the physical labourer-type occupation that I find K.M. would have done—with its attendant risks of labour force interruptions, unemployment, economic downturn, K.M. not being able to have worked to age 64, and injury and illness, especially as K.M. would have grown older in a field that is physically-demanding—I find that there is a real and substantial possibility, not mere speculation, that K.M. would be subject to negative labour market factors. I find that a modest general negative contingency of 15% to K.M.'s total loss of income claim is appropriate. I therefore assess K.M.'s total loss of income claim at \$2,300,000.00 less 15% (\$345,000.00), for a net total income loss claim for K.M. of \$1,955,000.00.

[674] Given that pre-judgment interest is owed on past income loss but not on future income loss, it is necessary to determine what amount of K.M.'s net income loss claim as assessed relates to past income loss as opposed to future income loss. The calculations in Scenario C provided by Professor Carr do not contain such a breakdown, nor was he asked to provide a breakdown. Without the benefit of accurate calculations to assess the allocation between past income loss and future income loss for this purpose from Scenario C's calculations, I look to the

allocation between past income and future income used in Scenarios A and B—for that purpose only. Scenario A and B figures approximate a 61%-39% allocation of past to future income loss. Applying this to the sum assessed of \$1,955,000.00, my assessment of the allocation of the total income loss claim of K.M. is \$1,192,550.000 in past loss of income and \$762,450.00 in future loss of income.

[675] I find the burdens of proof have been met by K.M. in this case. I therefore assess K.M.'s damages for past income loss at \$1,192,550.00. I assess K.M.'s damages for future income loss at \$762,450.00. I assess K.M.'s total loss of income at \$1,955,000.00.

[676] The above assessment of damages for past and future loss of income is reserved and subject to the terms as set out in Item #24 of this judgment ("Further Calculations, Errors, Omissions, and Ancillary Matters"), only as to further calculations as specified.

17. Damages for Future Loss of Interdependent Relationship:

[677] K.M. claims significant damages for the impairment of his ability to sustain a future interdependent relationship, or his future loss of interdependent relationship. This is a recognized claim for damages for pecuniary loss. Again, this loss is a matter of assessment of damages, not a matter of strict mathematical calculation.

[678] K.M. submits that as a result of the sexual abuse by Marson, he has been unable to sustain long-term interdependent relationships and his prospects for such relationships in the future are poor. K.M. claims \$178,366 in damages for his future loss of interdependent relationship. Interestingly, as opined by Professor Carr, on the unusual facts and evidence in this case, a claim for past loss of interdependent relationship is theoretically possible from age 30 (the age people usually marry or form an interdependent relationship) until K.M.'s age of 51 as at the date of trial. K.M. has, however, limited his claim to his future losses, in recognition of the fact that he has had prior spousal relationships and that the savings from such relationships would have been offset by other expenses related to family life, and the evidentiary difficulty and expense of proving a past claim for such loss.

[679] It is K.M.'s position that Ontario courts have recognized that damage to a plaintiff's ability to form and maintain an interdependent domestic relationship can cause a measurable financial loss to the plaintiff, for which he or she is entitled to compensation. K.M. submits that sizeable awards have been made under this head of damages (see e.g. *Jane Doe v. John Doe*, [2003] O.J. No. 4839 (S.C.), where damages were assessed at \$150,000 but the claim failed on the issue of causation). He submits that he must present evidence to show, first, that his ability to form and maintain an interdependent relationship has in fact been damaged, and second, that there is a substantial possibility that he would have benefited, in economic terms, had that damage not been sustained. K.M. submits that on the evidence, he will probably not form another spousal relationship during his lifetime and that Professor Carr provides reliable expert evidence as to the extent of K.M.'s loss.

[680] Professor Carr was asked to assume that, due to K.M.'s injuries, K.M. is unable to form a permanent interdependent relationship. K.M. says that Professor Carr explained that we can

conceptualize two kinds of loss from the lost ability to form an interdependent relationship: (1) the loss of the opportunity to save expenses (the form of loss he calculated), and (2) the loss of a possible benefit accruing to the plaintiff by virtue of being able to share in the income of the other spouse. Professor Carr calculated the first category—based on the premise that “two can live cheaper than one”—which, in his evidence, is generally the way he approaches the analysis of the calculation of loss of interdependent relationship. When economists have to predict what they think is going to happen to income in the future in this area of loss, the big element to predict is consumption. The Nobel Prize-winning theory of consumption is crucial in determining damages for loss of interdependent relationship. The consumption savings of having an interdependent relationship, in the evidence of Professor Carr, is 29.5%. Professor Carr’s approach does not require any notional partner.

[681] Professor Carr explained that Statistics Canada data shows that a one-person household needs 71% of the income of a two-person household to maintain the identical standard of living. As such, expenses in a two-person household are 141% ($1 \div 0.71$) of expenses in a one-person household. Average per-person expenses in a two-person household are 70.5% ($1.41 \div 2$) of expenses in a one-person household. Hence, there is a savings of 29.5% ($1 - 0.705$) in living expenses resulting from forming a permanent interdependent relationship. Professor Carr’s calculations then flow from Scenarios A and B, as described in the income loss claim above.

[682] In Scenario A, Statistics Canada data shows that a person earning between \$40,000 and \$49,999 in 1992 dollars consumes 78.1% of household income. Professor Carr estimates that K.M. would consume \$53,289 a year ($0.781 \times \$68,232$, K.M.’s 2016 income figure in Scenario A), and K.M. would save \$15,720 ($0.295 \times \$53,289$) in living expenses if he were able to form a permanent interdependent relationship. The present value, as of May 9, 2016 (the original date of trial), of \$1,000 a year until K.M. attains age 64 is \$13,530.485. Therefore, the present value as of May 9, 2016 of the loss of \$15,720 a year to age 64 is \$212,699 ($13.530485 \times \$15,720$). In Scenario A, Professor Carr estimates K.M.’s damages for future loss of interdependent relationship at \$212,699.

[683] In Scenario B, Statistics Canada data shows that a person earning between \$30,000 and \$39,999 in 1992 dollars consumes 67.4% of household income. Professor Carr estimates that K.M. would consume \$36,084 a year ($0.674 \times \$53,537$, K.M.’s 2016 income figure in Scenario B), and K.M. would save \$10,645 ($0.295 \times \$36,084$) in living expenses if he were able to form a permanent interdependent relationship. The present value, as of May 9, 2016 (the original date of trial), of \$1,000 a year until K.M. attains age 64 is \$13,530.485. Therefore, the present value as of May 9, 2016 of the loss of \$10,645 a year to age 64 is \$144,032 ($13.530485 \times \$10,645$). In Scenario B, Professor Carr estimates K.M.’s damages for future loss of interdependent relationship at \$144,032.

[684] K.M. claims the midpoint of Scenarios A and B (\$178,366), which is K.M.’s beginning or starting number. He submits that the evidence does not support any significant discount for contingencies.

[685] K.M. relies on two Ontario cases, *Walker v. Ritchie* (2005), 197 O.A.C. 81 (C.A.), varied on other grounds, 2006 SCC 45, and *Greenhalgh v. Douro-Dummer (Township)*, [2009]

O.J. No. 5438 (S.C.), to rebut the School Board’s submission that Professor Carr’s “two can live cheaper than one” approach is non-standard. K.M. acknowledges that the exact nature of the expert evidence in *Walker* and *Greenhalgh* is not totally clear, but submits that the general approach described in each case appears to be essentially the approach adopted by Professor Carr. In *Walker*, where \$125,000.00 was awarded for this type loss, K.M. submits that the trial judge and Court of Appeal framed the loss as being based on the proven and well-known fact that two people can live together less expensively than they can live apart. In *Greenhalgh*, the claim, assessed by the plaintiff at \$200,000, failed for lack of evidence on causation and on her risk of loss of interdependent relationship, but K.M. submits that the Court again seemed to be accepting of an approach being put forward that “two can live cheaper than one”. It is K.M.’s position that, contrary to the School Board’s view, the authorities have sanctioned Professor Carr’s “two can live cheaper than one” approach.

[686] The School Board submits that \$10,000 is the appropriate figure of damages for K.M.’s future loss of interdependent relationship. It submits that loss of interdependent relationship is a reasonably new head of damage and one that requires a fairly high level of proof. However, Dr. Silver’s opinion—which the School Board accepts—is that K.M. in all probability will not develop a future interdependent relationship in the course of the next 13 years to age 64. The School Board therefore acknowledges that an assessment of K.M.’s damages is appropriate for this loss.

[687] The School Board’s position is that “two can live cheaper than one” does not inform the approach to be taken in this case, or any other, for the calculation of interdependent loss. Professor Charette, who again provided calculations for the School Board, described Professor Carr’s approach as non-standard in that it is not a proper measure of what an individual gains in terms of their own purchasing power and standard of living by having entered a relationship. It is the School Board’s position that *Greenhalgh* does not assist K.M. here, because while plaintiff’s counsel in *Greenhalgh* presented the claim in terms of “two can live cheaper than one”, the judge concluded there was insufficient evidence and said nothing about the actual approach. Additionally, in *Walker* there was no expert evidence to rebut the plaintiff’s expert, and the Court of Appeal said that the plaintiff’s loss was measured as the loss of the contribution she would have received but for the accident—as Professor Charette measured in this case.

[688] Professor Charette’s approach to calculating damages again differed from that of Professor Carr. Professor Charette’s view was that to the extent that an individual’s prospects of forming an interdependent relationship have been diminished, K.M. can be expected to suffer a dependency loss analogous to that suffered due to the death of a spouse. Counsel submitted that the loss here is the loss of a notional spouse, and what you have to calculate is the financial consequence to the plaintiff of the loss of this notional spouse. The School Board submits that if you are going to consider the economic consequence of the loss of a notional spouse, you have to use the methodology endorsed by *Nielsen v. Kaufmann* (1986), 54 O.R. (2d) 188 (C.A.), which Professor Charette indicated is the basis of calculation in all fatality claims.

[689] In Professor Charette’s calculations, the earning profile for the notional spouse is that of a female of the same age and with the same level of education as K.M. Professor Charette assumed that the notional spouse would generate the average earnings of an Ontario female who

did not graduate from high school. He then tabulated the gross earnings for K.M. and his notional spouse, with federal and provincial income tax, C.P.P. and E.I. premium payments subtracted from gross earnings, yielding the funds available to the household on an annual basis.

[690] Professor Charette then applied a sole dependency rate of 70%, a rate consistent with the results of expenditure analysis of North American households. Professor Charette then relied on *Nielsen v. Kaufmann*, which sets out the principle of net (or cross) dependency loss that might be applicable to some households. He stated that the following net dependency loss formula is consistent with *Nielsen*: 70% of after-tax income of spouse, less 30% of after-tax income of K.M.

[691] His calculations produced \$3,659 in annual baseline dependency losses to age 55, and \$1,095 annually thereafter to age 64. Assumptions were then used to determine the present value of the baseline dependency shortfall. He assumed that the interdependent relationship and dependency loss would last to age 64 (as in Professor Carr's report); the Rule 53.09 discount rate for trials beginning in 2016 is 0% for the first 15 years and 2.5% thereafter; and that both K.M. and his notional spouse would have been subject to standard population mortality rates. The present value multiplier for annual payments from age 51.61 to 54 years is 3.355, multiplied by \$3,659 to produce a present value future dependency loss of \$12,276 from age 51.61 to age 54. The present value multiplier for annual payments from age 55 to 64 years is 8.394, multiplied by \$1,095 to produce a present value future dependency loss of \$9,191 from age 55 to 64. Under those assumptions, the present value of the total future dependency shortfall is \$21,467. The School Board submits that, as with its claim for economic loss and based on its position on causation, it is necessary to then determine the extent to which this \$21,467 loss is in fact related to the sexual abuse as opposed to K.M.'s original position. The School Board submits that the sum of \$21,467 should be reduced by 50% to \$10,733.50, generously rounded down to an award of \$10,000 as the appropriate assessment of K.M.'s damages for this loss. I have already rejected a 50% reduction of damages based on K.M. original position on causation as being inappropriate on the facts of this case. The School Board's beginning or starting number is therefore \$21,467.00.

[692] The School Board submits that Professor Charette's assumption relating to K.M.'s notional spouse—an Ontario female who did not complete a high school education—was reasonable. It generates a value that is higher than zero (i.e., that of the notional spouse who does not generate any income), but lower than a notional spouse who is a professional and earning a substantial income. It contemplates a notional spouse whom K.M. would have interacted with, and who generated income at that level. The School Board relies on two Ontario cases, *Osborne (Litigation Guardian of) v. Bruce (County)* (1999), 39 M.V.R. (3d) 159 (Ont. Gen. Div.), and *Bartosek (Litigation Guardian of) v. Turret Realities Inc.*, [2001] O.T.C. 856 (S.C.), as support for Professor Charette's approach. In *Osborne*, the economist's evidence involved the income-earning capacity of a notional spouse—the same fundamental approach as Professor Charette's. In *Bartosek*, there were references that seemed to relate to *Nielsen*, like Professor Charette. It is the School Board's position that Professor Charette's approach is logical, because it is measuring the economic loss of a notional spouse in accordance with well-established principles as to how that loss is calculated, and entirely consistent with the cases we have seen.

[693] In response, K.M. submits that the submission that Professor Charette's approach is the only tenable approach is not supported. While Professor Charette asserts that his approach somehow contained the loss that was calculated by Professor Carr, Professor Charette never explained this in any understandable way. K.M. submits that there does not appear to be any reported case in which the court has accepted Professor Charette's approach of using the *Nielsen v. Kaufmann* fatal injury model in a personal injury case. Professor Carr pointed out that Professor Charette's approach, imported from the fatal accident cases of *Nielsen*, entails calculating the claims on an after-tax basis, which would greatly affect those numbers—contrary, K.M. submits, to the approach accepted for decades in personal injury cases that damages are to be valued on a pre-tax basis subject to very limited exceptions. It is Professor Carr's opinion that Professor Charette did not apply *Nielsen* correctly, because if it were to be used so that tax considerations came into play, then the figures had to be grossed up for tax, and that further, *Nielsen* looks only at the income of the person that dies and uses 60%, whereas Professor Charette used a dependency rate of 70%. K.M. submits that Professor Charette's approach forces you to make additional, arbitrary assumptions about what the other spouse would have earned, complicating the whole process and the consequent calculations.

[694] The only loss Professor Carr looks at in this type of claim is “two can live cheaper than one”, not the difference in income levels (which is more speculative). Professor Charette's approach reflected a different sort of loss, I find, than that addressed by Professor Carr. Professor Carr knows how Professor Charette did his calculations, but he does not understand the approach taken by Professor Charette to the determination of loss of interdependent relationship. Professor Charette viewed this as a fatality case, which it is not. There are notable differences between fatality cases and personal injury cases.

[695] Loss of interdependent relationship is a serious head of damages that recognizes that compensation may be given for the pecuniary consequences of a lost opportunity to form a permanent interdependent relationship, such as marriage or a common law partnership. While there is a both a pecuniary and non-pecuniary aspect to the loss of the opportunity to form an interdependent relationship, care must be taken to keep these elements separate and avoid double compensation: *Reekie v. Messervey* (1989), 59 D.L.R. (4th) 481 (B.C. C.A.), at pp. 494-95. In this case, as I have discussed, K.M.'s claim deals only with the financial aspects of his future, not past, loss.

[696] The rationale or theory behind this head of damages flows from the general proposition that marriage or similar interdependent relationships produce economic benefits through household efficiency and shared expenses, as well as the ability to share in the income of a spouse: *Walker v. Ritchie*, at paras. 53-54; *Kelly v. Perth (County)*, at para. 263; *Bartosek*, at para. 34; *Latta v. Ontario*, [2004] O.T.C. 802 (S.C.), at para. 152. Given the nature of these marital benefits, some cases have held that these damages are properly considered part of the plaintiff's lost income: see e.g. *Reekie v. Messervey*, at pp. 495-98; *Wilhelmson v. Dumma*, 2017 BCSC 616, at paras. 351-52. I reject this approach for the reasons that follow below.

[697] To receive damages under this head, K.M. must establish: first, that as a result of his injuries, there is a real and substantial possibility that his chances of forming an interdependent relationship have been detrimentally affected or impaired; and second, that such a relationship

would have been economically advantageous to him: see *Kelly v. Perth (County)*, at para. 263; *Wilhelmson v. Dumma*, at paras. 331, 340. Absent significant or catastrophic loss, this head of damages must be substantiated by expert evidence and “pointed testimony” from persons who know the plaintiff: *Hartwick v. Simser*, [2004] O.T.C. 917 (S.C.), at para. 325.

[698] Regarding the first element, namely a real and substantial possibility that K.M.’s chances of forming an interdependent relationship have been detrimentally affected or impaired, no definitive standard exists in the case law. While claimants have been most successful if they have suffered a catastrophic injury (see e.g. *Reekie v. Messervey*), this is not a necessary requirement to prove impairment of the ability to form an interdependent relationship. For example, in *Wilhelmson v. Dumma*, while I reject the calculation of including this type of claim in income loss, the claimant was awarded damages under this head despite having been in two relationships since a catastrophic motor vehicle collision. The expert evidence and the claimant’s own evidence supported the fact that she had suffered psychological trauma and diminished self-worth related to the accident, impairing her ability to form a permanent interdependent relationship (paras. 333-40). In contrast, a claimant whose demeanor does not establish an inability to form a relationship, or who simply asserts that he or she is “damaged goods”, will not meet the required threshold, nor will a claimant who is in a relationship: *Afonina v. Jansson*, 2015 BCSC 10, at paras. 110, 177; *Latta*, at para. 160.

[699] For the first element, the parties agree that on the evidence, K.M. will not in all likelihood form another interdependent relationship. I agree. I accept and rely on the evidence of K.M., whom I found to be a credible witness, that he would not form a future interdependent relationship. At this point, he has lived alone since 1999. His three previous relationships with Y.G., K.W. and C.P. have all failed.

[700] K.M. has provided “pointed testimony” (*Hartwick*, at para. 325) from persons who know him, as well as expert psychiatric medical evidence, to support his claim.

[701] I accept and rely on the evidence of Y.G., whom I found to be a compelling and credible witness. K.M. and Y.G.’s first relationship struggled due to his moping around, his substance abuse, his mood swings and his lack of interest in sex (even as a young adult). His and Y.G.’s second dating type relationship struggled because K.M. kept his emotions to himself, he was withdrawn and had low energy, he had mood swings, and he used pot daily. He discussed thoughts of suicide with Y.G. in both relationships, and she felt she was being pushed away; she testified that K.M. was declining mentally even before their first relationship ended.

[702] I accept and rely on the evidence of K.M.’s sister S.G., whom I found to be credible. She testified that had K.M. not been assaulted, he would have been married, with a whole bunch of kids and lots of animals around. I find that but for the sexual abuse, K.M. would have been able to form and maintain an interdependent relationship.

[703] The expert psychiatric medical opinion evidence is consistent with the evidence of K.M., his family and Y.G. Dr. Singh found that K.M. has difficulty in building interpersonal relationships and that he will have difficulty in keeping relationships of any type. Dr. Silver agreed that K.M.’s chance of a stable, spousal relationship in the future is small.

[704] I find that there is cogent and reliable evidence to support a real and substantial possibility that K.M.'s chances of forming an interdependent relationship have been detrimentally affected or impaired by the sexual abuse perpetrated by Marson. I find that K.M.'s mood swings, drug abuse, depression, suicidal thoughts, mistrust, inability to keep a job, and lack of interaction with others arose as a result of the sexual abuse and made K.M. unable to sustain an interdependent relationship. I further find that the sexual abuse by Marson made K.M. angry, which at times crossed over into his domestic relationships. I find that the likelihood of K.M. being able to maintain an interdependent relationship in the future is realistically almost non-existent.

[705] Establishing the second element—namely that an interdependent relationship would have been economically advantageous to the claimant—is more complex. Many claims of this nature have been rejected because damages are too remote or the claimant fails to establish a statistical foundation for calculating the loss. It is not enough to simply rely on case law establishing financial dependency between spouses (see *Hartwick v. Simser*, at paras. 317, 324-26); the loss must be quantifiable in a manner that can be understood, is not too remote, and does not rely on antiquated social notions of dependency: *Bartosek*, at para. 38; *Afonina*, at para. 178; *Belyea v. Hammond*, 2000 NBCA 41, at para. 21. All the claims in these above cases either failed on their facts or from the lack of proper economic evidence to establish a foundation for the loss.

[706] I find that this is a serious economic claim that warrants a serious approach to compensate for K.M.'s economic loss. K.M. is permanently alone and will likely be for the rest of his life because of the sexual abuse by Marson. I accept Professor Carr's evidence and opinion that the pecuniary shortfall from the loss of an interdependent relationship can be conceptualized in two ways—the loss of the opportunity to save expenses, and the loss of a possible benefit through the shared income of another spouse—but that the most appropriate and accepted way is using the consumption analysis. In this case, K.M. has presented his claim in terms of the first category, his future lost opportunity to share in household expenses, which I find to be reasonable and the only appropriate approach on the facts of this case.

[707] Professor Carr's evidence, which I accept as being credible and reliable, is that the approach he has taken in this case is the one he generally uses in approaching the analysis of a loss of interdependent relationship. Professor Carr would not confirm that there is a common or general practice as to the calculation of this loss. Professor Carr testified that he has never seen the approach taken to the analysis as done by Professor Charette. There is no case law or personal injury cases that have accepted the approach to the calculation of this loss used by Professor Charette, which is based on a fatal injury model. I find Professor Carr has considerably greater experience in this field and knowledge of the governing legal authorities.

[708] I find that there is no dominant approach in the case law as to how to calculate a loss of interdependent relationship claim. Many cases that have allowed a claim under this head have not described the exact nature of the expert evidence relied upon. I do not accept and I disagree that the fatal accident approach taken by Professor Charette is more "standard" than the approach taken by Professor Carr. It is certainly not standard in a personal injury case. Professor Charette worked from after-tax figures, which I reject and find inappropriate in this case. If to be relied

upon, these figures would have at least been required to be grossed up for tax. While *Bartosek* was relied on by the School Board in oral submissions to support the *Nielsen v. Kaufmann*-type calculations done by Professor Charette, the School Board's argument here can be compared to K.M.'s reliance on the *Greenhalgh* case: while plaintiff's counsel advocated for a particular approach, that case failed on its facts and does not support that any particular approach is standard. Rather, what these cases actually show is that claimants approach this head of damages in different ways, and the facts and evidence in a particular case will dictate the appropriate approach.

[709] I further reject that K.M.'s expense-oriented approach is any more "non-standard" than other approaches; K.M. has rebutted that argument. In *Walker v. Ritchie*, the Court of Appeal noted that the plaintiff had "lost the opportunity to share household expenses" and that "[h]er loss, accordingly, is measured by the loss of the contribution that she would have received but for the accident" (para. 54; see also *Kelly v. Perth (County)*, at para. 263).

[710] I recognize that *Walker* factored some information regarding the claimant's notional spouse, such as his socio-economic background (para. 57, citing trial decision, [2003] O.T.C. 3 (S.C.)). However, as I said earlier, the facts of every case are important in determining the best manner of approaching the economic loss analysis. The future loss of interdependent relationship in K.M.'s case is very particular to its facts. The parties agreed—and the evidence was compelling—that K.M. will never again form an interdependent relationship. I find on the facts of this case, there is no notional spouse to consider. The School Board's approach, by definition, takes into account a notional spouse and would require me to make additional assumptions about a spouse that does not and in all likelihood will never exist. I reject this approach.

[711] Because there will never be a notional spouse on these peculiar facts, I find there is no need to make assumptions about such a spouse; the economic analysis for the second element of K.M.'s specific claim must be done on a different basis. I agree with K.M.'s position that making additional, arbitrary assumptions about a notional spouse complicates the process, and I find that the School Board's approach is inappropriate for the particular and compelling facts of this case. For this reason, I reject the approach taken in *Osborne* and I reject the approach taken by Professor Charette, which I find untenable both on the facts of this case and given that his figures have not been grossed up for tax. I accept and rely on Professor Carr's approach and opinion as being credible and reliable. His approach isolates the claim to K.M.'s own projected income figure and focuses on K.M.'s future loss of expense-sharing ability. I find that it is the only appropriate analysis for the unique and particular facts of this case, and does not attempt to quantify the loss of a notional spouse's income. The Court of Appeal in *Walker* agreed that the lost opportunity to share expenses is an economic claim that can be compensated, and I find that Professor Carr's approach offers the necessary statistical foundation and calculates K.M.'s loss in a quantifiable, understandable manner that permits the court to assess the loss and compensate K.M. accordingly. I prefer and accept, and find credible, the evidence and opinions of Professor Carr over those of Professor Charette.

[712] I find that K.M. has lost the opportunity to share a household and household expenses with a partner. K.M. will pay 100% of his household expenses instead of the lesser share of expenses had a partner assisted him with those expenses. The loss is the measure accorded by the

lost ability to share household expenses that K.M. would have had but for the sexual abuse by Marson, based on the theory of consumption as opined by Professor Carr.

[713] While I have accepted Professor Carr's approach to calculating K.M.'s claim for future loss of interdependent relationship, his approach is tied to the income levels for K.M. in Scenarios A and B. I have already accepted and relied on Scenario C in my income loss calculations as being the appropriate scenario, subject to a general contingency discount of 15%. I similarly accept Scenario C and the mean income of \$58,000 in 2016 dollars to determine K.M.'s future loss of interdependent relationship claim, which is income-dependent. Professor Carr's calculations for Scenario C did not address the future loss of interdependent relationship claim, nor was he asked to do so.

[714] While I do not have Professor Carr's calculations for K.M.'s future loss of interdependent relationship claim for Scenario C, the midpoint of K.M.'s future loss of interdependent relationship claims in Scenarios A and B is \$178,366. Scenario A is based on an income of \$68,232 in 2016 dollars and produces a loss of \$212,699. Scenario B is based on an income of \$53,537 in 2016 dollars and produces a loss of \$144,032. The mean income I have already found to be appropriate for K.M.'s income loss is \$58,000.00 in 2016 dollars, as in Scenario C. While I do not have the percentage consumption rate as to the loss that the mean income of \$58,000.00 would generate, I find \$58,000 would fall below the midpoint income of \$60,885 by about 4.7% (or 95.3% of \$60,885).

[715] To determine K.M.'s future loss of interdependent relationship claim under Scenario C, I therefore multiply the midpoint of the future loss of interdependent relationship claim (\$178,366) by 95.3%, to reach my beginning or starting number in assessing the damages for this claim at \$169,983.

[716] I appreciate that this sum includes an additional approximate 10 months of loss of future interdependent relationship, as the valuation date of the midpoint of this claim by Professor Carr is as of May 9, 2016 (the original commencement of trial date) and not as of March 20, 2017 (the actual commencement of trial date), approximately 10 months later. I estimate this 10-month period based on the midpoint of annual savings in Scenario A (\$15,720) and B (\$10,645) to be \$13,183, times 95.3% to be \$12,563 for 12 months, or \$10,469 for a period of 10 months. I reduce the total damages by the amount of \$10,469, for a net total assessment of this loss at \$159,514.

[717] I apply a 15% general contingency to K.M.'s future loss of interdependent relationship. I recognize that larger deductions have been made in prior cases to reflect that the plaintiff may never have sustained a relationship even without the injury, or that the plaintiff may be able to maintain one despite the injury (see e.g. *Walker* ONCA, at para. 57). In *Walker*, the plaintiff was 17 years old at the time of the accident, and the trial judge applied substantial contingencies. However, I accept the evidence of S.G. that absent the sexual abuse, K.M., now 51 years old, would have been married. There is still some risk that K.M. never would have formed a relationship, or that his relationships would have been of an intermittent nature. There is a "small" possibility that he will form a relationship in the future. The evidence is clear, however, that K.M.'s chances of a relationship post-sexual abuse are very limited. A contingency discount

rate of higher than 15%, I find, would not be appropriate. Accordingly, I find that the evidence supports a modest contingency discount of 15% of K.M.'s claim for loss of future interdependent relationship as being appropriate. With a 15% contingency discount applied to \$159,514, this produces a total future loss of interdependent relationship of \$135,587.

[718] As this is a claim for future loss, I find that K.M. has met the burden of proof that there is a real and substantial possibility, not mere speculation, that he will suffer a future economic loss of an interdependent relationship. The parties are agreed that K.M. is entitled to an award of damages for this loss. I therefore assess K.M.'s damages for loss of future interdependent relationship at \$135,587.

[719] The above assessment of damages for future loss of interdependent relationship is reserved and subject to the terms as set out in Item #24 of this judgment ("Further Calculations, Errors, Omissions, and Ancillary Matters"), only as to further calculations as specified and with a 10-month period deducted from the total award to reflect a valuation date of March 20, 2017.

18. O.H.I.P. Past Loss Subrogated Claim:

[720] O.H.I.P. seeks compensation for its subrogated claim in the amount of \$40,246.80 for the cost of K.M.'s past medical care and related services that were reasonably incurred in his treatment as a result of the sexual abuse by Marson. The details of the amounts claimed are before the court (Exhibit #3, Tabs #H1-10). On February 29, 2016, plaintiff's counsel was advised by O.H.I.P. that it was not pursuing a claim for future services.

[721] O.H.I.P.'s subrogated claim is a special damage claim and must be strictly proven, subject only to the standard of reasonableness. K.M. must prove that the special damages being claimed were in fact caused by the tort in question. The issue is, has K.M. proven that but for the sexual abuse committed by Marson, the specific medical expenses would not have been incurred?

[722] K.M.'s position is that O.H.I.P.'s subrogated claim of \$40,246.80 is for the cost of K.M.'s medical care and services that were reasonably incurred to treat his injuries. K.M. relies on the evidence of Dr. Singh, who testified that he had reviewed all of the O.H.I.P. expenses that were based on services rendered to K.M. and that in his view, they were reasonably incurred to treat K.M.'s psychiatric disorders at the time. K.M.'s position is that if the sexual abuse was at least "a cause" of his psychological disorders requiring treatment, then the School Board is 100% liable for O.H.I.P.'s subrogated claim.

[723] The School Board's position is that it admits that medical services were incurred in treating K.M. The School Board will not stipulate nor admit, however, that these expenses were reasonably incurred to treat K.M. as a result of the sexual abuse committed by Marson. The School Board's position is that as causation has not been established, it is not responsible for these expenses, which must be strictly proven. K.M. must specifically prove that the special damages were in fact caused by the tort.

[724] In support of its position, the School Board uses the example of K.M.’s January 28, 2000 hospital stay. This visit reports a suicide attempt by K.M. after the breakup of his relationship with C.P. K.M. was still suffering at that time from his treatment resistant dysthymia and substance abuse. The School Board relies on the evidence of Dr. Silver, who testified that he could not attribute any particular treatment to any particular cause. Dr. Silver testified that he was unable to say that any of the medical expenses can be directly attributed to the sexual abuse committed on K.M. It was Dr. Silver’s opinion that as he could not say whether the psychiatric disorders being treated were caused by the childhood sexual abuse, as opposed to all other causal factors, the same analysis applied to the medical expenses. In summary, the School Board’s position is that while the services rendered by O.H.I.P. may have been reasonable from a medical point of view, they have not been proven to have been caused by the sexual abuse committed by Marson on K.M.

[725] O.H.I.P.’s claim must, like all damages, be proven on the evidence. K.M. must show, as in the main argument on causation, that on a balance of probabilities, the sexual abuse by Marson is a “but for” cause of his injuries and of his medical expenses: *Athey, supra*. The but for test dominates this inquiry. Damages can be reduced where pre-existing medical conditions would have required treatment regardless (e.g. *Park v. Abd El Malak*, 2015 BCSC 223, at para. 136), but they can also be fully recovered where the tort does not cause a latent condition but instead irritates it: see e.g. *Harder v. Poettcker*, 2017 BCSC 312, at para. 111.

[726] In *M.(K.I.) v. Perri Estate*, [2001] O.J. No. 1691 (S.C.), for example, the plaintiff was sexually exploited by the defendant as a young adult. Though she had a personality disorder, she had never been under psychiatric care prior to her involvement with the defendant (para. 35). The Court allowed O.H.I.P.’s subrogated claim for psychiatric treatments, noting that since all the treatments were incurred after her involvement with the defendant, it was “satisfied on the balance of probability that they are as a result of the psychological damages arising from [the defendant’s] conduct” (para. 106).

[727] Once causation is established, the test for special damages claims is whether the expenses were reasonably incurred: *Derkson v. Nicholson*, 2015 BCSC 1268, at para. 78. Where the special damages are medical expenses, the claim should be substantiated by some evidence or indication that they relate to medically-required treatments: *Meady v. Greyhound Canada Transportation Corp*, 2012 ONSC 657, at para. 419. While medical justification is a factor in determining reasonableness, explicit medical recommendation is not a prerequisite: *Derkson*, at para. 78. The courts have also allowed treatment sought by a plaintiff without medical advice to be “retroactively approved or recommended” by experts: see e.g. *Halliday v. Wigmore*, [1996] B.C.J. No. 1536 (S.C.), at para. 423.

[728] I find that the School Board’s position is an extension of its main argument on causation, which I have rejected. The School Board’s position is contrary to *Athey*. I am satisfied that all the medical treatments were incurred after K.M.’s involvement with Marson. I agree with plaintiff’s counsel’s submission that the School Board is responsible for 100% of K.M.’s injuries and consequent damages because the sexual abuse by Marson—for which it is responsible—I find, constitutes a major cause of K.M.’s disorders. This would include, among others, K.M.’s dysthymia, his personality disorders, his post-traumatic stress disorder, and his relationship and

employment issues or a combination of them. When K.M. obtained medical services for these mental health injuries, for example in the January 2000 hospital visit, I find that it was for a combination of reasons. Part of the combination of reasons, in 2000 and for K.M.'s other O.H.I.P. expenses, I find, was his underlying mental health injuries of dysthymia, substance abuse and personality disorder caused by the sexual abuse by Marson. This does not change the fact that the sexual abuse by Marson was, as I have found on a balance of probabilities, a major cause of the injuries. The influence of other potential causal factors does not change the School Board's liability for the medical expenses incurred by K.M. in the treatment of his mental health injuries. I find that the treatments would not have been required, and the medical expenses would not have been incurred, but for the sexual abuse by Marson. I find that K.M. has established causation as to these expenses, for which the School Board is 100% liable. I further accept the evidence of Dr. Singh and find that the medical expenses claimed were reasonably incurred in treating K.M.'s disorders.

[729] Strict proof of the special expenses claimed is required. There are two areas of concern in the O.H.I.P. claim. Firstly, the O.H.I.P.'s claim starts with a "previous total" balance of \$4,542.69 (Exhibit #3, Tab #H1, p. 579). No breakdown of this amount is found in the evidence. As I am unable to assess what medical care was undertaken and who provided the services, this "previous total" amount of the O.H.I.P. claim is disallowed and dismissed as not proven. O.H.I.P.'s claim is accordingly reduced from \$40,246.80 less \$4,542.69 to \$35,704.11.

[730] Secondly, the O.H.I.P. claim contains a claim for two hospital stays by K.M. The first hospital stay from December 27, 1989 until January 19, 1990 is supported by the evidence in this case and is allowed. The second hospital stay claimed is from December 27, 1990 until January 19, 1991 and is not supported by the evidence. This second hospital admission date, I find, is as a result of a typographical error in the hospital records, carried forward by others, when in reality there was no such hospital stay by K.M. on these dates. The claim for the hospital stay from December 27, 1990 to January 19, 1991 of \$7,751.00 is therefore disallowed and dismissed as not proven. O.H.I.P.'s claim is accordingly reduced from \$35,704.11 less \$7,751.00 to \$27,953.11.

[731] A summary of the O.H.I.P. Subrogated Claim Amounts is attached as Schedule 'B'. The summary lists—by doctor, hospital, and laboratory—the amount of the expense claimed, the date the expense was incurred, and the location in the documentary exhibits where the reference is located, as well as the calculation of the total amount allowed from the records supplied by Mark McEwen, Subrogation Recovery Officer, Ministry of Health and Long Term Care.

[732] I find that K.M. has met the burden of proof of his past O.H.I.P. loss. The O.H.I.P. expenses allowed, I find, have been strictly proven and are reasonable. I therefore assess K.M.'s damages for his O.H.I.P. past loss subrogated claim at \$27,953.11.

19. Management Fee:

[733] The parties are agreed that a management fee of 5% of the future economic loss award is appropriate. K.M. will require the services of an investment manager to help him manage his award for economic loss. The point of a management fee is to invest and conserve K.M.'s funds

in order to provide a flow of income for K.M.'s needs into the future. The management fee sought is 5% of damages awarded for future losses, as in Professor Carr's evidence, which was not challenged by Professor Charette. I find that a management fee is necessary and reasonable for K.M. The management fee is assessed at 5% of the total damages for future loss of income of \$762,450.00 and future loss of interdependent relationship of \$135,587.00, or \$898,037.00 x .05, and is allowed at \$44,902.00.

[734] The parties are agreed as to K.M.'s entitlement to a management fee of 5% of his future economic loss award. I assess K.M.'s damages for a management fee at \$44,902.00.

[735] The above assessment of damages for a management fee is reserved and subject to the terms as set out in Item #24 of this judgment ("Further Calculations, Errors, Omissions, and Ancillary Matters"), only as to further calculations as specified.

20. Conclusion – Judgment to Issue:

[736] A judgment shall issue in favour of the plaintiff K.M. as against both named defendants, Robert Terrance Marson and The Hastings and Prince Edward District School Board, in accordance with these Reasons for Judgment, subject to the provisions as set out in Item #24 ("Further Calculations, Errors, Omissions, and Ancillary Matters") following below. The final judgment shall be signed only by MacLeod-Beliveau, J. after approval by counsel. Failing agreement, counsel may obtain a date to settle the order before me in Belleville. I remain seized of this matter until such time as the final judgment is issued.

[737] I therefore assess K.M.'s total damages as follows:

a. General and aggravated damages	\$ 250,000.00
b. Damages for past loss of income	\$1,192,550.00
c. Damages for future loss of income	\$762,450.00
Total damages for past and future income loss	\$1,955,000.00
d. Damages for future loss of interdependent relationship	\$ 135,587.00
e. O.H.I.P. past loss subrogated claim	\$ 27,953.11
f. Management fee	\$ 44,902.00
Total damages awarded	<u>\$2,413,442.11*</u>

[738] *Pre-judgment interest shall be assessed on the damages awarded as specified in the following paragraph.

21. Pre-judgment Interest:

[739] I award pre-judgment interest on the appropriate heads of damages to K.M. The calculation and quantum of the total pre-judgment interest to be awarded is reserved. The parties are agreed that pre-judgment interest on the non-pecuniary general damages at the rate of 5% is appropriate (Rule 53.10), and that pre-judgment interest on the past income loss and the O.H.I.P. past loss at the rate of 3.3% is appropriate (section 128 of the *Courts of Justice Act*) from October 1, 2006, approximately when the claim became discoverable. Counsel shall file a detailed calculation of pre-judgment interest to the date of this judgment with a draft judgment on or before **July 31, 2018**. If there is lack of agreement, written submissions and/or a further court attendance may be required.

22. Costs:

[740] The issue of costs of the trial and costs of the motion heard by me on May 1, 2017 is reserved. If there is lack of agreement between the parties as to costs, I will receive written submissions on costs no later than **September 14, 2018** after which time I will determine the issue of costs based on the material filed. Counsel shall set their own schedule during that time period for the orderly exchange of their submissions on costs and shall file them with the Superior Court of Justice trial co-ordinator in Belleville on or before the date specified. A further court attendance may be required. The costs order may issue as a separate order.

23. Cross-claim:

[741] The School Board cross-claims against Marson for contribution and indemnity for any amount for which the School Board may be found liable to K.M., costs of the main action on a solicitor and client basis, costs of the cross-claim on a solicitor and client basis, and interest on all amounts awarded pursuant to the *Courts of Justice Act*. Rule 1.04(5) stipulates that reference in a document to solicitor and client costs now applies as if the reference were to substantial indemnity costs. I find that the School Board is entitled to a judgment for contribution and indemnity from the co-defendant Marson, as claimed in the cross-claim, for the total sum awarded to K.M.

[742] The cross-claim by the School Board as against Marson is therefore allowed in the same amount as the total judgment awarded against the School Board to K.M., for all damages, pre-judgment interest, and costs that may be awarded to K.M. If the School Board claims any additional costs arising from the cross-claim beyond the sum that may be awarded to K.M., the School Board may include submissions about such costs with their costs submissions in the main action by **September 14, 2018**.

24. Further Calculations, Errors, Omissions, and Ancillary Matters:

[743] Accurate economic calculations, once factual findings have been made, are preferred in assessing significant claims for damages. I made the factual finding that K.M. would not have graduated from high school or obtained his high school equivalency, which affects the quantum of damages assessed. My assessments of damages for K.M.'s past and future income loss,

K.M.'s future loss of interdependent relationship, and K.M.'s management fee are based on calculations resulting from Scenario C (Exhibit #10), which is based on the cohort of average Ontario males who did not graduate from high school at a mean income which I have found to be \$58,000 in 2016 dollars. Complete calculations were not available to me at trial, as it was not anticipated and arose from the evidence heard during the trial, which is why I have reserved on these damage awards.

[744] As a result, unless counsel otherwise agree for purposes of this judgment, counsel for K.M. shall obtain a schedule of calculations from Professor Carr, based on Scenario C, to determine K.M.'s total income loss, including the allocation between K.M.'s past and future income loss, and to determine K.M.'s future loss of interdependent relationship for Scenario C. The schedule of calculations shall be based on my finding of a mean income of \$58,000 in 2016 dollars and the otherwise same assumptions, including fringe benefits and productivity, as used in Professor Carr's March 15, 2016 report. These calculations shall then be adjusted to the commencement date of this trial of March 20, 2017. Counsel for K.M. shall provide the schedule of calculations to counsel for the School Board and to me on or before **July 16, 2018**. Counsel for the School Board shall provide to counsel for K.M. and to me any responding calculations on or before **July 23, 2018**. Counsel for K.M. shall provide a reply, if any, to counsel for the School Board and to me on or before **July 27, 2018**. These calculations will be for my review and my final assessment of these damages, failing which the original amounts as determined by me for these damages in this judgment shall be found to be accurately calculated and shall be final.

[745] If there is lack of agreement between counsel as to the schedule of calculations specified above, counsel shall so advise me on or before **July 31, 2018** as to how they wish to proceed, and I will determine whether written submissions and/or a further court attendance with additional evidence is required, on a date to be set in Belleville.

[746] My request in respect for further calculations and submissions should not be considered an opportunity to re-argue any finding that I have otherwise made in these Reasons for Judgment. After receipt and review of these calculations, I will apply the 15% negative contingencies to K.M.'s total income loss claim and his claim for loss of future interdependent relationship, and I will adjust the calculation of the management fee accordingly, in determining my final assessment of these damages. I note the schedule of calculations will affect the quantum of pre-judgment interest as well.

[747] I appreciate that there may be legitimate reasons, such as vacations and other matters, that may make the timelines that I have set impractical. If additional time is required by any party in meeting any of the above timelines in any respect, counsel are to so advise me prior to **July 31, 2018** and this issue may be dealt with by way of teleconference on the record.

[748] Should there be other matters arising from these Reasons for Judgment which may have to be addressed or calculated, I would be obliged if counsel would bring such issues to my attention by **July 31, 2018** so that they can be addressed. If there are such circumstances and there is lack of agreement between the parties, written submissions and/or a further court attendance may be required.

[749] Counsel did not address how the \$50,000.00 advance payment by Marson is to be considered for purposes of this judgment. Submissions on this issue shall be received on or before **July 31, 2018**.

[750] To be clear, I am reserving the final quantum of the assessment of damages to be awarded in this case that are affected by Scenario C, pending further calculations from counsel in relation to Scenario C and other matters raised above. I reserve my right to make any such adjustments to this judgment as required prior to the judgment being issued, with supplementary reasons if necessary.

[751] If a further court date is required, I will reconvene court at a reasonably early date that is convenient to counsel and to the court, on a date to be set by the Superior Court trial co-ordinator in Belleville. All filings and communications on this file shall be through the Superior Court trial co-ordinator in Belleville whose contact information has been provided to counsel.

Madam Justice H. MacLeod-Beliveau

Released: June 29, 2018

[752] The following schedule details K.M.'s earnings income from his income tax returns:

Schedule 'A'
K.M.'s Earnings and Taxable Income Sources, 1987-2014
(In Dollars)

<u>Year</u>	<u>Earnings</u>	<u>UI</u>	<u>Social Assistance</u>	<u>Other Income</u>	<u>Line 150 total income</u>
1987	7,182	1,064			7,746
1988	27,041	412		691	28,144
1989	28,217				28,217
1990	518	1,065		14,040	15,623
1991	6,122			2,949	9,071
1992	3,494	3,915	1,189		8,598
1993	0	1,755	3,218		4,973
1994	3,248		5,879		9,127
1995	1,842		6,420		8,262
1996	37,212		923		38,135
1997	17,209	9,477			26,686
1998	19,965	7,867			27,832
1999	6,255	7,171	666		14,092
2000	4,427	2,768	2,858		10,053
2001	0		5,347		5,347
2002	0		3,768		3,768
2003	1,800		9,039		10,839
2004	0		10,460		10,460
2005	0		11,479		11,479
2006	0		11,608		11,608
2007	0		12,448		12,448
2008	0		12,551		12,551
2009	0		12,934		12,934
2010	0		13,217		13,217
2011	0			12,759	12,759
2012	0		10,640	7,405	*18,045
2013	0			7,573	7,573
2014	0			12,829	12,829

*Does not include CPP retroactive disability payments for 2009, 2010, 2011

[753] The following schedule details the O.H.I.P past loss claim:

Schedule 'B'
O.H.I.P. Past Loss Subrogated Claim Amounts

PREVIOUS TOTAL: \$4,542.69 * (Exhibit #3, Tab #H1, p. 579)

Doctors:

Dr. Manderville

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$22.90	12/27/89	Tab 3
TOTAL: \$ 22.90		

Dr. Brant

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$49.20	11/23/89	Tab 3
\$393.60	02/01/90 to 02/28/90 (8 visits x \$49.20)	Tab 3
\$49.20	02/16/90	Tab 3
\$147.60	03/01/90 to 03/31/90 (3 visits x \$49.20)	Tab 3
\$152.40	01/01/93 to 03/31/93 (3 visits x \$50.80)	Tab 3
\$50.80	09/01/93	Tab 3
\$50.80	02/01/94	Tab 3
TOTAL: \$ 893.60		

Dr. Conn

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$50.30	02/07/92	Tab 3
\$50.30	12/23/92	Tab 3
\$50.80	03/21/95	Tab 3
\$50.80	02/24/98	Tab 3
\$52.55	07/21/00	Tab 3
\$62.20	01/17/06	Tab 2
\$62.20	03/14/06	Tab 2
\$62.20	05/17/06	Tab 2
TOTAL: \$ 441.35		

Dr. Schwarz-Michorowski

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$50.80	02/25/97	Tab 3
TOTAL: \$ 50.80		

Dr. Ash

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$52.55	01/28/00	Tab 3
TOTAL: \$ 52.55		

Dr. Singh

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$131.30	05/12/09	Tab 1
\$131.30	08/27/09	Tab 1
\$71.00	10/15/09	Tab 1
\$142.00	12/17/09	Tab 1
\$142.00	01/25/10	Tab 1
\$142.00	02/11/10	Tab 1
\$142.00	03/05/10	Tab 1
\$142.00	05/03/10	Tab 1
\$142.00	08/03/10	Tab 1
\$142.00	09/16/10	Tab 2
\$149.40	11/09/10	Tab 2
\$185.50	12/17/10	Tab 2
\$149.40	01/21/11	Tab 2
\$149.40	03/07/11	Tab 2
\$149.40	05/12/11	Tab 4
\$149.40	07/04/11	Tab 4
\$160.60	12/01/11	Tab 5
\$160.60	01/30/12	Tab 5
\$80.30	03/16/12	Tab 5
\$160.60	05/28/12	Tab 8
\$160.60	06/25/12	Tab 8
TOTAL: \$2,982.80		

Dr. Matveyev

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$300.70	06/12/13 **	Tab 8
\$300.70	10/08/15	Tab 9
TOTAL: \$ 601.40		

Dr. Holmes

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$62.75	12/10/15	Tab 9
TOTAL: \$ 62.75		

Inpatient:**Kingston Psychiatric Hospital**

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$3,213.00	07/19/90 to 08/15/90 (27 days x \$119.00)	Tab 3
TOTAL: \$3,213.00		

Belleville General Hospital

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$7,337.00	12/27/89 to 01/19/90 (23 days x \$319.00)	Tab 3
\$7,751.00 ****	12/27/90 to 01/19/91 (23 days x \$337.00)	Tab 3
TOTAL: \$15,088.00		

Quinte Health Care Corporation

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$2,089.80	01/28/00 to 02/03/00 (6 days x \$348.30)	Tab 3
\$3,831.30	02/03/00 to 02/14/00 (11 days x \$348.30)	Tab 3
TOTAL: \$5,921.10		

Outpatient:**Belleville General Hospital**

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$89.00	02/24/98	Tab 3
TOTAL: \$ 89.00		

Quinte Health Care Corporation

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
\$110.00	07/21/00	Tab 3
\$110.00	05/04/01	Tab 2
\$164.00	11/07/06	Tab 2
\$238.00	05/12/09	Tab 1
\$238.00	08/27/09	Tab 1
\$238.00	10/15/09	Tab 1
\$238.00	12/17/09	Tab 1
\$238.00	01/25/10	Tab 1
\$238.00	02/11/10	Tab 1
\$238.00	03/05/10	Tab 1
\$247.00	05/03/10	Tab 1
\$247.00	08/03/10	Tab 1
\$247.00	09/16/10	Tab 2
\$247.00	11/09/10	Tab 2
\$247.00	12/17/10	Tab 2
\$247.00	01/21/11	Tab 2
\$247.00	03/07/11	Tab 2
\$260.00	05/12/11	Tab 4
\$260.00	07/04/11	Tab 4
\$260.00	12/01/11	Tab 5
\$260.00	01/30/12	Tab 5
\$260.00	03/16/12	Tab 5
\$270.00	05/28/12	Tab 8
\$270.00	06/25/12	Tab 8
\$287.00	06/12/13	Tab 8
\$309.00	10/08/15	Tab 9
TOTAL: \$6,215.00		

Other:**BPC Ontario Labs LP**

Expense	Date Incurred	Tab # (within Exhibit #3, Tab H)
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\$45.56	08/03/07	Tab 2
\$24.30	08/03/07	Tab 2
TOTAL: \$ 69.86		

TOTAL: \$40,246.80 * no breakdown of amount; ** correct date is 06/14/13; *** duplicate charge in error

CITATION: K.M. v. Marson, 2018 ONSC 3493

COURT FILE NO.: CV-08-0283-00

DATE: 2018 June 29

ONTARIO

SUPERIOR COURT OF JUSTICE

BETWEEN:

K.M.

Plaintiff

– and –

ROBERT TERRANCE MARSON and THE
HASTINGS AND PRINCE EDWARD DISTRICT
SCHOOL BOARD

Defendants

REASONS FOR JUDGMENT

MacLeod-Beliveau J.

Released: June 29, 2018

